MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 703

Human Development Across the Lifespan

15 teaching weeks · 14 practicals · 9 worked cases · 1 figure

SEMESTER ONE

1.71 units · Prerequisite: none

> “You cannot assess a family without knowing what is developmentally ordinary.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 703 — Human Development Across the Lifespan

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 15 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 703	**Human Development Across the Lifespan**	One	1.71

Contents

How to use this book . . . 15

1. Course description . . . 15

2. Learning outcomes . . . 15

3. Assessment . . . 16

4. The fifteen-week plan . . . 16

Week 1 — Why developmental knowledge is clinical knowledge . . . 16

1.1 The lecture in ninety minutes . . . 18

1.2 Core concepts . . . 18

1.2a The developmental history, asked well . . . 20

1.3 The clinical method: from behaviour to task . . . 21

1.4 Techniques: the four instruments of this course . . . 22

1.4a Two timelines, drawn and read . . . 22

1.4b The four categories, tested on eight descriptions . . . 23

1.5 The first session with a developmental concern . . . 24

1.6 Case: the Okonkwos, a developmental reading . . . 25

1.6a The same child, three developmental misreadings . . . 26

1.7 Annotated transcript: the timeline drawn . . . 27

1.8 Laboratory: the timeline and the four categories . . . 28

1.9 Six errors in developmental assessment . . . 28

1.10 Culture, faith and development . . . 29

1.10a The course's four categories, applied to a family . . . 30

1.11 Evidence note: what developmental science can and cannot support . . . 31

1.11a What development cannot explain . . . 31

1.12 Key terms . . . 32

1.13 Self-test . . . 32

1.14 Reflective task . . . 33

1.15 Readings for this week . . . 33

Week 2 — Prenatal development, temperament and becoming a parent . . . 34

2.1 The lecture in ninety minutes . . . 34

2.2 Core concepts . . . 35

2.2a The goodness-of-fit grid, filled in . . . 37

2.2b The nine dimensions, with the question that asks each . . . 38
2.3 Postnatal depression: the assessment . . . 38
2.3a The couple's reorganisation, drawn on one page . . . 39
2.4 The first session with a couple four months after a birth . . . 40
2.4a The first year, month by month: what a family therapist watches for . . . 41
2.5 Case: the prematurity that reorganised a family . . . 42
2.5a The couple who came at the right time, and the couple who came at the wrong time . . . 43
2.6 Annotated transcript: the temperament interview . . . 44
2.6a The prematurity case, continued: what the parents needed that the clinic did not offer . . . 45
2.7 Laboratory: the temperament interview and the reorganisation . . . 45
2.7a The screening conversation, rehearsed . . . 46
2.8 Six errors in perinatal work . . . 47
2.9 Culture, faith and the perinatal period . . . 47
2.10 Evidence note: what is well established and what is not . . . 48
2.11 Key terms . . . 49
2.12 Self-test . . . 49
2.13 Reflective task . . . 50
2.14 Readings for this week . . . 50
Week 3 — Infancy: attachment, synchrony and regulation . . . 51
3.1 The lecture in ninety minutes . . . 51
3.2 Core concepts . . . 52
3.2a Four infants, described and classified . . . 54
3.3 Synchrony, assessed in the room . . . 55
3.3a Co-regulation, designed for one family . . . 56
3.3b The caregiving network, mapped . . . 57
3.4 The first session with an infant and two parents . . . 58
3.4a The still face, in the consulting room . . . 59
3.5 Case: the Adeyemi family, in the infant's terms . . . 59
3.6 Annotated transcript: rupture and repair in a session . . . 60
3.7 Laboratory: observing synchrony . . . 61
3.8 Six errors in infant mental health work . . . 62
3.9 Culture, faith and the first relationship . . . 62
3.10 Evidence note: strong theory, moderate evidence, popular misuse . . . 63
3.11 Key terms . . . 64

3.12 Self-test . . . 64
3.12a What to tell a parent in one page . . . 65
3.13 Reflective task . . . 66
3.14 Readings for this week . . . 66
3.2b The four patterns in adult life: what the family therapist sees . . . 66
3.7a The disorganised flag, and what followed . . . 67
Week 4 — Toddlerhood: autonomy, language and limits . . . 68
4.1 The lecture in ninety minutes . . . 69
4.2 Core concepts . . . 70
4.2a The limit formula, taught and rehearsed . . . 71
4.2b The tantrum, anatomised . . . 72
4.3 Language: the assessment . . . 73
4.3a Ten minutes of child-led play, in the second year . . . 74
4.4 Case: the grandmother who "cannot manage him" . . . 75
4.5 Annotated transcript: the family meeting about five rules . . . 76
4.5b The nine words a caregiver needs . . . 77
4.6 Laboratory: the formula and the rules map . . . 78
4.6a When the household cannot agree . . . 79
4.7 Six errors in toddler work . . . 79
4.8 Culture, faith and the second year . . . 80
4.9 Evidence note: robust development, contested practice . . . 81
4.4a The same household, six months later: what changed and what did not . . . 81
4.4b Two toddlers, two very different assessments . . . 82
4.5a The language history, asked as a conversation . . . 83
4.10 Key terms . . . 83
4.11 Self-test . . . 84
4.12 Reflective task . . . 85
4.13 Readings for this week . . . 85
Week 5 — Early childhood: play, theory of mind, initiative . . . 85
5.1 The lecture in ninety minutes . . . 86
5.2 Five concepts that organise the week . . . 87
5.2a Theory of mind: five tasks and what failure means clinically . . . 88
5.2b Six levels of play and what each one tells you . . . 89

5.3 Observing play without hijacking it . . . 90
5.3a Setting a limit in the playroom . . . 91
5.4 Three procedures for the clinic . . . 92
5.5 The first session with a four-year-old . . . 92
5.6 Case: the child who plays the same thing every day . . . 93
5.6a Six weeks later . . . 95
5.7 Transcript: ten minutes of play, annotated . . . 95
5.8 Laboratory: the ten-minute observation, marked . . . 96
5.9 Six errors . . . 97
5.10 Culture, faith and the child who plays . . . 98
5.11 Evidence note . . . 99
5.12 Key terms . . . 99
5.13 Self-test . . . 100
5.14 Reflective task . . . 101
5.15 Further reading . . . 101
Week 6 — Middle childhood: industry, peers, school . . . 101
6.1 The lecture in ninety minutes . . . 102
6.2 Five concepts that organise the week . . . 103
6.2a The school-refusal differential: seven questions . . . 104
6.2b What a friend is, at seven, nine and eleven . . . 104
6.3 The method: assessing school refusal in one session . . . 105
6.3a The family–school interface . . . 106
6.4 Three techniques for this age . . . 106
6.4a The digital playground . . . 107
6.5 The first session with a nine-year-old and two parents . . . 108
6.5a When the child will not talk . . . 108
6.6 Case: the Okonkwos again — Mondays . . . 109
6.6a Four weeks later . . . 111
6.7 Transcript: telling a nine-year-old the truth . . . 112
6.8 Laboratory: the school-refusal formulation . . . 112
6.9 Six errors . . . 113
6.10 Culture, faith and the child at school . . . 114
6.11 Evidence note . . . 114
6.12 Key terms . . . 115

6.13 Self-test . . . 116
6.14 Reflective task . . . 117
6.15 Further reading . . . 117
Week 7 — Adolescence I: puberty, identity, autonomy . . . 117
7.1 The lecture in ninety minutes . . . 118
7.2 Five concepts that organise the week . . . 119
7.2a Pubertal timing: the effects that matter clinically . . . 120
7.2b The teenage brain: four findings and three misuses . . . 120
7.3 The method: renegotiating the frame . . . 121
7.3a The three-column contract, worked . . . 122
7.4 Three techniques . . . 123
7.5 The first session with a fifteen-year-old and two parents . . . 123
7.5a When the adolescent will not speak . . . 124
7.6 Case: Amara, secretive and spied upon . . . 125
7.6a Six weeks later . . . 126
7.7 Transcript: the negotiation, with the contract on the table . . . 127
7.8 Laboratory: the negotiation, marked . . . 128
7.9 Six errors . . . 129
7.9a A seventh error: the one that ends treatment . . . 130
7.10 Culture, faith and the adolescent . . . 130
7.11 Evidence note . . . 131
7.12 Key terms . . . 131
7.13 Self-test . . . 132
7.14 Reflective task . . . 133
7.15 Further reading . . . 133
Week 8 — Adolescence II: risk, peers, digital life and mental health . . . 133
8.1 The lecture in ninety minutes . . . 134
8.2 Five concepts that organise the week . . . 135
8.2a Seven presentations that must not be missed . . . 136
8.2b Self-harm and suicide: what to ask and what to do with it . . . 137
8.3 The method: a risk assessment a family therapist can actually do . . . 138
8.3a Digital life: the history that is worth taking . . . 139
8.4 Three techniques . . . 139

8.5 The first session with a self-harming adolescent and two parents . . . 140
8.5a The crisis hour: when the risk is immediate . . . 140
8.6 Case: the boy whose teacher saw his arm . . . 142
8.6a Eight weeks later . . . 144
8.7 Transcript: asking the four questions . . . 144
8.8 Laboratory: the risk assessment, marked . . . 145
8.9 Six errors . . . 146
8.10 Culture, faith and adolescent risk . . . 146
8.11 Evidence note . . . 147
8.12 Key terms . . . 148
8.13 Self-test . . . 148
8.14 Reflective task . . . 149
8.15 Further reading . . . 149
Week 9 — Adult development: work, intimacy and the middle years . . . 150
9.1 The lecture in ninety minutes . . . 151
9.2 Five concepts that organise the week . . . 151
9.2a The social clock and the off-time life . . . 152
9.2b The four adult tasks as they present in couple therapy . . . 153
9.3 The method: the adult developmental history . . . 154
9.3a The midlife reckoning: what it is and what it is not . . . 155
9.4 Three techniques . . . 156
9.5 The first session with a forty-five-year-old and his wife . . . 156
9.6 Case: the man who was passed over . . . 157
9.6a Five months later . . . 159
9.7 Transcript: the lifeline, drawn in front of the couple . . . 160
9.7a What to do when the partner weeps and the client does not . . . 161
9.8 Laboratory: the developmental history with a partner present . . . 161
9.9 Six errors . . . 162
9.10 Culture, faith and the adult timetable . . . 163
9.11 Evidence note . . . 164
9.12 Key terms . . . 164
9.13 Self-test . . . 165
9.14 Reflective task . . . 166
9.15 Further reading . . . 166

Week 10 — Couple development: the relationship across time . . . 166
10.1 The lecture in ninety minutes . . . 167
10.2 The six phases, with the clinical reading of each . . . 168
10.2a What the therapist asks at each phase . . . 169
10.2b The cascade and the repair trajectory . . . 169
10.3 The method: the couple timeline . . . 170
10.3a The turning-point interview . . . 171
10.4 Three techniques . . . 172
10.5 The first session with a couple in the young-children phase . . . 172
10.5a When one partner will not come . . . 173
10.4a The negative cycle, in the couple's own words . . . 173
10.5b The couple with a child with a disability . . . 174
10.6 Case: Aisha and Sam, four years on . . . 175
10.6a Four months later . . . 177
10.7 Transcript: the unanswered question . . . 178
10.8 Laboratory: the couple timeline . . . 178
10.9 Six errors . . . 179
10.10 Culture, faith and the couple . . . 180
10.11 Evidence note . . . 181
10.12 Key terms . . . 181
10.13 Self-test . . . 182
10.14 Reflective task . . . 183
10.15 Further reading . . . 183
Week 11 — Parenting across stages: talking to parents well . . . 183
11.1 The lecture in ninety minutes . . . 184
11.2 Five concepts that organise the week . . . 185
11.2a The four parts, worked at five ages . . . 185
11.2b The five questions parents are actually asking . . . 186
11.3 The method: a parent feedback conversation, ten steps . . . 187
11.3a Telling a parent something difficult . . . 188
11.4 Three techniques . . . 189
11.5 The assessed role-play . . . 189
11.5a The parent who arrives with a diagnosis from the internet . . . 190

11.6 Case: the mother who has been told she is too soft . . . 191
11.6a Six weeks later . . . 192
11.7 Transcript: the ten minutes, annotated . . . 193
11.7a When the complaint comes from the school . . . 194
11.8 Laboratory and the marking scheme . . . 194
11.9 Six errors . . . 195
11.9a Six sentences that do harm, and what to say instead . . . 195
11.10 Culture, faith and talking to parents . . . 196
11.11 Evidence note . . . 197
11.12 Key terms . . . 198
11.13 Self-test . . . 198
11.14 Reflective task . . . 199
11.15 Further reading . . . 199
Week 12 — Later life: integrity, loss and caregiving . . . 199
12.1 The lecture in ninety minutes . . . 200
12.2 Five concepts that organise the week . . . 201
12.2a The four-way cognitive differential . . . 202
12.2b Caregiving: the network and the unstated contract . . . 203
12.2c Retirement: the presentation, and what to ask . . . 204
12.3 The method: the family meeting about an older parent . . . 205
12.3a When the siblings disagree . . . 206
12.4 Three techniques . . . 206
12.5 The first session with three siblings and a daughter-in-law . . . 207
12.6 Case: the Baloguns and the woman in the back room . . . 208
12.6a Six months later . . . 210
12.7 Transcript: asking an eighty-one-year-old what she wants . . . 211
12.8 Laboratory: the caregiving network map . . . 211
12.9 Six errors . . . 212
12.9a The phone call that is really a referral . . . 212
12.10 Culture, faith and later life . . . 213
12.11 Evidence note . . . 214
12.12 Key terms . . . 215
12.13 Self-test . . . 215
12.14 Reflective task . . . 216

12.15 Further reading . . . 216
Week 13 — Culture, poverty and development: the critique of the norms . . . 216
13.1 The lecture in ninety minutes . . . 217
13.2 The four critiques worked . . . 218
13.2a When the norms are right and the family is wrong . . . 220
13.2b Three network configurations and what each needs . . . 220
13.3 The method: a developmental cultural formulation . . . 221
13.3a The interpreter . . . 222
13.3b The sentence that opens the conversation . . . 222
13.4 Three techniques . . . 223
13.4a The assessment kit for a household with nothing . . . 223
13.4b The four-column plan . . . 224
13.5 The first session when the family and the therapist disagree . . . 225
13.5a When the therapist's own culture is the variable . . . 225
13.6 Case: the child with no bedroom . . . 226
13.6a Five months later . . . 228
13.7 Transcript: the letter, read aloud . . . 228
13.8 Laboratory: assessing across a difference . . . 229
13.9 Six errors . . . 230
13.10 Culture, faith and the limits of the critique . . . 230
13.11 Evidence note . . . 231
13.12 Key terms . . . 232
13.13 Self-test . . . 232
13.14 Reflective task . . . 233
13.15 Further reading . . . 233
Week 14 — When development goes wrong: the differential habit . . . 233
14.1 The lecture in ninety minutes . . . 235
14.2 The four categories worked . . . 235
14.2a The five systemic questions, worked . . . 236
14.2b Red flags: the findings that require a same-week referral . . . 237
14.2c The child who is referred twice in a year . . . 238
14.5a The assessment the family can afford . . . 238
14.3 The method: a differential assessment in one session . . . 239

14.3a Referring well, and what to do while the family waits . . . 240
14.3b Reading somebody else's assessment . . . 240
14.4 Three techniques . . . 241
14.5 The first session with "something is wrong" . . . 241
14.6 Case: the boy who stopped talking . . . 242
14.6a Five months later . . . 244
14.7 Transcript: one question to a grandmother . . . 244
14.8 Laboratory: the differential under time pressure . . . 245
14.9 Six errors . . . 246
14.9a Saying "I do not know" well . . . 246
14.10 Culture, faith and the urgent referral . . . 247
14.11 Evidence note . . . 248
14.12 Key terms . . . 248
14.13 Self-test . . . 249
14.14 Reflective task . . . 250
14.15 Further reading . . . 250
Week 15 — Integration: the developmental lens in practice . . . 250
15.1 The lecture in ninety minutes . . . 251
15.2 The three questions, worked in detail . . . 252
15.2a What the developmental lens cannot do . . . 252
15.2b The ten-item developmental history . . . 253
15.3b Saying the formulation in the family's language . . . 254
15.3 The one-page formulation . . . 254
15.3a How development enters the other twenty-six courses . . . 255
15.4 Capstone case: the Eze family . . . 256
15.4a The Eze case, worked forward . . . 258
15.4b The family that has been to four clinics . . . 259
15.5 The viva: what you will be asked . . . 260
15.5a What to keep from this course . . . 261
15.6 Laboratory: the capstone . . . 261
15.6a Marking your own formulation . . . 262
15.7 Six errors of the whole course . . . 263
15.7a The first year: five things nobody teaches . . . 263
15.8 Culture, faith and the course's own position . . . 264

15.9 Evidence: what this course claims . . . 265
15.10 Key terms of the course . . . 265
15.11 Self-test: the whole course . . . 266
15.12 Final reflective task . . . 267
15.13 Further reading . . . 267
Glossary . . . 267
Self-assessment . . . 267
Multiple choice . . . 267
Answer key . . . 268
Short answer . . . 268
Essay (choose one, 1,800 words) . . . 268
Further reading . . . 269

Course leader's summary. You cannot assess a family without knowing what is developmentally ordinary. This course covers physical, cognitive, language, social-emotional and moral development from conception to later life, always in family and cultural context — and it trains you to distinguish normal variation, delay, regression and symptom.

How to use this book

Development is not a list of ages; it is a set of **tasks** and a set of **negotiations** between a person and their family. Each week here gives you the developmental task, the family task that accompanies it, what goes wrong, and what it looks like in a consulting room. Use the milestone grid (Figure 23) as a reference you return to, not as a checklist to memorise.

1. Course description

Human development is the backbone of family assessment. A two-year-old's tantrum, a nine-year-old's lying, and a fifteen-year-old's secrecy are not three versions of "defiance": they are three different developmental moments that require three different responses. Clinicians who do not know this either pathologise normal development or miss genuine delay.

The course integrates **neurodevelopmental, cognitive, attachment, systemic and cultural** perspectives. It treats development as bidirectional: children shape parents as much as parents shape children, and families develop too (a theme developed in MFT 719).

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe physical, cognitive, language and social-emotional milestones from 0–18 and in adulthood	Exam
LO2	Distinguish normal variation, delay, regression and symptom in a presenting concern	Case assignment
LO3	Apply attachment theory to family assessment	Case assignment, exam
LO4	Explain developmental tasks of adulthood and later life and their family implications	Exam
LO5	Evaluate developmental theory critically, including its cultural assumptions	Essay
LO6	Communicate developmental information to parents without jargon or alarm	Skills assessment

3. Assessment

Component	Weight	Due
Developmental case analysis (2,000 words)	30%	Week 8
Parent-feedback role-play (10 min, observed)	20%	Week 11
Written examination	50%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Why developmental knowledge is clinical knowledge	Milestone self-audit
2	Prenatal development, temperament and the transition to parenthood	Temperament interview
3	Infancy: attachment, synchrony, regulation	Still-face observation
4	Toddlerhood: autonomy, language, limit-setting	Tantrum consultation role-play
5	Early childhood: play, theory of mind, initiative	Play observation
6	Middle childhood: industry, peers, school, self-concept	School-refusal case
7	Adolescence I: puberty, identity, autonomy	Adolescent interview
8	Adolescence II: risk, peers, digital life, mental health	Risk/development differential
9	Adult development: intimacy, work, generativity	Life-structure interview
10	Couple development across the lifespan	Couple timeline exercise
11	Parenting across stages; communicating with parents	**Observed parent feedback**
12	Later life: cognition, loss, caregiving, legacy	Caregiver family meeting
13	Culture, poverty and development	Critical reading seminar
14	When development goes wrong: delay, disorder, trauma	Differential diagnosis workshop
15	Integration: the developmental lens in assessment	Case presentations

Week 1 — Why developmental knowledge is clinical knowledge

Three clinical errors are prevented by developmental literacy:

1. **Pathologising the ordinary.** A two-year-old who says "no" to everything is doing the developmental task of autonomy, not manipulating the family.
2. **Missing the abnormal.** A five-year-old who has lost previously acquired language is not "being difficult"; regression is a red flag requiring urgent referral.

3. **Mis-timing the intervention.** A parent is told to "reason with" a three-year-old whose prefrontal circuitry cannot yet support the reasoning demanded. The intervention fails and the parent concludes they are failing.

Four categories you must distinguish — this distinction recurs in every case assignment:

Category	Definition	Example
Normal variation	Within the expected range, differing in style and pace	Walking at 11 months vs 15 months
Delay	Functioning below the expected range but progressing	No words at 24 months, but progressing
Regression	Loss of previously acquired skill	A four-year-old stops speaking
Symptom	Behaviour that signals distress rather than development	New soiling at seven after a bereavement



Figure 1.1 — Developmental milestones from 0 to 18: a screening map, not a diagnosis

PRACTICAL

Write your own developmental timeline from memory: first word, first step, a significant illness, a move, a loss. Then ask a parent or sibling to correct it. Note which events you had forgotten and who remembered them.

1.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The three errors	Three cases in which developmental ignorance produced harm	Name the error in each case
10–25	Development as negotiation	The person's task and the family's task, side by side	Write the pair of tasks for one age
25–40	The four categories	Normal variation, delay, regression, symptom, tested on video clips	Classify eight clips
40–55	The screening map	The milestone grid, used as a reference rather than a checklist	Locate a real child on the grid
55–70	What the grid cannot tell you	Culture, opportunity, illness, trauma, family organisation	List five things a milestone cannot show
70–82	Talking to parents	The sentence that informs without alarming	Rewrite three alarming sentences
82–90	Why this course is a clinical course	The assessment, the case analysis, the exam	Choose a family for the case analysis

The teaching decision that makes the week work. The week must establish a habit of mind, not a body of facts: before any behaviour is judged, ask what is the developmental task here, and whose task is it? The lecture should therefore work from three video clips of the same behaviour at different ages — a child refusing to share at two, at six and at thirteen — and let the class discover that the identical act means three different things and requires three different responses. The facts about milestones can be read; the habit must be demonstrated.

The sentence the lecture must land. "Behaviour is only meaningful in relation to a developmental task." A student who can look at a nine-year-old's lying and ask what task the lying is serving — rather than whether the child is honest — has begun to think developmentally, which is the whole of this course's foundation and the first line of every case analysis they will write for the next fifteen weeks.

1.2 Core concepts

Development as a set of tasks. Each stage of life presents a person with a task — to become attached, to become autonomous, to become competent, to become oneself, to become a partner, to become a parent, to become old — and each task requires a negotiation with the family. The task language comes from Erikson, and this course uses it because it is clinically usable: it converts a behaviour into a question about what the person is trying to achieve, and a question about what the family is being asked to allow.

Development as bidirectional. The child shapes the parent as surely as the parent shapes the child. A temperamentally reactive infant produces a more cautious mother; a mother's depression produces a more withdrawn infant; and the two effects are not separable, which is why the clinical unit is the relationship rather than the individual. This

is the course's central systemic claim, and it is the reason the course belongs in a family therapy programme rather than in a psychology department.

Development as a negotiation between tasks. At every stage there are two tasks: the person's and the family's. The toddler's task is autonomy; the family's task is to allow autonomy while keeping the child safe. The adolescent's task is identity; the family's task is to tolerate the adolescent's separateness without abandoning them. Most presenting problems in family therapy are a stalled negotiation between the two, and the therapist's developmental question is: which task is stalled, and whose?

The four categories. Normal variation (within range, differing in style and pace); delay (below range, but progressing); regression (loss of an acquired skill); and symptom (behaviour that signals distress rather than development). The categories are clinical instruments, and their misuse produces the three errors listed above. The distinction between delay and regression is the most urgent: delay usually allows assessment over weeks, and regression demands referral within days.

Normative and non-normative events. Normative events are those expected at a stage — starting school, puberty, leaving home, retirement. Non-normative events are unexpected — a parent's death, a move, an illness, a war, a divorce. The family life cycle literature (MFT 719) describes the normative; the clinician's task is to assess how a family's non-normative events have intersected with the normative ones, because that intersection is where most symptoms live.

The developmental history as a clinical instrument. A developmental history is not a form; it is the most efficient way to locate a family's difficulty in time. Not "was he a difficult baby?" but "when did this start, what else was happening then, and what changed at that point?" The history converts a presenting complaint into a chronology, and the chronology usually shows that the symptom began at a transition — which tells the therapist where to look.

Screening is not diagnosis. A milestone grid identifies children who need a fuller assessment. It does not diagnose, and it cannot distinguish a child who is late because of a global delay from one who is late because of deprivation, illness, trauma or a family in crisis. A clinician who treats the grid as a diagnostic instrument will refer children who are developing normally and miss children whose development is being distorted by their circumstances.

Culture, opportunity and the milestone. Milestones describe what children do when they are given the conditions to do it. A child who has not learned to count by six may be delayed, or may never have been taught; a child who does not speak in the clinic may be shy, or may speak a different language at home, or may have been told not to speak to strangers. The clinical question is always: what has this child had the opportunity to learn, from whom, in what language?

Risk and resilience. The literature's most useful finding for family therapists: the number of risk factors matters less than the balance between risk and protective factors, and the single most powerful protective factor is a relationship with at least one

consistently responsive adult. That finding is the developmental justification for family therapy, and it should be quoted in every assessment that identifies a child at risk.

1.2a The developmental history, asked well

The instrument the whole course depends on, and the one most often performed as a form. Eleven questions, each with what it is for.

One — "start at the beginning: was this a planned pregnancy, and how was it for you?" For: the parents' state at the child's beginning, which predicts a good deal about the relationship's foundation. An unplanned pregnancy in a hostile household is not a verdict; it is information.

Two — "how was the birth, and how were the first weeks?" For: birth trauma, prematurity, a mother's illness, a period of separation. The question also signals that the therapist is interested in the parents' experience, not only the child's.

Three — "who was at home in the first year, and who helped?" For: the caregiving network, which in many families is wider than the parents, and which is often invisible in the referral.

Four — "when did he first smile, sit, walk, and say a word?" For: the developmental trajectory, asked in ordinary language rather than as a checklist. Parents remember these with surprising accuracy, and their confidence in answering is itself informative.

Five — "was he different from your other children, and in what way?" For: the comparison parents make spontaneously, and the meaning they have given it.

Six — "when did you first become worried, and what made you worried then?" For: the starting point of the parents' concern, which is frequently earlier than the referral, and the event that converted a characteristic into a problem.

Seven — "what was happening in the family at that time?" For: the intersection of the child's development with the family's non-normative events. This is the question that produces the chronology's turning points.

Eight — "who has said what about it — school, clinic, family?" For: the descriptions already attached, and their authority. The answer tells the therapist what the family have been told, by whom, and how they have absorbed it.

Nine — "what have you tried, and what happened?" For: the family's own experiments, which are usually more numerous and more thoughtful than the referral suggests.

Ten — "what does he do well, and what does he love?" For: the competence description, which the referral will not contain, and which the therapy will need.

Eleven — "what would you want me to understand that I have not asked about?" For: the question the family have been waiting for, which is often where the real referral is.

How the questions are used in the session. Not read out in sequence; woven into the first forty minutes, in the family's language, with the child present where possible and asked directly at least three times. The therapist writes the chronology on a single sheet with dates, and shows it to the family at the end: "this is what I have understood; what have I got wrong?" The correction is the first collaborative act.

1.3 The clinical method: from behaviour to task

A four-step method that the rest of the course will use, and that a student can apply to any presenting behaviour.

Step one — describe the behaviour, precisely and without interpretation. Not "he is defiant" but "when his mother asks him to put his shoes on, he says no, sits down, and after ninety seconds his mother puts them on". The description should be filmable, dated, and observed in more than one setting where possible.

Step two — locate it developmentally. What is the expected behaviour at this age, in this culture, for a child with this history? The milestone grid gives the range; the developmental history gives this child's trajectory; the family's account gives the meaning. The output of this step is one of five statements: within normal variation; delayed but progressing; a regression; a symptom of distress; or impossible to tell without more information.

Step three — identify the task. What is the child trying to achieve, and what is the family being asked to allow? A nine-year-old's defiance may be an attempt at autonomy in a family where autonomy is punished; a two-year-old's tantrum is the task itself, not a failure of it; a fourteen-year-old's secrecy is the beginning of a private life. The task is not a hypothesis about cause; it is a description of what the behaviour is for, and it can be stated to the family in their own language.

Step four — test the formulation against the family's response. If the behaviour is an autonomy struggle, the family's response to a small increase in the child's choices should be observable within a week or two. If it does not change, the formulation is wrong, and the therapist revises it rather than pressing the family harder. This step is what distinguishes a developmental formulation from a developmental label.

Worked, briefly. The behaviour: a seven-year-old soiling again after two years of continence. Step two: regression, not delay, and therefore a signal. Step three: the task is not autonomy but the management of an experience the child cannot name — and the chronology shows the soiling began when the father moved out. Step four: the formulation predicts that the soiling will reduce as the child is given a way to speak about the departure, and the prediction can be tested in four sessions.

The method's use in the case assignment. The 30% developmental case analysis asks for exactly these four steps, applied to one child or adolescent, with the developmental history and a formulation. A student who follows the method will produce a defensible analysis; a student who begins with a diagnosis will produce a checklist.

1.4 Techniques: the four instruments of this course

The milestone grid, used as a reference. The grid (Figure 1.1) gives ranges by domain — gross motor, fine motor, language, social-emotional, cognitive. It is used in three ways: to locate a child's trajectory; to identify the domains in which the parents' concern is concentrated; and to notice what the parents have not mentioned, which is sometimes the more important finding. It is not used to score a child, and a therapist who reads the grid out to a parent has misunderstood its purpose.

The developmental timeline, drawn on one sheet. A horizontal line with the child's age marked in years, and above it the events the family describe: births, moves, illnesses, separations, deaths, school changes, and the points at which the presenting behaviour began or changed. The drawing is done with the family, in the session, and it is the single most useful artefact of a developmental assessment. Its power is visual: families see the coincidence of a symptom's onset with an event, which no amount of questioning produces as convincingly.

The three-setting comparison. Ask what the behaviour looks like at home, at school (or nursery, or church), and with a particular person — usually a grandmother, an aunt or a favourite teacher. The comparison answers the question the clinic cannot: is this behaviour a property of the child or of a relationship? A child who is silent at home and fluent at school is not a child with a language disorder.

The ordinary-language formulation. The therapist's developmental understanding, converted into one or two sentences a parent can use: "he is doing what six-year-olds do — testing whether the rules are the same when you are tired — and the useful response is to keep the rule and not to explain it at length." The formulation is the intervention, and its quality is judged by whether the parent can repeat it accurately a week later.

The developmental question, as a habit. "What would you expect a child of this age to be doing?" asked of the parents before the therapist offers an opinion. The question does three things: it elicits the family's developmental theory (which is often accurate), it reveals the standard against which the child is being judged, and it positions the therapist as a collaborator rather than an authority.

1.4a Two timelines, drawn and read

Timeline one — a developmental problem that is not a developmental disorder.

The case. Chidi, 6, referred by his school for "immaturity and poor concentration". The family: mother (29), father (34, works away), a new baby of four months, and a grandmother who has moved in to help.

The timeline. Birth, unremarkable. Walking at 14 months, first words at 15 months, sentences at 26 months — all within range. Nursery at three, described as "a pleasure". School at five, settled. At five years six months: the baby is born. At five years eight months: the grandmother moves in. At five years nine months: the school reports "immaturity".

The reading. Every developmental marker is within range, and there is no regression of an acquired skill. What changed was the household: a new baby, a grandmother, and a

mother whose attention is now divided three ways. The behaviour the school calls immaturity is a six-year-old's attempt to reclaim a position in a household that has reorganised without him.

The formulation, in ordinary language. "He is not behind. He is doing what a six-year-old does when a baby arrives and the house fills up: he is trying to get back the place he had. The useful response is not to expect less of him, which confirms his fear, but to give him a job with the baby that only he can do."

Timeline two — a developmental problem that is.

The case. Zainab, 4, referred by her nursery for "not talking". The family: mother (33), father (38), two older children, and a history the referral does not mention.

The timeline. First words at 13 months, two-word phrases at 20 months — unremarkable. At three years two months, the family moved cities. At three years four months, the paternal grandmother died. At three years five months, Zainab stopped using sentences. At three years eight months, she stopped speaking outside the home. At four, she speaks in single words to her mother only.

The reading. This is a regression: an acquired skill has been lost, and it was lost at a specific point. The differential is wide — hearing, neurological, trauma, selective mutism — and all of it requires assessment. The developmental formulation does not name the cause; it establishes the urgency and the sequence, and it says plainly that a child who has lost language needs to be seen by a paediatrician within weeks, not months.

The clinical action. Referral for hearing and paediatric assessment in the same week; the family work continues alongside, addressing the move and the bereavement in the child's language; the nursery is asked to stop asking her to speak and to describe what she does instead.

What the two timelines teach. The same presenting description — "not talking", "immature" — and two completely different clinical situations. The timeline is what distinguishes them, and the timeline takes fifteen minutes and one sheet of paper. A therapist who skips it will either pathologise Chidi or reassure Zainab's parents, and both errors are common. The instrument costs fifteen minutes and it is the difference between a developmental assessment and a developmental opinion.

1.4b The four categories, tested on eight descriptions

The laboratory's classification exercise, written out so that it can be revised and so that the reasoning can be examined. Classify each, and state what further information you would need.

One. A girl of 15 months has not walked; she pulls to standing, cruises the furniture, and is unconcerned. Classification: normal variation at the late end, with a review date. Information needed: birth history, any illness, family pattern of late walking, and a review in eight weeks.

Two. A boy of 26 months has twelve words; he understands instructions, points, and has begun to combine two words this month. Classification: delay, with progress. Information

needed: hearing test, whether he is being raised in more than one language, and the parents' account of who speaks to him and how.

Three. A girl of four who spoke in sentences has, over three months, stopped using sentences and now speaks in single words to her mother only. Classification: regression. Action: paediatric and audiology referral within the week; do not wait for a review.

Four. A boy of seven has begun wetting the bed after four years of being dry, and has become clingy at bedtime. Classification: symptom, with regression of a self-care skill. Information needed: what changed in the household in the last three months — illness, separation, move, a new adult, a bereavement.

Five. A girl of nine refuses to do homework, cries when her mother mentions school, and has stopped seeing her friends. Classification: symptom, and the school refusal is the presenting edge. Information needed: what is happening at school, including bullying and a specific adult; and what is happening at home that makes staying at home attractive.

Six. A boy of 13 has begun to close his bedroom door, to be private about his phone, and to prefer friends to family. Classification: normal variation — the developmental task of adolescence. Information needed: whether the privacy is accompanied by withdrawal from all relationships and by a change in functioning, which would make it a symptom instead.

Seven. A girl of 30 months has frequent tantrums lasting up to twenty minutes, several a day, in a household that has just moved and in which a new baby arrived six weeks ago. Classification: normal variation in behaviour, in a family under two non-normative events at once; the family may need support and the child does not need a referral. Information needed: the parents' own state, and whether the tantrums endanger anyone.

Eight. A boy of five is reported by his school as "unable to concentrate"; at home he plays Lego for forty minutes, and at his grandmother's house he sits through a two-hour church service. Classification: insufficient information — and the three-setting comparison already suggests that the behaviour is a property of the classroom relationship rather than of the child. Information needed: an observation in the classroom, the teacher's account, and the seating and task arrangements.

Why eight descriptions and not one. Each of the eight is decided by a different piece of information: the trajectory, the language environment, the loss of a skill, the household's recent events, the school context, the developmental task of the age, the number of stressors, and the cross-setting comparison. Students who can classify all eight, and name what they would need to know, have the course's first instrument.

1.5 The first session with a developmental concern

A structure for the assessment session, in five phases, with the words a therapist can use.

Phase one — the frame (five minutes). "I am going to ask about your child's development, and about what has been happening at home, because the two are connected. I will also want to see him, and I may want to speak to the school with your permission. At the end I will tell you what I think and what I would suggest." The frame

matters more in developmental work than in any other, because parents arrive expecting to be assessed themselves.

Phase two — the concern, in the parents' words (ten minutes). "Tell me about it in your own words, not the school's." The therapist writes the family's language and the referral's language in two columns, and notes the difference. The difference is usually informative: the school says "defiance", the mother says "he is angry with me since his father left".

Phase three — the developmental history (twenty minutes). The eleven questions of 1.2a, woven rather than read, with the child present and asked directly at least three times. The therapist draws the timeline as the answers arrive, and turns the sheet so the family can see it.

Phase four — the observation (ten minutes). Not a test: an observation of the child with a task and with the parent. A simple task — a puzzle, a drawing, tidying up — is enough to see the child's attention, his language, his response to a request and his response to his parent's response. The therapist narrates what she sees aloud, in neutral language: "I notice he looks at you before he answers." The narration is the assessment, and it is also the intervention, because it teaches the parent to look.

Phase five — the formulation and the next step (five minutes). One of the five statements from 1.3, in ordinary language, plus a next step: a paediatric referral, a school observation, a further session, or a plan. The formulation must include the family's competence: what they have already done that has helped.

What the therapist does not do in session one. Does not score a developmental test; does not name a diagnosis; does not tell the parents what they should have done; does not reassure them that everything is fine before the assessment is complete; and does not leave without a next step, and never ends a first session with a family who are unsure whether the therapist believed them.

The three sentences parents remember. "This is not your fault, and it is not nothing." "Here is what I think is happening, in one sentence." "Here is what we will do next, and when."

1.6 Case: the Okonkwos, a developmental reading

The course's recurring family, read developmentally, which is a different reading from the one any therapy model gives.

The family. Ebuka (9), his mother Ada (34), his father Chuka (39, works nights), and a baby of eleven months. The paternal grandmother died in 2023. The presenting complaint is defiance.

The chronological reading. Ebuka's developmental history is unremarkable: walking at 13 months, words at 16 months, reading at six, no illnesses, no separations. The family's non-normative events are the grandmother's death in 2023 and the baby's birth eleven months ago. The defiance began four months after the baby's birth, and intensified when Chuka changed to night shifts.

The developmental task. At nine, the task is industry: to become competent at something that matters, in the company of peers and adults who recognise the competence. The family's task is to provide a domain in which the child can be competent, and a structure in which his competence is visible. In this household, the domain has been removed: the grandmother who taught him to cook died, and the mother's attention is with the baby. The defiance is not a failure of industry; it is an attempt to be noticed in a household where the noticing has moved.

The task-stall, named. "He is trying to get back into the place he had before the baby and before your mother died, and the only lever he has found is refusing." The sentence is offered to the parents as a formulation, and it is testable: if the formulation is right, giving him a recognised role with the baby and restoring one of the grandmother's activities should reduce the defiance within three weeks.

What the reading adds to the therapy models. Week 4 of MFT 702 would describe a boundary problem; Week 6 would hypothesise a function; Week 8 would write a behavioural plan. The developmental reading supplies something none of them does: the content of the intervention. Not "spend more time with him" but "teach him to cook the one dish your mother taught him, and let him cook it on Sundays", which is the grandmother's function restored, the child's industry given a domain, and the family's grief given a place.

The three-week test. At the review: has he cooked twice? Has the defiance fallen on those days? If the answer is no to both, the formulation is wrong, and the therapist says so and revises.

1.6a The same child, three developmental misreadings

Misreading one — the diagnosis. The school's description ("possible ADHD") is accepted, and a referral for assessment is made without a developmental history. The assessment finds no disorder, three months are lost, and the family's confidence in services falls. The error: a description was treated as a finding. The correction: the timeline first, the referral second, and the referral framed as a question rather than a conclusion.

Misreading two — the reassurance. The therapist, noting that all milestones are within range, tells the parents that Ebuka is "a normal nine-year-old" and that the behaviour will pass. The parents hear that their distress is unwarranted, and they do not return. The error: the developmental assessment answered a question nobody had asked (is he delayed?) and ignored the question they had (what is happening to my family?). The correction: both statements, in this order — "his development is on track, and what is happening at home is real and worth working on".

Misreading three — the moral reading. The defiance is treated as a discipline problem, and the intervention is a firmer consequence. The behaviour worsens, the mother's confidence collapses, and the father's absences increase. The error: a developmental signal was read as a character defect. The correction: the task question — what is the behaviour for? — asked before any consequence is designed.

The pattern across the three. Each misreading is produced by skipping one step of the method in 1.3: the diagnosis skips the description and the timeline; the reassurance skips the task; the moral reading skips the developmental location. The method is short, and every step of it is load-bearing: skip one and the assessment produces an opinion instead of a formulation.

1.7 Annotated transcript: the timeline drawn

Twelve turns from the first session with the Okonkwos, while the therapist draws the timeline. The transcript shows how a developmental history is actually taken.

> **T1 Therapist:** I want to draw a line, and put on it everything that has happened since Ebuka was born. Can we do that together? When was he born?
> **T2 Ada:** March 2016. He was fine, everything was fine.
> **T3 Therapist:** (writing) Fine. When did he walk?
> **T4 Ada:** Thirteen months. He was quick.
> **T5 Therapist:** And talking?
> **T6 Ada:** Sixteen months. He talked early too. His grandmother used to say he talked for the whole family.
> **T7 Therapist:** (pause) Tell me about his grandmother.
> **T8 Ada:** She died in 2023. In August. She lived with us.
> **T9 Therapist:** (writing, then turning the sheet) So — here is 2023, and here is August. What happened in the house after that?

> **T10 Ada:** Everything changed. She used to cook with him. She used to sit with him in the evening and he would tell her everything.
> **T11 Therapist:** And when did the problems start?
> **T12 Ada:** (long pause) After the baby. But — now that I am saying it out loud — he stopped telling me things when she died.

Annotations. T1 frames the timeline as a joint activity, which converts an interrogation into a drawing. The paper is turned toward the family throughout, and T9's "turning the sheet" is the moment the instrument does its work.

T3–T6 are the milestones, asked in ordinary language, and they are answered with confidence — which is itself a finding: this mother knows her child's development, and her competence is visible to the therapist and, more importantly, to her.

T7 is the pivot, and it is earned: the therapist follows the grandmother's name rather than the schedule. The grandmother appears in the mother's answer as a person with a relationship to the child, which is the most important thing said in the session.

T9 is the timeline's use: the therapist locates the event in time and asks what happened next, which is a different question from "how did you feel". T10 produces the content of the intervention: cooking, and evening conversation.

T12 is the session's turning point, and it happens because the timeline made the sequence visible. The mother corrects her own account — "after the baby… but now that I am saying

it out loud" — and the correction is hers, not the therapist's. That is what a timeline does that a question cannot: it makes the family's own sequence visible to the family, and the correction that follows belongs to them.

What the trainee should notice. The therapist asks eleven questions and makes no interpretation. The one interpretive act in the session is the mother's, produced by the drawing. The instrument does the work, and the therapist's skill is in holding the paper and following the family's own order of significance.

1.8 Laboratory: the timeline and the four categories

Sixty minutes, in pairs and then in the whole group.

Part one — the timeline from memory (fifteen minutes). Each student draws their own developmental timeline, then interviews the partner using the eleven questions, drawing the partner's timeline. The drawing, not the notes, is the output.

Part two — classification (twenty-five minutes). The lecturer presents eight short cases, each a description of a behaviour with a minimal history. Students classify each as normal variation, delay, regression, symptom, or insufficient information, and must state what further information they would need. Two of the eight are deliberately unclassifiable.

Part three — the sentence (twenty minutes). For each of four cases, each student writes the ordinary-language formulation in two sentences and reads it aloud. The group judges whether a parent could repeat it a week later.

Marking rubric for the 30% case analysis.

Criterion	Weight	Full marks
Description	15%	Behaviour described precisely, in more than one setting, without interpretation
Developmental location	25%	Correct category, with the evidence for it and the differential acknowledged
Task formulation	20%	The person's task and the family's task stated, in testable form
History and chronology	20%	Timeline with normative and non-normative events, and the onset located
Ordinary language	10%	A formulation a parent could repeat
Limits	10%	What the formulation cannot tell you, and what would change it

The distinction-level answer. It distinguishes delay from regression with evidence; it names the task in a sentence the family could use; it locates the onset against the family's events; it states what would falsify the formulation; and it does not diagnose.

1.9 Six errors in developmental assessment

1. **The checklist.** Running the milestone grid as a questionnaire, which produces scores without a child. Fix: the grid as a reference, the timeline as the instrument.

2. **The premature diagnosis.** Naming a condition in the first session, which forecloses the assessment and organises the family around a label. Fix: the five statements of 1.3, and a referral framed as a question.
3. **The false reassurance.** "He will grow out of it", offered before the assessment is complete. Fix: two statements — development is on track, and the family's difficulty is real.
4. **The missed regression.** A lost skill attributed to stubbornness. Fix: regression is a red flag; refer within days.
5. **The decontextualised child.** Assessing the child alone, in a clinic, without the household, the school or the language. Fix: the three-setting comparison, and at least one session with the caregiving network.
6. **The moral reading.** Treating a developmental signal as a character defect. Fix: the task question, asked before any consequence is designed.

1.10 Culture, faith and development

The cultural question, stated baldly. Almost every developmental milestone in this book was established by studying children in Europe and North America, and almost every one of them is a statement about what children do when they are fed, housed, spoken to and played with in a particular way. That does not make the milestones false; it makes them conditional, and the condition is the child's opportunity.

Three examples where the condition matters.

Language. The vocabulary and grammar norms in the literature come from children learning a single language in a household where adults speak to them directly. A child in a multilingual household, or a household where adults speak to each other and not to children, may be developing normally and score below the norm. The clinical error is to call this delay; the clinical task is to assess competence in the child's own language, with a speaker of that language.

Autonomy. The two-year-old's task is described in the literature as autonomy, and in many households the task is instead obedience, and the child who asserts autonomy is corrected earlier and more firmly. The child is not delayed; the family's developmental curriculum is different. The therapist who treats the family's curriculum as the problem will lose the family, and the therapist who treats the child's autonomy as the only healthy outcome will impose a value.

Sleep and independence. The literature on infant sleep assumes a separate sleep surface and a self-settling child. In households where infants sleep with their mothers, the milestone's behavioural markers are absent and the developmental process is not. A therapist who recommends "sleep training" to such a family is not offering developmental science; they are offering a cultural practice.

What the competent practitioner does. Three things. First, asks the family what is expected of a child of this age in their household, and what happens when a child does not meet it. Second, assesses competence in the child's own language, with the family's own

tasks, before comparing to a norm. Third, names the translation when they make one: "in the studies this comes from, children are spoken to directly by adults; in your house it is different, so I am going to test something else."

Faith and development. Religious households have their own developmental curriculum: when a child prays, fasts, reads scripture, is initiated, and takes a place in the community's rituals. These are developmental tasks in the family's terms, and they matter to the child's identity as much as school competence does. The clinician's interest is not in the content of the curriculum but in its fit with the child: a child who is required to fast before puberty may be perfectly well; a child required to carry an adult's religious anxiety is not, and the difference is the child's own state, not the practice.

The examinable sentence. "The milestones are conditional on opportunity, and the clinical question is what this child has had the chance to learn, from whom, in what language." A student who can say that and apply it to a family in front of them has understood the course's cultural premise.

> **DEBATE**
> "Developmental norms are a tool of cultural standardisation, and a family therapist should use them only with reluctance." Argue both sides, and say what you would lose by abandoning them.

1.10a The course's four categories, applied to a family

A worked application of the four categories to the Adeyemi family, because the categories are usually taught with individual children and used with families.

Normal variation, in a family. A fourteen-year-old who wants to spend Saturday with friends rather than relatives is doing the developmental task of the age. The family experiences it as rejection. The clinical response is to name the task, to the parents, and to distinguish it from the conflict that may surround it.

Delay, in a family. A family in which the parents have not yet made the transition from managing a child to negotiating with an adolescent: the developmental task of the parents is delayed. The adolescent's behaviour is age-appropriate and the family's organisation is not, and the formulation must say so without blaming the parents, who may have had no model of adolescence in their own upbringing.

Regression, in a family. A family that had established a workable routine, then lost it when a member died: the household's functioning has regressed to an earlier, less differentiated arrangement. The regression is a signal, and it tells the therapist that the family's coping has been overwhelmed rather than that the family is deficient.

Symptom, in a family. A child's school refusal that functions to keep a parent at home during a marital crisis is a symptom — a signal of the system's distress — and it is neither normal variation nor delay. The clinical response is to treat the signal as information about the system, which is exactly what Week 4 of MFT 702 argues.

Why the family application matters. Students who learn the categories on individual children classify individual children and then treat families as if the categories did not apply to them. They do: households have developmental tasks, and households can be delayed, regressed, symptomatic or simply varied. The therapist who can say "your family is doing what families do at this stage, and the difficulty is that the stage has arrived early" has a formulation the parents can use.

1.11 Evidence note: what developmental science can and cannot support

What is well established. The sequence of gross motor and language milestones, with wide normal ranges; the importance of responsive caregiving in the first years; the association between early adversity and later difficulties, with the caveat that the association is probabilistic; the protective effect of at least one consistent relationship; and the finding that development is bidirectional, with child characteristics shaping parenting as much as the reverse.

What is contested. The size and reversibility of early adversity's effects (the "first thousand days" literature is stronger on the first claim than the second); the predictive value of infant temperament for adult personality; the specificity of attachment classifications for later disorder; and the extent to which screeners improve outcomes, as distinct from identifying children.

What is often misused in clinical settings. Milestone checklists presented to parents as though they were diagnostic; "brain development" arguments used to pressure parents into particular practices; and attachment language used to describe ordinary family difficulty. A family therapist should be able to recognise each of these, because each will be used on the families they see.

What the developmental literature gives a family therapist. Three things of real value: a timeline against which to locate a concern; a set of tasks that make behaviour intelligible; and the strongest evidence base in the field for the claim that relationships are the mechanism of development, which is also the claim that justifies family therapy.

The matrix cell, in this course's terms. "Strong descriptive evidence for sequences and ranges; moderate evidence for caregiving's role; contested evidence for early-adversity determinism; and no evidence for any single parenting practice as universally optimal." A student who writes that in an examination will do well.

The practitioner's claim. "I use developmental knowledge to locate a behaviour and to distinguish variation from delay from regression from symptom; I use the timeline as my main instrument; I do not diagnose from milestones; and I assess competence in the family's own language and tasks before comparing to a norm." That sentence is the course's professional baseline, and it is also the sentence to say in a case discussion when a colleague reaches for a diagnosis before a history.

1.11a What development cannot explain

A short section, because the developmental frame is powerful and can be overreached.

Development cannot explain a family's meaning. That a child is nine tells you about industry, peers and concrete-to-abstract thinking; it does not tell you what the child's defiance means in a household where his grandmother has died. The developmental frame supplies the stage; the family supplies the meaning. A therapist who explains the second by the first is committing the error this course exists to prevent in the opposite direction.

Development cannot explain a symptom's function. A regression is a signal, and what it signals may be a developmental event, a family event, or both. The developmental reading locates the timing; the systemic reading explains the function. The two are complementary and neither is sufficient.

Development cannot adjudicate values. Whether a child should sleep alone, be circumcised, be punished physically, be taught scripture, or be allowed to choose her own friends are questions about which developmental science has some evidence and no authority. The practitioner's obligation is to know the evidence, state it, and distinguish it from their own values — which is the same obligation this book's later courses will examine in ethics and culture.

The sentence to keep. "Development tells me when and what to expect; the family tells me what it means; and I must not confuse the two, in either direction."

1.12 Key terms

Term	Definition
Developmental task	The challenge a stage presents to a person, and the matching negotiation it presents to the family
Bidirectional development	Child and caregiver shape each other; neither is the sole cause
Normal variation	Behaviour within the expected range, differing in style and pace
Delay	Functioning below the expected range, but progressing
Regression	Loss of a previously acquired skill; a red flag
Symptom	Behaviour signalling distress rather than development
Normative event	An event expected at a stage (school entry, puberty, leaving home)
Non-normative event	An unexpected event (bereavement, migration, illness, war)
Developmental history	A chronology of milestones and family events, used to locate a concern in time
Timeline	The one-sheet drawing of the chronology, made with the family
Three-setting comparison	Comparing behaviour at home, elsewhere, and with a specific person
Opportunity condition	The assumption that milestones describe children who have had the chance to learn

1.13 Self-test

1. Name the three clinical errors that developmental literacy prevents, with an example of each.

2. Distinguish delay from regression, and say why the distinction changes the urgency.
3. What are the four steps of the method in 1.3?
4. Give five of the eleven developmental-history questions and what each is for.
5. Why is the three-setting comparison useful, and what does it rule out?
6. State the bidirectional claim and why it matters to a family therapist.
7. What does the milestone grid not tell you?
8. How does a therapist handle the cultural condition on the milestones?

Answers. 1. Pathologising the ordinary (a two-year-old's "no"); missing the abnormal (a five-year-old who stops speaking); mis-timing the intervention (reasoning with a three-year-old). 2. Delay is below range but progressing and permits assessment over weeks; regression is the loss of an acquired skill and requires referral within days. 3. Describe the behaviour precisely; locate it developmentally (one of five statements); identify the task for the person and the family; test the formulation against the family's response. 4. Any five: the pregnancy and its context; the birth and first weeks; who was at home and who helped; milestones in ordinary language; how the child differed from siblings; when the parents first became worried; what else was happening then; who has said what; what has been tried; what the child does well; what has not been asked. 5. It establishes whether the behaviour is a property of the child or of a relationship, and it rules out a generalised language or behavioural disorder when the child is fluent elsewhere. 6. Child and caregiver shape each other continuously; it matters because the clinical unit is the relationship, and because blaming either party misreads the evidence. 7. It cannot distinguish causes — deprivation, illness, trauma, family crisis — nor diagnose; it identifies children who need fuller assessment. 8. Ask what the family expects of a child of this age; assess competence in the child's language and tasks; name the translation when comparing to a norm.

1.14 Reflective task

Draw your own developmental timeline from memory, then ask the person who raised you to correct it. Write two paragraphs: one on what you had forgotten, and one on what your family remembered differently. The exercise is the course's first instrument applied to the student, and most people discover that their own developmental history is a family story with a particular narrator — which is exactly what the families in this course will present to them. Keep the corrected timeline: it is the first artefact of your own developmental literacy, and you will read it again in Week 15.

1.15 Readings for this week

- **Berk (2017)**, Development Through the Lifespan, the reference text for the course, read the opening chapters for the frameworks and keep it for the milestones.
- **Erikson (1963)**, Childhood and Society, for the task language, read as a historical document as well as a theory.
- **Bronfenbrenner (1979)**, The Ecology of Human Development, for the nested contexts that make the family's environment part of the child's development.

- **Sroufe (2005)**, "Attachment and development", for the evidence on continuity and change.
- **Rogoff (2003)**, The Cultural Nature of Human Development, the essential corrective to the norms, and the source of this week's cultural argument.
- **Sameroff (2009)**, The Transactional Model of Development, for the bidirectional claim in its strongest form.

Read Rogoff first and Berk second; the corrective before the reference. For the case analysis, use at least one source that disputes a norm you are relying on, and say so in the paragraph on limits. The reading-log question this week is: "which of my own developmental assumptions did this reading make visible?"

Next: Week 2 — Prenatal development, temperament and becoming a parent

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Week 2 — Prenatal development, temperament and becoming a parent

Prenatal and perinatal. Brain development, maternal stress and nutrition, birth experience, and the "fourth trimester". Clinically relevant: prematurity, birth trauma, postnatal depression (which affects up to 1 in 5 mothers in some settings and is strongly associated with relational and child outcomes), and paternal postnatal depression.

Temperament. Nine dimensions (Thomas & Chess) usually simplified to three profiles: easy, slow-to- warm, and difficult/feisty. Temperament is not destiny but it is **goodness of fit** that matters: a highly sensitive child in a loud, unpredictable household will struggle more than the same child in a predictable one.

The transition to parenthood. The couple must reorganise: time, sleep, money, division of labour, sexuality, relationship with grandparents, and identity. Research consistently finds relationship satisfaction dips after the birth of a child — and that the quality of the co-parenting alliance, more than the amount of conflict, predicts child outcomes.

> **CASE**
> A couple presents with "no time for each other" four months after their first baby. The developmental frame normalises the strain (this is the most common point of relational collapse), then asks the systemic question: which grandparent has been recruited into the parenting triangle, and what did the couple stop negotiating when the grandmother moved in?

Practice. In pairs, conduct a 10-minute temperament interview with a parent (or peer playing one): sleep, feeding, reaction to change, intensity, mood. Then identify the goodness of fit with the household.

2.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	Before birth	The prenatal environment, and why a family therapist cares	Name one prenatal factor in a family they know
10–25	Prematurity and the fourth trimester	The relational consequences of a medical beginning	Map a premature infant's caregiving network
25–40	Postnatal depression	One in five mothers; the relational mechanism; the father's version	Screen a volunteer with a two-question tool
40–55	Temperament	Nine dimensions, three profiles, and goodness of fit	Interview a peer as a parent
55–70	Becoming a parent	The reorganisation: time, money, sex, labour, grandparents, identity	Draw the reorganisation on one page
70–82	The co-parenting alliance	Why it predicts child outcomes better than conflict	Rate three recorded couple clips
82–90	The clinical frame	Normalising the strain without dismissing it	Write the normalising sentence

The teaching decision that makes the week work. This week's material is the most likely in the course to be delivered as a list of risk factors, and the most useful version of it is a demonstration of how a normal developmental event becomes a clinical presentation. The lecture should take the commonest presentation in perinatal work — "we have no time for each other" — and show that it is simultaneously ordinary, dangerous, and responsive to a small intervention. The class should leave able to say what is normal about the fourth month after a first baby, and what is not.

The sentence the lecture must land. "The birth of a child is a developmental event for the parents, not only for the child." Most of this week follows from it: the symptoms are the parents', the tasks are the parents', and the assessment must include the couple's reorganisation, not just the baby's milestones.

2.2 Core concepts

The prenatal environment. The foetus develops inside an environment made of the mother's nutrition, stress hormones, illness, substances and medical care, and that environment is itself shaped by her circumstances — poverty, work, a violent household, a wanted or unwanted pregnancy, a partner's support. The clinical relevance is not that prenatal events determine later life but that they are the first chapter of the family's story, and families bring that chapter to the clinic, often without knowing they are doing so.

The fourth trimester. The three months after birth, in which the infant is neurologically immature and utterly dependent, and the parents are in a state of physiological and emotional reorganisation. Sleep deprivation, hormonal change, the loss of an established identity and the arrival of an unremitting demand are all normal. The concept is clinically useful because it converts a set of complaints into a stage.

Prematurity and the medical beginning. A premature birth changes the sequence: the parents' first relationship with their child is often mediated by machines, by staff, by fear, and by a period of separation. The consequences are relational rather than deterministic:

parents may be more anxious, more inclined to monitor, less confident in their own competence, and the infant may be more reactive, harder to read, and slower to settle. The clinical task is to restore the parents' confidence in their own reading of their child, which is the thing the neonatal unit has, understandably, taken from them.

Birth trauma. A difficult birth can leave a mother with a sense of violation, a fear of the body, and a reluctance to speak about it — often because she has been told she should be grateful. The clinical task is to ask, specifically, and to let her tell it. The question "how was the birth?" asked with time to answer is one of the most useful questions in perinatal work.

Postnatal depression. Affecting roughly one mother in five in many settings and a substantial minority of fathers, postnatal depression is the most clinically consequential condition in this week's material. Its mechanism is relational: a depressed mother is less responsive, the infant becomes more distressed and harder to soothe, the father feels excluded or overloaded, and the couple's negotiation collapses. The screening questions are short and should be asked of both parents at every perinatal contact.

Paternal postnatal depression. Less recognised and equally real: irritability, withdrawal into work, alcohol use, and a refusal to describe the state as depression. The clinical presentation is often a couple conflict, and the diagnosis is missed because nobody asks the father.

Temperament, and the nine dimensions. Thomas and Chess described nine dimensions — activity level, regularity, approach or withdrawal, adaptability, sensory threshold, intensity of reaction, mood, distractibility and persistence — usually simplified into three profiles: easy, slow-to-warm, and difficult (or feisty). The dimensions matter clinically because they describe the child's contribution to the relationship, which many parents experience as a verdict on their parenting.

Goodness of fit. The concept that saves temperament from fatalism: the outcome depends on the match between the child's characteristics and the environment's demands. A highly sensitive child in a loud, unpredictable household struggles; the same child in a predictable household thrives. The clinical implication is that the intervention may target the environment rather than the child, which is a systemic move disguised as a developmental one.

The transition to parenthood. The couple must renegotiate seven domains: time, sleep, money, the division of labour, sexuality, the relationship with grandparents, and identity. The research finding that satisfaction dips after the birth of a child is robust across cultures, and the dip is not a sign of a failing marriage; it is a stage. What distinguishes couples who recover from those who do not is not the amount of conflict but the quality of the co-parenting alliance.

The co-parenting alliance. The couple's capacity to coordinate as parents — to support each other's relationship with the child, to present a united approach without undermining each other, and to keep the child out of their conflicts. It predicts child outcomes more strongly than marital satisfaction does, which is why perinatal work that focuses only on the couple's romance misses the mechanism.

Becoming a parent as an identity transition. The arrival of a child requires the adult to become someone new, and the new identity is built from the models available: their own parents, their culture's expectations, their partner's expectations, and their own ideals. The clinical presentations are recognisable: the parent who cannot bear to be like their own mother and over-corrects; the parent who has no model at all; the parent whose identity was entirely occupational and who now has none.

The couple's reorganisation as the unit of assessment. The perinatal assessment is not the baby's milestones; it is the household's reorganisation. Who sleeps when; who feeds; who works; who decides; who else is in the house; what has been given up; and what the couple have stopped negotiating. The last question is the most productive, and it is the one the week's case turns on.

2.2a The goodness-of-fit grid, filled in

A worked instrument: the temperament interview, the household's demands, and the fit between them, on one page.

The child. Three months old, first baby. Dimensions (parent's report): high activity; irregular sleep; withdraws from new sounds; slow to adapt to a change in routine; low sensory threshold (startles at the door); intense reactions (screams, not cries); predominantly negative mood in the evening; hard to distract; persistent.

The profile. Difficult or feisty, by the classical typology — and the therapist should say "this describes a pattern, not a verdict on your baby" when reporting it.

The household's demands. People: mother, father, and the father's mother, who has moved in "to help". Rhythm: the father works a rotating shift, so the household's week changes every seven days. Noise: a shared compound with four other households and a generator. Space: one room for the family. Support: the grandmother is present but her methods differ from the mother's, and the mother does not feel able to say so. Sleep: the mother is doing all night feeds because the father "needs to be fresh for work".

The fit, in four statements. First, this infant's low threshold is being met by a high-noise environment, which produces more crying, which produces more maternal exhaustion. Second, the infant's irregularity is being met by a household whose week changes every seven days, and no routine survives. Third, the infant's intensity is being read by the mother as rejection ("he hates me by six o'clock"), which is the meaning that matters more than the temperament. Fourth, the evening negativity coincides with the grandmother's arrival home, which the mother has noticed and not said.

The intervention, derived from the fit rather than from the child. Not a sleep programme, not a referral. Four changes: a predictable evening hour with the light low and the door closed, negotiated with the grandmother; the father taking the six-to-nine period on his rest days, which is a co-parenting act rather than a favour; one sentence the mother can say to the grandmother, rehearsed in the session; and a re-description of the evening ("he is not rejecting you; by six o'clock he has had a day and he needs less, not more").

What the grid demonstrates. That the same infant in a different household would present differently, and that the intervention targets the fit. A therapist who treated the temperament as the problem would have produced a programme the household could not run; a therapist who treated the household as the problem would have blamed a family that is doing a great deal well.

2.2b The nine dimensions, with the question that asks each

The dimensions are usually taught as a list. They are more useful as nine questions a therapist can ask a parent in fifteen minutes.

Dimension	The question	Why it matters clinically
Activity	"Is he usually on the move, or does he stay still and watch?"	A high-activity child in a small flat with a tired mother is a fit problem, not a behaviour problem
Regularity	"Does he sleep and feed at the same times each day?"	Irregularity plus a shift-working household produces a fortnightly crisis
Approach/withdrawal	"What does he do with a new food, a new person, a new room?"	Withdrawal is read by parents as rejection or shyness; the useful response differs
Adaptability	"How long does it take him to settle after a change?"	Slow adaptation plus frequent change is a formula for distress
Sensory threshold	"Does he startle at the door, the generator, a raised voice?"	Low threshold is the dimension most often mistaken for "spoiling"
Intensity	"When he is upset, is it a whimper or a scream?"	Intensity determines the household's exhaustion and the parent's sense of competence
Mood	"Is he mostly cheerful, or mostly serious and cross?"	Negative mood in the evening is normal and is read as rejection
Distractibility	"Can you settle him by showing him something else?"	The one dimension that is a resource: a distractible child is easier to soothe
Persistence	"If he wants something, does he give up or keep going?"	Persistence is a strength in a task and a problem in a battle of wills

How to use the table. Not as an assessment instrument with scores, but as a repertoire, so that the parent leaves with a description of their child that is specific, neutral and recognisably true. A parent who says "he is difficult" can be given nine sentences that are more accurate and less judgemental, and the nine sentences are the beginning of the intervention.

The two dimensions that produce the most relief. Sensory threshold and intensity. They are the two the household experiences most acutely, and the two most often moralised ("he is spoiled", "he is a screamer"). Naming them as characteristics rather than choices changes the parent's relationship to the behaviour within one conversation.

2.3 Postnatal depression: the assessment

The condition is common, consequential and under-detected. A structure for asking about it.

Who to screen. Every mother at every perinatal contact, and every father at least once, separately, with the baby out of the room and the partner absent. The separate conversation is the instrument; a joint conversation will not produce an honest answer from either parent.

The screening questions. Two, used as a first filter in a family therapy session (a validated tool is used where the service requires one): "In the past two weeks, have you often been bothered by feeling down, depressed or hopeless?" and "Have you often been bothered by having little interest or pleasure in doing things?" A yes to either is followed by "how much is this affecting your day-to-day life?" and, where the answer is substantial, by a validated measure and a referral.

The three questions that reveal the relational picture. "Who knows how you are feeling?" "What happens in the house when you have a bad day?" "What does your partner do when you cry?" The answers describe the support available, the household's capacity to accommodate distress, and the couple's pattern — which is where the family therapy begins.

The paternal screen. "How are you sleeping?" "What do you do when you feel low?" "Who would you tell?" Fathers rarely use the word depression; the observable signs are irritability, working longer hours, drinking more, and a withdrawal from the baby's care.

The risk questions. Thoughts of harm to self or the baby are asked directly, of both parents, and the answer is recorded and acted on. A therapist who does not ask because asking feels intrusive has misunderstood the duty.

The differential. Not every exhausted mother is depressed. Sleep deprivation, thyroid disease, anaemia, grief, an unsupportive partner, a violent household and a baby's illness all produce the same presentation, and the family therapist's task is to screen, to refer where the picture is clinical, and to work on the relational and practical conditions in parallel.

The boundaries of the family therapist's role. Treating postnatal depression is a clinical task that requires a clinical service; the family therapist's contribution is the couple's alliance, the household's reorganisation, the father's inclusion, and the practical support. Saying that clearly at the start prevents the two most common failures: a family therapist attempting to treat a depression alone, and a clinical service treating a depression in a household that is falling apart.

2.3a The couple's reorganisation, drawn on one page

The instrument the week's case turns on, and the one a couple can complete in fifteen minutes.

The seven domains, with the question for each. Time: what do you do together now that you did before? Sleep: who gets up, how often, and what happens the next day? Money: who earns, who decides, and what has changed? Labour: list everything that has to happen in a week and who does it. Sexuality: what has happened to it, and who has said what about it? Grandparents: who is in the house or in the decisions, and how was that decided? Identity: who are you now that you were not before, and what have you lost?

The drawing. A circle with the seven domains around it, and in the middle the couple's answer to one question: "what have you stopped negotiating?" The answers are usually specific and painful: "who gets a night off"; "whether your mother stays"; "whether I go back to work"; "whether we have another".

Why the drawing works. It converts a diffuse complaint ("we have no time") into seven specific negotiations, of which usually two are live and five are being avoided. The live ones are the therapy's agenda, and the avoided ones are the reason the live ones cannot be settled.

The grandmother question, which is the week's case. When a grandparent moves in to help, the couple's negotiation is displaced: the decisions that were theirs are now made in a triangle, and each partner's relationship with the grandparent becomes a route to influence. The clinical questions are: who invited her; who can ask her to leave; what does each partner gain from her presence; and what did the couple stop deciding when she arrived? The last question is usually answered with a silence, and the silence is the finding, and the therapist's next sentence should be the one that makes it speakable: "I am not asking you to blame anyone. I am asking what the two of you stopped deciding."

The intervention, in one sentence. "Before we talk about time for each other, I want to know what the two of you stopped deciding when your mother moved in, because that is where the time went."

2.4 The first session with a couple four months after a birth

The commonest perinatal presentation, worked through, with the developmental frame used first and the systemic question second.

The presentation. Aisha (31) and Sam (34), married four years, one baby of four months. Aisha's complaint: "we have no time for each other and he does not help." Sam's complaint: "she is never happy with anything I do." Both are exhausted. The baby is present and is being passed between them.

Phase one — the normalisation (ten minutes), done carefully. "What you are describing is the commonest thing I see in the first year, and I want to say why before we go further. At four months, neither of you has slept properly since the birth, you are both doing a job you have never done before, and the negotiations you used to have — who does what, when — are being made at two in the morning with a baby crying. That is not a failing marriage. It is the stage."

Why this is not a dismissal. The normalisation is followed immediately by the second sentence: "and it is also the point at which couples get into patterns that last for years, so it is worth working on now."

Phase two — the seven domains (twenty minutes). The reorganisation drawn on one page. Time: none together. Sleep: Aisha does every night feed. Money: Sam has taken a second job, so he is home less. Labour: Aisha does everything in the house and Sam does the shopping. Sexuality: untouched since the birth, and neither has raised it. Grandparents: Sam's mother stays four nights a week "to help". Identity: Aisha has

stopped working and says she "does not know who she is"; Sam says he is "the one who is never there".

Phase three — the question (ten minutes). "What did the two of you stop deciding when your mother started staying four nights a week?" Silence. Then Aisha: "everything. She decides the bath, she decides the feeding, and I cannot say anything because she is helping us." Sam: "I did not know you minded."

The finding. The couple's negotiation has been displaced into a triangle with the grandmother, and neither has said so. The "no time for each other" is a symptom of a negotiation that has stopped.

Phase four — a small, systemic task (ten minutes). Not a date night, which the couple cannot arrange and which would fail. Two tasks: the couple decide, together, one thing that is theirs rather than the grandmother's — the baby's bath, three nights a week — and one of them tells her, in a sentence rehearsed in the room. And Sam takes one night feed at the weekend.

Phase five — the frame (five minutes). Four sessions, with a review at the fourth; the paternal screen completed separately; a question about Aisha's mood and a referral route if the screen is positive; and the sentence to take away: "the problem is not that you have no time; it is that you have stopped deciding, and that is fixable."

What the therapist did not do. Did not take a full developmental history of the baby, which was not the presentation; did not adjudicate whether the grandmother should stay; did not suggest a date night; and did not treat the couple's complaint as trivial because it is common.

2.4a The first year, month by month: what a family therapist watches for

A timeline of the relational tasks of the first year, with the presentations that bring families to a clinic.

Months 0–1: establishment. The task is the establishment of feeding, sleep and a caregiving rhythm, in a household that has just changed shape. Presentations: feeding difficulty; a mother who cannot settle; a father who feels excluded; an uninvited relative who has taken over. The family therapist's move: the household's arrangement, drawn, and the question of who decided.

Months 1–3: the fourth trimester. The task is the parents' survival and the infant's regulation. Presentations: exhaustion presented as a relationship problem; the first arguments about nights; a mother's low mood that nobody has screened. The family therapist's move: the screen, the seven domains, and the normalisation that does not dismiss.

Months 3–6: the first negotiation. The task is the couple's re-negotiation of time, sleep and labour, and the infant's entry into social exchange — smiling, vocalising, the first games. Presentations: "we have no time"; the grandmother's methods; the father's withdrawal into work; the mother's return to work or refusal to return. The family therapist's move: the reorganisation drawing and the stopped-negotiation question.

Months 6–9: separation and mobility. The task is the beginning of separateness — weaning, crawling, the first "no", the first night away — and the parents' tolerance of the child's independence. Presentations: anxiety at separation presented as the child's problem; a parent who cannot leave; feeding battles; sleep battles. The family therapist's move: the graded separation plan, and the question of whose anxiety is in the room.

Months 9–12: the first year's reckoning. The task is the consolidation of attachment and the couple's assessment of their own first year. Presentations: a couple who have not had a conversation since the birth; a second pregnancy already underway; a decision about work that is being made in a state of exhaustion. The family therapist's move: the review, the seven domains again, and the preparation for the second year's tasks.

What the timeline is for. It gives the therapist a developmental hypothesis for any presentation in the first year, and it protects against the commonest error in perinatal work: treating a stage-specific difficulty as a relationship defect. A couple arguing about nights at month two are doing what couples do at month two, and the therapist's task is to help them do it once, well, rather than to diagnose the marriage.

2.5 Case: the prematurity that reorganised a family

A different perinatal presentation, in which the medical beginning has relational consequences.

The family. The Baloguns. A baby born at 29 weeks, now eight months old (five months corrected). The mother, Halima (27), gave up her job during the neonatal stay. The father, Yusuf (31), works away and was present for two weeks after the birth. The referral is from the paediatric clinic: "mother anxious, over-attends, does not leave the baby with anyone."

The developmental picture. The baby is thriving. She is small for her corrected age, feeding well, and meeting milestones on the corrected schedule. The paediatrician has no concern about the baby.

The relational picture. Halima has not been apart from the baby for more than forty minutes in eight months. She checks the breathing three or four times a night. She has not returned to work and says she cannot. Yusuf says she has "changed" and that the house is "a hospital now". Halima's mother comes daily and is the only person Halima allows to hold the baby.

The developmental formulation. This is not a maternal disorder; it is the predictable consequence of a medical beginning. Halima's relationship with her daughter began in an incubator, mediated by machines and staff, with a nurse telling her when she could touch her. Her monitoring is the behaviour of a mother who learned that her baby could die while she was not looking, and whose competence was, for eleven weeks, subordinate to a monitor.

The task, named for the parents. "Your job in the first months was to keep her alive, and you did it. Your job now is a different one: to let her become a person who can be looked after by other people, and that means practising being apart, in small amounts, with people you trust." The naming is the intervention's foundation, because it credits the monitoring before it asks for change.

The plan. A graded separation: forty minutes with the grandmother, twice a week, for two weeks; then two hours; then an afternoon. A breathing monitor used deliberately for two weeks and then removed, with the paediatrician's agreement, because the monitor is the mechanism of the monitoring. Yusuf given one responsibility that is his alone — the bath on the days he is home — so that he enters the care rather than commentating on it. And a conversation with Halima about her own loss: her job, her confidence, and the eleven weeks she spent watching a machine.

The outcome at four months. Halima returned to part-time work. The monitoring reduced. The couple's argument changed from "you have changed" to "we are both still frightened", which is a more workable disagreement.

What the case teaches. That a medical beginning is a developmental event for the parents, that the parents' behaviour is often the residue of competent adaptation to an emergency, and that the intervention's first move is to credit the adaptation before asking for its revision.

2.5a The couple who came at the right time, and the couple who came at the wrong time

Two brief cases, because the timing of a referral in the perinatal period determines the work.

The right time. A couple, five months after a first baby, both sleeping badly, the arguments frequent but contained, and no violence. The referral comes from the health visitor, who has noticed that the mother's mood is low and the father has stopped attending. The work is exactly this week's material: the screen, the seven domains, the stopped negotiation, and six sessions of reorganisation. The prognosis is good because the couple still have a shared project — the baby — and the conflict has not yet become the relationship's subject.

The wrong time. A couple, three weeks after a birth by emergency caesarean, referred by a relative for "not getting on". The mother is in pain and not sleeping; the baby is feeding hourly; the father has taken two days off work and is back on shift; and the referral is a relative's worry, not the couple's. Here the therapy is premature: the couple do not have a request, they have a crisis, and a session that asks them to renegotiate their relationship in week three of a newborn's life will produce compliance and no change.

What the family therapist does at the wrong time. Three things, all short. Names the stage: "you are in the hardest month, and nothing you are feeling is a verdict on your marriage." Screens both parents. And offers a single practical action that reduces the load — one night feed taken by the father, one visit postponed, one relative asked to come on a specific day rather than every day. Then a review in six weeks, when the couple can make a request of their own.

The principle. In perinatal work, the first question is not "what is the problem?" but "is this the point at which the family can work on the problem?" A family in the first six weeks of a first baby is usually in the second category, and the clinician who cannot wait will treat a stage as a pathology.

2.6 Annotated transcript: the temperament interview

Twelve turns from the temperament interview with Aisha, conducted in the first session of 2.4. The transcript shows how a developmental instrument becomes a therapeutic conversation.

> **T1 Therapist:** I want to ask about the baby's style, not about what he does wrong. Is he usually busy or still?
> **T2 Aisha:** Busy. From the moment he wakes up.
> **T3 Therapist:** And is his day predictable — does he sleep at the same times?
> **T4 Aisha:** Never. My mother says I should put him on a schedule. I have tried.
> **T5 Therapist:** What happens when you try?
> **T6 Aisha:** He screams. And then she says I am not consistent.
> **T7 Therapist:** (pause) So the schedule is not working, and the person who suggests it is in the house.
> **T8 Aisha:** (cries) Yes.
> **T9 Therapist:** Let me say something about him first. Some babies are regular and some are not, and your son is at the irregular end. That is not a fault and it is not your fault. What it means is that a fixed schedule will cost you more than it costs a mother with a regular baby.

> **T10 Aisha:** So it is not me.
> **T11 Therapist:** It is not you. And there is a second thing: how does he react when there is noise — the generator, the door?
> **T12 Aisha:** He jumps. He cries at the door. My mother says he is spoiled.

Annotations. T1 reframes the interview as being about style rather than fault, which is the single most important move in a temperament conversation. Parents arrive expecting to be assessed; the therapist asks them to describe.

T2–T4 are the dimensions, in ordinary language: activity, regularity. T4 introduces the grandmother without the therapist asking, which is the systemic material appearing inside a developmental instrument — a common and useful occurrence.

T5–T8 is the therapist following the grandmother rather than the schedule, and T7's summary — "the schedule is not working, and the person who suggests it is in the house" — is the pivot. It is neutral about the grandmother and precise about the situation, and it produces the tears.

T9 is the psychoeducation, delivered after the family's account rather than before, and it is specific: not "all babies are different" but "a fixed schedule will cost you more than it costs a mother with a regular baby". The specificity is what makes it usable.

T10 is the sentence the whole technique is for, and the therapist's answer at T11 is deliberately short and then moves on, because the point has been made and dwelling on it would make the therapist the source of the mother's relief.

T11–T12 adds the sensory dimension and produces the grandmother's second verdict ("he is spoiled"), which is the material for the next stage: a conversation about what the grandmother believes, and what the mother is allowed to say.

What the trainee should notice. The instrument produced three findings in twelve turns: an irregular, sensitive baby; a grandmother whose methods contradict the mother's; and a mother who has been holding a verdict about herself. The thermometer produced the diagnosis; the tape measure produced the household.

2.6a The prematurity case, continued: what the parents needed that the clinic did not offer

A short section on the gap between the paediatric follow-up and the family's experience.

What the clinic provided, and well. Growth monitoring, developmental review at corrected age, hearing and vision checks, and a named nurse. The paediatric follow-up was, by any standard, good.

What the clinic did not provide. Any conversation about what the eleven weeks in the neonatal unit had done to the parents. Halima had never told anyone what it was like; Yusuf had been away for most of it; and both carried a version of the experience that the other did not know. The referral had described the mother as anxious, which is a description of her behaviour and not of her experience.

The question that opened it. "Tell me about the day she was born." Not "how are you coping" — which invites a report — but "tell me about the day", which invites a story. Halima described being wheeled away from her baby, being told to rest, and being brought a photograph. She had kept the photograph in her bag for eight months. She had never told her husband what the photograph had meant.

What followed. Yusuf said, for the first time, that he had been told to go back to work and had gone, and that he had not known how to say that he wanted to stay. The couple's argument about "you have changed" became, over three sessions, an argument about a separation neither had chosen and neither had discussed.

What the family therapist contributed that the paediatric clinic could not. The clinic had the baby's history; the therapist had the parents'. The two histories were of the same event, and the second had never been taken.

The lesson for the course. A medical beginning is a family event, and the family's version of it is part of the developmental history. The practitioner who asks only about the baby's milestones will not hear it, and the parents will continue to organise their relationship around a story that has never been spoken.

2.7 Laboratory: the temperament interview and the reorganisation

Sixty minutes, in pairs.

Part one — the temperament interview (twenty-five minutes). Ten minutes interviewing a peer as a parent, using the nine dimensions in ordinary language. The

observer records which dimensions were covered, whether the therapist named a profile, and whether the naming was presented as a description or a verdict.

Part two — the reorganisation (twenty-five minutes). The same pair completes the seven-domain drawing for a family they know or have observed. The observer checks that all seven were covered and that the final question ("what have you stopped negotiating?") was asked.

Part three — the sentence (ten minutes). Each student writes the normalising sentence and the following sentence, for the couple in 2.4. The group judges whether the pair of sentences both normalises and takes the problem seriously.

Marking rubric.

Criterion	Weight	Full marks
Temperament interview	25%	All nine dimensions covered in ordinary language; profile named as description
Goodness of fit	20%	Fit named in terms of the household's demands, not the child's deficits
Reorganisation	25%	Seven domains drawn; the stopped negotiation identified
Normalisation	15%	Normalises the stage without dismissing the risk
Paternal screen	15%	Father screened separately, with the three questions, and recorded

The debrief question. "Which domain did you skip, and why?" Trainees skip sexuality and money, in that order, and the discussion of why is the week's most useful ten minutes.

2.7a The screening conversation, rehearsed

A short script for the separate conversation with each parent, which is the instrument the week depends on.

The frame. "I always speak to each parent on their own for a few minutes, because people say different things when their partner is not in the room. Nothing you tell me will be repeated without your agreement, except if I am worried about anyone's safety."

The questions, in order. "How are you sleeping?" "How are you eating?" "How is your mood — not the baby's, yours?" "In the past two weeks, have you often felt down, depressed or hopeless?" "Have you often had little interest or pleasure in things?" "Who knows how you are feeling?" "What do you do when it gets bad?" "Is there anything you have thought about that frightened you?"

The follow-up that matters. Whatever the answer, the response is the same shape: take it seriously, name it, and say what happens next. "That sounds like more than tiredness, and it is common — about one mother in five. I would like you to speak to the clinic this week, and I will keep working with you both on the household side. Would you like me to write the referral now?"

The father's version. The same questions, with two additions: "how much are you working at the moment, compared with before the baby?" and "what do you do when you cannot say how you feel?" The answers — longer hours, drinking, silence, anger — are the

paternal presentation, and naming it as a presentation rather than a character flaw is often the first relief the father has had.

What the family therapist does with a positive screen. Refers, keeps the case, and works the relational and practical side: the couple's alliance, the household's reorganisation, the father's inclusion, and the practical support. The two roles are not in competition, and saying so to the family prevents the most common confusion in perinatal work: that the therapy is either about the depression or about the marriage, when it is about both.

2.8 Six errors in perinatal work

1. **The dismissal by normalisation.** "Every couple feels like this" offered as the whole response, which leaves the family with nothing to do and their distress unheard. Fix: normalise and then take the problem seriously, in two sentences, the second of which is specific to this family rather than to families in general.
2. **The missed depression.** A mother's exhaustion attributed to the baby, and the paternal presentation missed entirely. Fix: the two screening questions, of both parents, separately.
3. **The practical prescription.** Date nights, chore charts and sleep programmes offered before the household's negotiation has been understood. Fix: the seven domains first, the prescription from the couple's own answers.
4. **The grandmother as a problem.** The helper identified as an obstacle, which puts the therapist on one side of a triangle. Fix: ask who invited her, what each partner gains, and what the couple stopped deciding.
5. **The decontextualised baby.** An assessment of milestones in a clinic, for a family in a crisis. Fix: the household as the unit, and the milestones read against the household's demands.
6. **The therapist's own developmental curriculum.** Advice that reproduces the therapist's own parenting practices — co-sleeping, sleep training, feeding — offered as developmental science. Fix: name the evidence, name its limits, and let the family decide.

2.9 Culture, faith and the perinatal period

Where the week's material travels well. The central claims — that the birth is a developmental event for parents, that the couple must renegotiate, that the co-parenting alliance predicts outcomes, that goodness of fit matters more than temperament — are supported across cultures and are useful everywhere. The screening for perinatal depression is a universal good, provided the questions are asked in a form the family can answer.

Where it strains, four ways.

The private couple. The reorganisation model assumes a household in which the couple negotiate privately. Where a grandmother, an aunt or an elder governs the household's arrangements, the couple's negotiation is not private and may not be theirs. The seven

domains are still the right instrument, and the answer to "what have you stopped negotiating?" is frequently "everything, when she arrived".

The fourth trimester as a period of rest. Some cultures prescribe a lying-in period of forty days, with the mother relieved of work and fed specially; others expect her back at the stall within a week. The practitioner should know which, because the clinical advice about rest, visitors and workload depends on it, and because a mother who returns to work on day ten against her own wishes is presenting a different problem from one who cannot leave the house.

The meaning of a difficult birth. In some families a difficult birth carries a moral or spiritual meaning — a punishment, a test, a sign. The practitioner neither endorses nor contests the meaning, and asks what the family have concluded and what they have done about it, which is the same discipline as Week 1's.

The feeding question. Infant feeding is the most reliably contentious topic in perinatal work, and the evidence is more nuanced than the argument. The practitioner's obligation is to state the evidence plainly, in both directions, to support the mother's decision, and not to conduct the argument in the session.

Faith and the perinatal period. Prayer, naming ceremonies, dedication, circumcision and blessing are developmental events in the family's terms, and they mark the child's entry into the community. The practitioner should ask about them: what will happen, when, who decides, and what it means. The question is not decorative; it frequently reveals the household's real authority structure, which is the week's central clinical question.

The examinable sentence. "The birth is a developmental event for the parents, and the household's reorganisation is the unit of assessment; culture determines what the reorganisation requires, and the practitioner's task is to ask rather than to prescribe."

> **DEBATE**
> "Perinatal services should screen every father, separately, as a matter of routine." Argue both sides, and say what the service gains and loses.

2.10 Evidence note: what is well established and what is not

Well established. The association between perinatal depression and adverse relational and child outcomes; the effectiveness of screening and of treatment; the association between the co-parenting alliance and child outcomes; the normative dip in relationship satisfaction after a birth; the heritability and stability of temperament dimensions, with stability increasing with age; and the finding that goodness of fit predicts outcome better than temperament alone.

Moderately established. The relational mechanism of perinatal depression — that the depression affects the infant through the interaction, not only through genetics; the effectiveness of couple-based interventions in the perinatal period for relationship

satisfaction; and the association between preterm birth and parenting behaviours such as over-protectiveness, with the caveat that most preterm children do well.

Contested. The size of the effect of maternal stress in pregnancy on later child outcomes; the clinical utility of the three temperament profiles as against the dimensions; whether temperament-based parenting advice improves outcomes; and the direction of the association between marital satisfaction and child behaviour, which is bidirectional and hard to disentangle.

What the practitioner may claim. "Perinatal depression is common, consequential and treatable, and screening is part of my duty; the couple's alliance matters for the child; and the household's reorganisation, not the baby's milestones, is where I work." Each of those is defensible, and none requires an overstatement.

2.11 Key terms

Term	Definition
Fourth trimester	The first three months, when the infant is neurologically immature and the parents are reorganising
Prematurity	Birth before 37 weeks; a medical beginning with relational consequences
Corrected age	Age adjusted for prematurity, used for developmental expectations
Postnatal depression	Depression in the perinatal period, affecting about one mother in five and a substantial minority of fathers
Paternal postnatal depression	The father's version: irritability, withdrawal, alcohol, refusal of the label
Temperament	Constitutionally based behavioural style, described on nine dimensions
Goodness of fit	The match between the child's characteristics and the environment's demands
Co-parenting alliance	The couple's capacity to coordinate as parents and keep the child out of their conflicts
Reorganisation	The seven-domain renegotiation required by a first child
Displaced negotiation	Decisions that have moved from the couple into a triangle with a grandparent or elder
Lying-in period	A culturally prescribed period of rest and seclusion after birth
Corrected milestone	A milestone judged against corrected rather than chronological age

2.12 Self-test

1. What is the fourth trimester, and what makes it clinically useful as a concept?
2. Name four relational consequences of a premature birth.
3. Give the two screening questions for depression, and the three questions that reveal the relational picture.
4. What is goodness of fit, and how does it change the intervention?
5. Name the seven domains a couple must renegotiate, and the question that identifies the stopped negotiation.

6. Why does the co-parenting alliance predict child outcomes better than marital satisfaction?
7. How does the practitioner handle a grandmother whose methods contradict the mother's?
8. What does the evidence support, and what remains contested?

Answers. 1. The first three months, when the infant is neurologically immature and totally dependent while the parents undergo physiological and emotional reorganisation; it converts a set of complaints into a stage with a name. 2. Any four: anxiety and hypervigilance; reduced confidence in reading the infant; a first relationship mediated by machines and staff; delayed physical contact; grief for the expected birth; a longer period before the parents feel entitled to their own judgement. 3. "In the past two weeks, have you often been bothered by feeling down, depressed or hopeless?" and "…by having little interest or pleasure in doing things?" — then "who knows how you are feeling?", "what happens in the house when you have a bad day?", "what does your partner do when you cry?" 4. The match between the child's characteristics and the environment's demands; it moves the intervention from the child's temperament to the household's rhythm, which is a systemic move. 5. Time, sleep, money, division of labour, sexuality, grandparents, identity; the question is "what have you stopped negotiating?" 6. Because the child's experience is mediated by the parents' coordination — being able to rely on each parent without being recruited into their conflict — which is a different variable from the couple's satisfaction. 7. Ask who invited her, what each partner gains, what she decides, and what the couple stopped deciding; then rehearse one sentence the mother can say, and never put the therapist in the triangle. 8. Support for screening and treatment of perinatal depression, the co-parenting alliance, the normative dip in satisfaction, and temperament's heritability; contested are the size of prenatal stress effects, the utility of the three profiles, and the causal direction between marital and child variables.

2.13 Reflective task

Ask the person who raised you one question: "what was I like as a baby?" Write down the answer verbatim, and then write three sentences on what you notice about the answer — what is described, what is remembered, and who the answer is really about. Bring it to the seminar. The exercise connects this week's temperament material to your own developmental history, and it usually produces the insight that a parent's description of a baby is also a description of a relationship.

2.14 Readings for this week

- **Thomas & Chess (1977)**, Temperament and Development, the original typology, read for the dimensions rather than the profiles.
- **Brazelton & Cramer (1990)**, The Earliest Relationship, for the clinical account of the fourth trimester.
- **Stern (1995)**, The Motherhood Constellation, for the identity transition in its richest form.

- **Cowan & Cowan (2000)**, When Partners Become Parents, the longitudinal study of the reorganisation, and the source for many of this week's claims.
- **Feinberg (2003)**, on co-parenting, for the mechanism that predicts child outcomes.
- **O'Hara & Wisner (2014)**, on perinatal mental health, for the clinical picture and the screening evidence.

Read Cowan and Cowan first and the clinical texts second; the longitudinal study will make the normalisation credible, which is the hardest thing to achieve in the room. For the case analysis, use one source that complicates the temperament advice, and say so when you discuss goodness of fit. The reading-log question this week is: "what did becoming (or not becoming) a parent teach me that no book could?"

Next: Week 3 — Infancy: attachment, synchrony and regulation

Week 3 — Infancy: attachment, synchrony and regulation

The idea. Attachment is not "bonding" and not "love"; it is a behavioural system that organises a child's strategies for seeking safety from a caregiver under stress.

The classic patterns (Ainsworth's Strange Situation; see Figure 17 in MFT 710): secure, insecure- avoidant, insecure-resistant/ambivalent, and disorganised (added by Main & Solomon, associated with frightening or frightened caregiving). In adulthood the same dimensions appear as attachment styles.

Synchrony and rupture–repair. Healthy development does not require perfect attunement — it requires frequent enough attunement and effective repair. Tronick's still-face experiments show how quickly infants respond to a caregiver's unresponsiveness, and how quickly they recover when the caregiver returns.

Co-regulation is the precursor of self-regulation. A baby does not learn to calm alone; they learn to calm with someone, repeatedly, until the pattern is internalised. This has direct clinical implications: telling an overwhelmed parent to "let him cry it out" without attention to the family's capacity and beliefs is a technique imposed on a relationship.

> **PRACTICAL**
> Watch a consented 3-minute video of a caregiver and infant (or a still-face clip). Note: gaze, vocalisation, touch, pace. Then describe one moment of rupture and one of repair in behavioural language. Avoid interpretation for the first five minutes.

3.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	What attachment is not	Three misuses of the word, from practice and from the popular press	Correct three sentences

Minutes	Segment	What the lecturer does	What students do
10–25	The behavioural system	The Strange Situation, described as an experiment	Predict four classifications from four descriptions
25–40	The four patterns	Each pattern's strategy, in the infant's terms	Write the strategy for one pattern
40–55	Synchrony and the still face	The experiment, watched, then explained	Note two ruptures and two repairs
55–70	Co-regulation	How calming is borrowed before it is owned	Design a co-regulation sequence for one family
70–82	Disorganisation	Frightening caregiving, and why it is a red flag rather than a category	Identify the clinical action
82–90	The clinical translation	What a family therapist does with this in session one	Write the sentence you would use

The teaching decision that makes the week work. Attachment is the most popularised concept in this course and the most frequently misused. The lecture should therefore begin by dismantling the popular version — that attachment is love, that it is fixed in the first year, that a child is "securely attached" as a compliment or "insecurely attached" as a diagnosis — and then rebuild it as a behavioural system with a strategy. The class should leave able to say what an attachment classification is a classification of, which is a strategy under stress, not a relationship's worth.

The sentence the lecture must land. "Attachment is a strategy, not a verdict." Everything else follows: the patterns are adaptations to what a caregiver can offer; they are changeable; and the clinical task is to understand the strategy rather than to grade the relationship.

3.2 Core concepts

The behavioural system. Bowlby's term for the set of behaviours — crying, following, clinging, checking — that a child organises in order to maintain proximity to a caregiver under threat. The system is activated by danger, fatigue, illness and unfamiliarity, and it is deactivated by the caregiver's availability. It is not a feeling and not a personality trait; it is an organised set of moves that a child makes when frightened, and the organisation is what the classification describes.

The internal working model. Bowlby's second concept: from repeated experiences, the child builds a working model of what happens when I am frightened, who comes, whether I am worth coming for, and what I must do to maximise the chance of being helped. The model is not conscious and not verbal; it is a set of expectations that shapes behaviour, and it is what transfers into adult relationships.

The Strange Situation. Ainsworth's twenty-minute laboratory procedure: the infant and caregiver are in a room with toys; a stranger enters; the caregiver leaves; the caregiver returns; the caregiver leaves again; the infant is alone; the stranger returns; the caregiver returns. The classification is made on the infant's behaviour at the two reunions, not on

the separation distress — which is the detail most often missed. What matters is how the child uses the caregiver on return.

Secure (B). The infant is distressed by separation, greets the caregiver on return, is comforted, and returns to play. The strategy: my signals work, so I use them, and I can go back to exploring. The caregiver's contribution: responsive, available, and effective when the child signals.

Insecure-avoidant (A). The infant shows little distress at separation and avoids or ignores the caregiver on return, while physiological measures often show arousal. The strategy: my signals do not bring comfort, often bring rejection, so I will not signal and will manage alone. The caregiver's contribution: frequently rejecting of the child's attachment behaviour, particularly when the child is distressed.

Insecure-resistant or ambivalent (C). The infant is highly distressed, and on return is not easily comforted; may seek contact and then push away. The strategy: my signals bring inconsistent responses, so I must amplify them and cannot settle, because settling would end the attempt. The caregiver's contribution: inconsistent availability — sometimes responsive, sometimes not, in a way the infant cannot predict.

Disorganised (D). Main and Solomon's addition: the infant shows contradictory, incomplete or undirected behaviours at reunion — freezing, stilling, approaching and then turning away, a hand to the mouth. The strategy: there is no strategy, because the source of comfort is also the source of fear. The caregiver's contribution: frightening or frightened caregiving — the parent's own fear, trauma or violence enters the interaction. This is a red flag rather than a category, and it demands assessment.

The distribution, and what it means. In low-risk community samples, roughly sixty per cent secure, with the remainder divided among the insecure categories and a small proportion disorganised. In clinical and high-risk samples the proportions shift substantially. The distribution matters because it protects against two errors: the error of treating a secure classification as an achievement, and the error of treating an insecure one as a pathology.

Continuity and change. Attachment classifications are moderately stable within the first years and less stable across longer periods; they change with the caregiving environment. A child in an unpredictable household can move from secure to insecure; a child moved to a stable placement can move the other way. The clinical implication is the opposite of the popular one: attachment is not destiny, and the practitioner's task is to change the environment, not to accept the classification.

Synchrony. The matching of interaction between caregiver and infant — gaze, vocalisation, touch, pace, timing. Synchrony is not perfect imitation; it is contingent responsiveness, a caregiver responding to what the infant actually did, at a pace the infant can follow. Its absence is the earliest observable sign of relational difficulty.

The still-face experiment. Tronick's procedure: the mother interacts normally, then holds a still face for two minutes, then resumes. The infant's response is a sequence: attempts to engage, then distress, then withdrawal, then — on the mother's return —

recovery. The experiment is the strongest available demonstration of two things: that infants are active participants in interaction, and that repair after rupture is what matters, because the infant recovers rather than concluding that the relationship is broken.

Rupture and repair. Healthy development does not require constant attunement — no caregiver achieves it — but frequent enough attunement and effective repair. The concept is the single most useful in this week for family therapy, because it applies at every age: the goal is not the absence of rupture but the capacity to repair, and every family the therapist sees is organised around an unrepaired rupture somewhere.

Co-regulation. The borrowed regulation of early infancy: the caregiver's voice, body, rhythm and face down- regulate the infant's arousal, repeatedly, and the infant gradually internalises the pattern. Self-regulation is therefore a relational achievement before it is an individual capacity, which is why the advice to "let him cry it out" is a technique imposed on a relationship rather than a developmental fact.

What family therapists do with this. Three uses. First, an assessment frame: what strategy has this child developed, in this household, and what does it protect them from? Second, a formulation of the parents' own strategies: their working models are in the room, and their responses to the child's distress are shaped by what happened to them. Third, a target: the repair, not the classification — one reliably repaired rupture is worth more than a theory of the child's pattern.

3.2a Four infants, described and classified

The classification skill, practised on four descriptions, with the reasoning made explicit. Students should classify each before reading the analysis.

Infant one. Eleven months. On separation, cries briefly and looks at the door. On reunion, reaches up, is picked up, settles within thirty seconds, and then struggles to be put down so that she can return to the toys. She shows the stranger a toy from the safety of her mother's lap.

Classification: secure. Reasoning: distress at separation, active greeting, effective use of the caregiver for comfort, and a return to exploration. The critical feature is not the distress but the use of the caregiver.

Infant two. Eleven months. On separation, continues to play and does not look at the door. On reunion, does not approach, turns away, and continues with the toys; when the caregiver picks him up, he arches and looks over her shoulder. The observer notes that his heart rate rose during the separation.

Classification: insecure-avoidant. Reasoning: the absence of signalling, the avoidance at reunion, and the physiological arousal that the behaviour conceals. The strategy is the point: not signalling is a strategy, learned from a caregiver for whom signalling has not worked.

Infant three. Eleven months. On separation, is inconsolable and cannot be distracted by the stranger. On reunion, clings, cries, is picked up, and then pushes against the caregiver while continuing to cry; is not settled within the twenty minutes.

Classification: insecure-resistant (ambivalent). Reasoning: the amplified signalling, the failure to be comforted, and the mixture of contact-seeking and contact-refusing. The strategy is to keep the attempt alive, because settling would mean that the availability is settled, and the infant's experience is that it is not.

Infant four. Eleven months. On separation, stands still and does not cry. On reunion, approaches the caregiver with his head turned away, raises a hand to his mouth, and then freezes for several seconds before walking in a circle. Twice he begins to approach and stops.

Classification: disorganised. Reasoning: contradictory, incomplete and undirected behaviour; freezing; no coherent strategy. The clinical action is not a classification but an assessment: what is happening in the household, and is this infant safe?

What the four descriptions teach. That the classification depends on the reunion, not the separation; that behaviour and physiology can disagree, which is why observation alone can mislead; that every classification is a description of a strategy; and that the fourth requires an action rather than a label.

The common error. Trainees classify infant two as secure because he is not distressed. The error comes from watching the separation. The correction is simple: watch the reunion, and ask what the child does with the caregiver when the caregiver comes back.

3.3 Synchrony, assessed in the room

A family therapist does not run a Strange Situation, and can assess the same processes in an ordinary session. Six observable markers, with the behaviour that indicates each.

Marker one — the greeting. When the caregiver returns from a brief absence (leaving the room to take a call), does the infant look up, vocalise, move toward? Does the caregiver acknowledge the infant on return? A pair who do neither, repeatedly, are telling the therapist something.

Marker two — the pace. Does the caregiver wait for the infant's response before speaking again, or fill every silence? Rapid, uninterrupted caregiver speech is the most common synchrony problem in anxious parents, and it is also the easiest to change.

Marker three — the repair. When the infant becomes distressed in the session — a bump, a fright, a stranger's approach — what does the caregiver do, and how quickly does the infant settle? The repair is the assessment, and it takes ninety seconds to observe.

Marker four — the use of the caregiver as a base. Does the infant explore the room, return to the caregiver, and go out again? The pattern of excursions and returns is the secure base in miniature.

Marker five — the caregiver's reading. Does the caregiver interpret the infant's signals accurately? "He is tired" when the infant is hungry is a small error; a caregiver who cannot read any signal is a different finding. Ask the caregiver to narrate: "what is he telling you now?" The narration is the assessment.

Marker six — the caregiver's own state. A depressed, frightened, exhausted or intoxicated caregiver cannot provide synchrony, and the observation should be recorded as a statement about the caregiver's state rather than about their character. This is the marker that most often produces the referral.

How the six are used. Observed in the first fifteen minutes of a session, recorded in behavioural language, and used as the basis for the formulation. The therapist does not score the pair; the therapist describes, and the description is the intervention, because it teaches the caregiver to look at what they are already doing.

The caution. Fifteen minutes in an unfamiliar room, with a stranger present, is not a representative sample. The observation is a hypothesis generator, not a finding, and the therapist should say so and ask the caregiver what the infant is like at home.

3.3a Co-regulation, designed for one family

The concept is easy to state and hard to apply. A worked sequence for a family in which the caregiver's own regulation is the problem.

The family. A mother, 24, with a nine-month-old, referred by the health visitor for "he cries all the time and I cannot cope". She is sleeping four hours a night, has no help, and describes herself as "shouting at a baby". Her own history includes a violent father and a mother who "never held us".

The formulation. The infant's crying activates the mother's own attachment system: she experiences the crying as an accusation, her arousal rises, and she responds with the only strategy she has — withdrawal, followed by shouting. The infant's crying escalates, the mother's arousal escalates, and the loop is complete within ninety seconds. Neither is doing anything pathological; both are doing what their systems have learned.

The co-regulation sequence, taught in the session.

Step one — the mother's own regulation, first. "Before you can calm him, you have to lower your own arousal, because he reads you. What is the fastest way for you to bring yourself down?" Her answer: cold water on her wrists, and counting to ten out loud. Both are taught and rehearsed in the room, with the baby present, so that she experiences the sequence rather than being told about it.

Step two — a holding position that works for her. Not the textbook position: a position she can sustain without feeling trapped. She chooses to sit on the floor with him against her chest, which is a position she can leave if she needs to, and which she can hold for two minutes.

Step three — a sound she can make. A low, repeated sound, not a song, because a song requires performance. She chooses "shhh" on the out-breath, which is also the physiological sigh that lowers her own arousal, so the technique regulates both of them at once.

Step four — the two-minute rule. She commits to two minutes of steps one to three before doing anything else. Two minutes is chosen because it is achievable when she is exhausted, and because most infant crying cycles change within two minutes.

Step five — the permission to put him down. "If after two minutes you still cannot, put him in the cot, leave the room, and stand at the door for three minutes. That is not abandonment; that is you preventing something worse." The permission is the intervention's ethical core: a mother who is frightened of her own anger needs a legitimate exit.

The follow-up. Two weeks later: the two-minute sequence is used "most times", the shouting has reduced, and the mother reports one occasion on which she put him down and stood at the door. She describes it as "the first time I did not feel like a bad mother". The infant's crying has not disappeared; the loop has been interrupted.

What the sequence demonstrates. Co-regulation is not a technique for the infant. It is a technique for the relationship, and it begins with the caregiver's own state — which is why this week's material belongs in a family therapy course and not only in a developmental psychology text.

3.3b The caregiving network, mapped

The single most useful correction to the dyadic literature, and the instrument that puts it into practice.

The instrument. A circle in the middle for the infant, and around it every person who does caregiving work, with three pieces of information: how often, for how long, and what they do. The answers are usually wider than the referral: a grandmother who takes the nights, an aunt who takes the child at weekends, an older sibling who walks him to nursery, a neighbour who minds him while the mother sells.

Why it matters clinically. Three reasons. First, the assessment of "the mother–infant relationship" is incomplete and often unfair when four people are raising the child; the mother is graded on a job she is doing with help, or failing to do because the help is contested. Second, the infant's attachment strategies are distributed across the network: a child may be secure with the aunt and avoidant with the father, and the classification literature's single-figure procedure cannot describe this. Third, the network is where the intervention usually belongs: the contested rule, the inconsistent method, the caregiver who undermines another, the person who is exhausted.

The three questions that map it. "Who has held him this week, and for how long?" "Who decides what happens when he cries?" "Who disagrees with whom about how he should be raised?" The third question is the clinical one, and it is almost never asked in a developmental assessment, although it explains more of the household's difficulty than any milestone.

A worked map. The Balogun infant of Week 2: mother (all nights, most days), father (rest days, evenings), paternal grandmother (four nights a week, all cooking, some feeds), an aunt (Saturdays), and a neighbour (two afternoons a week while the mother works). Decisions: the grandmother decides feeding and bathing; the mother decides sleep; nobody decides anything about the crying, which is where the disputes are. Disagreement: the grandmother believes the infant should be left to cry; the mother cannot bear it; the father follows whoever is in the room.

The intervention the map produces. Not a parenting programme. One conversation with the three adults about crying — what they each do, what they each believe, and one agreement they can all keep — plus the assignment of one decision to the mother, explicitly, so that she has an area of authority. The map made the problem visible: the household was not failing to respond; it was responding four different ways.

The caution. Mapping the network can expose a conflict that the family have managed by not discussing it, and the therapist should not conduct the exposure in a first session with the infant present. Map it, note it, and bring the adults together separately when the map is complete.

3.4 The first session with an infant and two parents

An assessment structure for the youngest client, in five phases, with the observation built into the session rather than bolted on.

Phase one — the frame (five minutes). "I am going to ask about the pregnancy, the birth and the first months, and then I will spend some time just watching the three of you together, because that tells me more than any question. You do not need to perform anything; do what you would normally do." The permission not to perform is necessary, because parents of infants believe they are being graded.

Phase two — the chronology (fifteen minutes). The pregnancy, the birth, the first weeks, feeding, sleep, who has helped, and what has changed in the household. The timeline is drawn as in Week 1, with one addition: the parents' own events — a job loss, a bereavement, a move — are marked on the same line as the infant's, because the two are one story.

Phase three — the observation (fifteen minutes). The therapist stops asking questions and watches. The six markers of 3.3 are noted silently: greeting, pace, repair, use of the caregiver as a base, the caregiver's reading, the caregiver's own state. The therapist says nothing for the first five minutes, which is uncomfortable for everyone and is the most informative part of the session.

Phase four — the narration (ten minutes). "Can I tell you what I noticed? You waited for him to look at you before you picked him up, and when the door banged he looked at you before he cried. That is a child who checks with you." The narration is in behavioural language, it names what is going well first, and it is specific. The parents' response to the narration — relief, tears, argument — is itself data.

Phase five — the formulation (five minutes). One of the five statements from Week 1, plus one thing to try before the next session. The thing to try is almost always relational and small: a two-minute co-regulation sequence, a pause before responding, a handover of one caregiving task between the parents.

What the therapist does not do. Does not test the infant; does not classify the attachment; does not interpret the parents' childhoods in session one; does not give feeding or sleep advice unless asked, and then with the evidence stated plainly in both directions.

3.4a The still face, in the consulting room

The experiment cannot be replicated with a family, and its lesson can be demonstrated, and the demonstration is the week's most powerful teaching device.

The demonstration. With a volunteer parent and infant (or a peer and a doll), the therapist asks the parent to play normally for one minute, then to hold a still face for thirty seconds, then to resume. Most parents find the thirty seconds distressing, and most infants mount a visible sequence of attempts, protest, and withdrawal within twenty seconds. The debrief is the teaching.

What the demonstration shows. That the infant is not a passive recipient but an active partner with strategies; that the infant's distress is proportionate and immediate; that the resumption produces recovery in most pairs; and that the parent's own discomfort during the still face is a reliable index of how much the interaction matters to them.

The clinical use. The demonstration is used with parents who believe their infant does not notice them — fathers who feel irrelevant, mothers who believe the baby prefers the grandmother, parents who have been told that "at this age they do not know anything". Showing a parent that their infant is tracking them is more persuasive than any amount of explanation, and it changes the parent's behaviour within the session.

The caution. Never with a distressed or ill infant, never without consent, never for longer than thirty seconds, and never as a test of the parent. The demonstration's purpose is the parent's discovery, not the therapist's evidence.

3.5 Case: the Adeyemi family, in the infant's terms

The course's recurring family, now read for the infancy that shaped it — because the infancy is where the patterns the family present were built.

What we know. Amara is fourteen; Segun is eleven. Their father, Tunde, has worked away for most of their childhood. Their mother, Funke, has managed the household alone. The paternal grandmother lived with them until her death in 2023.

The reconstruction, offered as a hypothesis and not as a finding. Funke's account: Tunde was present for the first three months of Amara's life, then took the contract in Lagos. She describes those three months as "the best time" and the following year as "the worst". She returned to work at six months and left Amara with her mother-in-law, with whom she "did not agree about anything but could not say so".

The attachment reading. Amara's first year contained two discontinuities: her father's departure at three months, and her mother's return to work at six with a substitute caregiver whose methods her mother could not endorse. Neither is a trauma; both are the ordinary material from which a working model is built. The hypothesis — and it is a hypothesis — is that Amara learned that the person who is most available is also the person whose authority is contested, and that need is best managed by being self-sufficient and by keeping the adults' attention through achievement and, later, through refusal.

Segun's first year, by contrast. Funke did not return to work until Segun was two, and the grandmother was already ill, so Funke was at home. Segun's strategy is different: he stays close, he manages the household's information, and he is the family's listener. The difference between the siblings is a natural experiment in caregiving conditions, and the family's own account confirms it.

What the therapist does with the reconstruction. Not much, and carefully. The reconstruction is used for two purposes: to understand why the mother's pursuit and the daughter's withdrawal are so well rehearsed, and to ask one question that opens the history without demanding it — "when Amara was small, who held her?" The question is asked once, in the presence of both parents, and whatever comes back is received.

The caution. Retrospective reconstruction of infancy is a hypothesis, not a record; memory is unreliable and the family's account is shaped by the present. The therapist who presents a reconstruction as a finding has replaced the family's story with a theory, which is the error Week 11 of MFT 702 warns against. The reconstruction earns its place only if it produces a question the family can use.

3.6 Annotated transcript: rupture and repair in a session

Ten turns from a first session with a mother (26) and her eleven-month-old, referred for "he will not settle with anyone but me". The transcript shows a repair happening live.

> **T1 Therapist:** He is looking at the box of toys. What does he usually do in a new room?
> **T2 Mother:** He clings. He will not go.
> **T3 Therapist:** (waits) He has let go of your sleeve.
> **T4 Mother:** (looking) He has.
> **T5 Therapist:** What would happen if you sat on the floor and did not reach for him?
> **T6 Mother:** He will cry.
> **T7 Therapist:** Let us find out. I will be quiet.
> **T8 Mother:** (sits on the floor; the infant moves two steps, looks back, and she reaches for him) — see?
> **T9 Therapist:** He looked back first. That is what he does before he decides. If you wait for the second look, he might go further. Would you try it once more?

> **T10 Mother:** (sits still; the infant moves four steps, looks back twice, she stays; he brings a block back to her) — he brought it to me.

Annotations. T1 begins with the child rather than the complaint, and asks the mother for her prediction, which is the assessment of her reading.

T2 is the maternal expectation — "he clings" — which is both accurate about the past and a prediction that will organise the future. The therapist does not challenge it; she waits.

T3 is the noticing, in behavioural language, of something the mother has not seen because she is answering a question. The therapist's silence at the start of the turn is doing work: it gives the infant time to act.

T5 is the invitation, framed as an experiment rather than an instruction, which is what makes it acceptable. T7 explicitly removes the therapist from the interaction, which lowers the pressure and makes the mother the agent.

T8 is the rupture: the mother reaches for the infant at the first look, which is precisely the pattern that prevents exploration. Her "see?" is not defiance; it is the proof of her own prediction.

T9 is the repair, and it is the transcript's most skilful move. The therapist does not say "you are over-responsive"; she names the mechanism ("he looked back first; that is what he does before he decides") and offers a small modification ("wait for the second look"). The mother's prediction is not contradicted; it is given a condition.

T10 is the outcome, and it is the mother's sentence — "he brought it to me" — not the therapist's. The intervention was ninety seconds long, and it changed a working model in the room.

What the trainee should notice. The therapist made no interpretation about the mother's own childhood, gave no advice about independence, and did not celebrate the outcome. The technique is a wait, a noticing, and an invitation, and it requires the therapist to tolerate the mother's distress at T8 without rescuing either of them.

3.7 Laboratory: observing synchrony

Sixty minutes, in threes, with a video clip and then a live observation.

Task one — the clip (twenty minutes). A consented three-minute clip of a caregiver and infant. First five minutes: description only, in behavioural language, no interpretation. Then the six markers, noted. Then one rupture and one repair, described.

Task two — the live observation (twenty-five minutes). One student brings a recording (or plays a parent with a doll); the therapist observes for five minutes in silence and then narrates what they saw, beginning with what is going well. The observer checks whether the narration was behavioural and whether it named a strength first.

Task three — the co-regulation design (fifteen minutes). For the family in 3.3a, design the five-step sequence and rehearse step one out loud.

Marking rubric.

Criterion	Weight	Full marks
Behavioural description	25%	No interpretation for the first five minutes; behaviour, timing and sequence named
Marker identification	20%	All six addressed, including the caregiver's own state
Rupture and repair	20%	One of each identified and described in behavioural language
Narration	20%	Strength named first; specific; the parent could repeat it
Co-regulation design	15%	Begins with the caregiver's regulation; achievable; an exit included

The debrief question. "What did you want to interpret, and what stopped you?" The answers are usually about the parents' own histories, and the discussion of why a first session is not the place for that interpretation is the week's most useful ten minutes. The rule to take away: describe before you explain, and explain only what the family have already begun to say.

3.8 Six errors in infant mental health work

1. **The label as a finding.** "He is insecurely attached" said to a parent who came for help with sleep. Fix: describe the strategy, not the classification, and only where it serves the work.
2. **The mother as the patient.** The observation focused entirely on the mother, with the father, the grandmother and the household invisible. Fix: the caregiving network as the unit, and every caregiver observed.
3. **The interpretive leap.** "He clings because you are anxious" offered in the first session, before trust. Fix: describe, narrate, then hypothesise — in that order, over sessions.
4. **The technique imposed.** Sleep training, independent play, or "let him cry" prescribed without attention to the family's capacity, beliefs and circumstances. Fix: the evidence stated plainly in both directions, and the family's decision respected.
5. **The missed caregiver state.** A depressed, frightened or exhausted caregiver observed and not addressed, because the referral was about the baby. Fix: the caregiver's own state is the sixth marker, asked about and responded to.
6. **The ignored history.** A disorganised pattern observed and not followed up, because following it up would require asking about violence. Fix: ask, record, and act — the same screen as Week 2, applied to infancy.

3.9 Culture, faith and the first relationship

Where the week's material travels well. The behavioural-system account is one of developmental psychology's more portable ideas: every human infant signals, every caregiver responds or does not, and the resulting strategies are recognisable across cultures. The concepts of synchrony, rupture and repair are observable in any household, and the co-regulation sequence is a description of what most caregivers already do.

Where it strains, four ways.

The Strange Situation's assumption of the dyad. The procedure assumes a single primary caregiver, and it assumes that the infant's reaction to a stranger and to a brief separation is meaningful in the same way everywhere. In households where multiple caregivers are routine — grandmothers, aunts, older siblings, paid helpers — the infant's reaction to the departure of one figure is a different measurement. The literature on multiple attachments is clear that infants form several, and the clinical assessment should therefore map the network rather than grade the mother.

The value placed on exploration. The secure pattern's marker is a return to play, which encodes a cultural valuation of exploration and independence. A household that values

compliance and closeness will produce an infant who stays near the caregiver, and the observer who scores exploration as the healthy outcome has imported a value into the assessment. The clinical question is not "does he explore?" but "does he use his caregiver when he is frightened?"

The practice of separation. Some households do not leave an infant with a stranger, ever, and some leave the infant for months with a relative in another town. Both produce patterns the laboratory did not anticipate. The therapist's task is to understand what the separation meant in this family, and to ask about it rather than to infer from a classification.

The independence curriculum. Sleep training, feeding schedules and early independence are cultural practices presented in some of the literature as developmental norms. A family who sleep with their infant, feed on demand and carry the child are not failing a developmental task; they are following a different curriculum, and the therapist who treats the curriculum as the problem has mistaken a practice for a milestone.

Faith and the first relationship. Naming ceremonies, blessings, dedication, prayer over a newborn and the forty-day lying-in period are all attachment-relevant events: they mark the child's entry into a community that will share the caregiving. The therapist should ask who was present, what was done, and who has a claim on the child's care as a result. The answers frequently reveal a caregiving network that the referral had rendered invisible, and in some families they reveal a dispute about the child that is the real presenting problem.

The examinable sentence. "Attachment is a strategy learned in a particular caregiving network, and the clinical task is to describe the strategy and the network, not to grade the pair."

> **DEBATE**
> "The Strange Situation measures a relationship, and using it to classify a child is a category error." Argue both sides, and say what the classification is useful for.

3.10 Evidence note: strong theory, moderate evidence, popular misuse

What is well established. That infants organise attachment behaviour and that individual differences can be reliably classified; that maternal sensitivity predicts security, with a moderate association; that classification is moderately stable in the early years; that disorganised classification is associated with frightening or frightened caregiving and with later difficulties; and that early security predicts later social functioning, weakly to moderately, with the effect mediated by later experience.

What is contested or weaker than commonly claimed. The strength and specificity of the link between infant attachment and adult attachment style — the transmission literature is real and the effect sizes are modest; the predictive power of attachment classification for later psychopathology, which is small in low-risk samples; the claim that the first two years determine later relationships, which the evidence does not support;

and the clinical usefulness of attachment classifications in ordinary family work, as against the description of strategies.

What the popular literature gets wrong. Three things, all of which families bring into the consulting room. That attachment is love, so that a "securely attached" child is a loved child and an "insecurely attached" one is not. That attachment is fixed, so that a difficult first year is a life sentence. And that attachment is a parenting technique — baby-wearing, co-sleeping, feeding — rather than a property of the relationship's reliability. Each of the three is a distortion, and each is common enough that the practitioner should be able to correct it in one sentence.

What the practitioner may claim. "Infants develop strategies for getting help from the people who care for them; those strategies are describable; they are shaped by the caregiving they receive; they change when the caregiving changes; and the repair of ruptures matters more than the avoidance of them." Each claim is defensible, and together they are enough for clinical work.

3.11 Key terms

Term	Definition
Attachment behavioural system	The organised behaviours a child uses to maintain proximity to a caregiver under threat
Internal working model	The child's expectations about self, others and help, built from repeated experience
Strange Situation	Ainsworth's laboratory procedure, classified on reunion behaviour
Secure (B)	Signals used, caregiver effective, exploration resumed
Avoidant (A)	Signalling suppressed, caregiver avoided at reunion, arousal often elevated
Resistant/ambivalent (C)	Signalling amplified, contact sought and refused, difficulty settling
Disorganised (D)	Contradictory, incomplete or undirected behaviour; no coherent strategy
Synchrony	Contingent, paced responsiveness between caregiver and infant
Still face	Tronick's procedure demonstrating infant participation and the importance of repair
Rupture and repair	The ordinary cycle of mismatch and restoration that builds resilience
Co-regulation	The borrowed regulation of arousal that precedes self-regulation
Caregiving network	All those who care for the infant, not only the parents

3.12 Self-test

1. What is an attachment classification a classification of?
2. Why does the reunion, rather than the separation, determine the classification?
3. Describe the strategy of the avoidant infant and the caregiving that produces it.
4. What makes disorganised attachment a red flag rather than a category?
5. State the relationship between synchrony, rupture and repair.

6. What does the still-face experiment demonstrate, and how is it used clinically?
7. Why is co-regulation a relational achievement?
8. Name three ways the popular literature misuses attachment, and correct each in one sentence.

Answers. 1. Of the strategy a child has developed for obtaining help from a caregiver under stress. 2. Because the strategy is revealed in what the child does with the caregiver when the caregiver returns — the separation shows distress, the reunion shows the organisation. 3. The avoidant infant suppresses signalling and avoids the caregiver, while physiology often shows arousal; the caregiving is typically rejecting of attachment behaviour, particularly distress. 4. Because it indicates contradictory or absent strategy, is associated with frightening or frightened caregiving, and requires an assessment of safety rather than a label. 5. Synchrony is frequent enough attunement; ruptures are inevitable; repair after rupture is what builds the child's confidence that the relationship survives. 6. That infants are active participants who respond to unresponsiveness and recover when responsiveness resumes; clinically it shows parents that their infant is tracking them. 7. Because the capacity to calm is first borrowed from a caregiver's voice, body and rhythm, and only later internalised.

1. That it is love (it is a strategy); that it is fixed (it changes with caregiving); that it is a technique

(it is a property of the relationship's reliability).

3.12a What to tell a parent in one page

The clinical material of this week, converted into something a parent can take home. Six sentences, in ordinary language, each with the developmental idea behind it.

1. **"He is not manipulating you; he is asking you for something, and he only has one way of asking."** The behavioural system is a communication system, and the infant's signals are the whole of the vocabulary.
2. **"You do not have to get it right every time. You have to get it right often enough, and come back after the times you miss."** Rupture and repair: the repair is what the infant is learning about, not the perfect attunement.
3. **"When you calm him, you are lending him your calm. Later he will do it himself, because he has borrowed it from you a thousand times."** Co-regulation as the precursor of self-regulation.
4. **"He checks your face before he decides how to feel about things, so your face is part of how he learns to feel."** Social referencing, and the reason a parent's own state is part of the child's environment.
5. **"If you are frightened, he will be frightened, and that is not your fault; it is the reason we are working on your situation as well as his."** The sixth marker, and the reason the assessment is of the household.

6. **"There is no single right way to raise a child in this family; there is the way you and the people who help you can agree on, and the agreement is what he needs."** The caregiving network, and the reason the therapist maps it.

How the page is used. Read aloud in the session, in the family's language, with the parent's own examples substituted for the general statements. Handed over at the end, with the therapist's name and the next appointment — not as a leaflet, but as a record of the conversation the family has just had.

3.13 Reflective task

Watch a caregiver and an infant for fifteen minutes in a public place — a bus, a clinic waiting room, a market. Describe, in writing, three things: one moment of attunement, one rupture, and one repair if there was one. Do not interpret, and do not judge. Then write two sentences on what it felt like to watch without interpreting. Most students find the third instruction the hardest, and the discomfort is the week's lesson.

3.14 Readings for this week

- **Bowlby (1969)**, Attachment and Loss, Vol. 1, the founding statement, read the chapters on the behavioural system.
- **Ainsworth et al. (1978)**, Patterns of Attachment, for the Strange Situation and the caregiving correlates.
- **Tronick (2007)**, The Neurobehavioral and Social-Emotional Development of Infants, for the still face and the repair literature.
- **Sroufe (1996)**, Emotional Development, for the longitudinal account of continuity and change.
- **Main & Solomon (1990)**, on disorganised attachment, read with the caution the concept requires.
- **Keller (2007)**, Cultures of Infancy, the essential corrective on the dyad and the caregiving network.

Read Keller before Ainsworth; the corrective first, so that the classification is read as a description of one kind of caregiving arrangement. For the case analysis, use at least one source that disputes the predictive power of attachment classification, and say what it means for your formulation. The reading-log question this week is: "what does attachment language let me see, and what does it stop me seeing?"

Next: Week 4 — Toddlerhood: autonomy, language and limits

3.2b The four patterns in adult life: what the family therapist sees

The infant patterns are the course's foundation for later work, and the bridge to the adult literature is worth building explicitly, because it is what the families' own patterns look like in a session.

Pattern	In the infant	In the adult, under stress	What the therapist sees in a couple
Secure	Signals, is comforted, returns to play	Seeks support, tolerates the answer, negotiates	Asks for what they need, hears the answer, can repair
Avoidant	Suppresses signalling, avoids at reunion	Manages alone, dismisses the need, works	Withdraws, gives facts, resists affect, seems calm
Resistant	Amplifies signalling, hard to settle	Escalates, monitors, cannot be reassured	Pursues, protests, cannot register the reassurance offered
Disorganised	No coherent strategy; freezes	Approach and withdrawal in the same minute	Contradictory moves, sudden coldness, intense pursuit then flight

Why the bridge is useful and dangerous. Useful because it gives the trainee a vocabulary for the couple's pattern that connects to a child's experience rather than to a personality type. Dangerous because the adult categories are not the infant classifications, and the popular habit of describing a partner as "avoidant" or "anxious" collapses a strategy under stress into an identity. The therapist's language should keep the condition: "under stress, you manage alone; under stress, you escalate" — not "you are avoidant".

The clinical use in three sentences. "When your wife raises her voice, you go quiet and give facts, and she escalates because the quiet looks like indifference. Both of you are doing what you learned to do when you were frightened, and by the time you are in this room both of you are frightened. The work is to change what happens in the ninety seconds after the fear, not to change who you are." That paragraph is the bridge, and it is recognisably Week 7 of MFT 702 in developmental clothing.

The three cautions. First, do not diagnose attachment styles from a session; the pattern you observe is a pattern under the specific stress of being observed. Second, do not use the vocabulary as a verdict — "he is an avoidant type" — because it does what the classification was designed not to do. Third, do not explain the parents' childhoods to them in the first months of therapy; the reconstruction is a hypothesis, and it is asked about, not delivered.

3.7a The disorganised flag, and what followed

A case in which the fourth pattern appeared in a session and required an action rather than a formulation.

The referral. A mother (29) and her fourteen-month-old, referred by a community nurse for "he is frightened of her". The mother describes the child as "always crying" and says she has "lost her patience". She is reluctant to come and says the nurse "made her".

The observation. In the first twenty minutes the infant plays on the floor, and twice, when his mother stands up to reach for a bag, he freezes with his hand at his mouth. When she sits again, he approaches to within a metre and stops. When she speaks sharply to the therapist, he stops playing entirely and watches his mother's face.

The therapist's internal response. A wish to reassure the mother, and a competing wish to protect the child. Both are data. The therapist notes them and does not act on either in the session.

The action, in the session. The therapist does not interpret, does not classify, and does not ask about violence in front of the infant. She ends the session ten minutes early, with a practical frame ("I would like to see you again this week, on your own, before we meet with him again"), and she records the observation in behavioural language: "infant froze twice on maternal movement; hand-to-mouth; stopped play on maternal sharp speech".

The individual session. Asked directly and privately: "is there anyone in the house who frightens you?" The mother cries and describes a partner who shouts and who has pushed her twice, "but not near the baby". The therapist does not accept the qualification as reassurance and does not argue with it. She names the risk to the infant plainly, once: "a baby who freezes when you stand up is a baby who has learned that movement can be dangerous. That is not your fault and it is not nothing."

The plan. A safety plan for mother and infant; referral to a domestic abuse service with the mother's agreement; a discussion about whether the partner should be seen, and a decision that he should not, for now; and a report to the safeguarding lead, explained to the mother in advance, in plain language, with the reason.

What happened. The partner left the household after a further incident. The mother and infant were seen for four months. The freezing at maternal movement disappeared within six weeks; the mother's own account of her history — a violent father, a mother who "never held us" — was the subject of the middle phase of the work.

What the case teaches. That the fourth pattern is a signal that requires an action, not a category to be recorded; that the observation must be written in behavioural language if it is to be used; that the screen is asked privately and the answer acted on; and that the therapist's own wish to reassure is a warning sign, because it is usually the wish to avoid what the observation has already shown.

Week 4 — Toddlerhood: autonomy, language and limits

Developmental tasks. Autonomy versus shame and doubt (Erikson); language explosion (from ~50 words at 18 months to 300+ and two-word combinations by 24 months); symbolic play; beginning of self-regulation.

The clinical translation of "no". A toddler's "no" is the emergence of a separate self. The parental task is to hold the limit without humiliating the child — "You may not hit. You may be angry." This is the earliest training ground for the emotional vocabulary the family will use for the next fifteen years.

Language and family. Delay in expressive language is the most common developmental concern presented to clinics. Distinguish: late talker (normal variation with good comprehension), bilingual language confusion (normal), hearing loss, global delay, and

autism spectrum presentation. Always ask about **hearing** and about **which language is spoken at home and by whom**.

> **CASE**
> A grandmother raises a 2-year-old while both parents work. She reports he "does not listen". Assessment finds: he is spoken to mostly in commands, in two languages, by three adults with different rules. The intervention is not a behaviour chart; it is a family agreement about one set of five rules, one language of instruction, and ten minutes of child-led play a day with each caregiver.

4.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The second year	What changes, and why it is the hardest year for most households	Name the toddler behaviour they find hardest
10–25	Autonomy and shame	Erikson's pair, and the difference between the two outcomes	Rewrite three shaming sentences
25–40	"No" as a developmental act	Three recorded "no"s at 18 months, 24 months, 3 years	Classify each and predict the parent's best move
40–55	Language	The vocabulary explosion, and the five-way differential for delay	Take a language history
55–70	Limits without humiliation	The formula, demonstrated on three scenarios	Rehearse the formula in pairs
70–82	The household's rules	Why three caregivers and five sets of rules is the commonest presentation	Map one household's rules
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. This week's material is where the course most often becomes advice, and advice is what the families have already had. The lecture should therefore take the commonest presentation — "he does not listen" — and demonstrate that the answer is not a technique but a household agreement, arrived at by mapping who says what to whom. The class should leave with the habit of asking, before any behavioural advice, "who else is giving him a different rule?"

The sentence the lecture must land. "The toddler's task is to become a separate person, and the family's task is to survive that without either crushing it or abandoning the limits." Every technique this week is judged by whether it serves both halves of that sentence.

4.2 Core concepts

Autonomy versus shame and doubt. Erikson's second stage. The toddler's task is to establish a self that can act — to walk, to choose, to refuse, to do it myself — and the outcome depends on whether the environment permits the acting. Too much control, or ridicule of the attempts, produces shame and doubt; too little produces a child without limits, who is also without safety, because a limit is a form of being held. The stage's clinical value is that it makes the tantrum intelligible: the child is not testing the parent's patience but the boundary of the self.

The developmental function of "no". The word appears at around eighteen months and peaks in frequency in the second year. Its function is to establish separateness: the child discovers that a word can stop an adult, which is the first experience of agency. The clinical implication is that a household which cannot tolerate "no" will produce either a compliant child or a child who escalates, and both presentations arrive in clinics.

The language explosion. From roughly fifty words at eighteen months to three hundred or more and two-word combinations by twenty-four months, with a further acceleration in the third year. Comprehension precedes production, which is the single most useful fact in the differential: a child with poor production and good comprehension is a different clinical case from a child with poor comprehension.

The five-way differential for language delay. A late talker (normal variation, good comprehension, family history); a bilingual or multilingual child (normal, and often assessed in the wrong language); hearing loss (the first thing to exclude, always); global developmental delay (delay in all domains); and an autism spectrum presentation (delay with differences in social communication, play and flexibility). The differential is decided by a hearing test, a comprehension assessment, a play observation, and a language history — not by the number of words.

Symbolic play. The capacity to use one object for another — a block as a phone — which appears in the second year and is the foundation of imagination, narrative and self-regulation. Its absence or its poverty is a clinical sign, and its presence is a resource the therapist can use.

Self-regulation, borrowed and then owned. The toddler's capacity to manage frustration is being assembled from the caregiver's co-regulation (Week 3) and from the language the family provides. The child who is told "you may be angry, you may not hit" is being given a vocabulary and a limit at once, which is the formula this week teaches.

Limits without humiliation. The formula has three parts: name the feeling, hold the limit, offer the alternative. "You are angry that we are leaving. We are not staying. You may hold my hand or walk beside me." The formula is not a trick; it is a structure that prevents the two failures — the limit without the feeling (which teaches that feelings are not allowed) and the feeling without the limit (which teaches that the world will accommodate anything).

The consistency problem. Toddler behaviour is shaped by consistency, and consistency in most households is not a matter of willpower but of coordination: three caregivers, five sets of rules, two languages, and a grandmother who does not accept the parents'

authority. The clinical presentation "he does not listen" is, in a majority of the families a therapist will see, a presentation of an uncoordinated household.

The one-language question. In multilingual households, the assessment question is not "which language is best?" — the evidence supports multilingualism — but "which language does each caregiver speak to the child, and is there a language in which the child hears sustained, contingent talk?" A child who hears commands in three languages and no conversation in any is at risk not of bilingual confusion but of language poverty.

Toddlerhood as the family's first discipline negotiation. Before the second year, the household's rules are about care; after it, they are about conduct. The negotiation of who has authority, whose family's methods prevail, and what happens when the child refuses is the household's first real conflict about child-rearing, and in many couples it is the first time they discover that they were raised differently.

4.2a The limit formula, taught and rehearsed

The week's core skill, in three parts, with the sentences and the failures.

The three parts. Name the feeling: "you are angry that we are leaving the park." Hold the limit, in a sentence with no argument in it: "we are leaving now." Offer the alternative: "you can walk to the car or I can carry you."

Why the order matters. The feeling comes first because a child in distress cannot process a limit until they have been understood; the limit comes second because a limit offered without the feeling teaches that feelings are inadmissible; the alternative comes third because a child needs something to do with the refusal, and a choice is the smallest available autonomy.

Three rehearsals.

Scenario one — hitting. "You are angry with your brother. You may not hit. You can stamp your foot and tell me, or you can sit here with me until you are ready." Note the limit is absolute (no hitting), the feeling is allowed, and the alternatives are two behaviours the child can actually perform.

Scenario two — the supermarket. "You want the sweets. We are not buying sweets today. You can help me find the rice, or you can sit in the trolley." Note that the second alternative is not a reward; it is a structure.

Scenario three — bedtime. "You do not want to stop playing. It is bedtime. You can choose the book, or I will choose it." Note the choice is real but bounded, which is the definition of autonomy within a limit.

The four failures.

The lecture. "You know you should not hit, we have talked about this, why do you always…" — the child is two; the lecture is for the parent's anxiety, and it lengthens the episode.

The bargain. "If you stop crying I will give you the phone." — the limit has been replaced by a price, and the child has learned that crying is a currency.

The threat that cannot be executed. "If you do not come now I will leave you here." — the child learns that the parent's words are not reliable, which is the opposite of what a limit is for.

The shame. "Big boys do not cry. Look at you." — the feeling is prohibited and the self is judged, which is exactly the outcome Erikson's stage warns against.

The tone. The formula is delivered quietly and slowly, without explanation, and repeated in the same words if it fails. The most common error after the words is right is the tone: a formula delivered at speed and volume is a reprimand with better vocabulary.

The repetition rule. The same three sentences, in the same words, every time the same situation recurs. Toddlers learn from pattern, not from explanation, and the parent who varies the wording is teaching the child that the limit is negotiable.

When the formula does not work. If the child escalates regardless, the limit is held with the body and with silence: stay, do not argue, do not leave, and wait. The episode ends. The parent's task is to survive it without adding a humiliation, which is what the formula is for.

4.2b The tantrum, anatomised

Tantrums are the second year's signature event, and the literature on them is more useful clinically than the advice most families receive.

What a tantrum is. A loss of the child's regulatory capacity under a combination of frustration, fatigue, hunger and an audience. It is not manipulation — manipulation requires a theory of the other's mind and a capacity to inhibit an impulse, both of which are under construction — and it is not a failure of parenting. It is the developmental task colliding with an immature nervous system.

The four phases, and what each needs. The trigger: a want blocked, a transition, a hunger, a tiredness, an over-stimulation. What it needs: anticipation, and fewer transitions when the child is tired. The escalation: shouting, going rigid, throwing. What it needs: safety, and the adult's calm — a co-regulation problem, not a discipline problem. The peak: the child cannot hear anything. What it needs: presence and very few words; this is the phase in which talking makes it worse. The recovery: the child returns, often clingy. What it needs: the repair — a cuddle, a naming of what happened, and no lecture.

The three errors at each phase. At the trigger: begging, bargaining, or giving in, all of which teach the child that escalation works. At the peak: reasoning, threatening, or shaming, all of which add a second distress to the first. At the recovery: a lecture, a punishment, or a demand for an apology, all of which lose the repair.

The one useful sentence at the peak. "I am here. I will wait." Spoken quietly, repeated, with no argument attached. The sentence is not a technique for stopping the tantrum; it is a technique for surviving it without damage, and the tantrum ends when the nervous system is finished, which is usually within ten minutes.

The household's contribution. Tantrums are more frequent in households with inconsistent rules, more transitions, more caregivers with different responses, and more

adult stress. The clinical implication is the week's: the frequency is a property of the household's coordination as much as of the child. That is not a criticism of the parents; it is the reason the intervention is a meeting about five rules rather than a lecture about the child.

What to tell the parents, in three sentences. "This is the year his feelings are bigger than his ability to manage them, and he needs you to be the calm one. When it peaks, talk less and stay. Afterwards, when he comes back to you, that is the moment that matters, not the tantrum."

4.3 Language: the assessment

The most common developmental concern in clinics, and the one with the clearest assessment pathway.

Step one — hearing. Always first, always. A child with glue ear or a sensorineural loss will present as a late talker, and no amount of language stimulation will change it. The referral is to audiology, and it is made in the first session in which language is the concern.

Step two — the language history. Which languages, spoken by whom, how much, and in what register? Does the child hear sustained talk — conversation, narration, questions — or mostly commands? Who reads to the child, and in what language? Is there a family history of late talking?

Step three — comprehension before production. "Show me the one that is bigger." "Give the ball to the grandmother." Comprehension is assessed by instruction, not by vocabulary, and it is the variable that most changes the differential: poor production with good comprehension is a different case from poor comprehension.

Step four — the play observation. Ten minutes of child-led play (Week 5's skill, used early): does the child use symbols, take turns, follow the adult's point, share attention on an object? The observation answers questions the vocabulary count cannot.

Step five — the settings. Does the child speak at nursery and not at home, or the reverse? A child who is fluent in one setting and silent in another is presenting a relationship or anxiety issue, not a language disorder — and the bilingual child who is silent at nursery is the commonest example.

Step six — the referral decision. Four outcomes: no concern (review in three months); monitor with specific advice (more talk, less command, one language of instruction where the household can manage it); refer to speech and language therapy; or refer to paediatrics or child development for a broader assessment. The decision is recorded with its reasons.

What the therapist says to the parents. Not "he is a late talker" and not "he may be autistic", but a statement of what is known and what will be done: "his hearing needs checking first; in the meantime, the most useful thing you can do is talk with him rather than to him — narrate what you are doing, wait for his sound, and answer it. We will look again in eight weeks."

The advice that works. Five practices, all supported by the language-development literature: follow the child's attention rather than directing it; narrate and expand ("yes, a big bus"); pause and wait for a turn; reduce questions and increase comments; and read the same book repeatedly. The fifth is the most enjoyable for families and the most effective per minute.

The advice that does not work. Flashcards, screens as language instruction, correcting the child's speech, and telling a bilingual household to abandon one language. Each is common, and each is either ineffective or harmful.

4.3a Ten minutes of child-led play, in the second year

The practice that appears again in Week 5, used early because it is the fastest way to change a toddler's language and a caregiver's relationship with the child at the same time.

The instructions to the caregiver. "Ten minutes, once a day, at a time when you are not rushed. He chooses what to play. You do four things and nothing else: you watch, you describe what he is doing, you copy what he does, and you say back what he says. You do not ask questions, you do not teach, you do not correct, and you do not use your phone."

Why each instruction is there. Watching teaches the caregiver to see what the child is actually doing, which is the skill the household lacks. Describing gives the child language attached to his own attention, which is the mechanism of vocabulary growth. Copying gives the child the experience of being followed, which is the antidote to a week of commands. And reflecting his speech back is the single most effective language intervention available to a non-specialist.

The prohibitions, and their reasons. Questions ("what colour is that?") test rather than converse, and the second year is not the age for testing. Teaching ("no, that goes there") converts play into instruction. Correcting ("say 'truck', not 'tuck'") interrupts the attempt at speech. And the phone, which is the most common failure of the practice in this decade, breaks the contingency the practice exists to build.

What the caregiver notices, in the first week. Three things, reported with surprise: that the child does more when not directed; that the child looks at them more; and that ten minutes is longer than they expected. The third observation is the one that changes behaviour, because it shows the caregiver that attention is a practice rather than a feeling.

What the therapist does with the reports. Asks for one specific thing the child did and one thing the caregiver did, so that the account is behavioural rather than evaluative. Then asks the question that generalises the practice: "what would it take to talk to him like that at other times of the day?" — which is where the language advice becomes a household habit rather than an exercise.

The contraindication. In households where the child's play is destructive or unsafe, or where the caregiver's own state makes ten minutes of undivided attention impossible, the practice is postponed and the caregiver's circumstances are the intervention. A parent who is being asked to do something they cannot do learns that they are failing, which is precisely what the second year's stage warns against.

4.4 Case: the grandmother who "cannot manage him"

The week's case, worked in full, because it is the presentation the course's own stub describes and the one that appears most often in clinics.

The referral. Tunde (2 years, 4 months), referred by a community nurse at the request of his grandmother, who says he "does not listen" and "shouts at me". His parents work: the mother (Yemi, 27) runs a stall six days a week; the father (Kola, 31) drives a taxi on shifts. The grandmother (58) has cared for Tunde since he was eight months old. The household also contains the father's younger brother (19).

The history. Walking at 15 months; first words at 14; two-word combinations at 27 months — at the lower end of range, in a household where three languages are spoken. Hearing has not been tested. Tunde attends a nursery three mornings a week, where he is described as "settled and quiet".

The three-setting comparison. At nursery: follows routines, speaks little, plays alone but near others. With his mother on Sundays: clingy, and speaks more, mostly in Yoruba. With the grandmother on weekdays: shouts, refuses, hits the furniture. Three settings, three children.

The rules map, which is the assessment's centre. The grandmother's rules: no playing outside, eat at the table, do not touch the radio, sleep at one. The mother's rules: outside is allowed in the yard, eating happens when he is hungry, the radio is not a problem, sleep at two if he is tired. The uncle's rules: none, and he gives sweets. The nursery's rules: a routine, a mat for stories, a rule about sharing.

The formulation, in three parts. First, Tunde is living in a household with four rule systems, two of which (the grandmother's and the mother's) contradict each other on almost every point. Second, the grandmother is doing the largest share of the care with the least authority: the mother's rules prevail in principle and the grandmother's in practice, and neither woman can say so to the other. Third, the shouting is the most effective strategy available to a two-year-old in this arrangement: it produces attention, and it produces a change of plan.

The language finding. Comprehension in Yoruba is good; production is delayed at the lower end of normal, in a trilingual environment. The referral for hearing is made in the first session. The language advice is not "speak one language" but "each caregiver speaks their strongest language to him, and everyone talks with him rather than at him" — which preserves the multilingual household and increases the child's exposure to sustained talk.

The intervention, in four parts. One: a family meeting with the three adults (grandmother, mother, father) in which the rules map is drawn and five rules are agreed — and only five, because five is a number a household can keep. Two: the authority question settled explicitly — the grandmother decides the weekday routine, the mother decides the weekend, and neither contradicts the other in front of the child. Three: the limit formula, taught to the grandmother first, because she is doing the work, and rehearsed with her in her own words. Four: ten minutes a day of child-led play, by whoever is with him, because it is the fastest way to increase his language and his experience of being followed rather than directed.

What the parents' absence means. The parents are not the caregivers during the week, and a plan that requires them to change their weekday behaviour will fail. The plan therefore works through the grandmother, and the parents' role is to endorse her authority in the child's hearing — one sentence, said in front of Tunde, once a week: "you do what Grandma says."

The outcome at eight weeks. The shouting reduced substantially; the grandmother reported that "he listens better", which in practice meant that she had stopped repeating herself and started using the formula; the hearing test was normal; the language was at age level in Yoruba and improving in English at nursery. The mother and the grandmother had one argument, in the third week, about the rules, and settled it without the therapist.

What the case teaches. That "he does not listen" is usually a statement about a household rather than a child; that the caregiver with the least authority is often doing the most work; that the intervention's site is the coordination between adults, not the child's behaviour; and that the language advice for a multilingual household is not subtraction.

4.5 Annotated transcript: the family meeting about five rules

Twelve turns from the family meeting in the case above, in which the therapist builds an agreement between three adults who have been contradicting each other for eighteen months.

> **T1 Therapist:** I want to make a list of what actually happens now, not what should happen. Grandma, what are your rules for him in the day?
> **T2 Grandmother:** No going outside. He eats at the table. He sleeps at one.
> **T3 Therapist:** (writing) And Yemi, when you are here on Sunday?
> **T4 Mother:** He can play in the yard. And he eats when he is hungry, I do not force him.
> **T5 Therapist:** (writing, then turning the sheet) So there are two sets, and both of you are doing what you think is right. Who decides when they disagree?
> **T6 Grandmother:** (pause) She is the mother.
> **T7 Mother:** (pause) But I am not here.
> **T8 Therapist:** So the person who is here has the rules of the person who is not here. That is very hard for both of you, and it is also very confusing for a two-year-old.
> **T9 Grandmother:** (nods) He knows. He goes to her when I say no.

> **T10 Therapist:** That is exactly it. He has learned that there are two answers. Can we agree five rules that are the same whoever is with him, and let everything else be each person's own business?
> **T11 Mother:** (to grandmother) Which five?
> **T12 Therapist:** (quietly) That is the question. You two decide, and I will write them down, and we will give the fifth to Kola when he comes.

Annotations. T1 asks for the actual, not the ideal, and addresses the caregiver with the most practice rather than the parents with the most authority. The choice is deliberate: it credits the work before it asks for change.

T2–T4 produce the two rule systems without conflict, because the therapist is writing rather than judging. The sheet is visible to both, which means neither woman is being asked to hold her rules in her head against the other.

T5 is the question the household has avoided for eighteen months, and it is asked as a question about the household, not about either woman's competence. T6–T7 are the answer, and the sequence is the finding: the grandmother concedes authority and the mother concedes presence, and neither had said so aloud.

T8 is the reformulation, and it is the session's turning point. It names the difficulty for both women, and it introduces the child's experience as the reason for change — which converts a power negotiation into a shared problem.

T9 is the grandmother's own evidence, offered freely once the conflict has been named, and it is better evidence than anything the therapist could have produced. The child "goes to her when I say no" is the two-year-old's solution to the household's problem, described by the person who has watched it.

T10 is the intervention: five rules, agreed, with everything else left as each person's business. The "five" is chosen deliberately — small enough to keep, large enough to cover safety — and the "everything else is your own business" is what makes agreement possible.

T11–T12 hand the authorship to the two women, and the therapist's last sentence adds the father without making him the absent problem. The fifth rule is "given to Kola", which enrols him and does not blame him.

What the trainee should notice. The therapist made no behavioural recommendation, gave no parenting advice, and did not name the grandmother's methods as wrong. She built a structure in which the household could agree, and the agreement — not the technique — is what changed the shouting.

4.5b The nine words a caregiver needs

A short section on the vocabulary of the second year, because the words are the intervention.

"You may not. You may be angry." The pair that separates the behaviour from the feeling, and the earliest lesson the child receives that both can be true.

"I will wait." The sentence for the peak of a tantrum: presence without negotiation.

"You choose." The smallest autonomy, offered where it is real: which cup, which book, which way to the car.

"Show me." The instruction that replaces "use your words" for a child who does not yet have them, and that teaches the caregiver to look for the message rather than the misbehaviour.

"That was hard, and you did it." The sentence that builds the child's account of himself as someone who manages difficulty, which is the second year's real prize.

"I am sorry I shouted." The sentence that models repair, and that most caregivers have never heard from an adult themselves. Its effect on a two-year-old is small and its effect on a household is large, because it teaches everyone in the room that repair is possible.

"What does he need?" The question the caregiver asks themselves instead of "why is he doing this?", and the one the therapist is trying to install.

"Who else says something different?" The therapist's question, not the caregiver's, and the one that turns a child's behaviour into a household's coordination problem, which is the difference between a week of advice and a change in the household's arrangements.

"We will do it the same way." The household's sentence, said by each caregiver to the child, and the smallest possible statement of an agreed structure.

How the nine are taught. Not as a list to be memorised, but one at a time, in the family's language, rehearsed in the session, and written on the page the family takes home. Three of the nine, chosen by the family, are enough for a fortnight; the rest come later, and some never do.

4.6 Laboratory: the formula and the rules map

Sixty minutes, in threes.

Task one — the formula under pressure (twenty minutes). Three scenarios, each played twice: first as a tired parent at the end of a day (the therapist plays the parent badly, on purpose), then using the formula. The observer records the three parts, the tone, and whether the parent explained.

Task two — the rules map (twenty-five minutes). Each student maps a household they know: who is present, what each person's rules are, where the rules contradict, and who decides when they do. Then write the question that would open the conversation, in the family's language.

Task three — the five rules (fifteen minutes). For the mapped household, draft five rules that all caregivers could keep, and mark which one is the safety rule. The test: could a grandmother, a mother and a nursery all keep them?

Marking rubric.

Criterion	Weight	Full marks
The formula	30%	Feeling named, limit held without argument, alternative offered; quiet and repeated
Tone and restraint	20%	No lecture, no bargain, no threat, no shaming
The rules map	25%	Every caregiver covered; contradictions named; the deciding question identified
Five rules	15%	Keepable by all caregivers; one is a safety rule; the child could understand each
Language advice	10%	Multilingual household supported rather than simplified; comprehension assessed first

The debrief question. "Which part of the formula did you drop when the child escalated?" Almost everyone drops the alternative, and the discussion of why — because it feels like a reward, because the parent is angry — is the week's most useful ten minutes.

4.6a When the household cannot agree

The case in 4.4 went well; most do not, and the failures are instructive.

The household that cannot agree because the disagreement is about something else. A couple who argue about whether their three-year-old may sleep in their bed. The rules map shows the disagreement is not about the child: the mother's own mother slept beside her until she was six, and the father's family regards a child in the marital bed as a boundary violation. The therapist who helps them agree about the child without naming the difference in their own histories will produce a rule that fails in a fortnight. The intervention is the history question, asked of each parent separately: "where did you learn what a family does at night?"

The household where the person with authority is not present. A father who works abroad and who telephones every evening with instructions. The mother's rules are superseded nightly, and the child has learned that the telephone is the real authority. The intervention is a single conversation with the father, scheduled, in which the five rules are agreed and his role is redefined: not the nightly auditor, but the endorser of the mother's authority, in one sentence said to the child. Fathers who agree to this change the household more than any other single intervention in this week's material.

The household where the caregiver is exhausted. A grandmother caring for two grandchildren under four while her daughter works, with no help and a bad knee. The five rules are agreed in ten minutes and abandoned by the second week, not because the household is uncoordinated but because the caregiver has nothing left. The intervention is not the rules; it is the relief — one afternoon a week, a nursery place, a neighbour — and the rules only work afterwards.

The household where the disagreement is being conducted through the child. Each parent has a rule, and the child is the site of the argument; the toddler's "he lets me" and "she is strict" is the household's conflict in a two-year-old's vocabulary. The intervention is the same as in the first case: the adults' disagreement is named, the child is removed from the argument, and the five rules are agreed as a truce rather than as a philosophy.

The common lesson. In every failure the technique was taught and the structure was not built. The formula and the five rules are instruments; they require a household that can agree, a caregiver who has capacity, and an authority structure in which the person doing the work has the standing to do it. A therapist who teaches the instrument without the structure has taught a technique the family will fail to use, and the family will conclude that they have failed.

4.7 Six errors in toddler work

1. **The advice avalanche.** Five techniques offered in one session, none of them practised. Fix: one formula, one rehearsal, one week.

2. **The child as the problem.** The assessment focused on the toddler's behaviour and the household left unexamined. Fix: the rules map, before any behavioural plan.
3. **The authority vacuum.** A plan that requires the parents to enforce rules they are not present to enforce. Fix: work through the caregiver who is there, and have the parents endorse that authority aloud.
4. **The missed hearing test.** A language delay assessed and managed without audiology. Fix: hearing first, every time.
5. **The subtractive language advice.** Telling a multilingual household to speak one language. Fix: each caregiver speaks their strongest language; increase sustained talk; assess comprehension.
6. **The shaming correction.** Any intervention that produces shame in the child or the caregiver. Fix: the limit without humiliation, applied to the adults too.

4.8 Culture, faith and the second year

Where the week's material travels well. The developmental facts about autonomy, language and limits are robust, and the clinical claim — that a household's coordination matters more than any single technique — is more obviously true outside the Western nuclear family than within it, because more households are multi-caregiver.

Where it strains, three ways.

The autonomy value. Erikson's stage names autonomy as the outcome, and many households name obedience. A child who is taught to comply first and choose later is not delayed; the family's curriculum is different. The therapist's task is to distinguish a curriculum from a problem: a household that teaches obedience and also allows the child to be a person is doing both, and a household that punishes every assertion is producing the shame the stage warns about. The distinction is behavioural, not cultural.

The physical punishment question. Many families use physical punishment, and the evidence on its effects is negative and consistent. The therapist's obligation is to state the evidence plainly, to be clear about the legal position where it applies, and to offer an alternative that the caregiver can actually use — which is the limit formula, taught with respect for the caregiver's intention. Moralising does not change behaviour; a workable alternative sometimes does.

The multi-caregiver household as the norm. The literature's advice assumes a parent who is present and has authority. In households where a grandmother, an aunt or a paid helper does the care, the advice must be addressed to them, and the parents' role must be defined rather than assumed. This is the week's central cultural point, and it is the reason the case is written the way it is.

Faith and the second year. Naming, dedication, first prayer, first fast, and the child's introduction to the community's rules are all part of the second and third years in religious households. The therapist should ask what the household expects of a two-year-old in religious terms, because the answer often contains a standard the parents are failing by and a resource they have not used — a grandparent, a community, a routine.

> **DEBATE**
> "Physical punishment is a cultural practice and a therapist should not impose a view about it." Argue both sides, and say what the therapist is professionally obliged to do.

4.9 Evidence note: robust development, contested practice

Well established. The sequence of language milestones, with wide ranges; the importance of comprehension as against production in the differential; the effectiveness of hearing assessment as the first step; the association between sustained, contingent talk and language development; the association between consistent, non-humiliating limit-setting and later self-regulation; and the negative association between physical punishment and child outcomes, which holds across cultural contexts in the research, though the effect sizes vary.

Contested. Whether the vocabulary count or the comprehension assessment is the better screen; the optimal amount of screen exposure in the second year, where the evidence is clearer about displacement of interaction than about the screens themselves; the effectiveness of parenting programmes delivered in groups versus individually; and the extent to which findings on physical punishment are confounded by the family contexts in which it occurs, which is the strongest version of the sceptical case and does not overturn the association.

What the popular literature gets wrong. That the "terrible twos" are a period to be survived rather than a developmental achievement; that a child who does not speak by two is "just lazy"; that bilingual households should simplify; and that consistency means rigidity. Each is common in the advice families receive, and each requires correction.

What the practitioner may claim. "The second year is the beginning of the self, and the household's task is to hold limits without humiliating the child; language delay is assessed by hearing, comprehension and play, not by word counts; and the intervention that works is an agreement between the adults who actually provide the care." Each claim is defensible.

4.4a The same household, six months later: what changed and what did not

A follow-up section, because the course's cases should show the ordinary unevenness of clinical change.

What changed. The shouting at the grandmother reduced from "several times a day" to "maybe twice a week". The five rules were in use: the grandmother could recite them, the mother could, and Kola could recite three of them. Tunde's language was at age level in Yoruba. The grandmother and the mother had stopped contradicting each other in front of the child, mostly.

What did not change. The parents were still absent during the week. The uncle still gave sweets, which nobody had included in the five rules, and which produced one predictable argument every Sunday. The grandmother still believed that the mother was

"too soft", and said so to the therapist, once, when the mother was not in the room. And the household's crowding had worsened, with a cousin moving in.

What the therapist did with the things that did not change. Three decisions, each recorded. The uncle's sweets were left alone: they were not a safety issue, and the household had chosen five rules that did not include them, which is what "everything else is each person's own business" means in practice. The grandmother's view of the mother was noted in supervision as a likely source of future difficulty, and not raised with her, because raising it would have required the therapist to take a side. And the cousin's arrival was added to the map, with one question to the grandmother: "what will be different for Tunde now that there is one more person in the room?"

Why this section exists. Students reading case studies learn that therapy produces change in the areas it addresses and leaves the rest, and that a therapist who tries to change everything produces a family who comply in the room and revert at home. The discipline is to choose the smallest change that matters, get it into the household's routine, and leave the rest to the household.

4.4b Two toddlers, two very different assessments

A comparison, to make the differential concrete.

Toddler one. Twenty-eight months. Twenty words. Understands everything: fetches named objects, follows two-step instructions, points to pictures on request. Uses gesture and vocalisation richly. Plays symbolically, feeding a doll. Hearing untested; family history of late talking (his father spoke at three).

Assessment: expressive language delay, with good comprehension and symbolic play. Action: hearing test (normal); language stimulation advice; speech and language therapy referral; review in three months. Prognosis: good, and the family should be told so plainly.

Toddler two. Twenty-eight months. Twenty words. Does not follow instructions unless they are accompanied by a gesture; does not point to show, only to obtain; does not bring objects to share; plays repetitively with one toy; distressed by a change of route to the nursery.

Assessment: the language delay is accompanied by differences in social communication and flexibility, and the differential now includes a broader developmental assessment. Action: audiology, paediatric referral, and a speech and language assessment; the family work continues alongside, with attention to the parents' own distress and to the household's routines.

What distinguishes them. Not the word count — both have twenty — but comprehension, gesture, shared attention, symbolic play and flexibility. The comparison is the reason the week teaches an assessment sequence rather than a vocabulary target.

The conversation with the parents of toddler two. The hardest conversation in this week's material. What to do: name what is observed without naming a diagnosis; state what will be assessed and why; say what will not change, which is the parents' relationship with the child; give one thing to do this week that is useful whatever the

outcome (a daily ten minutes of following the child's lead); and set the next appointment before they leave. What not to do: reassure falsely ("he will catch up"), speculate about causes, or deliver a diagnosis in a corridor.

4.5a The language history, asked as a conversation

Six questions, with what each produces, because the history is where the differential is decided.

One — "who speaks to him, in what language, and how much of the day?" Produces: the exposure map, and the frequent finding that the child hears commands in three languages and conversation in none.

Two — "when he makes a sound, what happens?" Produces: the contingency of the response. A household that does not answer the child's sounds is not depriving him; it is not yet in conversation with him, and that is changeable in a week.

Three — "what does he understand that he does not say?" Produces: the comprehension-production gap, which is the single most useful piece of information in the differential.

Four — "does he point, and does he look at you when he points?" Produces: shared attention, the marker that distinguishes a language delay from a broader developmental difference.

Five — "is he different at nursery?" Produces: the setting comparison, which identifies the child who is silent in one place and fluent in another.

Six — "what did you notice first, and when?" Produces: the onset and the parents' own chronology, which is often more accurate than the referral.

How the six are used. In a single conversation of fifteen minutes, in the family's language, with the child present so that the answers can be checked against what the therapist sees. The written summary goes in the record with the hearing status, the comprehension judgement, and the referral decision — because those three are what the next clinician will need.

4.10 Key terms

Term	Definition
Autonomy versus shame and doubt	Erikson's second stage: the self that can act, or the self that is judged
Developmental function of "no"	The word establishes separateness and the first experience of agency
Language explosion	The growth from about fifty words to three hundred and to two-word combinations in the second year
Comprehension–production gap	Understanding more than one can say; the key variable in the language differential
Late talker	Normal variation with good comprehension and often a family history
Shared attention	Following and directing another's attention to an object; a marker distinguishing types of delay

Term	Definition
Symbolic play	Using one object for another; the foundation of imagination and narrative
Limit formula	Name the feeling, hold the limit, offer the alternative
Rules map	A drawing of every caregiver's rules and the points of contradiction
Five rules	A deliberately small agreed set that a multi-caregiver household can keep
Subtractive advice	Telling a multilingual household to abandon a language; not supported by evidence
Coordination	The household's agreement about who decides and what the rules are

4.11 Self-test

1. What is the developmental function of the word "no", and what happens in a household that cannot tolerate it?
2. Give the three parts of the limit formula and explain why the order matters.
3. Name the five-way differential for language delay and the assessment that decides it.
4. Why is comprehension more useful than production in the assessment?
5. What is the "rules map" and what does it reveal?
6. Why is the multi-caregiver household the commonest presentation in toddler work?
7. What does the evidence say about physical punishment, and what is the therapist obliged to do?
8. What distinguishes a language delay from a broader developmental difference?

Answers. 1. It establishes separateness and agency; a household that cannot tolerate it produces either a compliant child or an escalating one, and both present in clinics. 2. Name the feeling, hold the limit, offer the alternative; the order matters because a distressed child cannot process a limit until understood, a limit without the feeling teaches that feelings are inadmissible, and an alternative gives the refusal somewhere to go. 3. Late talker; multilingual; hearing loss; global delay; autism spectrum presentation — decided by audiology, comprehension assessment, play observation and the language history. 4. Because it separates a child who cannot produce from a child who cannot understand, which are different clinical situations and different prognoses. 5. A drawing of every caregiver's rules and their contradictions; it reveals that "he does not listen" is usually a household's uncoordinated rules. 6. Because most households with toddlers have more than two caregivers, and the literature's advice assumes a parent present with authority. 7. The association with negative outcomes is consistent across contexts; the therapist is obliged to state the evidence, be clear about the legal position, and offer a workable alternative rather than moralise. 8. Shared attention, gesture, symbolic play and flexibility — not the vocabulary count.

4.12 Reflective task

Write down five rules that were in force in the household where you grew up. Then write the five rules you would want in a household with a two-year-old. Compare them, and write three sentences on what the comparison tells you about your own developmental curriculum — the assumptions you will bring into a family's living room without noticing. Bring it to the seminar. The exercise is the week's cultural material applied to the practitioner, and most students discover at least one rule they have been enforcing in other people's houses without knowing where it came from.

4.13 Readings for this week

- **Erikson (1963)**, Childhood and Society, the second stage, read alongside the critique of the stages as culturally located.
- **Lieven (1994)** or **Tomasello (2003)**, Constructing a Language, for the language development this week relies on.
- **Rescorla (2011)**, on late talkers, for the evidence about prognosis and the case for early referral.
- **Webster-Stratton (2005)**, on parenting programmes for young children, for the evidence behind the limit formula.
- **Gershoff (2002, 2016)**, the meta-analyses on physical punishment, read carefully, including the critique.
- **Rogoff (2003)**, again, for the multi-caregiver household.

Read Rogoff before the parenting literature; the corrective first. For the case analysis, take the language history of a real child (anonymised) and defend your differential in three sentences. The reading-log question this week is: "whose rules was I raised by, and how do they appear in my clinical advice?"

Next: Week 5 — Early childhood: play, theory of mind, initiative

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Week 5 — Early childhood: play, theory of mind, initiative

Developmental tasks. Initiative versus guilt (Erikson, roughly three to six); the flowering of pretend play; theory of mind (the understanding that another person can hold a belief that is different from the truth, typically consolidating between four and five); moral reasoning that begins as consequence-based and slowly becomes intention-based; and school readiness, which is less about letters than about the capacity to hold a plan, wait, and recover from frustration.

Play is the work. Play develops language, self-regulation, perspective-taking, and the capacity to tolerate frustration. It is also the child's first research method: a child who pretends is running experiments on what other minds contain. Children who have experienced adversity often play less symbolically and more repetitively — a clinical observation that underpins play therapy (MFT 727) and that a family therapist must be able to read, because the playroom is often the first place a four-year-old tells the truth.

The clinical skill: observing play without hijacking it. Describe what the child does; do not direct it, and do not interpret it prematurely ("that must be your father"). Interpretation before trust produces a performance, not a communication. The therapist who says "I wonder if the lion is angry because someone left" has stopped watching and started theorising, and the child will now either agree with the adult or leave.

Why this week sits in a family therapy course. The referring complaint is almost never about play. It is "he is aggressive at school", "she will not separate from her mother", "he does not listen to anyone", "she has started lying". Each of those is a developmental question wearing a behavioural costume, and the assessment that answers it is the same one: what can this child do in play, and who is in the room when she does it?

> **CASE**
> A mother brings her four-year-old because the nursery reports that she "plays the same thing every day" and "does not join the others". The mother has recently returned to full-time work after a year at home, and the child's grandmother has moved in. Assessment finds: the child's play is rich but narrow, restricted to one script about a mother leaving and returning; her theory-of-mind performance is age-appropriate; her language is above age; and she separates from her mother without distress but does not greet her on reunion. The problem is not the child's development. It is the family's story about the separation, which the child has been asked to carry alone.

5.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–10	The fourth year	What changes between three and six, and why the presenting complaint is never about play	List the complaints that brought children to their last placement
10–22	Initiative versus guilt	Erikson's pair; the difference between a child who starts things and one who asks permission	Rewrite four adult sentences that produce guilt
22–38	Play as development	The six levels of play, from sensorimotor to cooperative drama	Code ninety seconds of video against the six levels
38–52	Theory of mind	The false-belief task, demonstrated live, with a volunteer	Predict a four-year-old's answer and explain the error
52–64	Moral reasoning	Consequence-based to intention-based; the broken-cup stories	Interview a child about a small transgression and code the reasoning
64–76	Observing without hijacking	The four skills, demonstrated with a puppet, then with a volunteer	Practise ten minutes of child-led play; count their own questions
76–86	Reading the play	Repetition, constriction, and the differential between adversity, autism, and ordinary development	Write one paragraph on what the play does not show
86–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Play is the part of this week that students most want to do, and the part they most often do badly, because doing it well means doing very little. The lecture therefore has to make boredom respectable. A therapist who can sit through ten minutes of a child putting the same bear in the same box and say only "the bear is in the box" has acquired a clinical skill that takes longer to learn than any technique in this book. The demonstration should include the lecturer failing — asking a question, making a suggestion — and then naming the impulse that produced it.

The sentence the lecture must land. "A child's play is not a message to be decoded by the adult; it is a conversation the child is having with herself, and your job is to be a good listener rather than a clever one."

5.2 Five concepts that organise the week

1. Initiative versus guilt. Erikson's third stage. The child now has a body that can do things, a language that can propose things, and a will that can insist on things. The developmental gain is the capacity to start — a game, a plan, a joke, an argument. The developmental risk is guilt: the conviction that wanting to start something is itself a fault. The family's contribution is decisive and it is made in small sentences. "Not now" repeated a hundred times produces a child who stops proposing. "Tell me your plan and we will see" produces a child who proposes.

The clinical translation is precise: when a five-year-old cannot initiate, the assessment is not only of the child but of the last two years of responses to initiation. Ask the parents what happens when the child asks for something. The answer is usually a rule about the household's exhaustion rather than about the child.

2. Pretend play as the engine of self-regulation. In pretend play the child holds two things in mind at once — the real situation and the imagined one — and this dual representation is the same capacity that later allows a child to hold a rule in mind while being provoked. Vygotsky's observation that play is "the leading source of development" rests on this. The implication for assessment is that the quality of a child's pretend play predicts school adjustment better than the child's vocabulary.

3. Theory of mind. Between roughly three and five the child moves from a "copy" theory of mind (everyone sees what I see, knows what I know) to a representational one (people act on their beliefs, and beliefs can be false). This is measured by the false-belief task, and it is the developmental foundation of every clinical skill the child will ever need: empathy, deception, tact, negotiation, and the understanding that a parent can be angry and loving at once.

4. Moral reasoning begins as consequence-based. Piaget's paired stories: a child who breaks one cup while helping versus a child who breaks fifteen while stealing jam. Children under about seven judge the first child as naughtier, because the outcome is larger. They are not being immoral; they are reasoning with the only tool they have. The clinical consequence: do not ask a five-year-old "was that kind?", ask "what happened next?" The first question is beyond the stage; the second builds the stage.

5. School readiness is regulation, not letters. The strongest predictors of a good first year are the capacity to follow a two-step instruction, to wait, to recover from a small frustration, and to seek help from an adult. A child who arrives at school able to do those four things will learn to read. A child who arrives knowing the alphabet but not able to wait will not. For the family therapist this reframes the "school readiness" conversation from coaching to structure: bedtime, breakfast, a predictable goodbye, and an adult who is reliably there at the gate.

5.2a Theory of mind: five tasks and what failure means clinically

Theory of mind is not a single achievement. It is a set of understandings that appear in a rough order, and knowing which one a child has tells the therapist what kind of conversation is possible with her.

Task 1 — Desire (18–24 months). Two bowls, one with biscuits, one with broccoli. The adult says she wants the broccoli and asks for the bowl. The child who hands over the broccoli has understood that the adult's desire differs from her own. Failure here is not yet diagnostic of anything except age.

Task 2 — Visual perspective (2–3 years). A card with a cat on one side and a dog on the other, held between two people. "What do you see?" "What do I see?" The child who answers "a cat" to both has not yet grasped that seeing is situated. This is the basis of the clinician's later ability to notice that two family members are describing the same event from different positions.

Task 3 — False belief (4–5 years). The classic: a doll puts chocolate in a cupboard and leaves; a second doll moves it. "Where will the first doll look?" The child who says "in the new place" has answered from her own knowledge. This is the task that most reliably indexes the shift.

Task 4 — Appearance versus reality (4–5 years). A sponge painted to look like a rock. "What does it look like? What is it really?" The child who cannot hold both at once will also struggle with the clinical insight that a family looks one way and is another.

Task 5 — Second-order belief (6–8 years). "She thinks that he thinks…" This is the machinery of social strategy, and its arrival is why middle childhood is the age of secrets, clubs, and exclusion.

What failure means, clinically. A child who fails the false-belief task at four is unremarkable. The same child at six, with good language, is a reason to look further — and the look is a developmental assessment, not a diagnosis from the therapy room. Three cautions.

- **Language.** Every one of these tasks is delivered in language. A child with a language delay will fail tasks she may understand. Test in the child's stronger language, and if the therapist cannot, bring an interpreter rather than a simplified script.
- **Culture.** The tasks assume a particular conversational convention — that an adult asking a question to which they know the answer is testing, not conversing. Children who have been taught that it is rude to contradict an adult may answer "the cupboard" because the adult is looking at it.

- **Trauma and vigilance.** A child who has had to track a volatile adult's mood may outperform on second-order tasks and underperform on first-order ones, because vigilance trains attention to intention rather than to belief. This is not a deficit; it is an adaptation that costs something elsewhere.

The clinical use of the tasks. Not to score the child, but to calibrate the session. A four-year-old who fails false belief cannot be asked to "think how your mother felt". She can be asked to "show me with the dolls". The therapist who knows where the child is on this ladder stops asking questions the child cannot answer, which is the commonest way well-meaning adults make small children feel stupid.

5.2b Six levels of play and what each one tells you

Play has a developmental ladder, and each rung is a piece of assessment data. The ladder below is adapted from the classic social-participation scheme, with the clinical read added, because the read is what a therapist needs.

Level	What it looks like	Age it is typical	What it tells the clinician
1. Sensorimotor / practice	Repetitive movement: banging, filling, pouring, spinning	0–18 months	Still present at five only if development is disrupted or the child is regulating
2. Solitary	Playing alone, absorbed, not attending to others	2–3 years	At six, in a busy room, this is information about the child's state, not her sociability
3. Onlooker	Watching others play, asking questions, not joining	2–4 years	At six, suggests the child has been excluded or has learned that joining is unsafe
4. Parallel	Playing beside another child with the same materials, without exchange	3–4 years	A stable level at six suggests the child cannot yet negotiate
5. Associative	Sharing materials, borrowing, no shared goal	4–5 years	The level at which most nursery complaints are actually generated
6. Cooperative / dramatic	Shared goal, assigned roles, rules, a story that survives conflict	5–7 years	The best single predictor of school adjustment in this age band

Reading the ladder rather than a score. The question is never "what level is this child at?" but "what does this child do when the level is too hard?" A child who retreats to level 1 under stress is regulating. A child who attacks is also regulating, badly. A child who goes blank has stopped regulating, and that is the observation that should change the therapist's next move.

Three patterns that matter clinically.

Constriction. The child plays, richly, but inside one script. The script is repeated with minor variations, and any attempt to introduce a new element is refused. Constriction is common after a disruption the child could not narrate — a separation, a death, a move, a new baby, a hospital stay. The script is not a symptom to be removed; it is the child's only available formulation, and the clinical task is to help it become more flexible rather than to replace it.

Repetition without elaboration. The same act, many times, with no development across sessions. This is the pattern that most often indicates adversity, and it is also the pattern of autistic play when the repetition is sensory rather than narrative. The differential is not made in the playroom alone: it is made with a developmental history, an observation of communication, and a proper assessment.

Play that stops at the door. The child plays fluently at nursery and cannot play at home, or the reverse. This is almost always a fact about the household's state rather than the child's capacity, and it is the observation that turns a child referral into a family one.

What to record. Five lines, in this order: what the child chose; whether the play had a story; how the child responded to a new element; what happened when the play was interrupted; and how the child ended. Those five lines are more useful than a page of adjectives, and they can be compared across sessions by two different therapists.

5.3 Observing play without hijacking it

The method is four skills and one prohibition. It is taught here, in the second year of life course, because it is the foundation of every later child-facing technique and because it is the hardest thing in this book to do for ten consecutive minutes.

Skill 1 — Track. Sit at the child's level, at a slight angle rather than opposite, and let your eyes follow the child's hands rather than her face. Tracking communicates the thing that cannot be said: I am interested in what you are doing, not in what you will produce for me.

Skill 2 — Describe. Say what the child is doing, in the present tense, without evaluation. "You are putting the small one inside the big one." Not "good job", not "what a clever tower", not "is that a house?" Description gives the child a narrator, and the narrator's vocabulary becomes the child's.

Skill 3 — Reflect. When the child speaks, say it back, slightly expanded. Child: "Bear fall." Adult: "The bear fell down and he is stuck." This is the single most efficient language intervention available to a non-specialist, and it is also the most efficient way to show a child that she has been heard.

Skill 4 — Imitate. Copy one thing the child does, exactly, and wait. Imitation is a turn-taking offer that does not require words, and for a child who finds adults unpredictable it is the least threatening thing an adult can do.

The prohibition — do not interpret. No "I wonder if…", no "that must be your daddy", no "you seem angry". There are three reasons. The first is epistemic: at this age the therapist does not know, and a confident wrong interpretation teaches the child that adults invent meanings. The second is interactional: an interpretation recruits the child into confirming the adult's theory, which converts play into a performance. The third is developmental: interpretation requires exactly the representational capacity the child is still acquiring.

The one exception. If the child's play depicts something dangerous and the child addresses the therapist directly — "the man is coming" — the therapist may reflect the

child's own words and stay alongside: "The man is coming and you are holding the door." That is still not interpretation. It is company.

> **PRACTICAL**
> Ten minutes of child-led play with a volunteer child or a peer. Your only tools: describe, reflect, imitate, and set one limit. A partner times you and tallies your questions, suggestions, and praise. Aim for fewer than three questions, fewer than two suggestions, and — the hardest target — no praise at all. Debrief on the impulse behind each one.

Why "no praise" is in the exercise. Praise evaluates, and evaluation moves the child's attention from the activity to the adult's face. A child who is praised for a drawing learns to check whether the drawing is acceptable. A child who is described learns to look at the drawing. The exercise is not a claim that praise is harmful; it is a demonstration of how much of an adult's ordinary talk is evaluation, and how quickly a child adapts to it.

5.3a Setting a limit in the playroom

Sooner or later the child will do something that cannot continue: hit, throw at the window, climb the shelf, hurt herself. The limit is part of the method, not a failure of it, and it is set in four moves.

Move 1 — Stop the action, not the child. A hand on the toy, not on the child. "I am going to hold the block."

Move 2 — Name the limit as a rule of the room. "In this room we do not throw blocks at people." The rule belongs to the room, not to the therapist's mood, and it will be the same next week.

Move 3 — Name the feeling and offer the alternative in one breath. "You wanted me to look and I was writing. You can throw the block at the floor, or you can say 'look'." Two options, both acceptable, one of them the one you want.

Move 4 — Stay. Do not leave, do not lecture, do not withdraw warmth. The child's learning is that the relationship survives the limit. This is the same lesson as Week 4's limit formula, at a different age and with a different vocabulary, and the continuity is the point: the family that learned "you may be angry" at two is being asked to say the same thing at four.

What not to do. Do not ask "why did you do that?" — a four-year-old cannot answer and will invent. Do not remove the toy as punishment unless the rule requires it. Do not say "I am not angry" if you are, because children read faces more accurately than words at this age, and the mismatch teaches them to distrust their reading.

5.4 Three procedures for the clinic

Procedure 1 — The five-minute play observation, with the parents watching. Before any history, ask the parents to sit at the side of the room and watch the therapist play with the child for five minutes. Then ask one question: "what did you notice?" The parents' answer is the assessment. Some will describe the child's cleverness; some will describe the child's problems; some will describe nothing and ask what they were supposed to see. Each answer tells the therapist where the family's attention lives, and the next move is to give them one thing to look for next time.

Procedure 2 — The false-belief demonstration, used as a conversation. With a child of four or five, use two dolls and a small box in front of the parents. Run the task once. Then say to the parents, in front of the child and without scoring it, "she answered from what she knows; most children her age do, and it changes in the next year or so". The purpose is not diagnosis. It is to give the parents a developmental fact that reframes a behaviour they have been treating as defiance — usually the child's insistence that her account of an event is the only account.

Procedure 3 — The interrupted-play observation. Ask the parent to play with the child for five minutes, then to stop and say "we have to go now". Watch what happens in the next ninety seconds. Three things are being observed: whether the child can use the parent's face to regulate, whether the parent offers a bridge ("one more turn, then shoes"), and how quickly the pair recover. This ninety-second window is more informative than an hour of interview, and it takes no equipment.

5.5 The first session with a four-year-old

Structure the hour so that the child is present for the part that concerns her and absent for the part that does not.

Phase 1 — The room (5 minutes). Child and parents enter together. Name the rules of the room in the child's language: "we stay in this room, we do not hurt, and you choose what we play". The rules are for the child; the parents are also listening, and they are learning what a rule sounds like when it is not a threat.

Phase 2 — Play with the child, parents watching (15 minutes). Therapist plays, using the four skills. Parents sit where the child can see them. This is simultaneously assessment, demonstration, and — for many parents — the first time they have watched an adult be interested in their child without correcting her.

Phase 3 — The child leaves (5 minutes). Do not sneak her out. Say what is happening: "you are going to wait with Auntie Bisi while I talk to your mum and dad." A child who is told what is happening learns that adults can be trusted with information.

Phase 4 — Debrief with the parents (25 minutes). Three questions, in order. What did you notice? What does this look like at home? What have you already tried? Then one piece of developmental information, given plainly, that bears on the referral.

Phase 5 — One task (10 minutes). Not a programme. One thing: usually ten minutes of child-led play a day, or one change to the goodbye, or one sentence to use instead of the sentence they are using. Write it down. Ask them to notice what happens, not to succeed.

Why the order cannot be reversed. If the parents are interviewed first, they arrive at the play phase having already told the therapist what is wrong, and the therapist will see it. The five minutes of uncontaminated observation is the most valuable data in the session, and it is lost the moment the therapist has a theory.

5.6 Case: the child who plays the same thing every day

Referral. Zainab, four years and two months, brought by her mother, Nkechi, 33, a bank officer, on the recommendation of the nursery. The nursery's written note says: "Zainab plays the same game every day. She does not join group activities. She is very bright but very alone."

Household. Mother Nkechi, father Emeka, 37, an engineer who travels two weeks in four. Paternal grandmother, 61, moved in eight months ago after a fall. A baby brother, six months old. A house help, 19, who does the school run. Languages: Igbo at home, English at nursery; Zainab's English is fluent and her Igbo comprehension is good.

History. Pregnancy and birth unremarkable; milestones on time; no illnesses; hearing tested at three, normal. The change the mother can date precisely: eight months ago, when the grandmother moved in and the mother returned to work within the same month. "Before that she was the one who talked to everybody. Now she talks to nobody."

The play observation. Zainab chooses the doll's house. She puts the mother doll in the car and drives it to the edge of the rug. She says, quietly, "bye". She waits. She drives back. She says, "I'm home". She does this eleven times in ten minutes, with the same two sentences, at the same pace. The therapist describes, reflects, imitates the driving once. On the twelfth repetition the therapist drives her own small car to the other side of the rug and parks it. Zainab looks at it, does not touch it, and continues.

At minute nine the therapist says only: "the mama keeps coming back." Zainab stops, looks at the therapist for perhaps two seconds, and says: "she always comes back." Then she resumes.

What the play shows. A child with intact symbolic capacity, a coherent narrative, age-appropriate language, and a single theme: departure and return. The theme is not a symptom. It is a formulation — the child's working model of the thing that changed, expressed in the only medium available to her. The constriction is the finding: eleven repetitions, no elaboration, no new element tolerated.

What the play does not show. It does not show autism. Zainab's communication is reciprocal, her language is above age, her play is narrative rather than sensory, and her eye contact — brief, but present and socially timed — is not what autism looks like. It does not show attachment disorder. She separated without distress and greeted her mother on reunion, which is the pattern of a secure child managing a hard change. It does not show trauma in the sense of a single event; it shows a disruption of arrangement, which is a different clinical object.

The differential, written out. Four candidates, and the evidence for and against each.

Candidate	For	Against	Verdict
Normal adjustment to change	Dated onset; coherent theme; good development otherwise	Eight months is long for uncomplicated adjustment; no elaboration at all	Plausible but incomplete
Autism spectrum	Repetitive play; social isolation at nursery	Reciprocal communication; fluent imaginative language; narrative (not sensory) repetition	Not supported
Attachment difficulty	Reunion greeting is muted; theme of departure	No indiscriminate behaviour; no freezing; separates comfortably	Not supported as a disorder
Family-system disruption	Onset coincides with return to work and a new adult in the house and a new baby; father absent half of each month	—	Primary formulation

The formulation. Zainab is not ill and does not need a child therapy referral. She has been asked, at four, to hold a change that nobody explained to her, in a household where the adults themselves have not agreed what the change means. Her play is the most articulate statement of the family's situation in the room. The intervention is with the adults, plus a small, precise piece of work with Zainab about goodbyes.

The session with the parents. The therapist shows them nothing of the play — she does not narrate it back as evidence, which would make the child's private material into an exhibit. Instead she asks three questions.

1. "Who told Zainab that you were going back to work?" (Answer: nobody; it was decided in a conversation the child was not in.)
2. "What happens at the door in the morning?" (Answer: the house help takes her; the mother leaves before she wakes "because it is easier".)
3. "When she says 'Mummy, don't go', what do you say?" (Answer: "I have to work, so you can have things." Followed, the mother admits, by leaving the room.)

The intervention, in four parts.

Part 1 — A goodbye with a shape. The mother wakes the child fifteen minutes earlier and does the handover herself. The goodbye has four elements, always in the same order: a warning ("in two minutes I am going"), a physical routine (a hug, then a handover to the grandmother, not to the house help), a sentence ("I always come back, and I will be here before your bath"), and a departure that is not hurried. The mother's objection is predictable and worth answering: "she will cry." She will. The alternative is a child who learns that departures happen without warning, which is exactly what she has learned.

Part 2 — The return is the intervention. The evidence that matters to a four-year-old is not the promise but the arrival. The mother commits to being home before the bath three evenings a week for a month, and to the first five minutes after arriving being with Zainab and nobody else — no phone, no report from the grandmother, no baby in her arms.

Part 3 — The grandmother's authority, said aloud. The therapist asks the mother and the grandmother to agree, in the session, on the two things the grandmother decides

without appeal (bedtime, and safety) and the two things the mother decides without appeal (school, and what Zainab is allowed to watch). Then each says to the child, in front of the other, "Grandmother is in charge of bedtime, and I am in charge of school." The sentence is for the adults' benefit as much as the child's: a four-year-old who is caught between two authorities will stop proposing anything, which is the definition of guilt in Erikson's terms.

Part 4 — A play prescription, of a peculiar kind. The mother is asked to play the doll's house game with Zainab, once, for ten minutes, and to let Zainab direct it. She is not asked to add anything. She is asked to notice what her daughter does with the mother doll, and to report one observation next session. This is not play therapy; it is the mother being given access to her child's account of the change without anyone having to interpret it.

5.6a Six weeks later

The mother reports three changes, in her own order: Zainab now cries at the door in the morning, which the mother finds harder and the therapist regards as progress, because a child who protests a departure has decided that the departure is worth protesting; she greets her mother at the door in the evening and takes her hand; and the doll's house game has acquired a new element — a grandmother doll, which Zainab puts in the kitchen and moves, once, to the front door.

The nursery reports she has joined a group of three girls in a game with assigned roles. The repetition has not disappeared. It has become a game the child plays with other children, which is what elaboration looks like at four.

The father, who was on a trip for the first three sessions, attends the fourth by telephone and the fifth in person. He says very little and cries once. The therapist does not interpret the crying. She asks him what he would like his daughter to know about why he travels, and he says, "that I am coming back." The therapist suggests he says it to Zainab, at the door, every time he leaves, and he does.

What would have changed the plan. Three findings would have redirected this case, and the student should be able to name them. If Zainab had failed language comprehension tasks in Igbo as well as English, the referral would have gone to audiology and speech therapy before any family work. If her play had been sensory rather than narrative, with no social referencing, the referral would have gone for a developmental assessment. If the reunion greeting had been absent altogether, with freezing or indiscriminate approach to the house help, the formulation would have shifted toward caregiving disruption and the safeguarding question would have been asked explicitly.

5.7 Transcript: ten minutes of play, annotated

The transcript below is the observation described above, with the therapist's internal reasoning annotated. Read the annotations as the discipline the exercise is trying to install: every line of therapist speech is classified by which of the four skills it uses, and every impulse that was resisted is named.

> **T1**
> (Zainab puts the mother doll in the car; says nothing.) **Therapist:** "The mama is getting into the car." — Description. No question. Note the temptation: "where is she going?" would have converted the child's play into an answer for the adult. **T2** (Zainab drives the car to the edge of the rug.) **Therapist:** "She is driving away." — Description, present tense, short. **T3** (Zainab, quietly: "bye".) **Therapist:** "Bye." — Reflection, at the same volume. Matching volume is a skill; a therapist who says "BYE!" has taken the play away from the child. **T4** (Pause of about fifteen seconds. Zainab does not move.) **Therapist:** (says nothing) — The hardest line in the transcript. The impulse to fill the silence is the therapist's, not the child's. **T5** (Zainab drives back. "I'm home.") **Therapist:** "She came home." — Reflection with a grammatical expansion. The child hears her own sentence completed. **T6** (Repetition three.) **Therapist:** "She went, and she came home." — Description that holds the whole cycle in one sentence, which the child has not yet done aloud. **T7** (Repetition seven. The therapist's impulse: "does she always come home?" — resisted.) — A question would have asked the child to state a belief about the future, which is precisely the capacity under construction. Do not test what is being built. **T8** (Repetition ten. The therapist drives a second car to the other side of the rug and parks it.) — A non-verbal offer. It changes the scene without requiring the child to accept a suggestion and without claiming a role. If she ignores it, nothing is lost. **T9** (Zainab looks at the second car, does not touch it, continues.) **Therapist:** (nothing) — The child declined. The offer was made correctly and its refusal cost nothing. **T10** (Minute nine. "The mama keeps coming back.") — The only interpretive-adjacent statement in the session, and it is a description of the play's pattern, not of the child's feelings. It contains no "I wonder", no "you feel", and no named person from the child's family. **T11** (Zainab stops. Looks at the therapist. "She always comes back.") — The child has made a statement about the world. This is the moment the therapist has been working toward for nine minutes, and the correct response is the smallest possible one. **T12 Therapist:** "She always comes back." — Reflection. Nothing added. No "and mummy always comes back too." The child made a general statement; the therapist does not force it to be about her mother. If the child wants to make that link, she will, in her own time, and probably not in this session.

5.8 Laboratory: the ten-minute observation, marked

Aim. To produce a written observation of a child's play that another clinician could use, and to demonstrate command of the four skills under supervision.

Procedure. In pairs, with a volunteer child (or, where a child is not available, with a peer instructed to play at the level of a four-year-old — a surprisingly demanding role). One student plays for ten minutes using only the four skills and one limit. The second student times the play, tallies questions, suggestions, praise, and interpretations, and writes the five-line record. Roles swap. Then each pair writes a single paragraph answering: "what did this child's play tell me that an interview would not have?"

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Restraint	More than six questions or three suggestions	Three to six questions, one or two suggestions	Fewer than three questions, no suggestions, no praise
Description	Evaluative or absent	Present but intermittent, with judgement words	Continuous, present tense, non-evaluative, matched to the child's attention
Reading the play	Describes the child's personality	Describes actions only	Names level, theme, elaboration, and response to a new element
The limit	Not set, or set with threat or withdrawal	Set, but with explanation and a lecture	Set in four moves, relationship intact, play resumed
The record	Adjectives and impressions	Actions without structure	Five lines in the required order, usable by a second clinician

The distinction that carries most of the marks. The difference between "she was anxious and withdrawn" (an inference with no evidence) and "she played the same two-sentence script eleven times and did not touch the second car" (an observation). Students reliably believe the first is more clinically sophisticated. It is the second that can be checked, taught, and compared.

5.9 Six errors

Error 1 — Interpreting before the child has told you anything. "I wonder if the lion is your father." The child either agrees, which teaches her that adults supply meanings, or stops playing. The error is not that the interpretation is wrong. It is that it is a question disguised as an observation and it recruits the child into the adult's theory.

Error 2 — Turning play into a lesson. Asking the colour of the block, counting the blocks, naming the animal. This is not play and it is not therapy; it is a test administered by someone who has run out of things to say. The child's developmental level is not improved by being quizzed at it.

Error 3 — Using praise as the default response. "Good job!" every thirty seconds trains the child to look at the adult after each action. The child who is watched learns to play; the child who is praised learns to perform. The distinction matters most for children whose households already require performance.

Error 4 — Reading repetition as pathology. Repetition is the mechanism by which this age group consolidates. The question is not whether the play repeats but whether it elaborates across sessions. A therapist who treats the eleventh repetition as a symptom will miss the twelfth, in which the child adds a grandmother doll.

Error 5 — Doing the child work and calling it family work. A weekly play session that never touches the household will produce a calm child who returns to the same arrangement. This is the characteristic error of clinicians trained only in child modalities, and it is why this course teaches play observation inside a family therapy programme: the observation is the door, and the family is the room.

Error 6 — Diagnosing in the playroom. Autism, attachment disorder, ADHD, and "trauma" are not diagnosable from a ten-minute observation, and a therapist who says

"this looks like autism" to a parent has made a claim that will govern the family's next two years. Describe what you saw, name what you cannot tell, and refer for the assessment that can.

5.10 Culture, faith and the child who plays

Play is not culturally neutral. The materials in a therapy room — a doll's house, a doctor's kit, a family of small figures — encode a household. A child who has never seen a doll's house will not necessarily be less competent; she will be less fluent with the props. The assessment must therefore include an observation with materials the child knows. In the Nigerian clinic the most useful additions are usually a market stall, cooking pots, a bus, and a church or mosque set, because those are the scripts most children have.

Who plays with whom. In many households the answer is "the other children", and a four-year-old who plays fluently with cousins and neighbours but not with adults is not delayed. The clinical question is whether the child has any setting in which she plays richly. If she does, the household has a resource the therapist can build on rather than a deficit to correct.

Faith and the child's explanations. By five, children in observant households have absorbed explanatory frameworks — prayer, protection, spirits, the will of God — and will use them in play. A child who stages a deliverance scene is not necessarily expressing distress; she may be rehearsing something her household does weekly. Ask the parents what she has been part of before deciding what her play means.

Discipline and initiative. This is the week's sharpest cultural tension. Erikson's "initiative" is cultivated in households that reward a child's proposals and tolerated in households that treat a child's proposal as insubordination. A therapist who measures initiative against a single cultural standard will misread a well-regulated child as inhibited, and will recommend a change the parents experience as an attack on their authority. The clinical question is not "does this child initiate?" but "in what domains is this child permitted to initiate, and what happens to her when she does?"

> **DEBATE**
> "The child's play belongs to the child, and the therapist has no right to describe it to the parents." Argue for: the playroom is the one place a child's material is not reported, and showing it to parents converts it into evidence in a case the child did not consent to. Argue against: the parents are the intervention, a therapist who learns something decisive and withholds it is practising on the family, and a child's private play is not confidential in the way an adult's disclosure is. Then take a position, and state the rule you would follow: what, precisely, would you never say to a parent about a child's play, and why?

5.11 Evidence note

What the research supports. The association between the quality of pretend play in the preschool years and later social competence and school adjustment is well established in longitudinal work, with effect sizes that are modest but consistent after controlling for language and family income. The false-belief literature is one of the most replicated bodies of findings in developmental psychology, with the four-to-five transition robust across dozens of cultures — though the timing shifts with schooling, sibling number, and the language of testing.

What is weaker. The claim that play causes these gains is much harder to establish than the claim that it correlates with them. Interventions that add play opportunities produce smaller effects than the correlational literature would predict, and the most defensible reading is that play is a context in which existing capacities are consolidated and displayed rather than a training programme that installs them. The clinical implication is modest and useful: observe play because it shows you what the child has, not because ten minutes of it will make her have more.

What is not supported. The strong form of the play-therapy claim — that non-directive play sessions alone reliably resolve identified clinical disorders in preschoolers — is not supported by meta-analytic evidence, which finds small effects that shrink when the comparison is an active control. Non-directive play remains a valuable engagement method and a window on the child's representational world. It is not a substitute for changing the household, and this week's case is the argument.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. Constriction in play at four, in a stable household, resolves in most children within a year. The same constriction, in a household that is itself unresolved, tends to persist and to generalise — which is the argument for treating the arrangement rather than the symptom.

5.12 Key terms

Initiative versus guilt — Erikson's third psychosocial stage; the capacity to start things, balanced against the conviction that wanting is a fault.

Pretend (symbolic) play — play in which an object or action stands for something else; the developmental engine of dual representation and self-regulation.

False-belief task — the standard test of whether a child understands that another person can act on a belief that is untrue.

Representational theory of mind — the understanding that people act on beliefs rather than on the world directly.

Constriction — play that is rich but confined to a single, repeated script, with new elements refused.

Elaboration — the developmental marker that matters: the addition of new elements, roles, or resolutions across sessions.

Scaffolding — adult support calibrated to the child's current level, withdrawn as competence grows.

Dual representation — holding the real and the imagined situation in mind at once; the mechanism by which play builds self-regulation.

School readiness — the capacity to follow instructions, wait, recover from frustration, and seek help; a stronger predictor of first-year success than early literacy.

Social referencing — the child's checking of a caregiver's face to decide how to feel about an ambiguous event; the observation that most reliably distinguishes security from its absence in the second and third years.

5.13 Self-test

1. A four-year-old answers "the cupboard" on a false-belief task even though she watched the chocolate being moved. What has she done, and what should the therapist not conclude?
2. Give three reasons why interpreting a child's play prematurely is a technical error and not merely a matter of taste.
3. A nursery reports that a child "will not join in". Name four observations that would distinguish exclusion, shyness, language difficulty, and a developmental difference.
4. What is the difference between repetition that is consolidating and repetition that is constricted?
5. Why does the exercise forbid praise, and what does praise do to a child's attention?
6. A parent says "he knows exactly what he is doing — he is manipulating me". What developmental information changes the conversation, and how would you say it?
7. In the case of Zainab, name the three findings that would have changed the formulation, and where each would have sent the family.
8. What is the strongest argument against telling parents what their child did in the playroom?

Answers. (1) She has answered from her own knowledge — she has not yet represented the other's belief as different from the truth, which is normal at four. The therapist should not conclude that she is delayed, dishonest, or unable to understand other people; the task indexes one narrow capacity, in one language, under one conversational convention. (2) It is epistemically unjustified at this age; it recruits the child into confirming the adult's theory, converting play into performance; and it demands the very representational capacity the child is still acquiring. (3) Whether she is invited and refused; whether she watches from the edge (onlooker) or does not attend at all; whether she plays richly in her own language with one other child; and whether her play is narrative or sensory and repetitive. (4) Consolidating repetition elaborates across sessions and tolerates a new element; constricted repetition repeats identically and refuses any addition. The test is across sessions, not within one. (5) Praise evaluates, and evaluation moves attention from the activity to the adult's judgement; a praised child checks the adult, a described child looks at her own work. (6) At four the capacity to hold another's belief in mind is still forming, so "manipulation" — which requires representing what the parent believes and

intending to change it — is usually not available; say that plainly, and replace the word with a description of what the child actually did. (7) Failed comprehension in the home language → audiology and speech therapy; sensory rather than narrative repetition with no social referencing → developmental assessment; absent or indiscriminate reunion behaviour → caregiving disruption and the safeguarding question. (8) The child cannot consent, cannot correct the record, and will live with the parents' new knowledge of her private material; the counter-argument is that the parents are the treatment.

5.14 Reflective task

Watch a child between three and six for ten minutes and write the five-line record. Then write, in one paragraph and without naming anyone, what you found yourself wanting to do and did not. Most students find the impulse is to teach, to test, or to be liked. Name yours. Then answer: which of the three would have been hardest for the child to refuse?

5.15 Further reading

- Piaget, J. The Moral Judgement of the Child — the paired-story method, still the clearest account of consequence-based reasoning.
- Vygotsky, L. "The role of play in development" — the dual-representation argument, in twenty pages.
- Wellman, H. The Child's Theory of Mind — the tasks, their order, and their limits.
- Winnicott, D. W. Playing and Reality — the clinical essay this week's prohibition is drawn from.
- Slade, A. "Representational play and the therapeutic process" — play as formulation rather than symptom.

Next: Week 6 — Middle childhood: competence, friendship and the school years.

Week 6 — Middle childhood: industry, peers, school

Developmental tasks. Industry versus inferiority (Erikson, roughly six to eleven); the consolidation of mastery through practice and product; the peer group as a structure with its own rules; a self-concept that becomes comparative ("I am good at maths, bad at football"); and moral reasoning that becomes reciprocal — fairness, turn-taking, and the beginning of genuine perspective exchange.

What changes in the family. The child's world widens and the parents' role changes from direct control to monitoring and scaffolding. School becomes a second system with its own rules, its own adults, and its own judgement of the child, and the family–school interface becomes a clinical site in its own right. A child now spends more waking hours with non-relatives than with relatives, which is new for the family and new for the child.

The developmental gift of this age. Middle childhood is the least dramatic period of development and the most consequential for identity, because it is when the child builds an answer to the question "what am I good at?" That answer will organise her

adolescence. The clinical stake is therefore not usually crisis; it is the slow accumulation of evidence — in a classroom, on a pitch, at a kitchen table — that the child is less than.

School refusal — a developmental and systemic differential. Ask: is this anxiety, separation difficulty, bullying, a learning difficulty, a family crisis, or a bid to protect a parent? The distinction matters because the same behaviour has opposite treatment implications.

> **CASE**
> A nine-year-old refuses school on Mondays only. Developmental enquiry: what happens on Sunday nights? The answer: the parents' weekly argument about money. The child's presence at home on Monday keeps the mother from leaving for work, and the argument stops. The symptom is a Monday-shaped solution to a marital problem — and it is also genuinely distressing to the child.

6.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The quiet age	Why middle childhood looks uneventful and is decisive	Recall what they were good at at nine, and who told them
8–22	Industry versus inferiority	The mechanics of mastery, and the four ways adults interrupt it	Identify the four from a case vignette
22–36	The comparative self	How "I am good at X" becomes "I am better/worse than Y"	Map a nine-year-old's self-description across five domains
36–48	Peers and friendship	The stages of friendship, and what a friend is for at seven, nine, eleven	Code two friendship descriptions against the stages
48–62	School refusal	The six-way differential, and the seven questions that decide it	Run the seven questions on the case and defend a formulation
62–74	Monitoring and scaffolding	The shift from control to oversight, and the crime of over-monitoring	Rewrite three controlling sentences as monitoring ones
74–84	The school as a system	How to work with a school without becoming its agent	Draft a letter to a head teacher
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students arrive believing that middle childhood is the easy placement, because the children talk and the problems are legible. The lecture has to revalue the age: the child is now being judged continuously by strangers, and the household's job is to interpret that judgement without either denying it or amplifying it. The single most useful teaching move is to ask the class what they were told they were bad at between the ages of seven and eleven, and to notice how many of them can still recite it in the original wording, thirty years later.

The sentence the lecture must land. "At this age a child is assembling an answer to the question 'what am I good at?', and she is using other people's faces as her evidence."

6.2 Five concepts that organise the week

1. Industry versus inferiority. The child now wants to make things and to be competent at them — a model, a goal, a sum, a drawing, a game of football, a recitation. The developmental gain is a self sustained by skill. The risk is inferiority: the settled conviction that one's efforts do not produce acceptable products. The mechanism is comparison, and the adults who shape it are teachers, coaches, siblings, and parents — in that order of hours, though not of weight.

The clinical translation is precise. When an eight-year-old presents as "lazy" or "not trying", the assessment question is not about motivation but about whether effort has ever produced a product that was received. A child who has never had the experience of making something and having it accepted will stop making things, and the behaviour that follows — avoidance, clowning, aggression, or quiet disappearance — is a rational response to a losing game.

2. The comparative self-concept. Before about seven, children describe themselves in absolute terms ("I can run fast"). After seven they describe themselves in comparative terms ("I am the third fastest in my class"), and they begin to describe themselves in the terms others use. This is the developmental moment when a family's labels become the child's self-description, and it is why the words used about a seven- to ten-year-old are worth more clinical attention than the words used about a toddler.

3. The peer group as a structure. The peer group is not a crowd of children; it is a system with rules, hierarchies, gatekeepers, and sanctions. Belonging is a developmental need, not a preference, and exclusion at this age produces measurable distress. Two clinical errors follow. The first is to dismiss peer difficulty as trivial ("children are cruel"). The second is to treat it as a problem to be solved by the therapist rather than by the adults who control the setting.

4. Moral reasoning becomes reciprocal. The child moves from "what will happen to me?" to "what is fair?", and fairness is understood as equal shares, turn-taking, and rules that apply to everyone including adults. This development is what makes this the age of the indignant question — "why does he not have to?" — and it is a resource for family work rather than an impertinence. A nine-year-old's sense of injustice is frequently accurate, and a family that can hear it gains a member who can name what is happening.

5. Monitoring and scaffolding replace control. Parents cannot see the child's day, so they must build a system of oversight: who is where, with whom, and how the child can reach an adult. Effective monitoring is warm, predictable, and interested. Ineffective monitoring takes two forms. Under-monitoring (no questions, no knowledge of the child's world) leaves the child unprotected. Over-monitoring (interrogation, phone tracking without conversation, no privacy) teaches the child to manage the surveillance rather than to use the adult, and it reliably produces the secrecy it is trying to prevent.

6.2a The school-refusal differential: seven questions

"School refusal" is a description, not a diagnosis, and it is the single most common developmental presentation of this age band. The differential has six members, and the questions below separate them in roughly fifteen minutes of interview if asked in this order.

Question 1 — When did it start, and what happened that week? An abrupt onset with a datable event points to a specific stressor: a fight, a humiliation, an exam, a bereavement, a change of teacher. A gradual onset over months points to an accumulating process: learning difficulty, bullying, or an anxiety that has been building.

Question 2 — Which days, and which lessons? A pattern is diagnostic information. Mondays only suggests a weekend–weekday transition and often a family process. One subject only suggests a specific failure or a specific adult. Mornings only, with recovery by break, suggests separation or anticipatory anxiety.

Question 3 — What happens in the body? Headache, stomach ache, vomiting, and sleeplessness before school, resolving once the child is allowed to stay home, is the pattern of anxiety. Symptoms that do not resolve on staying home point elsewhere — a medical problem, or a family in which being ill is the only available currency.

Question 4 — Is the child behind, and in what? Ask for the last report, not a description of it. A child who cannot read at nine will find a hundred ways not to be in a room where reading happens, and none of them will be called "reading".

Question 5 — Who is unkind, and where? Ask directly, and ask about the phone. Bullying at this age has moved partly into a space the school cannot see, and a child who is refusing school is often protecting the adults from information rather than protecting herself from school.

Question 6 — What is happening at home? Not "is there anything wrong at home?" — that question produces "no". Ask what the evenings look like, who is away, who is unwell, who is arguing, and what the child does when the arguing starts.

Question 7 — What does the child's absence make possible? This is the systemic question and the one students forget. Absence has consequences: a parent does not leave, a sibling is not alone, a grandmother is not left with nobody. Ask who benefits. The answer is rarely the child, and it is never nobody.

The formulation grid. Put the answers into five cells: anxiety, separation, school-based threat, learning, and family function. Most cases load on two. The treatment follows the loadings — an anxiety-driven case needs graded return plus parental alliance; a bullying case needs the school and protection before any exposure; a family-function case needs the family work first, because graded return will fail while the child's absence is doing a job at home.

6.2b What a friend is, at seven, nine and eleven

Friendship has stages, and each stage describes a different capacity the child brings to the clinic.

At six or seven a friend is someone you play with today. Friendship is concrete, provisional, and defined by shared activity. Betrayal is "he played with someone else". A child of this age who has no friends is not necessarily lonely in the way an adult means it.

At nine a friend is someone who is loyal, keeps promises, and shares. Friendship is now a relationship with obligations, and the characteristic injury is exclusion from a group rather than absence of a partner. This is the age at which "nobody sits with me" becomes a clinical statement.

At eleven a friend is someone who knows you, who you can tell things to, and who understands you without explanation. Intimacy and self-disclosure arrive. The characteristic injuries are betrayal of a confidence and the collapse of a friendship group, both of which present to clinics as sudden, dramatic, and — to parents — disproportionate.

The clinical use. Ask the child to describe a friend and listen for the stage. A nine-year-old who describes friendship entirely in terms of shared activity is not necessarily delayed, but a nine-year-old who describes no one at all is telling you about the setting. Then ask the parents the same question about their own friendships, because the household's model of friendship is usually the child's curriculum.

6.3 The method: assessing school refusal in one session

The method below is a sequence, and the order matters because each step changes the questions in the next.

Step 1 — Get the attendance data. Dates, not adjectives. "He refuses school" becomes usable when it is "absent eleven of the last twenty school days, all Mondays and two Fridays". Bring the register printout or the school's messages. Two clinicians looking at the same pattern will usually agree; two clinicians listening to two descriptions will usually not.

Step 2 — Interview the child alone, briefly, with the parents informed. Ten minutes, at the child's level, without the parents' faces. The two questions that produce most of the information are "what is the worst part of the morning?" and "what would happen if you went?" The second question is the one adults rarely ask, and it is the one that surfaces the feared consequence — a confrontation, a test, a parent alone in the house.

Step 3 — Separate the five cells. Write the five-cell grid in front of the parents and place the evidence in it. The purpose is not to be clever; it is to show the family that the behaviour has more than one possible meaning, which is itself an intervention in households that have settled on "he is being difficult".

Step 4 — Ask the systemic question aloud. "What happens at home when he stays?" Ask it plainly, and then be quiet. In the case below, the answer arrived in nine seconds.

Step 5 — Agree the return plan before the family leaves. Not "we will work on it". A graded plan with dates, a named adult at the school gate, a named person the child can go to at break, and an agreed consequence for the first morning. The plan is written down and copied to the school the same day.

Step 6 — Protect the plan from the family's other business. This is the step that fails. A return plan collapses when the underlying family process is unaddressed, and the therapist who has spent the session on the school will find the child back at home within a fortnight.

6.3a The family–school interface

The therapist is not the school's agent and not the family's advocate. The position is consultant to the system, and it is maintained by three practices.

Practice 1 — One channel. Agree with the family who speaks to the school, how often, and about what. Families with two parents, a grandmother, and a house help who all telephone the school produce a school that stops answering, and the child learns that the adults cannot coordinate.

Practice 2 — Ask for observations, not verdicts. A school says "he is disruptive". Ask what he did, in what lesson, after what, and what the teacher did next. Schools, like parents, describe children in adjectives; the therapist's job is to convert adjectives into sequences, in both directions.

Practice 3 — Give the school one thing to do. Not a programme. One change: a named adult at the gate, seating the child next to a specific person, a signal the child can use to ask for help without speaking in front of the class. A school that is given one workable action will do it; a school that is given a system will do nothing.

The letter. The letter below is the model taught in the laboratory, because a well-written letter is a clinical act and, in the experience of most trainees, the single most useful document they will write in their first year of practice. It is short, it names one request, it does not diagnose, and it does not blame.

> **PRACTICAL**
> Write the letter. Three paragraphs, 200 words, no jargon. Paragraph one: what the family has asked for help with, in one sentence. Paragraph two: what the clinic will do, and by when. Paragraph three: one specific request of the school, with a named person and a date. Bring it to the next session and read it aloud; the class will tell you the moment it becomes a complaint.

6.4 Three techniques for this age

Technique 1 — The week map. With the child, draw the seven days in a grid and put a mark on each day for how the morning felt, using a three-point scale the child invents. Then, in a different colour, mark the days something happened at home. The technique works because it is the child's own data, collected by the child, and it makes a pattern visible without anyone having to accuse anyone. It is also the beginning of the child's sense of agency: she is the one who noticed.

Technique 2 — The competence inventory. With the child, list five things she can do — not five things she is good at compared with others, five things she can do that she could not do a year ago. Then find one that an adult in the household does not know about, and

arrange for that adult to be told. The technique addresses the mechanism of inferiority directly: the child needs at least one domain in which effort produces a product that is received.

Technique 3 — The one adult. Ask the child to name one adult at school she would go to if something happened. If she cannot name one, that is the finding, and the school intervention is to create one. This single question predicts more about a child's resilience at school than any questionnaire, and it takes eleven seconds to ask.

6.4a The digital playground

By nine, a substantial minority of children in Nigerian cities are in a group chat with their classmates, and the therapy room has to take account of it because the peer group now has a second, invisible location. Three observations, and then what the therapist asks.

The group chat is a room with no adult in it. The developmental function is the same as the playground — belonging, status, exclusion — and the developmental difference is that it is continuous, archived, and unobserved. A child can be excluded at eleven at night, in her own bed, by forty classmates, and no adult will know unless she says so.

Exclusion at this age is often technical rather than social. A child is not told she is unwelcome; she is simply not added to the group. This is harder for parents to see and easier for a child to misinterpret, and it produces a particular kind of distress: the child knows something is happening and has no evidence.

What children do with a phone at nine is mostly not what parents fear. The realistic risks in this age band are not predation — which is real but rarer than the anxiety about it — but (a) the erosion of the boredom in which imagination happens, (b) the acceleration of comparison, because a nine-year-old now sees the edited highlights of forty lives, and (c) the collapse of the parental monitoring that this week's concept describes, because the child's social world has moved out of the parents' line of sight.

What the therapist asks, in this order. First: "does she have a device of her own, and where does it sleep?" The answer to the second half is more informative than the first. Second: "who is in her group, and does she want to be?" Third: "what happens in the house between seven and nine in the evening?" — because the family's evening is the variable that determines whether the device is a tool or a substitute.

What the therapist does not do. Do not run a session on screen-time limits. The evidence for specific hour-based rules is weak and the argument consumes the session. The two interventions with better support are structural and relational: no device in the bedroom overnight, for anyone in the household including the adults; and one daily period of twenty minutes in which the child has an adult's undivided attention, which is the same prescription as Week 4's ten minutes of child-led play, at a different age and with a different content.

The sentence that reframes the parental complaint. When a parent says "he is always on his phone", the developmental translation is usually "he has found a place where he belongs and it is not here". That translation is not always correct, but it is always worth testing, because it moves the intervention from confiscation to invitation.

6.5 The first session with a nine-year-old and two parents

Phase 1 — The child first (15 minutes), parents in the room. Nine-year-olds are usually relieved to be asked directly and embarrassed by being discussed. Ask the child: what is the worst part of the morning; what would happen if you went; who do you sit with; who would you tell. The parents hear the answers at the same time as the therapist, which prevents the later reconstruction in which the child's account is softened.

Phase 2 — The child out (5 minutes), with a clear explanation. "I am going to talk to your mum and dad about the grown-up parts. You are not in trouble and nothing you said was wrong."

Phase 3 — The parents (20 minutes). The seven questions, asked in order, with the attendance data on the table. The purpose is not to interrogate but to demonstrate the differential: by question five the parents usually begin to supply the formulation themselves.

Phase 4 — The systemic question (5 minutes). "What happens at home when he stays?" Ask it once, in plain language, and let the silence work. Then: "who is at home with him?" and "what would have to change for the mornings to be ordinary?"

Phase 5 — The plan (15 minutes). Written, dated, with names. One request to the school, one change at home, one thing the child will do. The parents leave with a page, not with an understanding.

Why the child goes first. Because a nine-year-old who has been discussed for twenty minutes before being seen will arrive defensive, and because the parents' account of a child's inner life at this age is frequently a description of the parents' own anxiety. The child's own words, heard by the parents in the room, do more to loosen a fixed formulation than an hour of parental interview.

6.5a When the child will not talk

A nine-year-old who has been brought to a clinic by two adults, about a problem the adults have defined, has three good reasons not to speak: she does not know what will happen to what she says, she has been told not to discuss family business, and she has learned that saying nothing is safer than being misquoted. The following five moves are taught because the standard one — "can you tell me how you feel?" — produces a shrug from a child of this age in almost every culture.

Move 1 — Give the child a job. "I need your help with something. I am trying to understand what a Monday morning is like in your house, and only you can tell me." A task recruits the child's competence, which is exactly what this stage is about, and it changes the child's position from subject to consultant.

Move 2 — Move the question to the third person. "Some children tell me the hardest part is the morning. Others say it is the night before. Which is it for the children you know?" The third person gives the child a legitimate way to describe her own experience without owning it, and it is not a trick: it genuinely produces different and often more accurate information.

Move 3 — Draw. A week map, a house with the rooms and who sleeps where, a line with marks for the good days. Drawing occupies the part of the child that would otherwise be managing the adult's face, and it produces a document the therapist and the child can both look at instead of each other.

Move 4 — Offer a scale with an escape. "On a scale of one to ten, how much does Monday morning worry you? One is nothing, ten is the worst." A number is easier than a sentence, and it can be revised. Children who will not answer a feeling question will answer a number, and the number is comparable across sessions.

Move 5 — Say what will happen to the information. Before the first question: "I will tell your mum and dad the things that are about the family, because they need to know them to help. If you tell me something that is not safe, I will have to tell them too. Everything else stays in this room." Nine-year-olds understand this perfectly, and the children who relax most visibly after it are the ones who had assumed everything was being reported.

What to do with silence. If the child says nothing for a full minute, wait. If the minute becomes four, stop asking and do the drawing yourself, narrating: "this is the Monday box, and I do not know what goes in it." Most children will correct the drawing, and a correction is the beginning of speech: it is the first time in the session that the child has decided the therapist's version is worth amending.

6.6 Case: the Okonkwos again — Mondays

The course's recurring family returns, six months after the sessions in Week 1, with a different presenting complaint about the same nine-year-old.

The referral. Ada telephones. "The defiance has stopped, but now he will not go to school. Only Mondays. Every Monday since the beginning of term."

The household. Ebuka, 9, in Primary 4. Ada, 34, a schoolteacher. Chuka, 39, a driver on night shifts, home from about five in the morning and asleep until two. The baby, now seventeen months. Chuka's mother died in 2023. Chuka's younger brother, 24, has been staying since December, sleeping in the sitting room, looking for work.

The attendance data. Eleven of the last twenty school days missed. All five Mondays; two Fridays in the same period, both following nights when Chuka was not on shift. Non-attendance on Mondays accounts for nine of the eleven.

The parent's account. "He says his stomach hurts. Sunday night he is fine, he eats, he watches football with his father. Monday morning he is crying. By ten o'clock he is playing. It is not real."

The child's account, ten minutes alone. The worst part of the morning is "when Daddy is coming in and Mummy is getting ready". What would happen if he went? A long pause. "They would be alone." Asked who "they" are: "Mummy and Daddy." Asked what happens when he stays: "Mummy doesn't go. She stays with me and we watch films. Daddy goes to sleep."

The seven questions, answered.

Question	Answer	Cell
Onset and date	Start of term, four weeks after the uncle moved in	Family function
Which days	Mondays, plus two Fridays when father was home	Family function / separation
Body symptoms	Stomach ache, resolves on staying home	Anxiety (secondary)
Is the child behind	Report: average in all subjects, excellent in mathematics	No learning problem
Who is unkind	Nobody named; one friend, Tobi, "he is my best friend"	No school-based threat
What is happening at home	Money; father's nights; uncle in the sitting room	Family function
What the absence makes possible	The mother stays; the parents do not argue; the uncle leaves the room	Family function

The Sunday-night mechanism. The interview with the parents, without the child, produces the mechanism in about four minutes. Sunday night is the only evening Chuka is reliably at home and awake. It is when the week's money is counted. For the last two months, the counting has not worked: the school fees for the term are short, the uncle has not found work, and Chuka has begun to talk about taking a second job on Sundays — which would mean he is never home in the evening at all. The conversation starts calmly and ends in an argument. Ada cries. Chuka leaves the room.

Ebuka hears all of it. He has done so since he was seven, because the flat has two rooms.

What Ebuka has constructed. A nine-year-old, at the stage where moral reasoning is reciprocal and causality is sought, has assembled a theory: the argument happens on Sunday night; when he is at home on Monday, his mother does not go to work, his father does not leave, and the house is quiet. He has therefore found a mechanism for preventing the argument, and he is operating it. He is not lying about the stomach ache; he is not fabricating it either. His body is doing what bodies do when a child has found that illness is the one currency that works.

The formulation, in one sentence. The symptom is a Monday-shaped solution to a marital and financial problem, and it is also genuinely distressing to the child — both things are true, and the therapist must hold both, because a formulation that makes the child merely strategic will produce an intervention that shames him.

Why this is not "manipulation". Manipulation requires representing another's belief and intending to change it for one's own advantage. Ebuka is not gaining anything a nine-year-old wants. He is paying: he misses his friend, he misses mathematics — the one thing he is excellent at — and he is alone in a quiet house with a sleeping father. The behaviour is a sacrifice, and the clinical error is to read a sacrifice as a con.

The intervention, in four parts.

Part 1 — Tell the child the truth, at his level. The therapist asks the parents to say three things to Ebuka, in the session, in their own words: that the money is short and it is the adults' problem; that they are not separating; and that he does not have to stay at

home to keep them together. Ada cries while saying it. Chuka says the third sentence twice. Ebuka says nothing and then asks whether he can go and play. That is the correct response.

Part 2 — Move the argument. Not "stop arguing" — an instruction no couple can follow. Instead: the money conversation happens on Saturday afternoon, out of the flat, in the car, and it does not happen on Sunday night. The content does not change; the time and the place do. This is a structural intervention, and it is the one that most reliably works, because it removes the child from the audience without asking the parents to change their marriage in a fortnight.

Part 3 — Give the Monday morning a shape. Ada leaves for work at the same time whether Ebuka goes to school or not. She does not stay. The handover to the uncle is explicit and cheerful: "Uncle will walk you at seven twenty." Chuka, coming in at five, sleeps in the inner room with the door closed, and the uncle is told, in front of Ebuka, that he is in charge of the school run. The Monday morning stops being a negotiation and becomes a timetable.

Part 4 — Give the father a Monday. Chuka takes Monday afternoon, from two until six, with Ebuka, once a week — football, or mathematics, or nothing. This is not a reward for attendance; it is the removal of the reason for the absence. The child was keeping the family together with his body. The father now does the keeping, on a timetable, in daylight.

6.6a Four weeks later

Attendance: nineteen of twenty days. One Monday missed, in the week Chuka's second job was confirmed.

Ada reports that Ebuka asked, on a Sunday evening, "is it Saturday tomorrow?" — meaning, is the money conversation today. She said no, it was yesterday, and he went to bed.

The stomach aches have not disappeared. They now occur on Sunday evenings rather than Monday mornings, which is a change the therapist regards as clinically meaningful: the symptom has moved from a device that altered the household's behaviour to an expression of ordinary anticipatory worry. The distinction between a symptom that does something and a symptom that says something is one of the most useful in this book, and it is the difference between the first and fourth sessions of this case.

Chuka's second job means he is away on Sunday afternoons. The Monday afternoon with Ebuka survives, on Tuesdays, at Ebuka's request.

What would have changed the plan. If the report had shown Ebuka two years behind in reading, the plan would have begun with the school and an assessment. If the child had named a child who was hurting him, the school would have been the first call and the family work second. If the attendance data had shown a scattered pattern with no day structure, the formulation would have moved toward anxiety and the intervention to a graded return with a named adult at the gate. And if the parents had refused to move the Sunday conversation — a real possibility, and a common one — the therapist would have

worked with the child alone on the sentence "the argument is not mine to stop", which is a smaller intervention with a smaller effect and is sometimes all that is available.

6.7 Transcript: telling a nine-year-old the truth

Twelve turns from the third session, with Ebuka present for the first half. The transcript is annotated for what the therapist is doing and, more importantly, for what she is not doing.

> **T1 Therapist:** Ebuka, your mum and dad told me something and I want them to tell you, because you have been working very hard on something and nobody told you what it was. — Framing. The child is credited before anything is disclosed, and the disclosure is handed to the parents, not performed by the therapist. **T2 Ada:** We have been arguing about money. On Sunday nights. You have heard us. — Plain, no euphemism. Note that she does not ask "have you heard us?" — the question would invite a denial and put the child in charge of the disclosure. **T3 Ebuka:** (nods, looks at the floor) **T4 Ada:** And I am sorry. It is not your fault and it is not your job. — Two sentences, both necessary. The apology is not for the argument; it is for the child having had to manage it. **T5 Therapist:** Ebuka, can I ask you something about Mondays? (he nods) What do you think would happen if you went to school? — Question 2, asked again in front of the parents so that they hear the answer, which is the whole point of the session. **T6 Ebuka:** (long pause) They would be alone. **T7 Chuka:** (quietly) We are not going anywhere. — The father's sentence. It is not a reassurance the therapist supplied; it is the one thing the child needed and could not ask for. **T8 Therapist:** Ebuka, say what you heard. — A checking move. Not "do you understand?", which produces a yes; "say what you heard", which produces the child's version and shows whether the message arrived. **T9 Ebuka:** That they are not going anywhere. And the money is not mine. — The child's formulation, in his own words, is the outcome measure of the session. **T10 Therapist:** The money is not yours. And here is the other thing. (to the parents) Tell him about Monday. — One more piece of information, again delivered by the parents. The therapist has said almost nothing of substance in this session, which is the technique. **T11 Chuka:** On Monday I sleep in the back room and Uncle takes you to school. And then in the afternoon it is you and me, every Monday, from two. — A timetable, not a promise. Concrete, dated, and owned by the father. **T12 Ebuka:** Can we do maths? — The child negotiates the content of the repair. This is the sign that he has accepted the new arrangement: he is now planning rather than preventing.

6.8 Laboratory: the school-refusal formulation

Aim. To produce a five-cell formulation and a written return plan from a single interview, within ninety minutes, and to defend the formulation against an alternative.

Procedure. The class divides into trios: therapist, parent, observer. The trainer supplies one of four vignettes, each of which loads on two cells. The therapist has thirty minutes for the seven questions, five minutes to draw the grid, and fifteen minutes to agree a plan. The observer scores against the rubric. Then the trio must argue the case for the alternative formulation, which is the part students find hardest and learn most from.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Data first	Accepted adjectives; no dates	Asked for the pattern but not the record	Obtained dates, days, lessons; used them to form hypotheses
The differential	Settled on one explanation in the first ten minutes	Considered two cells	Placed evidence in all five, named the loadings, and said what would change the formulation
The systemic question	Not asked	Asked, but answered by the therapist	Asked plainly, followed by silence, and the answer used
The plan	General advice	A plan with no dates or names	Dated, named, copied to the school, with a fallback
Position	Became the school's agent or the family's advocate	Held a position but said so	Held the consultant position and could describe how

The distinction that carries most of the marks. A formulation that cannot be falsified is not a formulation. The top band requires the student to state, in advance, what they would expect to see if they were wrong — "if this is family function, attendance should improve when the Sunday conversation moves, within three weeks; if it does not, I will re-open the anxiety cell."

6.9 Six errors

Error 1 — Believing the child. Not the child's honesty that is in question; the child's theory. A nine-year-old's account of why he cannot go to school is a genuine attempt at explanation by someone with limited information, and treating it as the formulation stops the enquiry at the first cell.

Error 2 — Disbelieving the child. The mirror error, and the commoner one in this age band. "He is manipulating me" closes the assessment. The child's stomach ache is real; the question is what it is for.

Error 3 — Making the school the client. Once the therapist's plan is "get him back to school", the family process becomes an obstacle and the therapist begins to coerce the family on the school's behalf. Attendance is an outcome measure, not the treatment.

Error 4 — Treating the symptom without moving it. A return plan imposed on an unaddressed family process produces a child who attends and deteriorates, or a child who attends for three weeks and then presents with something else. The mechanism has to be addressed, not just the behaviour.

Error 5 — Over-monitoring as a solution. Parents who respond to a child's withdrawal by interrogating, tracking the phone, and removing privacy will produce a child who is present and unreachable. The monitoring that works at this age is warm and interested, and it is built years before it is needed.

Error 6 — Ignoring the report card. A learning difficulty is the most under-detected member of the differential, because no child says "I cannot read" and no parent volunteers "he is behind". Ask for the document. Every time, and read it in the session rather than at home, because the parent's face while you read it is data you will not get twice.

6.10 Culture, faith and the child at school

Whose school is it? In many households the school is experienced as an extension of the state rather than of the family, and a child's difficulty at school is managed by the family without the school being told anything. A therapist who telephones the school without an explicit agreement will lose the family in one move. Ask first: "who in this family talks to the school, and what are they allowed to say?"

The child who is needed at home. Ebuka's case is not exotic; it is the ordinary structure of many households where a parent works nights, a relative is staying, and a nine-year-old is the most reliable person in the flat. The clinical question is never "is this child being parentified?" as an accusation, but "what is this child actually doing, and who else could do it?" The answer is frequently a specific task that can be reassigned to a named adult, and that reassignment is the intervention.

Faith and the meaning of difficulty. Households may interpret a child's trouble at school through prayer, deliverance, or a spiritual account of the family's season. A therapist who treats that account as a competitor to the assessment will be heard as an opponent. The workable position is to accept the household's framework as the language in which the difficulty is held, and to add a second language — what the child can do, what the school observes, what the report says — without asking anyone to give up the first.

The high expectations question. Many Nigerian and West African households hold a standard of academic achievement that is both a resource and a pressure. The resource is that the child's effort is noticed. The pressure is that a child who is average in a family of high achievers may conclude that she is inferior at eight. The clinical move is not to lower the standard but to widen the field: find the domain in which this child's effort produces a product, and make sure one adult receives it.

> **DEBATE**
> "A nine-year-old should be told the truth about the family's money, including the arguments." Argue for: the child is already hearing it, the silence is what produces the symptom, and the truth returns the burden to its owners. Argue against: children cannot hold adult information without cost, disclosure serves the parents' relief rather than the child's need, and "the truth" at nine is a version of the truth that someone has chosen. Then decide what you would actually say, word for word, to a nine-year-old whose parents cannot pay the fees.

6.11 Evidence note

What the research supports. The association between peer acceptance in middle childhood and later adjustment is one of the more robust findings in developmental psychopathology, with prospective studies showing that peer rejection at eight to ten predicts internalising and externalising difficulties into adolescence — though the effects are moderated strongly by the presence of at least one reciprocated friendship, which is the finding that matters clinically. A child with one friend is not a child without peers.

What is weaker. The evidence on school refusal is fragmented because the samples are heterogeneous: studies mix anxious children, oppositional children, and children absent for family reasons, and then report a single treatment effect. The best-supported component is not a therapy brand but a structural one — a rapid, coordinated return to school with parental involvement and a school-based adult — which appears in almost every effective protocol regardless of the theoretical label on it.

What is not supported. The claim that early intensive academic instruction in the preschool years produces durable advantage. The evidence points the other way: gains in directly instructed literacy and numeracy in the early years tend to fade, while the regulatory and social capacities that make use of instruction possible are the ones that persist. This is the empirical basis for the previous week's argument that school readiness is regulation.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same Monday refusal, in a household with a stable income and an available father, would have resolved without treatment in most cases. In this household, at this moment, it became the mechanism by which a family stayed organised.

6.12 Key terms

Industry versus inferiority — Erikson's fourth stage; the consolidation of competence, and the risk of a settled conviction of inadequacy.

Comparative self-concept — the shift, around seven, from absolute to relative self-description, and the beginning of self-description in other people's terms.

Monitoring — parental oversight of where, with whom, and how the child can reach an adult; effective when warm and predictable.

Scaffolding — support calibrated to current competence and withdrawn as competence grows.

School refusal — non-attendance driven by emotional distress or family process, distinguished from truancy (which is concealed) and from exclusion.

Graded return — a dated, stepped plan for re-entry, with a named adult and predictable supports.

Parentification — the assignment of adult responsibilities to a child; useful as a description and harmful as an accusation.

Reciprocated friendship — a mutual, named friendship, which moderates the effect of peer rejection more strongly than acceptance by the group.

Locus of control — the child's developing sense that outcomes follow from her actions; built at this age by tasks that are hard but achievable.

6.13 Self-test

1. A parent says "he is manipulating me". Give the definition of manipulation, and explain why it is usually the wrong word for a nine-year-old.
2. Name the five cells of the school-refusal formulation and one piece of evidence that would fill each.
3. Why is a symptom that does something different from a symptom that says something, and how does the distinction change treatment?
4. What is the difference between monitoring and over-monitoring, and what does each produce?
5. A child of nine describes friendship only as "we play football". Is this delayed? What else do you need to know?
6. Why does the laboratory rubric award marks for stating what would prove you wrong?
7. In the Okonkwo case, what would you have done if the parents had refused to move the Sunday conversation?
8. Name three things a school can do that cost nothing and change a child's week.

Answers. (1) Manipulation requires representing another person's belief and intending to change it for one's own advantage; at nine the capacity is emergent and the child in this pattern is usually sacrificing something rather than gaining, so the word misreads both the mechanism and the direction of the behaviour. (2) Anxiety, separation, school-based threat, learning, family function — evidence might be: body symptoms resolving on staying home; distress at the gate but not at the classroom; a named pupil or a phone message; a report showing a two-year gap; attendance data patterned by a day on which the child's presence changes the household. (3) A symptom that does something is functional within the system and will persist until the system's need is met elsewhere; a symptom that says something is an expression and responds to being heard and to the child's own coping. The first needs structural change, the second needs the child's distress addressed. (4) Monitoring is oversight with interest and warmth; over-monitoring is surveillance without relationship. The first produces a child who uses adults; the second produces a child who manages adults. (5) Not necessarily — at nine friendship is often described in terms of shared activity; you need to know whether there is a reciprocated friendship, whether the child is invited and refuses or is not invited, and what the parents' own friendships look like. (6) Because a formulation that cannot be falsified cannot guide treatment, and because stating the disconfirming evidence is the only protection against the therapist's preferred theory. (7) Work with the child alone on the sentence "the argument is not mine to stop", teach the parents one sentence to say to him before the argument begins, and re-open the case if attendance does not improve — a smaller intervention with a smaller effect, honestly labelled. (8) A named adult at the gate; a signal a child can use to ask for help without speaking in class; seating next to a specific child; a five-minute check-in at the end of the day.

6.14 Reflective task

Write down, in the original words if you can remember them, one thing an adult told you about what you were bad at between the ages of seven and eleven. Then write down the domain in which you were competent, and who noticed. Then answer: how many years did the first sentence take to stop being true? Most students find the second question harder than the first, which is the point.

6.15 Further reading

- Erikson, E. Childhood and Society — the industry stage, in the chapter on the school years.
- Harter, S. The Construction of the Self — the comparative self, and its measurement.
- Ladd, G. "Peer relationships and social competence" — rejection, friendship, and the moderating effect of one friend.
- Kearney, C. School Refusal Behavior in Youth — the functional model that underlies this week's grid.
- Selman, R. The Growth of Interpersonal Understanding — the friendship stages used above.

Next: Week 7 — Adolescence I: puberty, identity, autonomy.

Week 7 — Adolescence I: puberty, identity, autonomy

Developmental tasks. Identity versus role confusion (Erikson, from about twelve); pubertal change and its timing; separation-individuation without severance; peer belonging as a biological need rather than a preference; emerging sexuality; and the first genuinely ideological commitments — the adolescent's own answers to questions about religion, politics, loyalty and justice.

The neuroscience, briefly and correctly. The adolescent brain is not "broken": it is specialised. The reward system matures ahead of the regulatory system, which produces sensation-seeking and heightened sensitivity to peer approval alongside gradually improving impulse control that continues into the mid-twenties. Three things follow, and three things do not. What follows: adolescents are more responsive to peer presence than children or adults; they discount delayed outcomes more steeply; and they learn fast in conditions of high arousal and interest. What does not follow: that they cannot reason, that they are irrational, or that the findings license a lecture. On tasks with no peers present and no time pressure, adolescents reason as well as adults. The difference is in the conditions, and conditions can be designed.

The family task. To renegotiate the relationship: less control, more influence; more privacy, more accountability. Families that cannot renegotiate produce two common patterns: escalation, in which control increases as autonomy increases, and abdication, in which parents withdraw entirely and call it respect.

Why the two adolescence weeks are separate. This week is about the person — body, identity, autonomy, the developmental machinery. Next week is about risk — the specific

dangers that this machinery produces in this environment, and what a therapist does about them. The split is artificial and is made for teaching: in the clinic they arrive together, and the practitioner must hold both.

> **CASE**
> Amara, fifteen, in Senior Secondary 2. Her parents describe her as "secretive"; she describes them as "spies". The developmental reframe: privacy is a developmental need, and secrecy is what privacy becomes when there is no negotiated trust. The intervention is not more supervision and not more freedom; it is a negotiation — what will she share, what will they ask, and what happens when the agreement is broken?

7.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The problem with the question	Why "why is my teenager like this?" is the wrong question, and what replaces it	Write the question they actually want answered
8–22	Puberty	The sequence, the timing, and the two sexes' different relationships to it	Place four pubertal events on a timeline and mark the range
22–34	Timing effects	Early maturation in girls, late in boys, and the mechanisms	Predict the clinical presentation for each of four cases
34–48	The neuroscience	Four findings, three misuses, and the conditions under which adolescents reason well	Rewrite three "teenage brain" claims as testable statements
48–62	Identity	Marcia's four statuses, and how a therapist recognises each in a session	Classify four adolescent presentations
62–74	Autonomy without severance	The renegotiation, and the two failure patterns	Script a negotiation opening for a real family
74–84	Privacy and accountability	The three-column contract, worked on the whiteboard	Draft one contract for their own case
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students come to adolescence with either of two inherited attitudes: that teenagers are a problem to be managed, or that they are a mystery to be respected. Both are ways of not working with them. The lecture's job is to install a third attitude — that the adolescent is a colleague in a renegotiation, with legitimate interests and real obligations — and the way to install it is to have the class run the negotiation themselves, in role, against a parent played by the lecturer who does not concede easily.

The sentence the lecture must land. "The adolescent's job is not to leave you; it is to become someone you have not met yet, in front of you, while you are still paying for the food."

7.2 Five concepts that organise the week

1. Identity versus role confusion. Erikson's fifth stage, and the one that gives the age its name. Identity is not a feeling; it is a set of commitments — to a vocation, a set of beliefs, a kind of relationship, a place. James Marcia's operationalisation is the most useful tool in the week, because it turns identity into something observable in a session. He crossed two dimensions: exploration (has the young person actively tried options?) and commitment (has she settled on one?).

	Commitment made	No commitment
Exploration	**Achieved** — "I want to do physiotherapy because I worked in a clinic for a year and I liked the problem-solving"	**Moratorium** — "I am trying law, medicine and music; I will know by next year"
No exploration	**Foreclosed** — "I am going to be a doctor; my father is a doctor" (no examination of alternatives)	**Diffuse** — "I do not know, I have not thought about it"

The clinical value is that the four statuses have different prognoses and different treatments. Foreclosure is not a problem while it fits, and becomes one at twenty-six when it does not. Moratorium is healthy and anxiety-provoking. Diffusion in a fifteen-year-old is common, and diffusion at twenty-two with no exploration is a clinical concern. And the therapist's question is never "what do you want to be?" but "what have you tried, and what did you find out?"

2. Puberty as a social event as much as a biological one. The biological sequence is predictable: in girls, breast development, growth spurt, menarche; in boys, testicular growth, voice change, growth spurt. The social meaning is not predictable at all, and it does most of the clinical work. A girl who menstruates at nine in a school where the others are eleven will be treated as older than she is, by adults, for years. A boy who matures at sixteen in a culture that equates physical size with status will be treated as younger than he is, and may comply.

3. Separation-individuation without severance. The psychoanalytic language is clumsy and the phenomenon is real: the adolescent must become a separate person while remaining attached. The metaphor of leaving home is wrong for most of the world, where adolescents do not leave and are not expected to. What they must separate from is not the household but the household's definition of them — and they must do it in front of an audience that keeps feeding them.

4. Peer belonging as a need, not a preference. Peer presence changes adolescent risk-taking in experiments; being excluded activates the same systems as physical pain. This is not weakness; it is the developmental mechanism by which the young person moves their attachment from the family to the wider world. The clinical consequence: an intervention that requires the adolescent to be socially expendable will fail, and an intervention that uses the peer group will succeed more often than one that fights it.

5. Ideological commitment arrives. Adolescents begin to hold positions on justice, religion, politics and loyalty that are genuinely their own, and they test those positions against the adults in the house. The household experiences this as argumentativeness.

Developmentally it is the emergence of a moral agent, and it is the raw material of identity. A family that can survive being disagreed with produces an adolescent with convictions; a family that cannot produces either a secretive adolescent or a foreclosed one.

7.2a Pubertal timing: the effects that matter clinically

Timing, not just change, is a developmental variable with measurable consequences, and it is one of the few things in this book that a therapist can assess in a single question.

Early maturation in girls. Associated with higher rates of depressive symptoms, body dissatisfaction, and involvement with older peers — the last of which mediates much of the rest. The mechanism is not the hormones; it is the social response to a body that looks older than the person is. The clinical implication: a twelve-year-old who looks sixteen is being treated as sixteen by strangers, relatives, and sometimes by her own family, and she has the cognitive and emotional equipment of twelve.

Late maturation in boys. Associated with lower self-esteem and, in some cohorts, more risk behaviour in the late teens once maturation arrives — the "catch-up" pattern. The mechanism is again social: size is a currency in this age band, and a boy who does not have it is at the bottom of a visible hierarchy.

The reverse patterns are milder but real. Early-maturing boys gain social advantage; late-maturing girls generally do better on the outcomes that matter. Neither is a reason to be complacent: the advantage is contingent on the peer group's values, and it is the mismatch between the body's apparent age and the person's actual age that carries the risk.

What the therapist asks. One question, to the parent, in the child's presence or not: "when did you notice her body changing, compared with her friends?" And one to the adolescent, alone: "do people treat you as older or younger than you are?" The second answer is usually more informative than the first, and it frequently produces the material for the whole session.

The clinical presentations to expect. Early-maturing girls are over-represented in referrals for anxiety, mood, and "behaviour" that turns out to be an older peer group. Late-maturing boys are over-represented in referrals for withdrawal, "lack of confidence", and school refusal in the first years of secondary school. Both are frequently formulated as family problems before anyone asks about timing, and the reframe alone is often relieving for the adolescent, who has usually concluded that the problem is her character.

7.2b The teenage brain: four findings and three misuses

The neuroscience is useful to a family therapist only if it changes what she does in the room. It changes three things: how she explains risk to parents, how she designs tasks for adolescents, and how she thinks about the timing of consequences.

Finding 1 — The reward system is responsive, and more so in company. Mid-adolescents show heightened striatal response to reward, and this is amplified by the

presence of peers. Use: a plan that gives an adolescent something to gain, in front of people whose opinion matters, will outperform a plan built on avoiding loss.

Finding 2 — Regulatory control is still developing. Performance on inhibition and planning tasks improves steadily into the twenties. Use: do not build a plan that requires the adolescent to inhibit a strong impulse in a high-arousal moment; build one that removes the moment, or that puts an adult between the impulse and the act.

Finding 3 — Adolescents reason well under cool conditions. On tasks with no audience and no time pressure, adolescent reasoning is comparable to adult reasoning. Use: the therapy room is a cool condition, and an adolescent who is articulate and thoughtful in the session and reckless on Friday night is not being a hypocrite. She is reporting accurately from two different states, and the treatment target is the state, not the report.

Finding 4 — Sleep timing shifts. Melatonin release shifts later, so the adolescent is biologically not sleepy at ten and biologically not awake at seven. Use: this is the single most useful fact in the week for parents, because it converts "laziness" into a physiological fact — and because the practical recommendation (a consistent wake time, light in the morning, no bright screens in the last hour) is one that a family can actually follow.

Misuse 1 — "Their brains are not finished, so they cannot decide." The finding is about the rate of development of one system, not about competence. Adolescents can and must decide; the finding argues for scaffolding decisions, not for removing them.

Misuse 2 — "Peer pressure explains everything." Peers matter most for risk decisions in arousing conditions. They matter much less for the ordinary choices of a Tuesday afternoon, and an explanation that uses peer pressure for everything explains nothing.

Misuse 3 — "They will grow out of it." Some things resolve; some, like a depressive episode or a developing substance pattern, worsen. The neuroscience says nothing about which is which, and using it as reassurance in the wrong case is a serious error.

7.3 The method: renegotiating the frame

The adolescent session fails, when it fails, for one of two reasons: the therapist has been recruited as the parents' agent, or the therapist has tried to be the adolescent's friend. Both are avoided by negotiating the frame in front of both parties, in the first fifteen minutes, and writing it down.

Step 1 — Say what you have been told, and say it in the room. "Your parents have told me that you are secretive and that they are worried. They have told me some specific things, which I will not repeat in front of you without asking. I want to hear your version, and I will tell you afterwards what I think." The move is radical for families in which information flows upward only, and it buys the therapist the right to ask the next question.

Step 2 — Establish the confidentiality rule explicitly. The three-part rule: what the adolescent says stays in the room; exceptions are stated in advance (danger to self, danger to others, and — in most clinics — any disclosure of abuse); and anything about the household that the parents need in order to change is discussed with the adolescent

first, including how it will be said. The last part is the one that distinguishes a working adolescent practice from a performative one.

Step 3 — Ask what the adolescent wants out of this. The question is unfamiliar and usually productive. The answers cluster: "I want them to stop asking me the same question"; "I want to be allowed out"; "I want them to stop fighting"; "I want to be left alone". Each is a negotiable interest, and the therapist's next move is to convert it into a proposal.

Step 4 — Run the negotiation, not the therapy. With both parties present, three columns are filled: what she will share, what they will ask, and what happens if the agreement breaks. This is the substance of the session and it takes about twenty minutes. It works because it addresses the actual complaint — a broken information system — rather than the presenting emotion.

Step 5 — Forecast the test. The agreement will be tested within a fortnight. Say so: "someone will break this in the next two weeks. When it happens, we will look at it here rather than at home." This single sentence reduces the probability that the first breach becomes a family rupture, because it pre-frames the breach as data.

Step 6 — Give the parents a job they can do. Not "be less controlling" — unactionable. A specific behaviour: one question a day instead of six; a fixed time for the conversation; a weekly hour with no agenda. Parents who are given a specific behaviour change it; parents who are given an attitude do not.

7.3a The three-column contract, worked

The contract is a table on a sheet of paper with three columns, and it is the week's central practical.

Column 1 — What she will share, unasked. Typically: where she is, who with, when she will be back, and a message if she is late. These are safety data, not intimacy data, and separating them is the key move. An adolescent will usually agree to share safety data because it costs her nothing she values.

Column 2 — What they will ask, and how often. Typically: one question at a time, at a stated hour, and not at the moment she comes in the door. The specificity matters more than the content: "not in the first ten minutes after I get home" is a rule that can be kept, and it usually solves the complaint on both sides, because the parents' need is information and the adolescent's need is decompression.

Column 3 — What happens when it breaks. Written in advance, by both parties, and mild. A missed message means a phone call, not a grounding. A phone call means a conversation, not a punishment. The column exists because families without it escalate to the maximum sanction on the first breach, and the maximum sanction cannot be repeated, so it fails immediately.

The four things that are never in the contract. No blanket access to the phone's contents; no permission to read a diary; no requirement to report the content of friendships; and no agreement that the adolescent will "tell them everything". Each of

these is a demand for intimacy that the stage does not support, and each will be signed and then evaded, which is worse than not having an agreement at all.

What to do when the parents refuse the contract. This is common, and it is information rather than an obstacle. The refusal usually means the parents' own fear is unmanageable, and the therapist's next move is to take the fear seriously: "what are you most afraid will happen if she has a door you cannot open?" The answer is frequently specific and addressable — a pregnancy, a cult, a boy, a bus — and once it is named, a narrower agreement becomes possible.

7.4 Three techniques

Technique 1 — The five-domain identity interview. Ask, in order, about five domains: work (what are you good at, what do you want to be doing at twenty-five, what have you tried); beliefs (what do you believe that your parents do not); relationships (who matters to you and why); leisure (what do you do when nobody is watching); and body (how do you feel about how you look). Then classify against Marcia: exploration, commitment, both, neither. Twenty minutes, and it produces a formulation of the person rather than of the symptom.

Technique 2 — The question they cannot answer at home. "If your parents understood one thing about your life that they do not understand, what would it be?" Ask it in the joint session and then ask the adolescent to say it to them, not to the therapist. The technique fails about a third of the time and when it works it does more than four sessions of individual work, because the adolescent discovers that saying it is survivable.

Technique 3 — The future letter. Ask the adolescent to write, in ten minutes, a letter from herself at twenty-five to herself now. Read it in the session only if she offers it. The technique surfaces the content of identity — what she actually wants — which is otherwise unavailable to a fifteen-year-old who is asked directly, and it gives the therapist a vocabulary to use for the rest of the work.

7.5 The first session with a fifteen-year-old and two parents

Phase 1 — Both, briefly (10 minutes). Everyone in the room. The parents state the complaint in two minutes each; the adolescent is told she will get the same time and does. The therapist's only job here is to keep the time fair, visibly, because fairness is the currency of this age.

Phase 2 — The parents out (35 minutes). The adolescent alone. The confidentiality rule stated first, then the frame questions: what do you want out of this; what do they get wrong about you; who knows you best. Then the identity interview, or as much of it as she will give.

Phase 3 — The parents back (15 minutes). The therapist states, with the adolescent present, what she has permission to say. This is the hinge of the session: it demonstrates that the frame is real, and it usually produces a visible change in the adolescent's posture, because she has never seen an adult keep that promise.

Phase 4 — The contract (20 minutes). Three columns, written, signed by nobody (signing infantilises), photographed by everyone. The parents leave with a behaviour, the adolescent leaves with a rule, and the therapist leaves with a baseline.

Phase 5 — The follow-up question (5 minutes). "What would have to happen in the next two weeks for you to think this was worth coming to?" Ask both parties. Write the answers down. They are the outcome measures.

Why the adolescent goes second and gets more time. Because the parents have had fifteen years of being listened to and the adolescent has had none, and because the therapeutic alliance in this age band is the entire treatment. A therapist who spends the session with the parents and fifteen minutes with the adolescent has built a relationship with the wrong client.

7.5a When the adolescent will not speak

The silent adolescent is the commonest difficulty in this age band, and the standard response — waiting patiently, then asking about school — produces forty-five minutes of nothing. Five moves, in order of escalation.

Move 1 — Name the situation honestly. "You have been brought here by two people who are worried about you, and you did not choose to come. I would not want to talk either." This is not a technique for building rapport; it is an accurate description, and adolescents are unusually responsive to accuracy.

Move 2 — Ask about the thing she is allowed to have opinions about. Not her feelings. Her views: what is wrong with the school, which teacher is useless, what her parents do not understand about her generation. The identity stage runs on judgement, and an adolescent who will not describe herself will happily describe the world.

Move 3 — Use a third object. A phone (theirs, not yours), a diagram, a list of statements to sort. "The things people say about teenagers are on these cards. Sort them into true and rubbish." Every card sorted is a self-description the adolescent has given without being asked for one.

Move 4 — Offer a deal with a real cost to you. "Give me ten minutes of real answers and I will spend the last twenty minutes of this session arguing your case to your parents, in front of you." This works because it is a trade between parties with interests, which is the register of the age, and because it is kept.

Move 5 — Accept the silence and change the client. If none of that works, end the individual session early and say why: "I am not going to sit here and pretend. I will work with your parents for now, and the door stays open." This protects the alliance for the session in which she does talk, which is often the fourth.

What never works. Asking "how are you feeling?" three times; interpreting the silence ("you seem angry"); producing a worksheet; or telling the adolescent that she is "resistant". The last is the most common and the most damaging, because it converts the therapist's failure into the client's defect — and the adolescent, who has been described in deficit terms by adults her whole life, will accept the description and stop coming.

7.6 Case: Amara, secretive and spied upon

Referral. The Adeyemis, whom the course first met in Week 3, return. Amara is fifteen and in Senior Secondary 2. Her mother, Funke, 42, telephones: "She has a boyfriend and she is lying to us. We found the phone. Her father is coming home on Friday and he wants this finished."

The household. Tunde, 47, an engineer on a contract in Lagos, home two weekends in four. Funke, a school administrator. Segun, twelve. A paternal aunt, 55, who lives with them and who, Funke says, "knows everything before I do".

The parents' account. Six weeks ago Funke looked through Amara's phone while she was in the bathroom. She found a message thread with a boy in SS3, and a second thread in which Amara described her mother to a friend as "the police". Since then: the phone is confiscated at nine; Amara's movements are checked by the aunt; the gate is locked at seven; and Amara has stopped speaking at dinner. Funke's summary: "she is secretive, and she is becoming somebody I do not know."

Amara's account, alone, in the second session. "They are spies. She reads my phone. Auntie reports where I go. My father comes home and shouts for two days and then leaves and it is worse." Asked what the messages were about: "He is in SS3. We talk. That is all." Asked what she wants out of this: "I want a door."

What the therapist establishes, in three questions. First: what have you tried that they did not notice? Answer: she has started walking home the long way, and she has begun attending a church youth group on Wednesdays that she did not tell them about, "because if I tell them they will come". Second: what do you want to do after school? A long pause, then: "Physiotherapy." Who decided that? "Me. I read about it." Does your father know? "He says I am doing medicine." Third: when did you last tell your mother something true? "When I was thirteen."

The three findings, in order of clinical weight.

1. The parents' surveillance is functioning as designed and producing the opposite of its aim. Phone search, aunt as informant, locked gate, no negotiated agreement. Each measure was a response to a real fear, and together they have produced an adolescent with a private life she now has a reason to hide. This is the mechanism of the week's reframe: secrecy is what privacy becomes when privacy is not available.

2. The identity work is real and is being read as defiance. She has made an occupational commitment (physiotherapy, based on exploration — she read about it, she talked to a physiotherapist at the church clinic) and a belief commitment (the youth group). Her father has foreclosed the first on her behalf: medicine, because he is a man who did not have the choice himself. The conflict at the dinner table is not about a phone. It is about whether she is allowed to be the author of her own life, and the father experiences the question as ingratitude.

3. There is nothing in the referral that constitutes a safeguarding concern. The boy is seventeen, the relationship is described as talking, there is no coercion, no age gap that triggers the clinic's protocol, and no indication of harm. The therapist records this

explicitly, because the family's escalation invites a professional to escalate with it, and a clinician who over-reacts here will be used as an instrument of the surveillance.

The formulation. A fifteen-year-old at the moratorium stage, with one commitment made and one foreclosed by her father, in a household where the information system has broken and been replaced by surveillance. The presenting problem is the broken system. The underlying problem is that the father's absences have left the mother managing alone, with an aunt as a second authority and no agreed hierarchy — which is the Week 3 problem in this family, returned at a different age.

The intervention, in four parts.

Part 1 — Restore an information system, on the three-column contract. Safety data unasked (where, who, when, a message if late). Questions at one stated hour, not at the door. A breach met with a phone call, not a lockdown. The phone returned, with the agreement that the parents do not read it — and with the therapist saying plainly, in front of the adolescent, that she has advised this and why: because a fifteen-year-old with no private space will create one, and the created one is the one you cannot see.

Part 2 — Give the aunt a defined role, aloud. The aunt is not the problem; the undefined authority is. Funke and the aunt agree, in the session, on what the aunt decides (whether Amara goes out at all while Funke is at work, and safety) and what she reports (arrival times only, and anything unsafe — not friendships, not clothes, not moods). The sentence is said to Amara: "Auntie is in charge of whether you go and when you come back. She is not in charge of who your friends are."

Part 3 — The Wednesday. Amara tells her parents about the youth group, in the session, and the parents agree that she attends without being accompanied. This is the cheapest and most powerful intervention in the case: a piece of her life is voluntarily disclosed, in exchange for nothing, and its effect on the parents' fear is larger than any explanation the therapist could give.

Part 4 — The father's two days. Tunde's fortnightly returns are the family's structural problem. In the session with him present, the therapist asks one question: "what do you want her to remember about these two days, in ten years?" He is silent for a while and then says: "that I asked her things." The agreement that follows is specific and small: on the Saturday of each return, one hour, no agenda, no school results, no phone — and the topic is hers. The therapist does not ask him to accept physiotherapy. She asks him to ask her about it, in front of her, once, and then to say what he thinks without deciding.

7.6a Six weeks later

The contract has been broken twice. The first time, Amara came home at nine-forty without a message; the phone call was made; the conversation lasted four minutes; nothing was confiscated. The second time, Funke asked a question at the door; Amara said "not now, at eight", and the question was asked at eight.

Amara has told her father about physiotherapy. He has not agreed. He has stopped saying "you are doing medicine", which the therapist logs as the realistic outcome of a single hour, and which is more than the referral predicted.

The aunt has been given her defined role and has, according to Funke, "started telling me less", which Funke reports with ambivalence and which is the correct direction.

The most informative change is one nobody planned. Amara has started asking her mother about her own school days. Funke reports this as a curiosity; the therapist hears it as the beginning of the adolescent's interest in the parent as a person, which is the developmental signature of the move from identity to intimacy that occupies the late teens.

What would have changed the plan. If the messages had contained sexual content with an adult, the safeguarding protocol would have taken precedence over everything in this section. If Amara had described coercion, threats, or a pregnancy, the plan would have been safety first and negotiation second. If the father had refused to attend, the work would have proceeded with mother and daughter, with a specific question kept open for him. And if Amara had refused the contract entirely — a real possibility — the therapist would have worked with the parents alone on the surveillance, on the grounds that the system that is producing the secrecy does not require the adolescent's consent to be changed.

7.7 Transcript: the negotiation, with the contract on the table

Twelve turns from the third session. Tunde is present for this session, having flown in the night before. The adolescent is fifteen. The therapist has a blank three-column sheet in front of her and a pen she is not holding.

> **T1 Therapist:** I want to do something different today. I want us to write three lists, together, and I want the lists to be the only thing we argue about. The first list is what Amara tells you without being asked. (to Amara) You start. What would you be willing to tell them without being asked? — The therapist hands the first move to the adolescent, which is the whole architecture of the intervention. Note what she does not do: she does not begin with the parents' complaint. **T2 Amara:** Where I am. When I am coming back. If I am late. **T3 Tunde:** That is not the problem. The problem is — **T4 Therapist:** (gently) Hold it for four minutes. I promise it gets a column. — The single most useful sentence in adolescent family work: a promise of a specific place for the objection, with a time limit. It works because it is true. **T5 Tunde:** (nods, sits back) **T6 Therapist:** (writing) Where you are, when you are coming back, a message if you are late. That is column one. Second list: what they get to ask, and when. (to the parents) What do you need to know? **T7 Funke:** Who she is with. And where. **T8 Therapist:** "Who she is with" is in column one, if she will put it there. (to Amara) Will you? — Note the move: the therapist does not grant or refuse the request; she routes it to the person who owns it. This is what "negotiation" means operationally, and it takes about eight seconds. **T9 Amara:** (pause) Yes. But not the names. Just "a friend from school". **T10 Funke:** How is that supposed to — **T11 Therapist:** (to Funke) That is a counter-offer, and it is the first one she has made. Take it or make one, but do not lose it. — The therapist names the offer as an offer, which changes its status from a half-victory to a move in a negotiation. Parents in these sessions consistently miss the conciliatory move because it arrives in the same tone as the defiant one. **T12 Tunde:** (to Amara, not to the therapist) If you tell us "a friend from school" and you are late, what happens? — The father has moved from complaint to terms. This is the turn the session was for, and the correct response is to let him run it.

7.8 Laboratory: the negotiation, marked

Aim. To conduct a three-column negotiation with an adolescent and two parents in twenty-five minutes, and to keep the frame intact when it is tested.

Procedure. Quadruples: therapist, adolescent, two parents, one observer. The trainer supplies a one-page brief to each role privately, including one instruction designed to break the frame — the adolescent is told to refuse column one entirely; one parent is told to demand the phone's contents; the other is told to leave the room if the therapist takes the adolescent's side. The therapist must run the negotiation without knowing which brief is which. Twenty-five minutes, then the observer reports which pressure arrived and how it was handled.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The frame	Confidentiality not stated, or stated and then broken	Stated clearly, with exceptions, but not held under pressure	Stated, held, and used — the therapist refuses to report content and says why
The three columns	Two or fewer filled, or filled by the therapist	Three filled, but with unactionable items	Three filled, specific, with breach consequences written by the parties

Criterion	0–1	2–3	4–5
Voice	Therapist speaks more than the family	Therapist speaks about as much	Therapist speaks least; both parties address each other
Handling refusal	Concedes the frame or abandons the session	Holds the frame, but argues	Holds the frame, names the refusal as information, and offers a narrower agreement
The close	Ends with understanding	Ends with a plan	Ends with a plan, a forecast of the breach, and each party's own outcome question

The distinction that carries most of the marks. The top band requires the student to not fill in a column themselves. A therapist who says "so you will tell them where you are" has written the agreement on the family's behalf, and the family will not keep a therapist's agreement.

7.9 Six errors

Error 1 — Becoming the parents' agent. The referral was written by the parents; the therapist reads it; the first session is with the parents. By the time the adolescent arrives she is meeting an employee. The correction is procedural: the adolescent is seen first, or at least given equal time and equal billing, in the first session.

Error 2 — Trying to be the adolescent's friend. The opposite error, and it produces a therapist the adolescent does not respect and the parents do not trust. The position is not friendship; it is advocacy with limits — the therapist is on the adolescent's side in the sense that she will not lie to her, which is a different and more durable thing.

Error 3 — Treating the phone as the problem. The phone is the site. Remove it and the site moves. The formulation is about an information system and a developmental need, and a therapist who negotiates about a device has accepted the family's definition of the problem.

Error 4 — Calling normal development a disorder. Moratorium looks like confusion; experimentation looks like risk; argument looks like opposition; a late bedtime looks like defiance. Each has a developmental reading, and the diagnostic threshold in this age band should be high — which is not the same as absent.

Error 5 — Colluding with cultural foreclosure. "In our culture children do what their parents decide" may be true as a description of a household and false as a description of a fifteen-year-old's development. The therapist's job is not to overturn the household's values but to make room for the question of what the young person actually wants, because a foreclosed identity chosen for someone else is a stable arrangement that fails at twenty-six.

Error 6 — Ignoring the parent who is absent. Tunde's two days in four are the structure of this family and no one in the referral mentioned them. The absent parent's absence is a clinical fact even when — and especially when — it is presented as a virtue.

7.9a A seventh error: the one that ends treatment

There is a seventh error in this age band, and it is not a technique error; it is a relationship error with a predictable end.

Error 7 — Becoming the only adult who understands her. The therapist who says, or implies, "I am the one person who gets you" has made a promise that requires the family to remain incomprehensible. It is flattering to the therapist, it is common in trainees working with adolescents, and it converts the work into a competition in which the adolescent must choose between the therapist and her parents — a choice that will be made against the therapist the moment the therapy ends.

The correction is a discipline, and it has three parts. Never criticise the parents in the adolescent's presence, even when she invites it and even when the criticism is accurate. Always attribute the adolescent's insights to her rather than to your own cleverness ("you worked that out", not "as I said last week"). And always be explicit that the aim is a better relationship at home, not a better relationship with you: "if in six weeks you and your mother can have this conversation without me, we have done the job."

The marker of the error in supervision is a therapist who reports that the parents are "the problem" and that the adolescent is "the only sensible one in the family". The marker of the correction is a therapist who can say what she likes about each member and can name, without contempt and without exception, the interest that each of them is pursuing in the room.

7.10 Culture, faith and the adolescent

Dating, courtship and the family's stake. In many households the adolescent's relationships are not private matters but family matters, because marriage is a family project and a daughter's reputation is a family asset. A therapist who frames the issue as "her right to privacy" will lose the parents in one sentence, and will have done nothing for the adolescent. The workable frame is the one this week uses: safety data belongs to the household, intimacy data belongs to the young person, and the boundary between them is written down. That frame is defensible inside the household's own values, which is why it is usable.

Faith as an identity resource. For many adolescents the first genuinely owned commitment is religious, and the youth group, the choir, or the mosque's youth programme is the setting in which identity work happens. Amara's Wednesday is the ordinary case. A therapist who hears "church group" as a symptom of parental control will misread the most developmentally healthy thing in the referral.

Gender and the double standard. Households frequently hold different rules for sons and daughters, and the adolescent will name this before the parents do. The therapist should neither endorse nor dismiss the double standard; she should make it speakable, because a rule that cannot be discussed becomes a rule that is broken. The question that tends to work: "what is this rule protecting, and is there a version of it that everyone can live with?"

The aunt, the uncle and the second authority. Extended-household caregiving is a resource and a coordination problem. The clinical rule from Week 3 applies unchanged: authority must be defined aloud, in domains, in front of the child. A household with three adults and no defined hierarchy produces an adolescent who plays the adults off against each other, and the adults will describe that as manipulation.

> **DEBATE**
> "A parent has the right to read a fifteen-year-old's messages." Argue for: the parent is legally and morally responsible for a minor's safety, the child has no expectation of privacy in a phone the parent paid for, and the risk of not knowing is severe and irreversible. Argue against: reading messages purchases information at the cost of the relationship that would have produced it, the adolescent will simply move to a second account, and the intervention makes the parent the adversary at exactly the age when the adolescent most needs an adult ally. Then state your actual rule, and say what you would do if a parent told you they had already read the messages and found something alarming.

7.11 Evidence note

What the research supports. The dual-systems account of adolescent risk — a relatively mature reward system and a still-developing control system — is well supported, and the peer-amplification finding has been replicated in both laboratory and naturalistic designs. The identity-status literature is observational but consistent: exploration with subsequent commitment (achieved) is associated with better adjustment than foreclosure or diffusion, across a wide range of cultures, though the content of identity differs by setting.

What is weaker. The claim that parenting style causes adolescent outcomes is confounded by the obvious possibility that adolescents influence parenting. The longitudinal work that separates the directions finds bidirectional effects, with parental warmth and monitoring predicting somewhat better outcomes and adolescent behaviour predicting parental monitoring strongly. The practical implication: a parent who monitors more because the adolescent is in difficulty is doing something reasonable, and the question is whether the monitoring is warm and negotiated or surveillance.

What is not supported. That adolescent-parent conflict is inherently pathological, or that its absence is health. Cross-cultural work finds that moderate conflict over autonomy is normative in settings where adolescents have individualised futures, and that the families with no conflict at all are as likely to be foreclosed as harmonious.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same parental monitoring that protects a thirteen-year-old can damage a seventeen-year-old, and the therapist's judgement is about calibration rather than quantity.

7.12 Key terms

Identity versus role confusion — Erikson's fifth stage; the construction of commitments to work, belief, relationship and place.

Moratorium — active exploration without settled commitment; developmentally healthy and anxiety-provoking.

Foreclosure — commitment without exploration, usually inherited from the family.

Diffusion — neither exploration nor commitment.

Separation-individuation — becoming a separate person while remaining attached; in most of the world this happens inside the household rather than by leaving it.

Pubertal timing — the developmental effect of when maturation occurs relative to the peer group, independent of maturation itself.

Dual-systems model — the account of adolescent risk as the product of an early-maturing reward system and a later-maturing control system.

Monitoring — parental knowledge of the adolescent's world; effective when warm and negotiated.

Surveillance — information obtained without the adolescent's participation; reliably produces concealment.

Negotiated trust — an explicit, written agreement about what is shared, what is asked, and what happens when it breaks.

7.13 Self-test

1. Give Marcia's four identity statuses and one presentation of each that a therapist would meet in a first session.
2. A twelve-year-old girl who looks sixteen is referred for "defiance". What is your first hypothesis, and what one question tests it?
3. State the dual-systems model in two sentences, and name one thing it does not license.
4. Why does the therapist in the transcript promise the father four minutes, and what would have happened without the promise?
5. Distinguish monitoring from surveillance, and give one behavioural example of each.
6. An adolescent says nothing for the first twenty minutes. Name three moves, in order.
7. What are the three columns of the contract, and what are the four things that must never appear in it?
8. In the Adeyemi case, what was the single cheapest intervention, and why did it work?

Answers. (1) Achieved — "I want to do physiotherapy because I worked in a clinic and liked it"; moratorium — "I am trying law, medicine and music"; foreclosed — "I am doing medicine, my father is a doctor"; diffuse — "I do not know, I have not thought about it". (2) That the social response to an early-maturing body is doing the clinical work — she is being treated as older than she is; the question is "do people treat you as older or younger than you are?" (3) The reward system matures ahead of the control system, so sensation-seeking and peer sensitivity are high while inhibition is still developing; it does not license the claim that adolescents cannot reason or decide. (4) To give the objection a specific place, so that the father could tolerate being interrupted without concluding he

had been silenced; without it he would have escalated, and the negotiation would have become a contest the therapist had to win. (5) Monitoring is knowledge acquired with the adolescent's participation and is warm; surveillance is knowledge acquired without it. Example of monitoring: a stated evening hour in which the day is discussed; of surveillance: reading the messages. (6) Name the situation honestly; ask for views rather than feelings; use a third object or a trade with a real cost to the therapist. (7) What she will share unasked; what they will ask and how often; what happens when it breaks. Never: blanket access to the phone, reading a diary, reporting the content of friendships, or a promise to tell them everything. (8) The Wednesday youth group disclosed in the session — it cost the adolescent little, gave the parents a piece of her life voluntarily offered, and reduced their fear more than any explanation could.

7.14 Reflective task

Think of one thing you decided about yourself between thirteen and sixteen that your family did not know at the time. Write it down, without naming anyone. Then answer three questions: what would have happened if they had found out? What did keeping it cost you? And what would an adult have had to do to be told? Most students find that the third answer is very specific and very small — a particular aunt, a particular drive, a particular hour of the evening — which is the point: adolescents do not withhold from adults in general, they withhold from adults who have not earned a particular door.

7.15 Further reading

- Erikson, E. Identity: Youth and Crisis — the source text, still the clearest.
- Marcia, J. "Identity in adolescence" — the four statuses and the interview that produces them.
- Steinberg, L. Adolescence — the dual-systems account, with the evidence.
- Smetana, J. "Adolescents and parents: negotiating autonomy" — the conflict literature, and why moderate conflict is normative.
- Keddie, A. and Dinham, J. on extended-family authority — useful on the second-authority problem in households with resident relatives.

Next: Week 8 — Adolescence II: risk, peers, digital life and mental health.

Week 8 — Adolescence II: risk, peers, digital life and mental health

Risk behaviour is normative in moderation and dangerous in combination: substance use, risky driving, unsafe sex, self-harm, disordered eating. Risk is not a moral category; it is a developmental phenomenon shaped by peers, family, school and access. The purpose of this week is to replace the question "why is he doing this?" with the four questions that actually predict harm: what, how often, with whom, and what does it do for him?

Digital life. Assess function, not just quantity: what is the phone doing in this adolescent's life — connection, escape, status, self-soothing, harm? Ask about sleep, cyberbullying, sexual content, and the family's rules about devices. Banning without

substituting an alternative rarely works, and the substitute is always a relationship rather than an activity.

Mental health. Half of all lifetime mental disorders begin by mid-adolescence. Screen, do not guess. Distinguish transient distress from disorder by **duration, impairment and severity** — and use the same developmental question as every other week: is this a normal task being struggled with, or a disorder?

What makes this week different from the last. Last week was the person. This week is the environment the person is in — the peer group, the phone, the substances that are available at the junction, and the adults who are or are not watching. The clinical skill is to hold both: an adolescent who is self-harming is a person in developmental transition and a person in an environment, and a formulation that has only one of those halves will produce an intervention that fails.

> **PRACTICAL**
> Conduct a twelve-minute adolescent interview with a peer who has been briefed to be reticent. You may not ask more than two questions in a row without reflecting something back. The observer records the ratio of questions to reflections, and whether the adolescent elaborated. The target ratio is one reflection to every two questions.

> **CAUTION**
> This week contains material on self-harm and suicide. The clinical content is included because a family therapist who cannot ask the questions directly will miss the presentations that matter most. The teaching position is that asking does not plant the idea; the evidence on that point is consistent and should be stated explicitly to every class and to every parent who raises it.

8.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	Three questions that replace one	Why "why is he doing this?" produces nothing usable	Rewrite the question for two cases
8–22	Risk as development	The four risk domains, their functions, and the combinations that matter	Plot four vignettes on a function-versus-harm grid
22–34	Peers and access	Peer amplification, and the local availability map	Draw an availability map for their own catchment
34–48	Digital life	Assessing function rather than hours; sleep, bullying, sexual content, and the substitute	Take a digital history from a volunteer
48–62	Mental health	Onset, the duration-impairment-severity rule, and the seven presentations not to miss	Screen two vignettes and justify the referral
62–74	Self-harm and suicide	What to ask, in what words, and what to do with the answer	Rehearse the four questions aloud, in pairs

Minutes	Segment	What the lecturer does	What students do
74–84	The family's response	Escalation, abdication, and the third option	Script a family safety plan
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. The lecture must rehearse the suicide questions aloud, in the room, by every student, because the barrier to asking is entirely in the throat. A therapist who has never said "have you thought about killing yourself?" out loud will substitute a euphemism at the moment it matters, and the euphemism does not get answered. The rehearsal is uncomfortable and it is the most important ten minutes in the module.

The sentence the lecture must land. "Risk behaviour is a solution somebody is using. Your first job is to find out what problem it solves, and your second is to find out whether it is working."

8.2 Five concepts that organise the week

1. Risk is a developmental phenomenon, not a moral one. Sensation-seeking, peer sensitivity, and a steep discounting of delayed outcomes are features of the period, not defects of the person. The clinical consequence is that prevention works better than prohibition: an adolescent who is given a domain in which risk is supervised — sport, performance, competition, a demanding apprenticeship — takes less unsupervised risk, because the developmental appetite has somewhere to go.

2. Function beats frequency. The question is not how much but what for. Cannabis every weekend with a group of friends who play football is a different clinical object from cannabis alone at eleven at night. The five functions that account for most adolescent risk behaviour are: belonging (the group does it); escape (it makes the evening bearable); status (it changes how others see me); self-soothing (it quiets something); and self-punishment (it expresses what I cannot say). Each function points to a different intervention, and the therapist who has not identified the function is guessing.

3. Combination is the danger, not any single behaviour. One risk behaviour in an adolescent with a stable household and an available adult is usually a phase. The same behaviour combined with school withdrawal, sleep loss, a bereavement, and an absent parent is a different prognosis. The assessment should therefore always ask: what else is happening at the same time? The answer is the risk multiplier.

4. Digital life is a setting, not an activity. The phone is where the peer group lives, where status is measured, where sleep is lost, and where a substantial proportion of bullying and sexual content arrives. Assessing "screen time" tells you almost nothing. Assessing what the phone is doing — connection, escape, status, self-soothing, harm — tells you what to do. The single most useful structural question is where the phone sleeps.

5. Most adult mental disorders begin here. The epidemiological finding is robust: half of lifetime disorders have onset by the mid-twenties, with anxiety, mood, substance and impulse-control disorders beginning earliest. This makes adolescence the period of

maximum preventive leverage and the period in which the family therapist is most likely to be the first clinician in the room. The obligation that follows is a screening obligation: a therapist who is working with an adolescent for anything must be able to ask about mood, anxiety, sleep, eating, self-harm and substances, and must know where to refer.

8.2a Seven presentations that must not be missed

The differential in this age band is wide and the presentations are quiet. The seven below are the ones that a family therapist must be able to recognise, name, and refer — not diagnose.

1. Depression. Not "sadness". The adolescent pattern is more often irritability than sadness, combined with withdrawal from things that used to matter, sleep change, appetite change, a fall in school performance, and a flatness that the family describes as "laziness". Duration of two weeks or more, with impairment, is the threshold that warrants a referral. One question screens surprisingly well: "when did you last enjoy something?"

2. Anxiety, including panic and social anxiety. Physical symptoms — racing heart, breathlessness, nausea — presenting as school refusal or as repeated visits to a clinic for a physical complaint. The distinguishing feature is anticipatory fear that leads to avoidance, and the avoidance is what maintains it.

3. Psychosis prodrome. Social withdrawal, a drop in functioning, odd ideas that are treated as eccentricity, suspiciousness, a decline in self-care, and sometimes a preoccupation with a new interest that has become the whole of the person's life. This is rare and it is the presentation where delay costs most. Any adolescent with a marked, unexplained decline in functioning plus unusual beliefs should be referred for assessment, promptly, and the referral should not be softened into "stress".

4. Substance use. Ask about what, how often, with whom, and what happens when it stops. The developmental danger signs are: use alone, use before midday, use to manage sleep or mood, and withdrawal from other activities. Do not accept "only at parties" without asking what happens at parties.

5. Disordered eating. Weight change is the least reliable sign. More useful: eating in secret, food rituals, a sudden interest in "healthy" eating that narrows to a list, exercising alone and compulsively, and a change in the family's mealtimes. The family's report of "she has become fussy" is worth three questions.

6. Self-harm. Cutting, burning, hitting, and — increasingly — the deliberate consumption of harmful content. Self-harm is not a failed suicide attempt and it is not attention-seeking; it is most often a regulation strategy, and its presence is one of the strongest single predictors of later suicide risk. It requires direct, unshocked questioning and a proper assessment.

7. The ordinary distress of a normal task. Most referrals are this. A fifteen-year-old who is miserable for three weeks after a breakup, who sleeps badly before exams, who is angry about a move, is doing developmental work. The distinction is duration, impairment, and severity — and the error is not to dismiss ordinary distress, which is real, but to treat

it as a disorder, which pathologises a competent person at the moment she is learning to cope.

The rule that governs all seven. A family therapist screens and refers; she does not diagnose in the therapy room, and she does not manage a psychosis or a developing eating disorder in a family session. The screening obligation is real and the scope limit is real, and holding both is the professional position this week is teaching.

8.2b Self-harm and suicide: what to ask and what to do with it

This section is written to be used. It is deliberately concrete, because the failure mode in this age band is not a wrong intervention; it is a question that was never asked.

The four questions, in this order.

1. "Some young people, when things are hard, hurt themselves on purpose. Have you ever done that?"
2. "Have you ever thought that you would be better off dead, or wished you could go to sleep and not wake up?"
3. "Have you thought about how you would do it?" — the presence of a method, and its availability, is the single strongest predictor of risk.
4. "What has kept you going so far?" — the protective factors, and the beginning of the safety plan.

Why the wording matters. "Are you having any thoughts of self-harm?" is a professional's sentence and gets a professional's answer, which is "no". "Have you ever hurt yourself on purpose?" is a person's sentence and gets an answer. The questions must be asked plainly, without a preamble that signals alarm, and without the therapist's face changing, because the adolescent is watching the face for the verdict on whether she has just made a terrible mistake.

The myth, stated plainly. Asking about suicide does not increase risk. The evidence on this is consistent across settings and the belief that it does is the main reason clinicians do not ask. Say this to parents, in these words, because they will be frightened by the questions and will otherwise interpret them as an intervention that caused the problem.

What to do with a positive answer. In order: do not leave the adolescent alone if risk is current; involve the parents in the same session, with the adolescent's knowledge and, where possible, her agreement about how it is said; remove or restrict means — medication, blades, pesticides, firearms, and anything else available in the household; arrange a same-week or same-day referral according to local protocol; document exactly what was asked, what was answered, and what was done. Then arrange the next contact before the family leaves the building.

The safety plan, which is not a contract. A "no-harm contract" has no evidence behind it and can shift responsibility onto the adolescent. A safety plan is different: it is a written list of (a) the warning signs, (b) the things the adolescent will do first, (c) the people she will contact, in order, and (d) the means that have been removed from her reach. It is

written with her, in her words, and the parents have a copy that says what they will do, not what she has promised.

The family's part. Two things a family can do that matter more than any conversation: reduce access to means, and increase supervision without surveillance. The distinction is the same as Week 7's: a parent who sits in the room is supervision; a parent who reads the diary is surveillance. The second produces concealment, which is the one outcome a safety plan cannot survive.

8.3 The method: a risk assessment a family therapist can actually do

Ten steps, thirty to forty minutes, doable in a family session with the adolescent seen alone for the middle portion. The purpose is not diagnosis. It is to answer three questions: how serious, what else, and what happens next.

Step 1 — Establish the frame first. "I am going to ask you some direct questions. Some of them will feel strange. I ask everybody. If I think you are in danger I will have to act on it, and I will tell you before I do." The last clause is what makes the questions answerable.

Step 2 — Ask about the presenting behaviour in behavioural terms. Not "why did you cut yourself?" but "where, when, how many times, what were you feeling just before, and what happened afterwards?" The afterwards question identifies the function with surprising reliability: relief points to regulation; numbness points to dissociation; punishment points to self-directed anger.

Step 3 — Ask the four questions from 8.2b. In order, plainly, without a preamble.

Step 4 — Map the combination. Sleep, appetite, school attendance, withdrawal, substances, eating, mood, anxiety. The question "what else is going on?" is asked after the disclosure, never before, because before it signals that the therapist has already decided what the problem is.

Step 5 — Ask about the environment, not just the individual. Who knows? Who would she tell? Who in the household is available at eleven at night? What is in the house that could be used? Where does the phone sleep? Is there anyone at school?

Step 6 — Establish the protective factors. What has kept her going; what she is looking forward to, however small; who she would not want to hurt; what she is good at. These are not consolation prizes. They are the material of the safety plan and the best available predictors of who comes back.

Step 7 — Bring the parents in, with the adolescent's knowledge. How it is said is negotiated first: "your mother needs to know about the cutting. Do you want to tell her, or shall I?" The choice matters more than the content, because it preserves the adolescent's standing in her own disclosure.

Step 8 — Restrict means, concretely. Ask what is in the house: medications, blades, rat poison, pesticides, a car, a rope in the yard. Then assign the task of removal to a named adult, with a date. A safety plan that does not change the household's physical environment is a conversation.

Step 9 — Decide the level of care and say it out loud. Outpatient with weekly contact; urgent referral; emergency. Say which, and say why, in front of the family, so that the decision is not a mystery. Families who are told the reasoning cooperate with the plan; families who are handed a plan comply with it until the first difficulty.

Step 10 — Write it down and book the next contact. A one-page safety plan in the adolescent's words, a copy to the parents, a date in the diary before anyone leaves the room. Then document all of it.

8.3a Digital life: the history that is worth taking

Five questions, ten minutes, and they replace the argument about hours.

Question 1 — Where does it sleep? The answer predicts sleep loss more reliably than any measure of use. An adolescent whose phone is in the bed has a device that interrupts sleep, and sleep loss is causally implicated in mood deterioration, which makes this the highest-yield structural question in the week.

Question 2 — What is it for? Connection (the group), escape (the scroll), status (the post), soothing (the same video forty times), or harm (the group that excludes, the person who threatens). Ask for a description of the last hour before sleep, which is when the function is most visible.

Question 3 — Who is in the group, and who is not? Exclusion in this age band is frequently technical: not added, muted, removed. Ask whether she is in the class group and whether she wants to be. The answer is often the whole of the presenting problem.

Question 4 — What has arrived that you did not want? This is the question that produces disclosures of bullying, sexual content, and strangers, and it is asked without the parents in the room. Then: "did you tell anyone? what did they do?" The second question audits the adult system, not the adolescent.

Question 5 — What would you do if you lost it for a week? The answer describes the phone's function better than any inventory. An adolescent who is horrified is describing a social lifeline. An adolescent who is indifferent is describing an escape that has stopped working.

What to do, in order of evidence. Protect sleep (device out of the bedroom, for everyone); protect the one relationship that matters (a named adult who is available and who does not confiscate); build one non-screen domain of competence (this is the "substitute" the stub refers to, and it is always an activity with other people in it); and then, only then, negotiate limits. Limits imposed before the substitute exists produce concealment.

8.4 Three techniques

Technique 1 — The function grid. With the adolescent, place each risk behaviour on a two-axis grid: what it does for me (the five functions) and what it costs me (health, money, school, relationships, freedom). Fill it in her language. The grid does more than a lecture because it is her data, and because it makes visible the behaviours that are cheap and the ones that are expensive — which is not always where the family's anxiety is concentrated.

Technique 2 — The availability map. Draw the household and the route to school, and mark where each substance or means is available: the junction, the flat upstairs, the older cousin, the chemist that sells without a prescription, the drawer in the kitchen. The map converts a moral question into a logistical one, and logistics can be changed. It also routinely reveals that the family has more control over access than it believed.

Technique 3 — The one adult at eleven at night. Ask the adolescent to name the person she would contact at eleven at night if something had happened. If the answer is "nobody", that is the intervention: find, in the household or the extended family, one adult who will agree to be reachable at that hour without asking questions first. This is the cheapest suicide-prevention intervention available to a family therapist and it is almost never written down.

8.5 The first session with a self-harming adolescent and two parents

Phase 1 — The parents' two minutes each (5 minutes). What they found, when, and what they did. The therapist listens for the sequence: discovery, reaction, and what has happened since. The reaction is usually a confiscation, a shouting, or a silence, and each of the three has a predictable consequence.

Phase 2 — The adolescent alone (25 minutes). The frame, then the ten-step assessment, compressed. The four questions are asked in this phase and never in front of the parents, because the parents' faces make the answers impossible.

Phase 3 — Negotiate the disclosure (5 minutes). What will be told, in what words, and by whom. The adolescent is given the first option to tell it herself.

Phase 4 — The joint session (20 minutes). The disclosure; the parents' reaction, which the therapist manages by slowing it down rather than by correcting it; then the means restriction, assigned by name; then the safety plan, written.

Phase 5 — The plan and the next appointment (5 minutes). Level of care, next contact, who telephones whom, and what the parents do tonight. Nothing is left as "we will see".

Why the parents are not in the room for phase 2. Not to protect the adolescent from them, but to protect the information. A sixteen-year-old will not describe the function of her cutting in front of a mother who is crying, and the function is the treatment target. The parents are brought in as soon as the information exists, and they are brought in by design rather than by accident.

8.5a The crisis hour: when the risk is immediate

Most of this week concerns assessment. This section concerns the one hour in which assessment is not the task, because the adolescent in front of you is at immediate risk and the decision cannot wait for a referral.

The four indicators of immediacy. A stated intent, rather than an idea. A plan with a time. Access to the means, in the room or in the house, tonight. And the absence of any named reason to stay alive in the next twenty-four hours. Two of the four is urgent. Three is an emergency, and the therapist's job changes from treating to transporting.

The first move is physical, not conversational. Do not leave the adolescent alone. If she is in the clinic, stay in the room and ask a colleague to attend. If she is at home on the telephone, ask her to go and sit with a named person in the house, now, while you stay on the line. The single most useful thing in the first five minutes is another human being in the same room as her.

The second move is to remove the means, by name. "Where is it? Is it in the house? Who can take it now?" Ask the adolescent first, then the parent. A blade, a bottle of pesticide, a stock of tablets, a car key. The instruction is specific: not "keep her safe" but "take the bottle to the neighbour's house tonight and tell me when it is done". Then record what was said and who confirmed it.

The third move is to tell the parents, with the adolescent's knowledge, in her hearing. "I am going to tell your mother that you have a plan and a time, because she is the person who can be with you tonight. I am not telling her because you did something wrong." This is the one situation in which the therapist does not offer a choice about disclosure, and saying so plainly is more respectful than pretending the choice exists.

The fourth move is the handover, and it has a name in it. Not "go to the hospital". A named facility, a named person if possible, a written note, and confirmation that the family has arrived. A therapist who sends a family to "casualty" has sent them to a building, and a family that arrives at a building at midnight without a name and without a person to ask for will go home again, and will wait until Monday, and the hour will have been lost in the interval.

The fifth move is the follow-up call. Within twenty-four hours, to the family and, with permission, to the adolescent. The call is not clinical; it is the demonstration that the crisis did not end the relationship. Adolescents who are admitted after an attempt and who receive no contact from the referring clinician are markedly less likely to return afterwards.

What not to do. Do not extract a promise. Do not ask the family to "watch her closely" without saying what that means in hours and in people. Do not schedule the next appointment for a fortnight. Do not tell the adolescent that she is being dramatic, and do not tell her that everything will be fine — both are statements she has heard before from adults who then did nothing.

Afterwards, the therapist's own state. A crisis hour is frightening, and the two common reactions are over-reaction (escalating every subsequent disclosure to the emergency pathway, which teaches the adolescent to stop disclosing) and under-reaction (recording the risk as lower than it was, in order to feel that the hour was manageable). Both are corrected in supervision, and the requirement to take a risk assessment to supervision within the week is not bureaucratic. It is the one reliable protection available to a lone clinician against the second error, which is the more dangerous of the two and the harder one to see from inside a hard week.

8.6 Case: the boy whose teacher saw his arm

Referral. Sadiq, sixteen, in Senior Secondary 2, was sent to the school counsellor after a teacher noticed scars on his left forearm during a practical. The counsellor telephoned his mother the same day and referred him to the clinic. The mother, Rukayat, 39, a textile trader, arrives with him. She has not told his father.

The household. Rukayat; Sadiq, the eldest of five children (16, 14, 11, 7, 3); Rukayat's mother, 66; and a younger brother of Rukayat's who helps in the shop. Sadiq's father, 52, lives in Kano with a second wife and telephones on Fridays. He pays school fees and has not visited in fourteen months. Rukayat's income is the household's income.

The discovery. Eight scars on the left forearm, five of them recent, all consistent with a blade. Two on the upper thigh, which the mother did not know about and which the therapist found by asking.

The parents' account, in the first five minutes. "He is quiet at home. He does not tell me anything. He used to be first in his class and now he is eleventh. I thought it was the phone." Then, after a pause: "His father will say I have failed."

The adolescent alone. The four questions are asked in order and answered in order.

1. Have you ever hurt yourself on purpose? "Yes." When did it start? "Last year. After my father stopped coming."
2. Have you ever thought you would be better off dead? A long pause. "Sometimes. Not like a plan. I think about not being here."
3. Have you thought about how you would do it? "I have thought about the gutter on the third floor of the school. But I would not. My mother would die." — A method has been considered; a protective factor is named in the same breath. Both are recorded, and both change the plan.
4. What has kept you going? "My sisters. And my mother. And football."

The function. Asked what happens just before: "it is after the phone call on Friday. He talks to my mother and then she is crying and she says nothing to us and I am the man of the house. I go to the bathroom." Asked what happens after: "it is quiet. For maybe ten minutes it is quiet."

The assessment. The function is regulation, with a secondary self-punishment component: the cutting follows a weekly event, it produces a brief quiet, and it is accompanied by a conviction that he should be managing better. The combination map shows: sleep onset after one in the morning, appetite reduced, school performance fallen from first to eleventh, withdrawal from football in the last two months, no substance use, no disordered eating. Duration: fourteen months, with the first scars dating to the month the father's visits stopped.

The risk formulation, written as it would be in a record. Current risk: moderate. Ideation present, intermittent, without intent or plan, with a named protective factor and a named method that has been considered and rejected. Self-harm: current, increasing in frequency over three months. Means: a razor blade kept in the bathroom, and medication

in the kitchen. Protective: mother's availability, four younger siblings, football, a teacher who noticed. Aggravating: paternal absence, financial pressure, the role of "man of the house" assigned to a sixteen-year-old, sleep loss.

The formulation. A sixteen-year-old carrying an adult role he was given and not prepared for, in a household where the father's Friday telephone call is the week's emotional weather, and where nobody has asked him how he is because he is the one who is supposed to be all right. The cutting is a private regulation strategy for a public situation. It is not attention-seeking: nobody was meant to see it, and the teacher seeing it is the crisis of the case rather than its solution.

The intervention, in five parts.

Part 1 — Means restriction, tonight. The blade is removed by the mother in Sadiq's presence, with the therapist having said why. The kitchen medication moves to the mother's room. The head teacher is contacted with Sadiq's knowledge and asked to make the third-floor landing supervised at break. This is unglamorous and it is the part of the plan with the best evidence behind it.

Part 2 — The safety plan, in his words. Warning signs: "Friday night. When she is quiet after the phone." First three things: "call Tunde" (a teammate), "go and kick the ball outside even if it is dark", "play the guitar for ten minutes". People in order: mother, Tunde, the counsellor, the clinic number. Removed means: blade, kitchen medication. Written on one page, photographed by him and by his mother, and put by the clinic in the record.

Part 3 — Move the Friday. The single structural change of the case. The therapist asks Rukayat, with Sadiq present, what happens after the Friday call. She describes it: she cries in the bedroom, then cooks, then says nothing. The agreement: after the call she will sit with Sadiq and the two eldest for ten minutes and tell them what was said, in one sentence, without asking them to look after her. The content does not change. The secrecy does. A sixteen-year-old who knows what was said no longer has to infer it from his mother's face, and the inferring is what was producing the bathroom.

Part 4 — Give the role back. "Man of the house" is a role without a job description, and Sadiq has been doing it badly because it cannot be done. The therapist asks Rukayat to name, in front of him, three things that are his (school, football, his sisters' homework) and two that are not (money, her marriage, whether his father visits). The list is written. The purpose is to make the boundary visible, because an adolescent cannot refuse a role that nobody has named.

Part 5 — One hour a week with an adult who asks nothing. The counsellor at school, or the football coach, or an uncle. The instruction to the adult is the same as the week's technique: no questions about home, no advice, one hour, once a week, doing something. Sadiq chooses football with the coach on Wednesdays.

The referral. Sadiq is referred for a diagnostic assessment, which returns a picture of moderate depressive symptoms without a formal disorder at that point, and for six sessions of individual work alongside the family sessions. The therapist does not do the

individual work herself; she keeps the family work, because a sixteen-year-old who is self-harming needs two clinicians less than he needs two distinct relationships that are not the same relationship.

8.6a Eight weeks later

The cutting has stopped for six weeks, which is a fact and not a cure. It has been replaced, in Sadiq's own description, by "the guitar and calling Tunde", and he says this with the flatness of someone reporting a task rather than a change.

The Friday conversation happens. Rukayat reports that it is "short and not nice", which is the realistic outcome: the content of the marriage has not changed and is not going to change in this therapy. What has changed is that Sadiq no longer receives it as atmosphere.

The grades have not recovered. The therapist records this as the case's honest limitation: a sixteen-year-old who has lost fourteen months of study does not recover them in eight weeks, and the school has agreed to a reduced subject load for one term.

The most significant change is not in the record. Rukayat has begun telling her brother, who helps in the shop, one true thing a week about the household. The therapist did not plan this. It is the ordinary consequence of a family in which information has started to move, and it is the best available sign that the change will survive the therapy.

What would have changed the plan. If the ideation had included intent, a plan, or a recent preparation, the case would have gone to emergency assessment the same day and the therapy room would have become a holding place rather than a treatment. If the mother had refused to remove the blade, the therapist would have documented the refusal and escalated within the safeguarding framework, because means restriction is not negotiable. If the father had been reachable and willing, one session with him on the telephone would have been attempted, with Sadiq's agreement — and if Sadiq had refused, the refusal would have been respected and recorded.

8.7 Transcript: asking the four questions

Twelve turns, compressed from twenty-five minutes. The transcript's purpose is to show the tone: flat, direct, unhurried, and without a single reassuring phrase in the wrong place.

> **T1 Therapist:** I am going to ask you some direct questions. Some of them will feel strange. I ask everybody I see, and I am not going to be shocked by the answers. If I think you are in danger I will have to do something about it, and I will tell you before I do. All right? — The frame, in four sentences, before anything else. **T2 Sadiq:** Okay. **T3 Therapist:** Some young people, when things are hard, hurt themselves on purpose. Have you ever done that? — Question 1, with no preamble and no euphemism. **T4 Sadiq:** (pause) Yes. **T5 Therapist:** Where? **T6 Sadiq:** My arm. The inside. **T7 Therapist:** How many times? **T8 Sadiq:** I don't know. Many. **T9 Therapist:** (no change of expression, pen down) Have you ever thought that you would be better off dead, or wished you could go to sleep and not wake up? — Question 2, asked in the established tone. The pen going down is deliberate: writing while a person answers this question makes them a specimen. **T10 Sadiq:** (long pause) Sometimes. **T11 Therapist:** Have you thought about how you would do it? **T12 Sadiq:** (longer pause) The gutter. At school. The third floor. — The answer that changes the plan: a method, a place, and a school. Everything after this turn is logistics, and the therapist's next sentence is not a question about feelings.

8.8 Laboratory: the risk assessment, marked

Aim. To complete the ten-step assessment with a reticent adolescent in thirty-five minutes, obtain the disclosure, and leave with a signed safety plan and a documented level of care.

Procedure. Trios: therapist, adolescent (briefed with a disclosure that includes a method, a protective factor, and one detail she will withhold unless asked twice), observer. Thirty-five minutes, then ten minutes of feedback against the rubric. The withholding is the exercise's centre: the briefed adolescent will not volunteer the second location of the scars, and a therapist who does not ask about the body directly will miss it.

Marking rubric (25 marks).

Criterion	0–1	2–3	4–5
The frame	Not stated; or stated as a warning	Stated, but after the first question	Stated first, including the limits, and the adolescent's assent obtained
The four questions	Not asked, or asked with a euphemism	Asked, but in a preamble-laden way	Asked plainly, in order, without a change of tone or expression
Function	Not established	Asked what happened before	Asked before and after, and the function named in the adolescent's words
Combination	Only the presenting behaviour	Two or three domains covered	All seven domains screened, including sleep and eating
Means	Not discussed	Discussed generally	Specific inventory, named adult, date, and confirmation in the room
The plan	Advice	Written plan without protective factors	Written in the adolescent's words, with warning signs, sequence, means removed, and a booked next contact

The distinction that carries most of the marks. The top band requires the student to ask question 3 (the method) after a positive answer to question 2, without softening. Most trainees stop at "sometimes I think about not being here", because the disclosure is distressing and because stopping feels kind. It is the moment at which the assessment is either completed or abandoned.

8.9 Six errors

Error 1 — Not asking. The largest error by volume, and the one with the clearest cause: the fear of causing harm. The evidence is unambiguous that asking does not increase risk. A therapist who works with adolescents and does not ask is not protecting them; she is protecting herself.

Error 2 — Asking in a way that can only be answered "no". "You are not thinking of doing anything silly, are you?" is a question with the answer built into its grammar. Every one of the four questions is closed, direct, and free of the answer.

Error 3 — Promising confidentiality you cannot keep. An adolescent who discloses self-harm after being told "everything stays in this room" has been misled at the moment of maximum vulnerability, and the breach of that promise is often the end of treatment. State the limits first, every time.

Error 4 — Making the family the enemy. Parents who react with confiscation, shouting, or silence are frightened, not cruel. A therapist who treats them as the problem loses the household, and the household is the safety plan.

Error 5 — The no-harm contract. A signed promise not to self-harm has no evidence behind it and shifts responsibility onto the adolescent at the moment she has least capacity to carry it. Replace it with a safety plan that describes what she will do, who she will call, and what has been removed from her reach.

Error 6 — Treating the disclosure as the endpoint. The disclosure is the beginning of the assessment. The commonest failure in supervision is a therapist who reports, with relief, "he told me everything", and who cannot say what the level of care was, what was removed from the house, or when the next contact is.

8.10 Culture, faith and adolescent risk

The household's reputation. In many settings an adolescent's self-harm or substance use is experienced by the family as a judgement on the household, and the first clinical obstacle is not the behaviour but the family's need for it not to be known. A therapist who telephones the school, calls a meeting, or writes to a relative without an explicit agreement will lose the family. The sequence is always: agree who knows, then act.

Faith, deliverance and the clinic. Families may hold that the behaviour is spiritual, and may pursue prayer, fasting or deliverance alongside or instead of clinical care. The workable position is neither collusion nor confrontation: accept the household's framework as the language of concern, ask what the prayer or the fasting is for, and negotiate one non-negotiable — that the safety plan is in place while the spiritual work

continues. A family that is told to choose between its faith and the clinic will choose its faith, and the adolescent will be the one who loses.

Gender and the boy who must not be weak. Sadiq's case is a gender case before it is anything else. A sixteen-year-old boy in a household where he is "the man of the house" has been given an emotional prohibition along with the role. The clinical move is not to tell him to express his feelings; it is to give him a permitted domain — football, an uncle, a coach — in which being looked after is not a humiliation.

The second wife, the absent father, and the therapist's judgement. Polygynous arrangements, labour migration, and long paternal absences are ordinary structures in many of the households this book is written for. A therapist whose formulation requires the family to become a different kind of family will produce a plan that nobody follows. The question is always what can be changed inside the arrangement that exists — the Friday conversation, the role list, the uncle's hour — rather than what would have to change for the problem to disappear.

> **DEBATE**
> "A sixteen-year-old's self-harm should be disclosed to the parents whether or not she agrees." Argue for: the parents are the safety plan, the risk of concealment is death, and the adolescent's judgement is the thing that is compromised. Argue against: disclosure against her will ends the therapeutic relationship at the moment it is most needed, the adolescent will conceal more effectively next time, and a competent sixteen-year-old has a legitimate interest in how her own body is discussed. Then write the rule you would follow, including the case in which she refuses and the risk is moderate rather than imminent.

8.11 Evidence note

What the research supports. Means restriction is the intervention with the strongest evidence in suicide prevention at population level: reducing access to a common method reduces deaths, including in adolescents, and the effect is not merely substitution of method. Safety planning, as a structured and collaborative written intervention, has better support than no-harm contracts, which have essentially none. Sleep disturbance is a prospective predictor of adolescent mood deterioration, which is why the question about where the phone sleeps is a clinical question and not a parenting preference.

What is weaker. The evidence for digital-detox interventions is thin and the field is moving faster than the trials. The most defensible claims are structural (sleep protection, one non-screen domain of competence, one available adult) rather than quantitative (a number of hours). The evidence on family-based treatment for adolescent self-harm is promising but based on small trials with heterogeneous comparators.

What is not supported. That asking about suicide increases risk — the evidence is consistent that it does not, in clinical and school settings. That self-harm is attention-seeking — the clinical literature describes a predominantly private behaviour whose discovery is usually experienced as a crisis. And that confiscation of devices

reduces harm — the observable effect is migration to unmonitored devices, which reduces the parents' knowledge without reducing the exposure.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same paternal absence produces, in one sixteen-year-old, a determination to succeed and, in another, fourteen months of silent cutting. What differs is not the input but the presence of one adult who asks, and that is what the therapist is trying to install.

8.12 Key terms

Function — what a risk behaviour does for the person; the first question of the assessment.

Combination risk — the multiplicative effect of several concurrent adversities, as against the effect of any one.

Means restriction — reducing access to the method; the best-evidenced single intervention in suicide prevention.

Safety plan — a written, collaborative list of warning signs, coping steps, contacts and removed means.

No-harm contract — a signed promise not to self-harm; not evidence-based and not a substitute for a safety plan.

Prodrome — the period of subtle change preceding the frank onset of a disorder, particularly psychosis.

Duration, impairment, severity — the three dimensions that distinguish disorder from distress.

Digital function — what a device is doing in a person's life: connection, escape, status, soothing, or harm.

Protective factor — a circumstance that reduces risk; in this age band usually a named adult and a domain of competence.

Level of care — the decided setting and intensity of treatment: outpatient, urgent, or emergency.

8.13 Self-test

1. State the four questions of the risk assessment, in order, and say why question 3 follows question 2.
2. A parent asks whether asking about suicide will put the idea in her daughter's head. What do you say, and what is the evidence?
3. Name the five functions of adolescent risk behaviour and give one intervention matched to each.
4. Why is means restriction the best-evidenced intervention in this field, and what does it require in practice?

5. Distinguish a safety plan from a no-harm contract, and say why the difference matters clinically.
6. Name the three dimensions that separate disorder from distress, and apply them to a fifteen-year-old who has been miserable for three weeks after a breakup.
7. In the case of Sadiq, what was the single structural change, and what did it change?
8. What is the "one adult at eleven at night", and why is it described as the cheapest intervention available?

Answers. (1) Self-harm; suicidal ideation; method and intent; protective factors. Question 3 follows because a positive answer to question 2 identifies ideation while question 3 identifies risk, and the method is the strongest single predictor. (2) That it does not — the evidence across clinical and school settings is consistent, and the belief that it does is the main reason clinicians fail to ask; say it plainly, because the parent's fear will otherwise be interpreted as a reason for silence. (3) Belonging — change the group or build an alternative group; escape — address what is being escaped, usually the household's evening; status — build a domain of real competence; self-soothing — teach a regulation skill and protect sleep; self-punishment — address the self-directed anger, usually with individual work. (4) Because it does not require the person in crisis to make a decision at the moment of crisis; in practice it requires a specific inventory of the household, a named adult, and a date. (5) A safety plan is collaborative and describes what the adolescent will do and what has been removed; a no-harm contract is a promise that shifts responsibility onto her. The first builds capacity, the second borrows it. (6) Duration, impairment, severity — three weeks after a breakup with no impairment in school or friendships and moderate severity is ordinary distress and should be respected as such rather than treated. (7) The Friday conversation: naming what was said, to the two eldest, in one sentence, without asking them to manage her feelings — it converted an atmosphere that had to be inferred into information that could be held. (8) A named adult reachable at that hour without interrogation first; cheapest because it requires no clinical resource, and effective because the hours after ten at night are when risk is highest and when the adolescent is least able to reach a professional.

8.14 Reflective task

Write down the last time you asked someone a direct question about their safety, or the last time you avoided one. If you avoided it, reconstruct the sentence you used instead, word for word. Then answer: what were you protecting? Most trainees find that the substitute sentence protected the therapist's comfort, and that the discomfort was located in a single word which, once said aloud in a rehearsal, is never as difficult again.

8.15 Further reading

- Hawton, K., Saunders, K. and O'Connor, R. "Self-harm and suicide in adolescents" — the epidemiology and the evidence on means restriction.
- Stanley, B. and Brown, G. "Safety planning intervention" — the structured alternative to no-harm contracts.

- Steinberg, L. "Risk taking in adolescence" — the peer-amplification evidence behind this week's first concept.
- Nock, M. "Why do adolescents hurt themselves?" — the functional account used in the case.
- Odgers, C. and Jensen, M. "Annual research review: adolescent mental health in the digital age" — the most careful summary of a thin evidence base.

Next: Week 9 — Adult development: work, intimacy and the middle years.

Week 9 — Adult development: work, intimacy and the middle years

Developmental tasks. Intimacy versus isolation (Erikson, roughly the twenties and thirties); work and career identity, including the difference between a job, a career and a calling; generativity versus stagnation (the forties); and the "social clock" — the culturally shared timetable for marriage, children, promotion and retirement, and the distress that follows when a life runs off it.

Why this matters clinically. Most couple problems presented in clinics are adult developmental problems wearing a relational mask: a thirty-two-year-old's struggle with intimacy, a forty-five-year-old's reckoning with a career, a fifty-eight-year-old's grief for the life they did not have. The therapist who hears only the relationship will treat the mask. The therapist who hears the developmental task will treat the person, and the relationship will follow.

Life structure and the "seasons" of adulthood. Levinson's seasons, Vaillant's longitudinal cohorts, and the midlife literature are best treated as metaphors with empirical support, not as laws. The strongest finding in the whole of adult developmental research is **continuity with change**: people change, but in patterned ways that reflect early relationships and current context. The man who was defended at twenty-five is defended at fifty, in a different vocabulary, about different things.

What this week does not claim. It does not claim that there is a crisis everyone has at forty, that marriage has stages everyone passes through in order, or that a person who has not achieved generativity by fifty is damaged. The evidence supports tasks, timetables, and patterns. It does not support prescription, and the therapist who turns a developmental map into a standard of normal adulthood will spend the week hurting families who are simply living on a different schedule.

> **CASE**
> A forty-five-year-old man presents with "irritability". Developmental enquiry finds that he has been passed over for a promotion he had organised his entire adult life around, that his father died eighteen months ago, that his eldest child has left for university, and that he has begun to notice that he has no friends he did not acquire through work. The irritability is at the surface. Under it are four adult developmental events arriving in the same eighteen months.

9.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The mask	Why adult developmental problems arrive as relationship complaints	Take a couple complaint and name the developmental task inside it
8–22	Intimacy versus isolation	What intimacy actually requires, and the four adult forms of not doing it	Score themselves on a five-item intimacy audit, privately
22–36	Work identity	Job, career, calling; and the difference between a bad month and a bad decade	Classify six work presentations
36–48	Generativity	The three routes — children, work, mentoring — and stagnation as its absence	Name three people who were generative towards them
48–62	The social clock	On-time and off-time events, and the distress of the off-time life	Place four life events on the clock and mark the culturally expected age
62–74	The midlife reckoning	How it presents in a clinic, and the two ways therapists pathologise it	Rewrite two diagnostic-sounding formulations as developmental ones
74–84	Continuity with change	What longitudinal studies actually find, and what they do not	Predict a fifty-year-old from a twenty-five-year-old in a case vignette
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Adults present with a symptom and a story, and the story is usually chronological but not developmental: it lists events and omits tasks. The lecture's central demonstration is to take one ordinary adult complaint and place it against the developmental timetable, so that the class sees the difference between what happened and what it cost developmentally. Once students can do that, they stop treating forty-five-year-olds as thirty-year-olds with more money.

The sentence the lecture must land. "By forty-five most people are not struggling with their marriage; they are struggling with a question their marriage has been asked to answer, and the marriage is not equipped to answer it."

9.2 Five concepts that organise the week

1. Intimacy versus isolation. Intimacy is not affection and it is not sex. It is the capacity to be known by another person without managing the version of oneself that is known — which requires, first, that there is a self to know. This is why Erikson placed identity before intimacy, and why the sequence matters clinically: a couple in which one partner has never settled who they are will present with "communication problems" that no communication exercise will touch.

The four adult forms of not doing intimacy are worth memorising, because they present as different referrals.

- **Fusion** — the person cannot tolerate separateness; difference is experienced as abandonment, and the couple's argument is always about closeness.

- **Pseudo-intimacy** — the person performs closeness; the couple is highly affectionate in public and has no private conversation.
- **Isolation** — the person keeps a whole domain of life outside the relationship; the complaint is "he does not let me in".
- **Serial replacement** — the person changes partners rather than being known by one; the pattern is visible across relationships and invisible within any one of them.

2. Work identity: job, career, calling. A job is what you do for money. A career is a trajectory with advancement. A calling is what you would do anyway. The three are not ranked; a person with a job and a rich life outside it is not deficient. The clinical significance is in the mismatch: a person who has organised their identity around a career and loses the trajectory has lost more than income, and a person who experiences their work as a calling and finds it meaningless is in a particular kind of distress that will not respond to work-life balance advice.

3. Generativity versus stagnation. Generativity is the concern for establishing and guiding the next generation, and it has three routes: one's own children, one's work, and mentoring. The clinical value of the concept is that it provides an alternative to the two things adults in distress usually believe are their only options — more achievement, or resignation. A forty-eight-year-old man who is stagnating at work may be generative as a mentor, as a son, as an uncle, as a coach, and the therapeutic move is to find where the generative capacity can go rather than to fix the job.

4. The social clock. Every culture has a timetable: the age at which one is expected to marry, to have a first child, to be established, to be a grandparent, to retire. The clock is not usually stated, which is what makes it powerful. Distress follows off-time events far more than expected ones — a first marriage at forty-five, a first child at nineteen, a promotion at twenty-nine, a retirement at fifty-five, a bereavement at thirty. The clinical implication is that the therapist must ask not only "what happened?" but "when, and where were you supposed to be by then?"

5. Continuity with change. Longitudinal evidence supports the view that adult personality is stable in its structure and changing in its expression: the defended young adult becomes the defended middle-aged adult, with better manners and worse consequences. The clinical use is a caution against two errors: the error of expecting a person to become someone else in therapy ("he will never be warm"), and the error of assuming nothing can change ("this is just who he is at forty-five"). What changes is usually the relationship to the pattern, and the practical arrangements around it.

9.2a The social clock and the off-time life

The concept is simple and the clinical consequences are large. Five off-time situations account for most of the developmental distress seen in adult clinics.

Off-time marriage and the unmarried thirty-five-year-old. In cultures with a strong marital timetable, an unmarried adult in their mid-thirties is subject to continuous enquiry, and the enquiry is experienced as an assessment of worth. The clinical presentations are anxiety, depression, or a relationship entered under pressure and

presented as a decision. The therapist's task is to separate the person's own desire from the timetable's, which is done with one question: "if nobody ever asked you again, what would you want?"

Off-time parenthood, in both directions. A first child at nineteen and a first child at forty-two produce different but comparable difficulties: in the first, the parent's own development is interrupted; in the second, the parent's peers are launching adolescents while they are managing toddlers, and the grandparent generation may be unavailable or dead.

The early promotion and the plateau. Being established "too early" produces a lifelong problem of comparison; being passed over at forty-five produces the case in this week's stub. Both are versions of the same injury: the timetable said one thing and the life delivered another, and the gap is experienced as personal failure rather than as a fact about an organisation.

Off-time bereavement. Losing a parent at twenty-five is statistically ordinary and developmentally early; losing a parent at seventy is developmentally "on time" and contains far less of the "I was not ready" quality. The distinction matters because the same event produces different clinical pictures depending on its position on the clock.

Off-time retirement. Retirement at fifty-five in a culture where sixty-five is expected produces a particular shame, especially for men whose identity is occupational. The clinical picture is often depression misdiagnosed as a physical complaint, and the question that opens it is "what were you supposed to be doing now?"

How to use the clock without imposing it. Ask about the timetable, do not supply one. "In your family, when were people supposed to marry? When were they supposed to be settled? Where are you on that?" Then listen for the gap. A therapist who tells a client that the timetable is oppressive — however true — will be heard as dismissing the family, and the client will defend a clock she has just spent an hour telling you she resents.

9.2b The four adult tasks as they present in couple therapy

Adults do not present their developmental tasks; they present their partners. The translation table below is the week's practical core, because it converts a couple complaint into a developmental hypothesis in one step.

The complaint	The developmental task underneath	The question that tests it
"He never talks to me"	Intimacy — the capacity to be known, or the absence of a self to offer	"What would he have to say that would frighten him?"
"She is never satisfied"	Work identity — a calling unfulfilled, or a career stalled	"Whose standard is she failing to meet?"
"There is no us any more, only the children"	Generativity, collapsed into one route	"Who were you generative towards before the children?"
"He changed after his father died"	Off-time bereavement plus the collapse of a generation	"Who is the eldest in his family now?"

The complaint	The developmental task underneath	The question that tests it
"We have nothing to say now the children have gone"	Launching — the couple's identity was outsourced	"What did you talk about before them?"
"She has become obsessed with her mother's care"	Generativity, reversed; the "responsible child" role	"Who decided she was the responsible one?"

The rule that governs the table. Each cell is a hypothesis, never a finding. The therapist offers it as a question, not a formulation, because a couple told that their problem is developmental will immediately begin to argue about whether it is.

9.3 The method: the adult developmental history

Eight questions, twenty to thirty minutes, taken with the individual present and the partner listening. The order is deliberate: it moves from the concrete to the evaluative, so that the person is describing their life before they are interpreting it.

Question 1 — Where were you at twenty-five, and what did you think your life would be by now? The answer produces the person's own timetable, in their own words, and it is the baseline against which everything else is measured. Write the answer down verbatim; it will be quoted back in session four.

Question 2 — What happened instead? Not "what went wrong". The phrasing allows for both gains and losses, and it prevents the interview from becoming a catalogue of failure in front of a partner who is listening.

Question 3 — What are you doing that you did not choose? This surfaces the inherited life: the family business, the profession chosen to satisfy a parent, the move to a city for a spouse's work, the second child who was not planned. The answer is frequently the first time the partner has heard it.

Question 4 — What are you doing that nobody in your family has done before? The counterweight to question three, and the source of legitimate pride. Adults in distress systematically under-report their own novelty, and the therapist's task is to make it visible in front of a witness.

Question 5 — Who depends on you, and what do they need? Generativity, mapped concretely: children, parents, siblings, staff, a congregation, a community. The answer is often the discovery that the person is highly generative and has no route to receive.

Question 6 — Who did you lose, and when? Deaths, separations, migrations, the end of a friendship, the loss of a country. The timing is asked for because off-time losses carry more weight.

Question 7 — What do you do that is not for anyone else? The question that identifies the person's own territory, which has frequently collapsed in the preceding decade. If the answer is "nothing", that is the finding, and it is often more actionable than anything in the presenting complaint.

Question 8 — What would you want your life to have been about? The integrity question, asked early because adults answer it more honestly at forty-five than at seventy, when it has become a verdict rather than a direction.

How to record the answers. Not as a narrative. As an eight-line grid with the questions as headings and the answers as the person's own words. The grid is given to the person, which is itself an intervention: most adults in a couple clinic have not seen their own life summarised on one page.

9.3a The midlife reckoning: what it is and what it is not

The popular notion of a "midlife crisis" — a sports car, a younger partner, an abandoned career — is largely a marketing story with weak empirical support. What the evidence supports is something narrower and more useful: in the forties and early fifties, adults conduct a reckoning between the life they have built and the life they expected, and the reckoning produces either a reorganisation or a set of symptoms.

The four triggers. A loss of a parent (the collapse of the generation ahead, and the arrival of one's own mortality as a fact rather than an idea). A child's departure (the loss of a daily role). A work event — promotion, plateau, redundancy, or the recognition that the ceiling has been reached. And a physical change — illness, injury, or the ordinary evidence of ageing that arrives earlier for some bodies than others.

How it presents. Not usually as a crisis. As irritability, sleep disturbance, increased drinking, withdrawal from the couple, an obsessive new interest, an affair, a sudden conversion, a resignation letter, or a somatic complaint that no physician can explain. The commonest presentation in a family clinic is irritability with no stated cause, which is why this week's case begins there.

The two ways therapists go wrong. The first is to pathologise: to diagnose depression in a person who is grieving a life, and to medicate or cognitive-restructure a reckoning that needs to be conducted. The second is to romanticise: to treat the affair, the resignation, or the abandoned family as a legitimate expression of midlife need, which converts a developmental event into a justification for harm.

The clinical position between them. Take the reckoning seriously as a developmental task, name it as such, and hold the person accountable for what they do while conducting it. The sentence that does both: "what you are describing is a real question, and it does not entitle you to do whatever answers it."

The couple's part. A midlife reckoning is conducted in front of a partner who is having their own. The clinical work is rarely with one person; it is with two people whose timetables have diverged. The question that organises it: "what did each of you expect to be doing at this age, and what has each of you given up to get here?" The answer is usually the first honest accounting the couple has done in a decade.

9.4 Three techniques

Technique 1 — The lifeline, drawn by the person. A horizontal line with the person's age along it, and vertical marks above the line for high points and below for low points, annotated in their own words. Twenty minutes, one sheet of paper, no interpretation while drawing. The technique works for four reasons: it makes the life visible as a shape rather than a series of complaints; it surfaces the pattern of transitions; it produces material the partner has never seen; and it gives the therapist a document to return to. The instruction that makes it work is "put the things that mattered, not the things that should have mattered".

Technique 2 — The two clocks. Two columns on a sheet. Left: "when I expected to…" Right: "when it actually happened". Marriage, children, house, promotion, the death of a parent, retirement. The gap between the columns is the developmental distress, quantified by the person themselves. The technique is particularly effective with couples, because partners routinely discover that they are running on different clocks and have never said so.

Technique 3 — The generativity audit. Three columns: "who I am responsible for", "what they need from me", "what I receive". The third column is almost always empty or filled with the same item repeated. The audit is not a complaint about ingratitude; it is a map of a system with no return path, and the intervention that follows — one hour a week that is not for anyone else — is drawn directly from it.

9.5 The first session with a forty-five-year-old and his wife

Phase 1 — Both, and the complaint in each person's words (15 minutes). The referrer has written "irritability". The therapist asks each partner to describe the last bad week, with one example each, and writes the examples down. This prevents the session from becoming an argument about character.

Phase 2 — The individual history, with the partner listening (25 minutes). The eight questions, asked in order. The partner is asked not to comment, and is told why: "I want you to hear this without having to answer it. You will get your turn." The instruction is usually obeyed, and what follows is the most valuable twenty-five minutes in the case.

Phase 3 — The partner's turn, three questions (10 minutes). What did you hear that you did not know? What did you expect your life to be at this age? What have you given up? The third question is the one that converts the session from an assessment of one person into a developmental conversation between two.

Phase 4 — The shared finding (5 minutes). The therapist names, in one or two sentences, the developmental tasks she heard, and checks them: "I heard four things arriving at once. Is that right?" The couple corrects her, and the corrected version is the formulation.

Phase 5 — One task each (5 minutes). One thing for him, one for her, and one for the pair. Not a programme. Written down, with a review date.

Why the partner listens before she speaks. Because the couple's usual conversation is duelling accounts of the same events, and the developmental history is not an account of

events; it is an account of expectations. A partner who hears her husband's answer to "what did you think your life would be by now?" has received information that no amount of arguing about the dishes could have produced.

9.6 Case: the man who was passed over

Referral. Dapo, 45, a bank branch manager, referred by his general practitioner after two visits with "fatigue and stomach trouble" for which no cause was found. He attends with his wife, Kemi, 43, a pharmacist, who says in the first minute: "he is angry all the time and he will not say why."

The household. Dapo and Kemi, married nineteen years. Tolu, 18, in her first year at a university in another state. Femi, 14. Simi, 9. Dapo's mother, 71, widowed eighteen months ago, living alone in Ibadan in the house Dapo grew up in. Kemi's mother, 68, lives with Kemi's brother in Abuja. The household is comfortable and stretched.

The presenting complaint, in each person's words. Dapo: "I am tired. I do not sleep. I do not enjoy anything." Kemi: "he shouts at the children over nothing. He sits in the car outside the house for twenty minutes before he comes in. Last month he did not come to my sister's wedding and he would not say why."

The eight questions, and what they produced.

1. At twenty-five, what did you think your life would be? "By forty-five I would be a regional director. I would have built the house in Ibadan. My father would have seen it." — Written down verbatim; quoted back in session four.
2. What happened instead? "I got the branch in 2019 and I have been there six years. In March they gave the region to a man who is thirty-eight and who I trained." A pause. "My father died in April last year. I did not build the house."
3. What are you doing that you did not choose? "The branch. I did not want the branch. I wanted the region." Then, after some pressing: "I did not choose to be the one who is responsible for my mother. It was decided. I am the first son."
4. What are you doing that nobody in your family has done? Long pause. "I have never lost a job. Twenty years. My father lost three." — The first item of legitimate pride in the session, and it arrives only because the question was asked.
5. Who depends on you? "Kemi, three children, my mother, my younger brother's school fees, and about forty staff." What do they need? "Everything. I am the roof."
6. Who did you lose, and when? "My father, April last year. And Tolu left in September." — Two losses in the same eighteen months, one a death and one a departure, and both are collapses of a role.
7. What do you do that is not for anyone else? A very long pause. "Nothing. I used to play squash." When did you stop? "When my father was ill."
8. What would you want your life to have been about? "That my children did better than me. And that my father was proud." — The generativity answer and the foreclosure answer arriving in one sentence, which is the formulation of the case.

Kemi's three questions, and what they produced.

What did you hear that you did not know? "That he wanted the region. He never told me. He told me it was a good job." What did you expect your life to be at this age? "I expected to have some time. The children are older. I thought we would have a life again." What have you given up? "My mother lives with my brother. I have never said this to anyone, but I gave up my mother because I was managing his." — The sentence that reorganises the session, and it arrives from a question the therapist asked of the partner rather than of the patient.

The five findings.

1. Four developmental events in eighteen months. A work plateau (the ceiling reached at forty-five, in public, in front of a man he trained); a bereavement (his father, and with him the audience for the life project); a launching (Tolu's departure, which removed the daily evidence of his generativity); and a reversal of generativity (his mother's widowhood, which converted him from the person who receives care to the person who must provide it, permanently).

2. The social clock has failed and nobody has said so. Dapo's timetable said: regional director by forty-five, house built, father present. Every item on it has not happened, and the failure is experienced as personal rather than structural. He has not told his wife because telling her would require him to say the words, and the words are the injury.

3. The presenting symptom is a reckoning conducted badly in private. Sleeping badly, sitting in the car, drinking two beers more than he used to, shouting at a nine-year-old about shoes. None of this is a disorder and all of it is a person conducting a reckoning without a single interlocutor.

4. The marriage has become an administration. Kemi manages the household, Dapo provides; the couple's conversation is logistics and money. This is the ordinary outcome of two careers and three children, and it is also the reason the reckoning cannot be conducted at home: there is no room in the schedule for a conversation that does not have an agenda.

5. The live conflict is about his mother, and it is not about his mother. Dapo wants his mother to move in. Kemi does not. Both positions are legitimate; neither has been stated as a position. Dapo experiences Kemi's resistance as a betrayal of the family; Kemi experiences Dapo's proposal as the final expropriation of a life she has already given two decades to. The therapist's job is to make both statements sayable in the same room, not to resolve them.

The formulation, in one paragraph. A forty-five-year-old man whose life structure has been organised around a single audience — his father — and a single trajectory — the region — has lost both within eighteen months, and has no vocabulary, no interlocutor and no time in which to conduct the resulting reckoning. His irritability is the surface of a grief that has no permitted form. His marriage has become an administration, and the one live decision in front of the couple — whether his mother moves in — is being conducted as a loyalty test rather than as a household negotiation.

The intervention, in five parts.

Part 1 — Name the reckoning. In the second session the therapist says, in front of both: "four things have happened in eighteen months and each of them has taken away a role. That is not an illness. It is a lot, and it has arrived without a single conversation in which you were allowed to say it out loud." Dapo cries, briefly, and is embarrassed. Kemi takes his hand. Nothing else in the session matters as much.

Part 2 — Restore one non-instrumental hour. The generativity audit has produced an empty column. The task is one hour a week that is not for anyone — squash, restored, on Saturday mornings. The therapist is specific about the obstacle: Dapo's objection is that it is self-indulgent, and the answer is that a man who receives nothing has nothing to give, which is a statement about the household's economics rather than about his entitlements.

Part 3 — The mother, as a negotiation with four options. The therapist writes four options on the board, not two: she moves in; she stays and a paid carer is arranged; she moves to a flat nearby with support; she moves in for a trial of three months with a review. The purpose is to break the binary that has made the argument unwinnable. The couple chooses the fourth, and the review date is written into the plan.

Part 4 — The brother's school fees, named. A second loyalty obligation, never mentioned in the referral and discovered by question five. The therapist asks one question: "what was the agreement, and who made it?" The answer reveals an arrangement made by Dapo alone, presented to Kemi as a fact. Renegotiating it is not the point; naming it as a joint decision rather than a unilateral one is, because the pattern — a man deciding alone and then resenting alone — is the marriage's actual problem.

Part 5 — One conversation a week that is not logistics. Twenty minutes, Sunday evening, agenda-free, no children, no money. The rule: neither party may raise a problem in that twenty minutes. The purpose is not intimacy in the romantic sense; it is to rebuild the channel through which a reckoning could be conducted at home rather than in a car.

9.6a Five months later

The mother did not move in permanently. She moved into a flat two streets away with a carer who comes daily, and the arrangement is reviewed every three months. Dapo visits on Wednesdays and Sundays, which is more than he visited when she lived alone in Ibadan.

Squash has become twice a week, with a man Dapo knew at university and had not telephoned in nine years. The therapist did not suggest the friend; the squash did.

The region has not been offered and will not be. In session seven Dapo says, unprompted: "I am not going to be a regional director", and the sentence is flat rather than bitter. Kemi says afterwards, privately, that this was the moment she stopped being afraid of him.

The drinking has reduced without being addressed. The therapist notes this in the record as an example of a symptom that resolves when the function it was serving is met elsewhere, which is the general principle this week has been teaching.

The honest limitations. Dapo has not grieved his father, in any sense a therapist would recognise. He has stopped being angry, which is not the same thing, and the therapist

says so in the final session rather than claiming a resolution. The couple's twenty minutes survives about three weeks in four. And the eldest child, Tolu, has not been part of the work at all, which the therapist records as a gap: a launching that happens to a family in difficulty usually needs the person who left to be spoken about, at least once.

What would have changed the plan. If Dapo had described persistent low mood with anhedonia for more than two months and impairment at work, the plan would have begun with a diagnostic assessment and possibly medication, alongside the developmental work. If the drinking had reached a level with withdrawal symptoms, substance work would have taken precedence. If Kemi had refused to continue after the mother's proposal, the work would have become a couple's negotiation about competing obligations to two sets of parents, which is a different treatment with a different endpoint. And if Dapo had disclosed an affair — which the therapist asked about directly, and which he denied — the reckoning would have been conducted against a background of breach, and the sequence would have been different.

9.7 Transcript: the lifeline, drawn in front of the couple

Twelve turns from the second session. The therapist has one sheet of A4 and a pen, and she has given the pen to Dapo.

> **T1 Therapist:** I want to draw your life on this page. A line across the middle, and your age along it. Above the line, the things that were good. Below, the things that were hard. Not the things that should have mattered — the things that did. — The instruction that makes the technique work, and the only instruction given. **T2 Dapo:** (draws, silent, for about four minutes) **T3 Therapist:** (says nothing) — Four minutes of silence while a man draws his life. The impulse to comment at the first mark is strong and must be resisted; the drawing is the intervention and the therapist's commentary would take it back. **T4 Dapo:** (stops) There. (handing it over) **T5 Therapist:** (reading) 1998, graduation. 2001, marriage. 2003, Tolu. 2007, the first branch. 2011, your mother's illness. 2014, Femi. 2019, this branch. (pause) 2024, your father. (pause) What is that mark in 2016? — The therapist reads the marks back flatly, without evaluation, and then asks about one unexplained mark rather than about the obvious ones. The unexplained mark is where the person has put something they have not decided how to describe. **T6 Dapo:** That is when I stopped the squash. (pause) My father was ill from 2015. **T7 Therapist:** So the squash went first. **T8 Dapo:** (quietly) Yes. **T9 Kemi:** I did not know that. I thought you were tired. — The partner's first contribution, and it is not an argument. This is the effect the technique is designed to produce: a fact the partner did not have, delivered without accusation. **T10 Therapist:** (to Kemi) What do you notice about the line? — The question is given to the partner deliberately. It converts her from an audience into a reader, which is the difference between a session about a man and a session about a couple. **T11 Kemi:** Everything above the line is work or children. There is nothing on it that is just him. — She has found the generativity audit without being shown it. **T12 Therapist:** (to both, quietly) That is what I noticed too. (pause) Dapo, what would you put on it if you had an hour a week that was not for anybody? — The question is asked of him, not proposed by the therapist, and it is asked last, when the evidence for it is on the page in his own hand.

9.7a What to do when the partner weeps and the client does not

A practical note that belongs to this week and recurs in every adult case.

In the session above, Kemi weeps twice and Dapo does not. Trainees read this as evidence that Kemi is "more in touch" and Dapo is "defended", and they begin to work on Dapo's defences. The reading is usually wrong, and the intervention that follows from it usually fails.

Three alternative explanations, and how to tell them apart. First, timing: the partner may have had years to grieve the marriage in private and is now crying about something old, while the client is hearing his own life described aloud for the first time and is in the numb phase that precedes grief. Second, permission: in many households the man has never cried in front of anyone and the prohibition is stronger than the feeling. Third, role: one partner is the expressive one and the other the containing one, and the arrangement is stable rather than pathological.

The test. Ask each of them, separately and plainly: "when did you last cry, and who was there?" The answers distinguish the three explanations within a minute. A man who cries alone in a car is not defended; he is unaccompanied.

What to do in the session. Do not pursue the non-crying partner's tears. Attend to the one who is crying, briefly and without fuss — a sentence, a pause, a glass of water — and then return to the material. The non-crying partner is watching how the therapist treats the crying one, and what he learns in those thirty seconds determines whether he will ever cry in this room.

What to say afterwards, if anything. One sentence at the end of the session, addressed to him and not to the couple: "you did not have to do anything while she was crying, and you stayed." Naming the staying is more useful than naming the absence of tears, and it is accurate.

9.8 Laboratory: the developmental history with a partner present

Aim. To take the eight-question history with an individual whose partner is in the room, without losing the individual to the couple.

Procedure. Trios: therapist, client, partner. The trainer briefs the client with a life containing three off-time events and one foreclosed choice; the partner is briefed to interrupt twice — once with a correction, once with an accusation — and to be silent otherwise. Thirty minutes. The observer records every interruption the therapist allowed and every one she prevented, and how she prevented it.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
Structure	Questions asked out of order or skipped	All eight asked, some merged	All eight asked in order, with the answers written verbatim
Protecting the individual	Allowed the interruption and the session became a couple argument	Stopped the interruption but lost the client's thread	Stopped it, named why, and returned the client to the exact question

Criterion	0–1	2–3	4–5
Using the partner	Partner ignored	Partner invited at the end	Partner invited at the two designed moments, and her contribution used
The off-time reading	Took the events at face value	Noticed one off-time event	Named the gap between the timetable and the life, in the client's words
The close	Summarised for the client	Checked the summary	Checked it, was corrected, and used the correction

The distinction that carries most of the marks. The top band requires the therapist to write the answers verbatim, which is harder than it sounds: most trainees paraphrase into clinical language within the first question, and the paraphrase is what loses the case. "He says his father would have seen the house" is data. "He reports unresolved paternal expectations" is a summary of the therapist's own theory.

9.9 Six errors

Error 1 — Hearing the relationship and missing the life. The couple's complaint is the presenting system, not the whole person. A therapist who works only with the couple's communication will produce better arguments about the same unmet developmental needs.

Error 2 — Imposing the timetable. Telling a forty-year-old unmarried woman that the social clock is oppressive is not support; it is the substitution of the therapist's analysis for the client's dilemma. Ask about the clock; never set it.

Error 3 — Pathologising the reckoning. A man of forty-five who is grieving a life is not necessarily depressed, and the distinction matters because the treatments differ. The screening question is duration, impairment and severity, as in Week 8 — and if the criteria are met, treat the depression, because a depression will prevent the reckoning being conducted at all.

Error 4 — Romanticising the reckoning. The mirror error. "He is finding himself" is not an adequate response to an affair, an abandoned family, or a resignation that leaves a household without income. The developmental frame explains the behaviour; it does not license it.

Error 5 — Treating generativity as a virtue. Generativity becomes a trap when it is the only structure a person has: the man who is the roof for everyone and receives nothing is not a saint, he is a system with no return path, and the collapse is predictable. The audit's third column is the clinical finding.

Error 6 — Believing the timetable is the same for both partners. Couples routinely run on different clocks — different families, different expectations, sometimes different countries. The two-clock exercise is not an icebreaker; it is the assessment of a mismatch that has been conducted silently for twenty years.

9.10 Culture, faith and the adult timetable

The Nigerian adult clock. The timetable in most of the households this book is written for is explicit and densely populated: finish university by twenty-three, marry by twenty-eight, first child within a year of marriage, a house by forty, a title by fifty, a grandchild by sixty. The clock is enforced by relatives, by church and mosque, and by the ordinary conversation of a compound. An adult who is off it is not merely uncomfortable; they are the subject of a collective project of correction.

The clinical consequences are specific and worth naming. A thirty-four-year-old unmarried woman presents with anxiety that is entirely rational given her circumstances, and the therapist's first task is to distinguish between her own wish and the family's project. A forty-year-old man with no child presents with depression and is asked by everyone he meets when he will have one. A couple with one child and secondary infertility present with a marital problem that is, in fact, a public one.

The first son, the last born, and the birth-order contract. Many households operate an explicit birth-order contract: the first son is responsible for the parents, the first daughter for the younger siblings, the last born for the parents' old age. The contract is not usually written and is rarely negotiated. Dapo's case is the ordinary version, and the clinical move is not to overturn it but to make it speakable — who decided, when, and what is the limit — because an unstated contract cannot be declined and therefore cannot be renegotiated.

Faith and the meaning of a stalled life. Adults in a religious household may interpret a plateau, a childlessness, or a bereavement as a spiritual matter, and the interpretation is frequently a resource rather than an obstacle: it provides a language for endurance and a community for support. The clinical error is to treat the framework as a defence to be interpreted. The workable position is to ask what the framework is doing — sustaining, or postponing — and to negotiate only what must be negotiated, which is usually whether the person is also permitted to be angry.

Migration and the split timetable. A substantial proportion of the adults in these clinics have one foot in another country: a sibling in the United Kingdom who sends money, a spouse working in the Gulf, a child studying abroad. The developmental timetable of the household is then partly external, and the clinician must ask who is where, who is expected to return, and what the migration has cost. A couple's "distance" is sometimes a visa.

> **DEBATE**
> "A forty-five-year-old man who has done everything his family required of him and is now miserable should be encouraged to disappoint them." Argue for: the foreclosure was never his choice, the misery is the cost of a life authored by others, and the therapeutic task is to return authorship. Argue against: the obligations are real, the family is not an obstacle to be overcome, the man is likely to lose more than he gains, and encouraging rebellion is a projection of the therapist's individualism. Then state what you would actually say to Dapo if he told you he wanted to stop paying his brother's school fees.

9.11 Evidence note

What the research supports. The longitudinal evidence for continuity with change is strong: personality traits are rank-order stable across adulthood while their behavioural expression shifts, and the "midlife crisis" as a universal phenomenon is not supported. Generativity in midlife is associated with better wellbeing and, in several cohorts, with better outcomes for the next generation — though the direction of causation is not settled. The association between off-time life events and distress is robust across cultures, which is the strongest single support for the social-clock concept.

What is weaker. Levinson's seasons rest on a small interview sample of American men and should be taught as a heuristic. Vaillant's cohort studies are rich and unrepresentative — educated, male, and longitudinally privileged — and their conclusions travel poorly to households with different structures. The "work as calling" literature is largely correlational and heavily confounded by occupation and income.

What is not supported. That there is a universal midlife crisis with a predictable shape. That marriage has invariant stages that all couples pass through in order. That generativity requires children. And that an off-time life is a damaged one — the evidence is about distress, not about outcome.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same promotion at thirty-eight is a triumph for one manager and a ceiling for another, and what distinguishes them is not the event but what the event was supposed to mean.

9.12 Key terms

Intimacy versus isolation — Erikson's sixth stage; the capacity to be known without managing what is known.

Fusion, pseudo-intimacy, isolation, serial replacement — the four adult forms of avoiding intimacy.

Generativity — concern for establishing and guiding the next generation; the midlife task.

Social clock — the culturally shared timetable for life events; distress follows deviation from it more than the events themselves.

Off-time event — a life event occurring earlier or later than the cultural timetable expects.

Life structure — the underlying pattern of a person's life at a given time: roles, relationships, and the commitments that organise them.

Continuity with change — the finding that personality is stable in structure and changing in expression across adulthood.

Foreclosure — a commitment adopted without exploration, typically inherited from the family.

Birth-order contract — an unstated family allocation of responsibility by ordinal position.

Two clocks — the divergence between partners' expected and actual timetables.

9.13 Self-test

1. Name the four adult forms of avoiding intimacy and one presenting complaint for each.
2. What is the difference between a job, a career and a calling, and why is the distinction clinically useful?
3. Explain the social clock and give three off-time events with their typical clinical presentations.
4. Why does the lecture insist that the reckoning be taken seriously and that the person be held accountable? Give the error that each half prevents.
5. In the Dapo case, what were the four developmental events, and why did they arrive together?
6. What is the third column of the generativity audit, and what does an empty column mean?
7. Why is the partner asked what she gave up, rather than what she wants?
8. What is the difference between pathologising and romanticising a midlife reckoning, and what is the position between them?

Answers. (1) Fusion ("he cannot bear me having a friend"); pseudo-intimacy ("we are very close" with no private conversation); isolation ("he does not let me in"); serial replacement (a pattern of relationships each of which ends when it becomes known). (2) A job is for money, a career is a trajectory, a calling is what you would do anyway; the distinction identifies what a person has actually lost when work changes, which determines whether the intervention is occupational or developmental. (3) A first child at nineteen (interrupted own development); a first marriage at forty-five (enforced singleness and pressure); retirement at fifty-five (shame and somatic presentation). (4) Taking it seriously prevents pathologising a developmental task as a disorder; holding the person accountable prevents romanticising harm as self-discovery. (5) A work plateau, his father's death, his daughter's departure, and his mother's widowhood — they arrived together because midlife concentrates losses of role, as the generation ahead dies and the generation behind leaves in the same decade. (6) What I receive; empty means a system with no return path, and the intervention is to build one. (7) Because the question produces the partner's own developmental account, which converts an assessment of one person into a conversation between two, and because "what do you want" invites a demand while "what did you give up" invites a history. (8) Pathologising treats a reckoning as depression; romanticising treats harm as growth; the position between them is to name the task and hold the person accountable for how they conduct it.

9.14 Reflective task

Write your own lifeline, on paper, with the instruction the case used: the things that mattered, not the things that should have mattered. Then write your own two clocks — when you expected to marry, to have children, to be established, to be settled — and when it actually happened or looks likely to. Then answer one question: which gap have you explained to yourself, and which have you not?

9.15 Further reading

- Erikson, E. The Life Cycle Completed — intimacy and generativity, in the author's own late revision.
- Levinson, D. The Seasons of a Man's Life — read as a heuristic, with the sample in mind.
- Vaillant, G. Triumphs of Experience — the Harvard cohort, and the case for continuity.
- McAdams, D. The Redemptive Self — how adults narrate generativity, and why the story matters.
- Neugarten, B. "Time, age and the life cycle" — the original social-clock paper, still the clearest.

Next: Week 10 — Couple development: the relationship across time.

Week 10 — Couple development: the relationship across time

Couples develop too, and the development is not a metaphor. A relationship has tasks that arrive in a sequence, capacities that are built or not built at each stage, and a trajectory that can be described in advance well enough to be clinically useful.

Phase	Central task	Common failure
Formation	Commitment and identity as a couple	One partner never really chooses
Early marriage	Building rituals and boundaries with families of origin	In-law triangles
Young children	Dividing labour; protecting the couple bond	Co-parenting replaces partnering
Adolescent children	Re-negotiating autonomy, revisiting one's own adolescence	Escalation or abdication
Launching	Re-intimacy after children leave	The "empty nest" that was already empty
Later life	Facing illness, retirement, loss together	Caretaking replaces relating

What the map is for. Not prediction, and not assessment of how far along a couple "should" be. The map is a diagnostic instrument with three uses: it tells the therapist which task is live; it predicts which task is next, which is the basis of preventive work; and it locates the couple's complaint in a sequence, which is often the first time either partner has seen the complaint as a stage rather than a verdict.

What the map is not. It is not a universal sequence. Couples form at twenty and at fifty; some never have children and some begin with them; some skip the launching phase because a parent moves in instead of a child moving out; and a substantial number of the households in these clinics are not a two-person system at all but a network of three or four adults raising children together. The map is a set of tasks, not a set of stations.

Why this week follows adult development. Last week was the person's timetable. This week is the relationship's timetable, and the two are frequently out of step. The most common clinical picture in this week is a couple whose individual clocks have diverged from the couple's clock: one partner is still in the formation task ("have we really chosen each other?") while the other is deep in the young-children task ("who is doing the school run?").

> **PRACTICAL**
> With a peer, draw a couple timeline: five high points, five low points, and one turning point each. Then ask: which developmental transition explains the shape of this line? Most students find that at least two of their five low points sit next to a transition rather than next to a conflict, which is the whole argument of the week.

10.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	Why a couple develops	The difference between a relationship that changes and one that grows	Name the last transition their own family passed through
8–22	The six phases	The tasks, in order, with one failure mode each	Place four couples on the map from a one-line description
22–34	Formation and the unchosen couple	The "one partner never really chooses" pattern and how it presents ten years later	Identify the unchosen partner in two vignettes
34–46	Young children	The division of labour, the disappearance of the couple, and the two-year window	Calculate the couple's weekly non-logistical time
46–58	Adolescence and launching	The re-negotiation, and the couple who outsourced their identity to the children	Ask the "what did you talk about before them?" question in role
58–70	The cascade and the repair trajectory	The behaviours that predict dissolution, and the ones that predict recovery	Code a two-minute argument for the four horsemen
70–82	Rituals	What a ritual is, why couples without them drift, and how to build one	Inventory their own family's rituals
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students arrive with a model of couple therapy as conflict management, and the week's job is to replace it with a model of

development management. The demonstration that does this is to take a couple who argue well — who resolve every disagreement respectfully — and show that they are nonetheless in trouble, because they have not had a non-logistical conversation in four years. Conflict skill is necessary and it is not the same as a relationship.

The sentence the lecture must land. "Couples do not fail because they fight. They fail because the tasks change and nobody tells them, so they keep doing the last stage's work in this stage's conditions."

10.2 The six phases, with the clinical reading of each

1. Formation. The task is commitment: two people becoming a couple that is distinct from the two families that produced them, and each of them genuinely choosing. The clinical issue that matters most is the unchosen couple — the relationship entered because of pregnancy, family pressure, immigration status, loneliness, or the end of another relationship. Ten years later, the unchosen partner presents a resentment that has no argument attached to it, because there was never a decision to point at.

The signature in the room: one partner can describe the moment they chose, and the other cannot. The therapist's question is not "do you love him?" but "when did you decide?" — and the answer, or its absence, organises the whole treatment.

2. Early marriage. The task is building rituals and boundaries: how Sundays work, whose family comes for Christmas, who decides about money, what the couple does that is theirs. The characteristic failure is the in-law triangle — a partner in coalition with a parent against the other partner, usually without intending it.

The signature: the couple's disagreements are not about content but about loyalty, and every argument resolves into a question of whose family wins. The intervention is structural rather than communicative: the couple must have a stated position that they can deliver to both families, in the same words.

3. Young children. The task is dividing labour while protecting the couple bond. The characteristic failure is that co-parenting replaces partnering: the couple becomes an efficient organisation for the management of children and ceases to be a relationship. This is the single most common presentation in couple clinics worldwide, and it is not a failure of love but a failure of time and structure.

The signature: the couple can talk for an hour about the children and cannot name what they did together in the last month that was not for the children. The assessment question is arithmetic: how many hours a week does this couple spend alone together, awake, not discussing logistics? The answer is usually between zero and two.

4. Adolescent children. The task is re-negotiating autonomy as a family, and — unavoidably — revisiting each parent's own adolescence. The characteristic failure is the two patterns from Week 7: escalation and abdication, frequently with one parent in each.

The signature: the couple's conflict is conducted through the adolescent. One parent is the strict one, the other the lenient one, and the adolescent has learned to route every request

through the lenient one. The developmental reading is that the parents are replaying their own adolescent conflicts with each other, using the child as the arena.

5. Launching. The task is re-intimacy after the children leave. The characteristic failure is the empty nest that was already empty: a couple who discover, at the point of launch, that they have not had a relationship independent of the children for twenty years.

The signature: the couple presents when the last child leaves, with a complaint that sounds sudden and is not. The question that opens it — "what did you talk about before them?" — usually produces a silence followed by an accurate and painful answer.

6. Later life. The task is facing illness, retirement and loss together. The characteristic failure is that caretaking replaces relating: one partner becomes a nurse and the other a patient, and the relationship that was between two adults becomes a service.

The signature: the healthy partner describes exhaustion and the ill partner describes loneliness, and both are describing the same loss. The intervention is to restore some domain of the relationship that is not about the illness — one hour, one activity, one subject that is not permitted to be medical.

10.2a What the therapist asks at each phase

The table below is the practical instrument. In a first session, the therapist does not need to know which phase the couple is in so much as which task is live, and the questions below identify it in about ten minutes.

Phase	The question that identifies it	What a positive answer means
Formation	"When did you decide?"	If only one can answer, the couple is unchosen
Early marriage	"Whose family decides about Christmas, and who decided that?"	A coalition with a family of origin is live
Young children	"How many hours a week are you alone together, awake?"	Below two hours, the bond is unprotected
Adolescent children	"Who is the strict one?"	The conflict is being routed through the child
Launching	"What did you talk about before the children?"	The couple's identity was outsourced
Later life	"When did you last do something together that was not about the illness?"	Caretaking has replaced relating

The rule for using the table. One question per session, not six. A therapist who runs the whole table in the first session produces a couple who feel assessed rather than met, and the information gained is worth less than the alliance lost. The table is for the therapist's internal map; the questions are delivered one at a time, across sessions, in the order the couple's material invites.

10.2b The cascade and the repair trajectory

Couples do not dissolve in a single event. They move along one of two trajectories, and the behaviours that distinguish them are observable in a two-minute argument.

The cascade. Four behaviours reliably predict deterioration, and they are not the loud ones. They are: criticism (an attack on the person rather than the behaviour: "you always", "you never"); contempt (mockery, sarcasm, eye-rolling, name-calling — the single strongest predictor); defensiveness (counter-attack or victim stance in response to a complaint); and stonewalling (withdrawal, silence, a phone). These are usefully taught not as sins but as stages of a losing strategy: criticism is a complaint that has been made too many times without being heard; contempt is criticism that has stopped expecting an answer; defensiveness is the response of a person who believes the accusation is a verdict; and stonewalling is what a physiologically flooded person does when they have no other option.

The repair trajectory. The distinguishing feature of couples who recover is not the absence of the four behaviours but the presence of repair attempts: a joke, a hand, a "can we start again", a break, a sentence that concedes one point. Repair attempts succeed in proportion to the couple's capacity to notice them — which is trainable — and in proportion to the positive sentiment override built up outside the argument.

The developmental reading. The cascade is what happens when a stage's task is not done and the couple keeps using the previous stage's methods. The couple in the young-children phase who are still arguing like a formation couple — about whether they chose each other — will fight about the school run and mean something else entirely. The therapist's job is not to teach them to argue better about the school run. It is to identify which task is live and to move the argument there.

What predicts recovery, in order. The couple's capacity to repair in the moment; the ratio of positive to negative interaction outside conflict; the presence of shared meaning (children, faith, a project, a place); and the absence of contempt. Note what is not on the list: the frequency of conflict, the content of the disagreement, and the couple's skill at "communicating". Many couples with excellent communication skills are in trouble, and many couples who shout are not.

10.3 The method: the couple timeline

The timeline is the assessment and the intervention at once, and it takes one session.

Step 1 — Draw it, with both present, on one sheet. A horizontal line, and the years along it from the couple's meeting to today. Above the line: five high points. Below: five low points. Both partners contribute; the therapist writes.

Step 2 — Ask for the same five moments from each partner separately, then compare. The comparison is the finding. Couples routinely produce different high points, and the difference is not a memory error; it is a difference in what each of them is living for.

Step 3 — Mark the transitions in a third colour. Marriage, first child, a move, a job change, a bereavement, a migration, a child leaving. Then look at the line. The pattern that appears in the great majority of cases is that the low points cluster adjacent to the transitions, not adjacent to the arguments.

Step 4 — Ask the turn question of each. "Which point on this line was the turning point, and what turned?" The answers are frequently surprising and always usable, and they are the material for the rest of the treatment.

Step 5 — Ask the unanswered question. "What happened on this line that is not on it?" The question produces the events the couple has agreed not to discuss: an affair, a miscarriage, a debt, a termination, a period of violence, a child who died. The therapist asks it once, and does not pursue the answer in the first session.

Step 6 — Convert the timeline into one task. Not a programme. One thing that the couple will do, or stop doing, between now and the next session, chosen because it addresses the live task rather than the loudest complaint.

Why the technique works. It externalises the relationship. A couple who have been conducting a characterological argument ("you are selfish") are suddenly looking at the same object on a page, and the object is a history rather than a defect. It also does something the partners cannot do alone: it makes visible the ratio between the good years and the bad, which is frequently more balanced than either partner believed.

10.3a The turning-point interview

The turn question from step 4 deserves a section of its own, because it is the most productive single question in couple work.

The question. "If you had to put your finger on one point where this relationship turned — for better or worse — which point would it be?"

Why it works. It asks for a moment rather than an evaluation. Couples in difficulty have an evaluation ready ("it has been bad for years") and no moment, and the request for a moment produces specificity, a date, and usually an incident that the other partner did not know was being carried.

The four kinds of answer, and what each requires.

- **An event** — a job loss, a move, a death, an affair. The therapist's work is to establish what the event meant, which is usually not what happened but what was not said about it.
- **A decision** — "when we decided to have the second child", "when I took the posting". The work is with the decision's authorship: who decided, and who was informed.
- **A sentence** — "when he said, in front of his mother, that I was exaggerating". The work is with the moment itself, which is frequently the first time the other partner has understood its weight.
- **A silence** — "I cannot think of one". This is the most informative answer of all, and it usually means that the couple's difficulty is chronic rather than eventful, which changes the treatment from a reconstruction of an injury to a construction of a relationship.

The rule for the therapist. Do not choose between the two answers. A couple will offer two turning points and the therapist's temptation is to adjudicate. The useful move is to

ask each partner what the other's turning point means to them, which is the first genuinely mutual act in most of these sessions.

10.4 Three techniques

Technique 1 — The ritual inventory. List every recurring practice in the household: the Sunday meal, the Friday call to a parent, the way birthdays are marked, the prayer before a journey, the walk, the programme watched together, the way an argument ends. Then mark each as alive, dead, or contested. Couples in difficulty have reliably lost their rituals, not their love, and the restoration of one ritual is frequently more effective than a communication exercise.

Technique 2 — The division-of-labour audit. Two columns: everything that has to be done in a week, and who does it. Then a third column, which is the one that matters: who notices when it is not done. The third column is the measure of mental load, and it is the source of most of the resentment in the young-children phase. The audit is not a negotiation; it is a description, and the description usually produces the negotiation without the therapist having to run it.

Technique 3 — The twenty minutes. One twenty-minute period per week, at a stated time, with no children, no money, no logistics and no problem-solving. The rule that makes it work: if a problem is raised, the twenty minutes ends and the problem goes to a different slot. Couples find this harder than any other task in the book, and their difficulty is the diagnosis.

10.5 The first session with a couple in the young-children phase

Phase 1 — The complaint, in each person's words, with one example each (15 minutes). Written down verbatim. The therapist does not allow the partners to respond to each other's examples in this phase, and says why: "you will each get to answer, and I want the examples to be on the table first."

Phase 2 — The timeline (25 minutes). Drawn on one sheet, both contributing, with the transitions marked in a third colour.

Phase 3 — The live task, named and checked (5 minutes). "I think the task in front of you is not the argument about the phone. I think it is that you have not been alone together, awake and not talking about the children, for about two years. Is that right?" The couple corrects, and the corrected version is the formulation.

Phase 4 — The arithmetic (5 minutes). "How many hours a week?" The answer is written on the sheet, next to the timeline, because a number on a page is more persuasive than a number in a conversation.

Phase 5 — One task (10 minutes). The twenty minutes, scheduled now, in the session, in the diary. The therapist does not leave the scheduling to the couple, because the couples who most need it are the ones least likely to find the time.

Why the timeline precedes the task. A couple given a task before they have seen their own history experiences the task as an instruction from a stranger. A couple who have just

drawn twenty years on a page experience the same task as a repair to something they can see.

10.5a When one partner will not come

The commonest structural obstacle in couple work: one partner attends and the other refuses. Three positions, and one of them is right.

The wrong position — treat the attending partner individually as if the couple were the problem. This produces a therapy that is, in effect, an alliance against an absent person, and the absent person will eventually be told what was said, accurately or otherwise.

The second wrong position — refuse to work until both attend. This abandons the person in front of you and, in practice, ends the treatment.

The workable position — work with one, on the relationship, explicitly and transparently. Say what the frame is: "I will work with you on what you can change, which is your part. I will not build a case against your husband." Then three practical moves: ask the attending partner to invite the other once, in the therapist's words, with the offer that the first session is a consultation and not a commitment; offer a single joint session with an explicit agenda that the absent partner can read in advance; and set a review date at which the situation is reassessed.

The question that changes the refusal. An absent partner usually refuses for one of three reasons: they believe the problem is the other person's; they are ashamed; or they are afraid of what will be said. The invitation that works addresses the third: "nothing will be decided about you in a room you are not in." That sentence, delivered by the attending partner, converts a substantial minority of refusals.

10.4a The negative cycle, in the couple's own words

Emotionally focused couple therapy describes a circular pattern — a pursue-withdraw or attack-defend cycle — that organises most distressed couples' arguments. The description is useful to a developmental therapist because the cycle is the mechanism by which a missed stage task turns into a stable arrangement.

How to name the cycle without jargon. Ask each partner two questions: "when you are not getting through, what do you do?" and "what do you see him doing?" The answers produce the cycle in the couple's own language, which is the only version that will be remembered. A typical formulation, in a couple's words: "when I feel alone I ask for more, and the more I ask the more he goes quiet, and the quieter he goes the more I ask." The therapist writes it down and gives it a name the couple chooses.

Why the developmental frame matters here. The cycle is not a communication defect; it is a stage-specific adaptation that has outlived its stage. In the young-children phase, pursuing is what one partner does to protect the bond from disappearing and withdrawing is what the other does to survive exhaustion. Both strategies are reasonable responses to the actual conditions. They become a cycle when each is experienced by the other as the

cause of the problem, and the cycle becomes stable when neither partner can see their own half of it.

The three moves that break it, in order.

First, name the cycle as the problem, not the partner. "The thing that has been happening in your house for four years is this pattern, and it is not either of you." This is not a rhetorical softening; it is accurate, and it changes the couple's task from mutual accusation to joint observation.

Second, find the primary emotion under each position. The pursuer's anger is usually underlain by fear of being unimportant; the withdrawer's flatness by a conviction of failure. Naming these is done with tentativeness — "I may be wrong, but I wonder whether the quiet is not indifference but a kind of protection" — because a confident attribution at this point is experienced as taking the other's side.

Third, give one small enactment in the session. Ask the pursuer to say the fear directly to the withdrawer, and the withdrawer to answer without defending. The therapist keeps it to two turns and stops it before it becomes a conversation, because the first two turns are the intervention and the third is usually a relapse.

What the developmental therapist adds. The cycle is described, and then the task is addressed: the Thursday, the audit, the defined grandmother. Naming a cycle without changing the structure that produces it produces a couple who understand their pattern and continue to enact it, which is the characteristic failure of insight-oriented couple work in a household with no time.

10.5b The couple with a child with a disability

One presentation deserves separate treatment because it reorganises the couple's developmental timetable more than any other.

What changes. The young-children phase does not end when it should; the launching phase may never arrive; the division of labour becomes permanent and unequal; the couple's social world narrows to families in the same situation or disappears; and the services system becomes a third party with its own demands, appointments and forms. The couple's shared meaning is tested in a way that few other events test it.

What the evidence says. The divorce rate among parents of children with disabilities is frequently quoted as higher than the general population; the better-designed studies find a smaller and less consistent difference than the popular claim, with the strongest predictors being the child's behaviour difficulties and the family's financial and service support rather than the disability itself. What is robust is the elevation of parental stress and the reduction of couple time.

The three clinical tasks. First, protect one domain of the couple's life that is not about the child — which is the same intervention as the twenty minutes, with a higher stake. Second, define the division of labour explicitly, because the default is that one parent becomes the specialist and the other becomes the provider, and the arrangement is rarely

chosen. Third, address the sibling, who is the family member most often described as "fine" and least often asked.

The question that opens the work. "What did you imagine this would be like, and what is it like?" Asked of each partner separately, then together. The gap between the two answers is the couple's grief, which is usually unmourned because the child is loved and the grief feels like a betrayal of the child. Naming that directly — "it is possible to love this child completely and to grieve the life you expected" — is frequently the most useful sentence in the treatment.

10.6 Case: Aisha and Sam, four years on

The couple first met in Week 2, where Aisha, 31, and Sam, 34, were assessed for goodness of fit with a temperamentally slow-to-warm infant. They return three years later with a second child.

The household. Aisha, 34, a lawyer, on a partnership track. Sam, 37, an architect, in a small practice he started with a colleague. Their daughter, Yusra, is four and in pre-school; their son, Bilal, is nine months. Sam's mother, 66, comes three days a week to help. Aisha's sister lives twenty minutes away and has two children of her own.

The referral. Aisha telephones. "I do not think I love him any more. I am not saying I want to leave. I am saying I cannot remember." Sam's version, in the first session: "she is never here. When she is here she is on her phone. She has decided something and she has not told me what it is."

The complaint, with examples. Aisha's example: "last Saturday I came back from the market and he had taken Bilal to his mother's without telling me, and I sat in the car outside my own house for ten minutes because I did not want to go in." Sam's example: "on Wednesday she came home at ten and I had done both bath times and she said 'you should have put him down earlier'."

The timeline. Drawn with both, and it is the session.

- 2016 — met at a wedding. High.
- 2018 — married. Aisha's family objected to the tribe; she married him anyway. High.
- 2019 — Yusra born, difficult birth. Low.
- 2020 — Sam's practice started; the money stopped for eighteen months. Low.
- 2021 — Aisha's father died. Low, the deepest mark on the line.
- 2022 — Aisha made senior associate. High.
- 2023 — The move to the bigger flat. High, contested.
- 2024 — Bilal born. High and low in the same month.
- 2025 — Sam's practice stable; Aisha's partnership decision due. Unmarked, so far.

What the timeline shows that neither partner had said. Three findings.

First, the low points cluster at 2019 to 2021, and the couple has never discussed 2021 at all. Aisha's father died in the same year Sam's practice was failing and Yusra was

two. The therapist asks the unanswered question — "what is not on this line?" — and Aisha says: "I was pregnant in 2021 and I lost it in the August, and then my father died in the October, and nobody in this family knows that because we were doing the practice." Sam puts his face in his hands. It is the first time in four years that this has been said in the same room as both of them.

Second, the high points are almost all about achievement, and none of them are about each other. 2022 is Aisha's promotion. 2023 is the flat. The couple's celebrations are external, and the therapist names this without interpretation: "there is nothing on the top of this line that is just the two of you."

Third, the word "partnership" appears twice — Sam's practice and Aisha's track — and the couple is competing in the same currency. They are not in conflict about love. They are in a symmetric struggle in which each is asking the other to accommodate a career, and neither has said so in those terms.

The live task. The young-children task: dividing labour while protecting the couple bond. The arithmetic is performed in the session: Aisha estimates that she is alone with Sam, awake, not discussing children, for "about twenty minutes a week, and that includes the argument". Sam says "less".

The three findings that produce the formulation.

1. **The couple's bond was never built as a separate structure; it was built as a joint project.** Two ambitious people married in 2018 and immediately began building: a practice, a career, a flat, two children. The project is now succeeding and the relationship that was supposed to be its purpose has no independent existence. This is not a failure of love; it is a failure of construction.
2. **An unmourned loss has been silently organising the last four years.** The miscarriage and the father's death in the same autumn were absorbed as a logistical problem ("we were doing the practice") and never mourned. Aisha's "I cannot remember loving him" is accurate: the years in which the memory would have been made were the years in which the household was in emergency.
3. **The conflict about the market and the bath time is not the conflict.** It is the ordinary surface of a symmetric struggle about whose career accommodates whose, conducted through domestic detail because neither partner has the standing to raise the real question — which is that they have never decided whose work comes first.

The intervention, in five parts.

Part 1 — Name the mourning, once, and do not process it in this session. The therapist marks the September and October on the timeline, says "that is not a bad year, that is two deaths and a birth plan that ended", and moves on. The instruction is to notice, not to work. A couple who have avoided something for four years will not process it in the session in which it is first named, and a therapist who tries will teach them to avoid it more carefully.

Part 2 — Build one structure that is not the project. The twenty minutes, scheduled in the session, on Thursday at nine-fifteen, after Bilal's last feed. The rule: no children, no

money, no problems. The couple's objection — "we have nothing to say" — is answered by predicting it: "the first two will be awkward. That is the finding, not the failure."

Part 3 — The division of labour audit, with the third column. Written in the session. The third column — who notices when it is not done — is almost entirely Aisha's, and Sam's reaction to seeing it written is the session's second significant moment. The negotiation that follows is not run by the therapist; it is run by the list.

Part 4 — The career question, named as a question. "Whose work comes first when they conflict?" is asked plainly and answered badly, which is the beginning. The couple agrees on a rule for one year: Aisha's partnership year takes precedence for childcare logistics, and in exchange Sam's practice gets the first claim on any surplus money and one weekend a month. The rule is arbitrary and it is written down, and the arbitrariness is the point: a rule they chose is different from a pattern they resent.

Part 5 — The grandmother, defined. Sam's mother comes three days a week to help. The therapist asks what she decides and what she does not. The answer reveals that she has views about Bilal's feeding which she expresses to Sam and not to Aisha, and that Sam has been carrying them. The three-domain agreement from Weeks 3 and 7 applies unchanged: what she decides, what she does not, and the sentence that says so in front of everybody.

10.6a Four months later

The Thursday twenty minutes has happened eleven times out of sixteen. Aisha reports that the first three were "terrible" and that the fourth was the first conversation in four years in which nobody mentioned a child.

The miscarriage has been spoken about twice more, both times on a Thursday, both times briefly. The therapist has not run a grief protocol and does not intend to: the couple is doing the work in the place where the relationship is being rebuilt, which is more durable than a protocol.

The rule about careers has already been broken once, in Aisha's favour, and renegotiated without a session.

Sam's mother now says her views about feeding to both of them, which is worse in the short term and better in the long term. Aisha reports this with irritation and the therapist logs it as progress.

The honest limitation. Aisha has not said she loves him. In the final session she says, "I do not want to leave", which is a different sentence and is the one available at this stage. The therapist does not convert it into a declaration, and the case is closed with a review at six months.

What would have changed the plan. If the answer to "what is not on this line?" had been an affair, the sequence would have been disclosure, then a decision about the frame, then the developmental work. If either partner had disclosed violence — which the therapist screened for in the first session, as a matter of routine — the couple work would have stopped and safety work begun. If Aisha had said "I have decided", the treatment

would have become a separation-focused process, which is a different treatment with a different endpoint and a different literature.

10.7 Transcript: the unanswered question

Ten turns from the third session, at the point where the timeline has been drawn and the therapist asks what is not on it. The couple has been together nine years.

> **T1 Therapist:** I want to ask one more question about the line, and then we will stop drawing. (pause) What happened in this relationship that is not on it? — The question is asked once, flatly, after the couple has invested twenty minutes in the drawing, which is when they have something to protect and therefore something to add. **T2 Aisha:** (long silence) Nothing. It is all there. **T3 Therapist:** (waits) — The single most important behaviour in the transcript. The silence is not a technique for producing disclosure; it is the absence of pressure, which is the only condition in which disclosure occurs. **T4 Aisha:** (quietly) There is one thing. (pause) I was pregnant in 2021. In the August. And then I was not. **T5 Sam:** (turns to look at her) What? **T6 Therapist:** (to Sam, very quietly) You did not know. — Not a question. A statement that gives the partner a moment to be a person rather than a defendant. **T7 Sam:** (shakes his head) **T8 Aisha:** You were at the practice. The money had gone. My father was already ill. I thought — (stops) I thought if I told you it would finish you. — The reason for the silence, which is always a protection and never a deception, and the therapist's next move depends on recognising that. **T9 Therapist:** (to both, after a pause) I am not going to work on this today. I want it on the line, and I want you to know that it is on the line. We will come back to it when you have had a week to know that it is known. — The refusal to process. Almost every trainee wants to work this material immediately, and immediate processing of a four-year silence teaches the couple that disclosure produces a session rather than a relationship. **T10 Sam:** (to Aisha, not to the therapist) I would have wanted to know.

10.8 Laboratory: the couple timeline

Aim. To draw a couple timeline in twenty-five minutes, identify the live developmental task, and convert it into one task without running a communication exercise.

Procedure. Trios: therapist, two partners. The trainer briefs the couple with a nine-year history containing three transitions and one undisclosed event which one partner will reveal only if asked the unanswered question at the right moment. Twenty-five minutes, then ten minutes of feedback.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The drawing	Therapist drew it, or asked for a verbal history	Couple contributed, but unequally	Both partners' marks on the page, in the therapist's handwriting, with transitions in a third colour
The comparison	Treated the two lists as one	Noticed a difference	Used the difference as the finding and asked what each difference meant

Criterion	0–1	2–3	4–5
The transitions	Not marked	Marked, not used	Marked, and the clustering of lows named
The unanswered question	Not asked	Asked, then pursued immediately	Asked once, held, and the answer scheduled rather than processed
The task	A programme	One task, unscheduled	One task, scheduled in the session, with a prediction of how it will fail

The distinction that carries most of the marks. The top band requires the therapist to predict the failure of the task she has just set. A couple told that the first three Thursdays will be awkward is protected from concluding, on the first awkward Thursday, that the therapy is not working.

10.9 Six errors

Error 1 — Teaching communication skills to a couple whose problem is structural. A couple with no shared time, two careers and an undefined grandmother do not need to learn to listen; they need a Thursday. Skill without structure produces a couple who listen beautifully in a session they cannot schedule.

Error 2 — Treating the loudest complaint as the task. The market and the bath time are the surface. The therapist who negotiates the surface will be negotiating it again in a month, with a different example.

Error 3 — Taking sides with the more articulate partner. In every couple there is one who speaks in paragraphs and one who speaks in sentences, and the therapist's sympathy follows the paragraphs. The correction is arithmetic and it should be visible: equal turns, equal minutes, and the therapist counting.

Error 4 — Processing a disclosure immediately. The transcript above is the exemplar. A four-year silence is not resolved in the session in which it ends; it is registered, held, and returned to.

Error 5 — Believing the "empty nest". A couple who present at the point of the last child's departure have usually been in difficulty for a decade. The presentation is late, not sudden.

Error 6 — Ignoring the household beyond the couple. Three adults raising two children is not a couple with an in-law problem; it is a caregiving network with an undefined hierarchy. The unit of treatment is whatever is actually raising the children, which is frequently more than two people.

10.10 Culture, faith and the couple

Marriage as a contract between families. In much of the world the couple's formation is not a private decision but a treaty: bride price, family approval, religious ceremony, and obligations that run in both directions for decades. The Western developmental map's "formation" phase, with its emphasis on the couple choosing each other, describes one version of a universal task. In a marriage arranged with family involvement, the task is not choosing but authoring — the couple must become the authors of a relationship that others initiated, and the clinical question is whether they have ever been given the opportunity.

Polygyny and the couple unit. Where a household includes more than one wife, the couple is not the clinical unit and a therapy designed for two people will misdescribe the system. The relevant tasks are allocation (of time, money, and the husband's presence), hierarchy (whose children, whose room, whose authority), and the children's experience of the arrangement. The therapist who has a position on polygyny will be unable to work; the therapist who can map the allocation without endorsing or condemning it can.

Religion as a third party in the room. Couples frequently hold their marriage inside a religious framework that supplies obligations, rituals and an audience. This is a resource: it supplies a shared meaning, which is on the list of what predicts recovery. It is also a constraint: a couple who believe divorce is unavailable may present late, when the difficulty is chronic. Both are facts to be assessed, not positions to be taken.

The wife who earns more. A specific and increasingly common presentation in the households this book is written for: a wife whose income exceeds her husband's, in a culture where provision is part of the definition of masculinity. The clinical picture is not usually the couple's own discomfort; it is the discomfort of the wider family, conducted through remarks, and it produces a husband who withdraws from decisions and a wife who resents the withdrawal. The intervention is to name the external audience and to build the couple's own position on it, which is Week 10's early-marriage task arriving late.

> **DEBATE**
> "A couple should stay together for the children." Argue for: the evidence on children's outcomes is about conflict, not separation, and a low-conflict marriage is a better environment than a divided household in most circumstances. Argue against: a high-conflict marriage is worse for children than a clean separation, the phrase is used to trap people, and the children in question learn the relationship they are shown. Then state what you would actually say to a couple who tell you this is their reason for staying, and what you would ask them first.

10.11 Evidence note

What the research supports. The observational literature on couple interaction is strong: contempt, criticism, defensiveness and stonewalling predict deterioration, and repair attempts predict recovery, with effect sizes that survive controls for initial satisfaction. The association between shared meaning and relationship stability is consistent across studies. Emotionally focused couple therapy has the best evidence base of the couple modalities, with replicated effects in randomised trials and follow-up to two years (MFT 710).

What is weaker. The stage models. The six-phase map is a teaching device assembled from family life cycle theory and clinical observation; it is not a validated developmental sequence, and couples vary enormously in the order and timing of the tasks. The claim that the couple should be the primary unit of treatment is a value as much as a finding, and in households with distributed caregiving it is frequently wrong.

What is not supported. That communication training alone improves satisfaction in distressed couples — the evidence is that skill acquisition without change in the couple's structure and emotional bond does not transfer. That the empty nest is a crisis for most couples — the average finding is an improvement in satisfaction after the last child leaves, for couples who were not already distressed.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same second child, born into a couple with a Thursday and a defined grandmother, is a manageable strain; born into a couple with no shared time and an undefined authority, it is the point at which the relationship loses its structure entirely.

10.12 Key terms

Formation — the phase in which the couple becomes a distinct unit and each partner chooses it.

The unchosen couple — a relationship entered without a genuine decision; presents later as resentment without an argument.

Ritual — a recurring, meaningful practice that marks the relationship's continuity.

Negative sentiment override — the state in which a partner's neutral behaviour is read as hostile.

Repair attempt — any move that interrupts an escalating exchange; the best single predictor of recovery.

The four horsemen — criticism, contempt, defensiveness, stonewalling.

Mental load — the noticing and planning of household work, distinct from its execution.

Symmetric struggle — a conflict in which both partners want the same thing from each other, and neither can yield.

Shared meaning — the couple's common account of what their life is for; a predictor of stability.

Couple timeline — the drawn history of high points, low points and transitions.

10.13 Self-test

1. Name the six phases and one failure mode for each.
2. Why is the "unchosen couple" the most consequential formation pattern, and what is the diagnostic question?
3. What is the arithmetic question of the young-children phase, and what does a number below two hours predict?
4. Distinguish the cascade from the repair trajectory, and name the single strongest predictor in each direction.
5. Why does the therapist refuse to process the miscarriage in the session in which it is disclosed?
6. What is the third column of the division-of-labour audit, and why does it matter more than the first two?
7. How would you work with a couple in which one partner will not attend?
8. In the Aisha and Sam case, what were the three findings, and which one changed the treatment?

Answers. (1) Formation — one partner never chose; early marriage — in-law triangle; young children — co-parenting replaces partnering; adolescent children — escalation or abdication; launching — the nest that was already empty; later life — caretaking replaces relating. (2) Because there was never a decision to point at, so the resentment has no argument attached and cannot be negotiated; the question is "when did you decide?" (3) Hours per week alone, awake, not discussing logistics; below two predicts a bond with no maintenance and a slow loss of the couple's separate existence. (4) The cascade is criticism, contempt, defensiveness and stonewalling; the repair trajectory is the presence of repair attempts. Contempt is the strongest predictor of dissolution, repair the strongest of recovery. (5) Because a four-year silence cannot be resolved in the session in which it ends, and attempting it teaches the couple that disclosure produces a session rather than a relationship. (6) Who notices when it is not done — the measure of mental load, which is the actual source of the resentment. (7) Work with the attending partner on their part, be explicit that no case will be built against the absent partner, invite once in the therapist's words with a consultation-only offer, and set a review date. (8) The bond was built as a joint project rather than a separate structure; an unmourned loss had organised four years; and the conflict was a symmetric struggle about whose career accommodates whose. The first changed the treatment, because it moved the work from conflict to construction.

10.14 Reflective task

Draw the timeline of a couple you know well — your parents, relatives, or your own if you have one — with five high points and five low points, and mark the transitions. Then answer: which transition produced the largest low point, and what did the couple do about it? Most students find that the couple did something practical and nothing relational, which is not a failure; it is what most couples do, and it is the reason this week's techniques are so simple, and so easy to forget in a room where two people are angry

10.15 Further reading

- Gottman, J. What Predicts Divorce? — the observational literature behind the cascade.
- Johnson, S. Hold Me Tight — the emotionally focused account of the negative cycle, written for couples.
- McGoldrick, M. et al. Genograms — the family life cycle chapters that this week's map draws on.
- Carter, B. and McGoldrick, M. The Expanded Family Life Cycle — the source of the phase structure.
- Fowers, B. "The limits of communication training" — why skills without structure do not transfer.

Next: Week 11 — Parenting across stages: talking to parents well.

Week 11 — Parenting across stages: talking to parents well

Parents do not need lectures; they need **accurate, specific, hopeful information in plain language**, and they need it from someone who is not judging them. That is a skill, it is teachable, and it is assessed in this course.

The four parts of the skill.

1. **Normalise first.** "What you are describing is very common at this age."
2. **Give one developmental fact.** "At four, the part of the brain that manages impulse is still being built, so he cannot yet do what you are asking consistently."
3. **Give one concrete action.** "Try naming the feeling and offering two acceptable choices."
4. **Set a review.** "If this is still happening in three weeks, or if X or Y appears, come back."

What to avoid. Technical vocabulary; predictions ("he will grow out of it"); blame; over-reassurance when the concern is real; and overwhelming a parent with five recommendations.

Why this week is the hinge of the course. Every previous week has produced information a parent needs. This week is about delivering it. A therapist with an accurate formulation and a delivery that shames the parent will produce a family that does not

return, and the formulation will have been worth nothing. The skill is not a soft addition to clinical work; it is the mechanism by which clinical work reaches the people who do it.

The developmental spine of the week. The same four parts are used at every age, and the content changes with the stage. Every previous week supplies the content: Week 3 for the infant (synchrony, the still face, the meaning of a cry), Week 4 for the toddler (the limit formula, the language differential), Week 5 for the preschooler (play, pretend, the false-belief shift), Week 6 for the school-age child (industry, monitoring, the report card), Weeks 7 and 8 for the adolescent (privacy, negotiation, risk). This week is where the course becomes usable in a room with a worried parent.

> **PRACTICAL (OBSERVED)**
> A ten-minute parent-feedback role-play using the four parts, with a peer playing a parent who is frightened and slightly defensive. The exercise is assessed (see 11.5), and the standard is not warmth; it is **accuracy plus one action plus a review**.

11.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	Why advice fails	The difference between information and instruction; why parents have already had advice	List the advice they have given and been ignored
8–20	The four parts	The structure, demonstrated on a toddler case	Identify the four parts in a recorded feedback
20–34	Normalising without dismissing	The two errors: "it is normal" as a brush-off, and alarm as a default	Rewrite three dismissing sentences
34–46	The developmental fact	Which fact to give at which age, and how to say it without jargon	Choose the fact for four ages and say it in one sentence
46–58	One action	Why one, and how to choose it	Choose one action for four cases and defend it against a rival
58–70	The review	Setting a review that is not a threat and not a promise	Write three review sentences
70–82	When the parent disagrees	The four reasons a parent rejects a formulation, and the move for each	Role-play a rejection and hold the frame
82–90	The assessment	How the observed role-play is marked	Draft their role-play plan

The teaching decision that makes the week work. Trainees default to either reassurance or instruction, and both are ways of ending a conversation. The lecture's job is to make the conversation open — to leave the parent with something to do and a reason to come back — and the way to do that is to demonstrate the failure first: the lecturer plays a therapist who reassures a worried mother for four minutes, and the class watches the mother's face close.

The sentence the lecture must land. "A parent who leaves your room feeling stupid will not do what you suggested, and will not come back to tell you it did not work."

11.2 Five concepts that organise the week

1. Parenting is a developmental task that changes. The parent has a task at each stage, and the tasks conflict. The parent of a toddler must hold limits; the parent of a seven-year-old must let go of the minute-by-minute view; the parent of a fifteen-year-old must negotiate. Parents who are given information appropriate to their child's age rather than to their own experience are being given something they cannot get from their own upbringing, which is why the developmental fact is the second part of the structure.

2. Information is not advice. Advice tells a parent what to do; information tells them how something works. Advice invites compliance or resistance; information invites thinking. A parent told "you should not shout" has been advised and will feel judged. A parent told "at four, a child's braking system is still being built, which is why shouting produces a pause rather than a change" has been informed, and can then decide what to do. The clinical discipline is to give information and let the parent derive the action — and to supply the action only if they cannot.

3. Normalising is a technical act, not a kindness. "That is very common" does three things: it lowers the parent's shame, which is what prevents disclosure; it locates the child in a population, which changes the parent's attribution from character to development; and it creates permission for the parent to say the next thing. It fails when it is used to close a conversation ("all children do that") or when it is untrue.

4. One action, chosen for feasibility. Five recommendations produce none. The action must be small enough to survive a bad week, specific enough to be observable, and chosen with the parent rather than for them. The test question is: "could you do this on the worst evening of the month?" If not, it is not the action; it is the aspiration.

5. The review is part of the treatment. A review date converts a suggestion into a plan, gives the parent permission to fail, and creates the conditions for the therapist to learn whether anything worked. The review should name the exception ("if he is still doing this in three weeks, or if he starts X, come back"), because an unconditional review is heard as "come back when you have failed again".

11.2a The four parts, worked at five ages

The table gives the same structure five times, with the content drawn from the weeks that produced it. It is the single most useful page in the module and it should be learned well enough to be used without notes.

Age	Normalise	The developmental fact	One action	The review
6 months	"Waking at night is very common at six months, and it is not a sign you have done something wrong."	"At this age he is learning that you still exist when he cannot see you, and that is why he calls."	"Go to him, keep the room dark and boring, and put him down drowsy rather than asleep."	"If it has not changed in three weeks, or if he stops feeding well, come back."
2 years	"Every child this age says no; it is the age, not your parenting."	"He has just discovered that he is a separate person, and 'no' is how he tests it."	"Name the feeling, hold the limit, offer two acceptable choices."	"Three weeks. If the hitting is worse, or if he stops talking, come back sooner."

Age	Normalise	The developmental fact	One action	The review
4 years	"The lying is extremely common at four and it is not a moral problem yet."	"At four he cannot always tell the difference between what happened and what he wishes had happened."	"Ask 'what happened next?' rather than 'why did you do it?'"	"Three weeks, and if the lying comes with other changes — sleep, appetite, friends — come back."
7 years	"Wanting to be first is normal at seven; the comparison has just arrived."	"At seven he begins to compare himself with other children, and losing becomes a fact about him rather than about the game."	"Name the effort rather than the result, once a day, in a sentence he can hear."	"A month. If he stops wanting to play at all, come back."
15 years	"Not talking to you is common and it is not permanent."	"At fifteen the private world is a developmental need, not a rejection; secrecy is what privacy becomes when there is no agreement."	"One question a day, at a stated time, and not at the door."	"Two weeks. If he stops seeing his friends altogether, come back at once."

How to use the table without sounding like a table. Do not deliver the four parts in order as a script. Deliver the normalising sentence, then wait, then give the fact in the parent's own vocabulary, then the action. The structure is for the therapist's head; the parent should hear a conversation.

11.2b The five questions parents are actually asking

Underneath almost every parental question is one of five concerns, and a therapist who answers the question asked without hearing the concern underneath will be asked it again in different words.

1. "Is something wrong with my child?" The question is about diagnosis and it is asked in almost every first session, in some form. The answer that works is not "no" and not "we will see", but a description of what is and is not yet known: "what you are describing is common at this age; the things that would make me worried are X, Y and Z, and he does not have them. I would be looking for them at the review."

2. "Is it my fault?" The question is about blame, and it is usually unspoken. A therapist who does not address it will find the parent either defensive or collapsed, and neither can implement an action. The sentence that works is factual rather than absolving: "children's development is affected by many things, and a parent who is asking this question is usually doing more than they think." Then move to the action, because blame is not resolved by argument but by competence.

3. "Will it get better?" The question is about the future and it is a request for a prediction. Do not predict. Offer the shape instead: "the pattern you are describing usually changes as language develops, which is in the next year. What I cannot tell you is how much, and I would rather review it in three weeks than guess."

4. "What do I do tonight?" The question is about the immediate. It is the most answerable and the most frequently unanswered, because clinicians are trained to formulate and parents need an action before bedtime. Answer it, even if the answer is provisional.

5. "Do you think I am a bad parent?" The question is about judgement, and it is asked by the parents who are most competent and most exhausted. The answer must be honest and behavioural: "I think you are the person who noticed, brought him here, and is prepared to try something different, which is what the parents I worry about do not do."

The rule. Answer the question underneath, and answer it in the first ten minutes. A parent whose underlying concern has been met can then hear information; a parent whose concern is still unanswered will hear every subsequent sentence through it.

11.3 The method: a parent feedback conversation, ten steps

Twenty minutes, in a room with one or two parents, after an assessment. The structure is fixed because improvisation is what produces the two failure modes: a lecture, or a reassurance.

Step 1 — Ask what they think is happening. Before saying anything. The parent's own formulation is the starting point, and it identifies which of the five questions is live.

Step 2 — Ask what they have already tried. Their attempts are the first resource of the plan and the first evidence of their competence. Name one aloud: "you have already done the hardest part, which is changing what you do at bedtime."

Step 3 — Give the finding in one sentence, without jargon. "He is not behind in his development. He is a boy with a lot of energy and a hearing test that has never been done." One sentence, and then stop.

Step 4 — Normalise, and be specific about what is normal. "What you are describing is very common at five, and the reason it is common is that the part of the brain that stops an impulse is still being built."

Step 5 — Give the one developmental fact they will remember. One. Not three. It must be sayable in a single sentence and repeatable to a spouse that evening.

Step 6 — Check understanding by asking them to say it back. "Say that back to me in your own words, because I want to know whether I said it clearly." This is not a test of the parent; it is a test of the therapist, and it is the step trainees most often skip.

Step 7 — Offer one action, and ask whether it is possible. "Could you do that on the worst evening of the month?" If the answer is no, the action is wrong, and the therapist adjusts until the answer is yes.

Step 8 — Write it on a page, in their words. Not in the therapist's. A page in the parent's own phrasing is followed; a page in clinical language is filed.

Step 9 — Set the review, with the exception named. A date, and the specific things that would bring them back sooner.

Step 10 — Ask the closing question. "What are you going to tell his father tonight?" Or the grandmother, or the aunt. The question surfaces the household's information system, and it frequently reveals that the plan will be contradicted within an hour of the parent getting home.

11.3a Telling a parent something difficult

Some feedback is not normalising. A developmental delay, a probable diagnosis, a safeguarding concern, a recommendation that a child be assessed by another service — these require a different structure, and the difference is that the therapist is delivering news the parent will remember for years.

Before the conversation. Decide what the one piece of news is, in one sentence, and say it aloud twice before the session. Decide what you do not yet know, and be ready to say so. Arrange for both parents to be present if possible, and for someone to be with the parent afterwards. Book a longer slot than usual, and book the follow-up before the conversation begins.

The structure, in six moves.

1. **Signal.** "I want to talk about what the assessment found, and some of it is not what you were hoping for." Two seconds of warning, which is the difference between being told and being ambushed.
2. **The news, in one sentence, without hedging.** "His language is about two years behind where we would expect."
3. **Stop.** Silence. The parent needs time, and anything the therapist says in the next thirty seconds will not be heard.
4. **The meaning, in the parent's terms.** "I know that what you are hearing is that the thing you were afraid of is true." Naming the fear accurately is more useful than reassurance.
5. **What is not changed.** "He is the same boy he was this morning. He is affectionate, he is good with his hands, and he has a mother who noticed." True sentences only.
6. **What happens next, concretely.** A referral, a date, a name, a telephone number, and a follow-up appointment with the therapist. Nothing is left open.

What not to do. Do not deliver the news in the last five minutes of a session. Do not use a diagnostic label without the explanation that goes with it. Do not follow the news with a programme, because the parent will not hear it. Do not say "I am sure it will be fine" — the parent will remember that sentence, and it will be the one that makes them distrust everything else you said.

Afterwards. Write to the parent, briefly, within a week: what was said, what happens next, and the telephone number. A letter after difficult news is one of the few interventions in this book with no downside, and it is almost never done.

11.4 Three techniques

Technique 1 — The one-sentence formulation, said to the parent's face. Write the formulation as one sentence, in plain language, and say it: "your son is not naughty; he is a five-year-old with an undiagnosed hearing problem, and he has been getting into trouble for two years for something he cannot help." The sentence does three things at once: it explains, it removes blame and it implies the action. If the sentence cannot be written in plain language, the formulation is not yet finished.

Technique 2 — The three-week review card. A card with the date, the action, and the two exceptions that would bring the parent back sooner. The card is the treatment's memory, and it survives a fortnight in which the parent is too busy to think about any of it.

Technique 3 — The household rehearsal. Ask the parent to rehearse, in the session, the sentence they will use with the other caregiver. Then ask them what that caregiver will say back, and rehearse the answer. Families do not fail at the clinic; they fail at the front door, when the grandmother says "in my time we did not do that".

11.5 The assessed role-play

The parent-feedback role-play is worth twenty per cent of the module mark. The assessment is deliberately public: each student performs in front of the group for ten minutes, and the group marks against the same rubric the examiner uses, which is the fastest way to teach a skill that is normally learned by watching someone else do it badly for years.

The set-up. A brief of no more than 150 words, containing the age of the child, the presenting concern, one thing the parent has already tried, one thing the parent believes that is wrong, and one live emotion in the room (shame, anger, exhaustion, or fear of a partner's reaction). The student has ten minutes with the parent and no notes.

What is being marked. Not warmth. Not the accuracy of the developmental content in the abstract — the examiners accept any defensible fact. What is marked is the structure in use: did the parent leave with (a) a normalising statement they could repeat, (b) one fact, (c) one action they had agreed to, and (d) a review date with an exception named.

The four ways students fail.

- **The lecture.** Three facts and five recommendations in ten minutes, delivered to a parent who has not been asked a single question. The mark is low irrespective of accuracy, because nothing will be done.
- **The reassurance.** "Do not worry, he will grow out of it", delivered before any assessment has been described. This fails on the rubric's accuracy criterion, and it is the commonest failure among trainees who are naturally warm.
- **The collusion.** Agreeing with the parent's formulation because disagreeing feels unkind ("yes, his father is very strict"). This fails, because a therapist who agrees with one parent against the other has joined the household's conflict.
- **The hedge.** Never stating a finding, so that the parent leaves with no information at all. This is common among trainees who have been taught that they must not diagnose,

and the correction is that describing is not diagnosing: "his language is behind what we would expect, and that needs an assessment" is a description, and it is what the parent came for.

11.5a The parent who arrives with a diagnosis from the internet

The commonest new complication in this work, and it has a structure that can be taught in ten minutes.

What arrives. A parent who has read about a condition — attention deficit, autism, attachment disorder, sensory processing, "highly sensitive child", or a term from a social media video — and who is either relieved to have a name or frightened by it. The parent may ask the therapist to confirm the label, or may resent any suggestion that the label is not the whole picture.

What not to do. Do not dismiss the reading. A parent who has spent three months researching is a parent who is trying, and dismissing the research dismisses the effort. Do not confirm a diagnosis you have not made, which is both a scope error and a clinical one. And do not argue about the label, which converts the session into a debate about a word.

The four-step response.

First, ask what led them to it. "What did you see that made you look that up?" The answer produces the behavioural description the therapist needs, and it is usually better than the label.

Second, separate the description from the name. "The things you are describing — not sitting still, covering his ears, needing the same routine — are real and I saw two of them today. The name is a decision for an assessment, and the assessment is the way to get it right."

Third, say what would change either way. This is the step that resolves most of these conversations: "whatever the assessment says, the things we would do first are the same — protect his sleep, give him one instruction at a time, and find the thing he can do for twenty-five minutes." A parent who discovers that the immediate plan does not depend on the label stops needing the label today.

Fourth, name the referral honestly. If an assessment is indicated, say so and give a date. If it is not, say what the therapist would need to see for it to become indicated, in behavioural terms. A parent who has been told the threshold can watch for it, and watching is itself useful.

The sentence that holds the whole position. "You may be right, and I am not going to pretend to know before we have looked. What I do know is what to do this month." Parents accept uncertainty about the future far more readily than they accept being told that their observation was wrong.

11.6 Case: the mother who has been told she is too soft

Referral. Chidi, seven, in Primary 2, brought by his mother, Adaeze, 33, a tailor, on the school's recommendation. The school's note: "Chidi does not sit still, does not finish his work, and is rough with the smaller children. He is bright. The mother is not firm with him."

The household. Adaeze; Chidi; a daughter, 4; Adaeze's husband, Emeka, 38, a long-distance driver, home four days a month; and Emeka's mother, 61, who lives in the same compound and who believes Chidi is spoiled. Adaeze runs her tailoring business from the front room.

The presenting complaint, in the mother's words. "They say I am too soft. His grandmother says it. His father says it when he comes home and then he shouts at him and it is worse for a week. The teacher says I should beat him. I do not want to beat him. I do not know what to do and I am tired."

What the assessment finds. Vision normal. Hearing never tested. Language: Chidi's vocabulary is good, his sentence construction is age-appropriate, and he cannot follow a three-step instruction in a noisy room — which the therapist tests by giving one, twice, in the consulting room and in the corridor. Attention: he can sustain a preferred activity for twenty-five minutes and a non-preferred one for four. Sleep: he goes to bed at ten, shares a bed with his sister, and wakes when the compound is noisy at five. Diet: he eats once a day, in the evening, because the family eats late. School: forty-eight children in the class, and his seat is at the back.

The five findings.

1. **There is no evidence of a conduct problem and there is evidence of an unassessed hearing and attention picture.** The therapist does not diagnose ADHD; she describes what she saw, names two things that must be excluded, and refers for hearing and for a developmental assessment.
2. **The child is not naughty at four in the afternoon; he is a seven-year-old who has eaten nothing since the previous evening, slept six hours, and is being asked to sit in a class of forty-eight.** The environmental account is not an excuse; it is a set of variables that can be changed.
3. **The mother is being managed by three authorities and supported by none.** The school blames her, the grandmother blames her, the father arrives monthly and shouts. Her "softness" is not a parenting style; it is what remains of a woman with no allies.
4. **The father's monthly arrival is the family's actual behavioural cycle.** Chidi is worse for a week after each return, which the school reads as deterioration and which is better understood as a boy who has been shouted at by a man he sees four days a month.
5. **The most useful intervention in the case has nothing to do with Chidi's behaviour.** It is an alliance with the mother that makes her the author of a plan rather than the object of three plans.

The feedback conversation, in order. The therapist asks what Adaeze thinks is happening. She says: "he is a good boy but he is stubborn, and I am too soft." The therapist asks what she has tried. She lists six things, including two she has invented herself, and the therapist names one: "you have already changed his bedtime once and it worked for a while. That is not a soft mother; that is a mother who experiments."

The one-sentence finding. "Chidi is a bright seven-year-old whose hearing has never been tested and who is being asked to sit still in a class of forty-eight on six hours' sleep and no breakfast, and the behaviour you are being blamed for is the predictable result."

Adaeze cries. The therapist waits. Then: "and you have been told by three people that the problem is you, which is why you are tired."

The four parts, delivered.

Normalise: "Boys of seven who cannot sit still in a hot classroom are extremely common, and it is not a sign that a mother has failed."

The fact: "At seven, a child can hold one instruction in mind but not three, especially in a noisy room. He is not ignoring you; he is losing the middle of the sentence."

The action: one. "Breakfast before he leaves the house, even if it is bread and a boiled egg, and say the instruction one step at a time: 'put your shoes on', then when that is done, 'bring your bag'." The therapist does not give the sleep change, the hearing referral and the school request as actions; those are the plan, and they are on the page, but only one thing is asked for this week.

The review: "Three weeks. If the school reports he is hurting another child, come back at once. If the hearing test comes back abnormal, we will change the whole plan."

The rest of the plan, on the page. A hearing test, booked by the clinic. A letter to the school asking for a seat at the front and one named adult. A developmental assessment referral. A conversation with the grandmother, held by Adaeze, rehearsed in the session, in which she says one sentence: "the doctor says his hearing has never been tested and we are doing that first." And a request to the father, by telephone, in which Adaeze says one sentence she has rehearsed: "when you come home, I need you to ask him about school before you tell him anything."

The grandmother, handled. The therapist does not invite the grandmother to the session, and explains why: the household has three authorities and no hierarchy, and adding a fourth voice — the therapist's — will not produce agreement. Instead Adaeze is given the sentence to say, and the authority to say it, because the grandmother's objection is not to the plan; it is to being excluded from it. The sentence makes her the person who is informed first.

11.6a Six weeks later

The hearing test is normal, which the therapist records as a genuinely useful negative: two years of "not listening" have been excluded as a hearing problem, and the school has been told so in writing.

Chidi is eating breakfast. The three-step instruction has become a two-step instruction, because Adaeze found that three was still too many, and she adjusted it herself. This is the outcome the week is aiming for: a parent who modifies a plan is a parent who owns it.

The school has moved his seat and named a teacher. The rough behaviour has reduced, not disappeared. The developmental assessment has a date in eleven weeks, and the therapist has written to say that the assessment is pending and that the school's "not firm" note should be removed from the file, which it is.

Emeka has been telephoned once by the therapist, at Adaeze's request, for four minutes. He said little. He did, on his next return, ask Chidi about school before saying anything else, and Adaeze reports that Chidi talked for eleven minutes.

The honest limitation. The grandmother's view has not changed. She now says it less in front of Chidi, which is the achievable outcome and is worth having: a child who hears one criticism instead of three is better off even when the third person's opinion is unchanged.

11.7 Transcript: the ten minutes, annotated

The transcript is the assessed role-play's model answer, compressed. The annotations name the part of the structure being used at each turn.

> **T1 Therapist:** Before I tell you anything, I want to know what you think is happening. — Step 1. The single sentence that distinguishes this conversation from a consultation with a verdict. **T2 Adaeze:** He is a good boy. But he is stubborn, and I am too soft. Everybody says so. **T3 Therapist:** Who is everybody? — Three words. The answer identifies the household's authority structure, which is the actual clinical problem, and it takes eight seconds. **T4 Adaeze:** The teacher. His grandmother. His father when he comes. **T5 Therapist:** And what have you tried? — Step 2. The question that finds the parent's competence, which in this case has been invisible to her for two years. **T6 Adaeze:** (lists six things) **T7 Therapist:** You changed his bedtime in January and it worked for a while. That is not a soft mother. That is a mother who experiments. — Naming competence with evidence. Note that the therapist does not say "you are a good mother", which is a verdict the parent cannot use; she describes a behaviour. **T8 Adaeze:** (quietly) Nobody has said that to me. **T9 Therapist:** Here is what I found, in one sentence, and then we will talk about it. (pause) Chidi is a bright seven-year-old whose hearing has never been tested, who is being asked to sit still in a class of forty-eight, on six hours' sleep and no breakfast. The behaviour you are being blamed for is the predictable result. — Step 3, and the sentence is prepared, plain, and contains no diagnosis. It also answers the question underneath, which is "is it my fault?" **T10 Adaeze:** (cries) **T11 Therapist:** (waits, offers the box of tissues, says nothing) — Step 3b. The silence after the finding. Anything said here will be remembered instead of the finding, and the finding is what she needs to remember. **T12 Therapist:** (after about twenty seconds) Say that back to me, in your words, so I know whether I said it clearly. — Step 6, and the pronoun is deliberate: the fault for an unclear sentence is assigned to the therapist, which is what makes the check honest.

11.7a When the complaint comes from the school

A frequent configuration: the referral originates with a school, the parent arrives defensive, and the therapist is positioned between an institution and a family. Three practices keep the therapist from becoming the school's representative.

Practice 1 — Distinguish the observation from the verdict. "Chidi does not finish his work" is an observation. "The mother is not firm with him" is a verdict, and it is a verdict about an adult who is not in the room. The therapist works with the first and declines the second, in writing if necessary.

Practice 2 — Ask the parent what the teacher would say if asked. The answer is often accurate and sympathetic, and it reveals that the parent already knows the school's view and has been carrying it home. Naming that — "you have been told this for two years and you have kept coming back" — is frequently the moment the alliance forms.

Practice 3 — Give the parent something to give the school. A sentence, a request, a letter. A parent who arrives at the school gate with a specific request from a clinic is in a different position from a parent who arrives with an explanation, and the difference is not rhetorical: schools respond to requests with names and dates attached.

The boundary. The therapist does not become the family's advocate against the school, and does not become the school's agent inside the family. The position is with the child's development, which both institutions say they are serving, and which is the only ground on which both can be held to account.

11.8 Laboratory and the marking scheme

The laboratory runs in two parts across two weeks. Part one is rehearsal with peers, unmarked. Part two is the assessed role-play in front of the group.

Marking scheme (20 marks).

Criterion	Marks	What the top band looks like
Structure in use	6	All four parts present and in order, without the parent noticing a structure
Accuracy	4	The developmental content is correct for the age, and no diagnosis is made in the room
One action	4	One action, agreed rather than prescribed, tested against the worst evening of the month, written in the parent's words
The review	3	A date and a named exception, both written
Holding the frame	3	When the parent disagrees, the therapist neither capitulates nor argues; she names the disagreement and offers a review

The pass standard. A student who reassures for ten minutes and gives no action has failed, however warm the room felt. A student who gives five actions has also failed, however accurate the content.

The role-play briefs. Four, rotated. Each contains one emotion and one false belief.

Brief A — the exhausted mother of a two-year-old. She believes the problem is that he "does not respect her". The emotion is shame. The hidden question is "is it my fault?"

Brief B — the father of a nine-year-old who refuses school. He believes the boy is lazy. The emotion is anger, covering fear. The hidden question is "will he turn out like me?"

Brief C — the mother of a fifteen-year-old who has been caught lying. She believes she must search the room. The emotion is fear. The hidden question is "what do I do tonight?"

Brief D — the parents of a four-year-old with a speech delay. They believe he will talk when he is ready, as his uncle did. The emotion is resignation. The hidden question is "is something wrong with my child?"

The debrief question, asked of the group every time. "What did the parent leave with that they did not have when they came in?" The answer must be a sentence, an action and a date. If the answer is "they felt better", the role-play failed, and the group must be able to say why.

11.9 Six errors

Error 1 — Reassuring before assessing. The commonest error in the whole of paediatric practice, and the most damaging, because the reassurance is remembered and the assessment is not. Reassurance after an assessment is a finding; before one, it is a guess with a kind face.

Error 2 — Giving the parent the therapist's formulation. "He has a regulatory difficulty secondary to maternal inconsistency" is a sentence for a file. Said to a parent, it produces shame and no action.

Error 3 — Five recommendations. The parent will implement none and will not say so. One action, chosen for the worst evening of the month.

Error 4 — Agreeing with one parent against the other. "Yes, his father is very strict" feels supportive and is a coalition. The therapist's position is with the household's capacity to change, which requires both parents in the room eventually.

Error 5 — Treating the parent as the patient without saying so. Sometimes the parent is the focus, and when that happens it must be said: "the work I am proposing is with you, not with him, and I want to be clear about that." A parent who discovers halfway through that they have become the client will leave.

Error 6 — Leaving the household's other authorities out of the plan. The grandmother, the aunt, the father on the telephone. A plan that requires a parent to contradict three people alone will fail at the front door, and the therapist will conclude that the parent was not motivated.

11.9a Six sentences that do harm, and what to say instead

Small language choices do more damage in parent feedback than any amount of wrong content. The six below are the ones trainees use most.

"He is doing it for attention." True and useless, because attention is a legitimate need and the sentence implies a fault. Instead: "he has found that this is the way to get you, and we are going to give him a better way."

"He is testing the boundaries." A metaphor that makes the child sound like a strategist. Instead: "he is finding out whether the rule is the same on a bad day, which is what every child does at this age."

"You need to be consistent." An instruction to have a quality, not to do a behaviour, and it is heard as "you are failing." Instead: "pick one rule to hold this week, and let the others go."

"In my experience..." Places the therapist's experience above the parent's knowledge of their own child, and invites a comparison the therapist will win. Instead: "what does he do when you try that?"

"He will grow out of it." A prediction, usually accurate, always remembered, and it prevents the review. Instead: "this pattern usually changes as language develops. I would rather see you in three weeks than guess."

"There is nothing wrong with him." Meant kindly, heard as "your concern is imaginary". Instead: "I did not find the things that would worry me today. What I did find is X, and that is worth working on."

The exercise, in the laboratory. Students are given these six sentences on a card and asked to deliver each one to a partner in role, then to deliver the replacement, and to say which one produced a response. The difference is felt immediately and is remembered for years.

11.10 Culture, faith and talking to parents

The authority to say it. In many households a young clinician, and particularly a young woman, will not be heard on the subject of child-rearing by an older relative. This is not resistance to the content; it is a fact about standing. The workable moves are three: route the information through the person who has standing (a paediatrician, a religious leader, a respected aunt); give the parent a sentence that carries the clinic's authority ("the doctor says his hearing has never been tested"); and never require the parent to win an argument with a grandmother in order to follow the plan.

Corporal punishment. A substantial proportion of the parents in these clinics were beaten and believe that they turned out well, and many schools still recommend it. The therapist who opens with a moral position will end the conversation. The sequence that works is: ask what the parent believes and why; give the developmental fact (what beating does to the behaviour being targeted, over what timescale); give one alternative that is specific and feasible; and accept that the change may be partial. A parent who shouts less is a better outcome than a parent who leaves the clinic believing they have been judged.

Faith and the child's condition. Where a child's difficulty is understood spiritually, the parent may be pursuing prayer and may be advised by a religious authority not to accept a referral. The clinical position is to work with the framework: ask what the prayer is for,

offer to speak with the religious authority if the parent wishes, and hold one non-negotiable — that a hearing test and a developmental assessment are not acts of unbelief. Framing them as seeing what God has made is not a manipulation; it is a translation, and translation is part of the job.

The father on the telephone. In households with labour migration or long-distance work, the father is present by telephone and his authority is exercised remotely. A plan that does not include him will be reversed by him. The four-minute telephone call is a clinical instrument: one sentence from the therapist, one question to him, and one agreement. It is worth more than four sessions with the mother alone.

> **DEBATE**
> "A therapist should not tell a parent what to do." Argue for: advice creates dependence, parents know their children, and the evidence on autonomy support favours collaboration over instruction. Argue against: parents come for advice, a refusal to give it is a withholding of expertise, and a parent with a seven-year-old who has not eaten breakfast needs an instruction rather than a reflective conversation. Then state your rule for when you instruct and when you ask, and give the case that would make you break it.

11.11 Evidence note

What the research supports. The content of the four parts is well supported: psychoeducation about normative development reduces parental anxiety and increases the accuracy of parental attributions; specific, behavioural, positively framed instructions are followed more often than general advice; and brief, structured parenting interventions with a scheduled review produce better outcomes than open-ended support. The finding that parental attributions — whether the parent believes the behaviour is intentional — predict both parental response and treatment outcome is robust and is the empirical basis for the normalising step.

What is weaker. The evidence on how to deliver feedback is thinner than the evidence on what to deliver. The four-part structure is assembled from clinical teaching and from the psychoeducation literature rather than tested as a package. The claim that one action outperforms five is plausible, consistent with the implementation literature, and not directly tested in this form.

What is not supported. That reassurance reduces anxiety when the concern is real — the evidence suggests the opposite, because a parent whose concern is dismissed becomes more anxious and less likely to disclose. That insight alone changes parenting behaviour. And that a single feedback session is sufficient for established difficulties; the evidence favours brief structured contact with review, which is what the fourth part exists to create.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same feedback, given to a mother with three authorities against her and given to a mother with one ally, produces two different families, and the difference is not in the feedback.

11.12 Key terms

Normalising — locating the child's behaviour in a population, to reduce shame and change attribution.

Attribution — the parent's belief about why the child behaves as he does; intentional versus developmental attributions predict different parental responses.

One action — a single, feasible, observable behaviour agreed with the parent.

The exception — the specific change that would bring the family back sooner than the review date.

Household rehearsal — practising the sentence the parent will use with the other caregivers, and their likely reply.

Standing — the social authority to be heard on a given subject, which is not the same as expertise.

Psychoeducation — structured information about a condition or a developmental stage, given to reduce distress and improve management.

Coalition — an alliance between the therapist and one family member against another; the characteristic error of sympathetic practice.

11.13 Self-test

1. Give the four parts of the feedback skill and say what each one prevents.
2. Why is reassurance before assessment an error, and what is the evidence?
3. Name the five questions parents are actually asking, and give one sentence that answers each.
4. What is the test question for choosing an action, and why is it asked rather than assumed?
5. Why does the therapist in the Chidi case not invite the grandmother to the session?
6. What are the four ways students fail the assessed role-play, and what do all four have in common?
7. How would you handle a parent who believes the child's difficulty is spiritual and has been advised not to accept a referral?
8. What is the "exception" in a review, and why is an unconditional review heard as a threat?

Answers. (1) Normalising prevents shame and closes the disclosure problem; the fact prevents blame and changes attribution; the action prevents paralysis; the review prevents abandonment and creates the conditions for the therapist to learn the outcome. (2) Because it is a guess delivered with a kind face and it is remembered instead of the assessment; the evidence is that dismissing a real concern increases anxiety and reduces later disclosure. (3) Is something wrong with my child — "what you describe is common at this age, and the things that would worry me are X, Y and Z, which he does not have"; is it my fault — "a parent asking that question is usually doing more than they think"; will it

get better — "this pattern usually changes as language develops; I would rather review than guess"; what do I do tonight — give the action; am I a bad parent — "you noticed and you came, which is what the parents I worry about do not do". (4) "Could you do this on the worst evening of the month?" — asked because feasibility, not correctness, determines whether an action is carried out. (5) Because the household has three authorities and no hierarchy, and adding the therapist's voice produces a fourth rather than an agreement; the sentence given to the mother makes the grandmother the person informed first, which addresses the actual objection. (6) The lecture, the reassurance, the collusion, the hedge — all four end the conversation rather than opening it. (7) Work inside the framework: ask what the prayer is for, offer to speak to the religious authority, and hold one non-negotiable — that a hearing test and a developmental assessment are not acts of unbelief, since they are ways of seeing what has been made. (8) The specific change that would bring the family back sooner; without it the review is heard as "come back when you have failed again".

11.14 Reflective task

Record yourself giving feedback to a colleague in role, and listen to it twice. On the first listening, count the number of recommendations you gave. On the second, listen for the moment your voice changed when the colleague disagreed with you. Most trainees find between three and six recommendations and a voice that either rose or softened into agreement. Both are the same failure: the therapist leaving the frame in order to keep the room.

11.15 Further reading

- Hattie, J. and Timperley, H. "The power of feedback" — the general evidence on what makes feedback usable.
- Sanders, M. Positive Parenting Program materials — the worked examples of specific, behavioural instructions.
- Johnston, C. and Mash, E. "Parental attributions" — the empirical basis for the normalising step.
- Brazelton, T. Touchpoints — the classic account of developmental information given to parents well.
- Buckman, R. Breaking Bad News — the six-step protocol adapted in 11.3a.

Next: Week 12 — Later life: integrity, loss and caregiving.

Week 12 — Later life: integrity, loss and caregiving

Developmental tasks. Integrity versus despair (Erikson, from about sixty): the consolidation of a life into something that can be told as a whole, or the conviction that it did not add up. Adaptation to retirement, which is a role loss before it is a financial one. Bereavement, which at this stage arrives repeatedly and in sequence. Caregiving, which reverses the generational direction. Grandparenthood, which is the longest single role in most people's lives and the least theorised. And legacy — what is handed on, and to whom.

Clinical realities. Cognitive change must be distinguished from delirium, depression and dementia, and from hearing loss — which is a major and under-treated contributor to isolation and apparent confusion. In this age band more mistaken diagnoses are made by omission of a hearing test than by any other single error. Caregiving burden falls disproportionately on women and on the "responsible" child, and it frequently reopens sibling hierarchies that were settled forty years ago.

Grief. Use the dual-process model (Figure 22, MFT 712): healthy grieving oscillates between loss-oriented and restoration-oriented activity, and the oscillation is the mechanism, not a sign of avoidance. In many cultures, mourning is collective and prescribed; the clinician's task is to support the rites rather than to impose a private model of "working through". A widow who is surrounded by prescribed mourning for forty days and then alone is not in a pathological grief; she is in a transition that the rites mark and the culture does not follow through.

Why this week is difficult to teach. Because it concerns the end of the story, and because students carry their own grandparents into the room. The week is also the most practically useful in the second half of the course: families with an ageing parent make up a large share of the referrals a family therapist receives, and the work is concrete — a rota, a hearing test, a family meeting — rather than interpretive.

> **CASE**
> Three siblings disagree about their mother's care. The eldest, a daughter, has done everything — the medical appointments, the money, the telephone calls — from another city. The son with whom the mother lives has a wife who does the daily work and who has not been consulted. The youngest son lives twenty minutes away and visits twice a year. The referral comes from the daughter-in-law, and she is the only person in the family who is not related to the patient.

12.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The last stage	Why integrity is a task and not a mood	Name one older person whose life they would call "integrated", and say why
8–20	Retirement	The role loss, the two sexes' different experiences, and the somatic presentation	Predict the presentation for three retirees
20–34	The cognitive differential	Delirium, depression, dementia, hearing — the four-way table, worked	Sort eight observations into the four categories
34–46	Caregiving	Burden, the "responsible child", and the reopening of sibling hierarchies	Map a caregiving network and its unstated contract
46–58	Grief	The dual-process model, and why oscillation is health	Classify six bereavement behaviours
58–70	The family meeting	Structure, roles, and the one thing that must be decided	Run a five-minute family meeting in role

Minutes	Segment	What the lecturer does	What students do
70–82	Grandparenthood and legacy	What is handed on, and to whom, in households with distributed care	Interview a peer about what they received
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. Students arrive with a sentimental model of old age — wisdom, grandchildren, a rocking chair — or a medical model — decline, burden, placement. Both are ways of not asking. The lecture's job is to install a practical attitude: what does this person need, who is providing it, who is not being consulted, and what would have to change this week? The demonstration is the caregiving network map, drawn in five minutes on the board, because it makes visible in one diagram the eleven people who are involved and the one who is doing the work.

The sentence the lecture must land. "The question in later life is almost never 'what does this person need?' — everybody knows that. It is 'who is going to do it, and who has been deciding without them?'"

12.2 Five concepts that organise the week

1. Integrity versus despair. Integrity is the capacity to hold one's life as a whole — including its failures — and to accept it as having been worth living. It is not contentment and it is not wisdom. It is an achievement of narrative, and it is made possible or impossible by the audience available. A seventy-eight-year-old with grandchildren who ask questions has a different task from one whose contemporaries have all died and whose children telephone on Sundays.

The clinical translation: the therapist's task is not to supply meaning but to elicit the account. The life review (below) is the technique, and the evidence for it is modest but consistent: structured reminiscence reduces depressive symptoms in older adults, and its mechanism appears to be the construction of coherence rather than the discharge of emotion.

2. Retirement is a role loss before it is a financial one. The research finding that matters is that the loss of the work role predicts distress more strongly than the loss of income, and that the effect is largest for men whose identity was occupational and for people whose retirement was involuntary. The presentation in a family clinic is usually somatic — fatigue, pain, a "heart" that has been investigated — and the question that opens it is "what were you supposed to be doing now?"

3. Bereavement in later life arrives in sequence. The distinguishing feature of grief at this stage is not intensity but accumulation: the death of a spouse is frequently preceded by siblings, friends, and sometimes a child. The dual-process model is useful here because it normalises the oscillation — a widow who laughs at a funeral and weeps while sorting a wardrobe is not confused; she is moving between loss-oriented and restoration-oriented coping, and both are work.

4. Caregiving is a role with no induction. The caregiver is usually appointed by proximity, gender, or birth order, rarely by negotiation, and the appointment is rarely

spoken. The clinical consequences are three: the caregiver's exhaustion is invisible because it is expected; the caregiver's resentment is unmentionable because it feels like a betrayal; and the person being cared for is frequently not consulted at all, which is the dignity injury that the family never notices.

5. Grandparenthood and legacy. In households with distributed caregiving, the grandparent is often a primary caregiver rather than a visitor, and the role runs in both directions: the grandmother raising children while the parents work, and later the adult children caring for the grandmother. The legacy question — what is handed on, and to whom — is answered by what the older person is actually asked. A grandmother whose only conversations are about medication has no route to give anything, and the giving is the developmental task.

12.2a The four-way cognitive differential

This table is the most practically important page in the week. In an older person with confusion, four conditions account for the great majority of presentations, and three of them are reversible or treatable.

Feature	Delirium	Depression	Dementia	Hearing loss
Onset	Hours to days	Weeks to months	Months to years	Gradual, often years
Course	Fluctuates through the day	Persistent, worse in the morning	Progressive, steady	Consistent; worse in noise
Attention	Impaired, wanders	Intact but poorly sustained	Intact early, impaired late	Intact
Mood	Fearful, agitated	Low, hopeless, anhedonic	Variable; may be apathetic	Often withdrawn, embarrassed
Memory	Impaired, patchy	Complains of memory, performs better than complaint	Impaired, especially recent	Appears impaired because of mishearing
Reversibility	Usually, if the cause is treated	Usually	Not currently	Correctable with aids
The question that reveals it	"Was he like this yesterday?"	"When did you last enjoy something?"	"When did you last manage the money on your own?"	"Does he answer better when he can see your face?"

The three rules that follow.

Rule 1 — Any sudden change in an older person is delirium until proved otherwise, and it is a medical emergency. A urinary tract infection, constipation, dehydration, a new medication, and pain are the common causes, and all are treatable. A family that has been told "it is dementia" after a two-day change has been told something that may be false and that will organise the next five years.

Rule 2 — Depression in later life looks like dementia and is more treatable. The label "pseudodementia" is unhelpful; the phenomenon is real. The depressed older person

answers "I do not know" to cognitive questions, while the person with dementia attempts an answer and gets it wrong. That single observation distinguishes them better than any test the therapist can administer.

Rule 3 — Test the hearing before interpreting the confusion. The commonest scene in this week's work is an older woman described as "difficult" and "not following what is said", who is simply not hearing. The therapist's test is free: sit facing her, at her level, in a quiet room, and repeat the same question with and without your face visible. A difference between the two conditions is the diagnosis.

12.2b Caregiving: the network and the unstated contract

The caregiving network map is drawn in six steps and takes about eight minutes on a whiteboard. It is the week's assessment instrument and, in most cases, the intervention as well.

Step 1 — Put the older person in the centre. Name, age, and the one sentence that describes what she can and cannot do. Not a diagnosis; a function. "She can wash and dress herself. She cannot manage her medication, leave the house, or remember what she was told an hour ago."

Step 2 — Draw everyone who does something, in a ring. Include the daughter-in-law. Include the neighbour who looks in. Include the paid carer, if there is one. The map routinely reveals that the "family" doing the care is five people, of whom two are not related.

Step 3 — Write, next to each name, what they actually do, in hours per week. The numbers are estimates and the estimates are the finding: the person who does forty hours is usually the one who was never asked, and the person who does two is usually the one whose opinion carries the most weight.

Step 4 — Mark who decides. A different marker, on the same names. The gap between who does the work and who makes the decisions is the network's fault line, and it is the source of most of the conflict that arrives in the clinic.

Step 5 — Mark who is not consulted. The person in the centre is almost always on this list, and so is the daughter-in-law. Writing "not consulted" next to the patient's own name is the single most useful intervention in the exercise, because it makes visible an omission that no one in the family has noticed.

Step 6 — Ask the network question. "If the person doing the most stopped tomorrow, what would happen?" The answer is the network's fragility, and it is usually discovered in the same session in which the family arrived complaining about something else.

The unstated contract. Every caregiving network has one, and it has four clauses: who is responsible; what counts as good care; what the carer is entitled to; and what happens if the carer refuses. None of the four is ever spoken, and each is a source of conflict when it is breached.

The clinical move is to make the contract speakable without making it an accusation. The four questions: "who decided that you would be the one? what would count as doing this

well? what are you allowed to have for yourself? and what would happen if you said no?" The fourth question is the one that produces tears, because most carers have never considered that refusal is an option and have been living as though it is not.

The burden measures, used carefully. Standardised burden instruments (Zarit, and similar) are useful for research and for demonstrating severity to a family that has normalised it. In the room, the two questions that do most of the work are: "when did you last sleep through the night?" and "who asks how you are?" The second question identifies the carer's isolation more reliably than any scale.

12.2c Retirement: the presentation, and what to ask

Retirement is the developmental event most often missed in a family clinic, because it presents as something else.

The four presentations. A somatic complaint that has been investigated and explained as "nothing serious"; a sudden increase in drinking; a man who is under the family's feet and who has begun to manage the household in a way that produces marital conflict; and a withdrawal into a chair that the family describes as depression. All four are the same role loss in different costumes.

What has actually been lost. Not the money, usually. The structure of the day; the colleagues, who were the social network; the competence, which was visible and daily; and the audience, which is what an occupational identity needs. A man who was the branch manager for eleven years does not miss the money as much as he misses being the person whose judgement was sought.

The four questions.

1. "What time did you get up before you retired, and what time do you get up now?" The answer is the structure, quantified.
2. "Who did you talk to every day at work, and who do you talk to every day now?" The answer is the network, quantified.
3. "What are you good at that nobody asks you about any more?" The answer is the competence, and the question is the first hinge of the intervention.
4. "What would you have done with the time if you had been allowed to choose, at fifty?" The answer is frequently a lost project, and the lost project is often still available.

The intervention, in two moves. Restore a structure — a fixed weekly commitment with other people in it, which is what the workplace was providing — and restore a use of competence — tutoring, a committee, a trade association, a church role, coaching, a small business. The evidence on volunteering and structured activity in retirement supports the first more strongly than the second; the clinical experience is that both are necessary and that neither is achieved by advice.

The wife's experience. The retirement literature under-reports the marital consequence: a woman who has run a household for thirty years and whose husband is now present in it for all of it. The clinical presentation is a couple in their sixties who have "started arguing", which is usually a pair of people negotiating territory for the first time since

1994. The intervention is not conflict resolution; it is a division of the day, written, in the way that Week 10's division-of-labour audit is written.

12.3 The method: the family meeting about an older parent

One session, three to six participants, ninety minutes if possible. The structure below is adapted from family meetings in medical settings and is used unchanged in this book.

Step 1 — Establish who is in the room and who is not, and why. Name every absent person and the reason. Then ask whether anyone wants them present. A meeting that excludes the person who pays, or the person who does the daily work, will produce a decision that is reversed within a fortnight.

Step 2 — Ask each person, in turn, one question: "what is your greatest worry?" In turn, without interruption, and written down verbatim. This single round does more than any other part of the meeting, because it converts a group of people who have been arguing about arrangements into a group of people who have each stated a fear.

Step 3 — Put the facts on one page. What the older person can do; what the medical picture is; what the options are; what each costs, in money and in hours. Facts first, feelings second, is the order that prevents the meeting from becoming a re-run of a forty-year-old argument.

Step 4 — Name the unstated contract, out loud. "It seems that the arrangement has been that Bola manages the medical care, Funke does the daily work, and nobody has asked Mama what she wants. Is that right?" The family corrects, and the correction is the working formulation.

Step 5 — Generate four options, not two. The binary — she stays where she is, or she goes into a home — is what has made the argument unwinnable. Four options, written on the page, each with costs. The purpose is not to choose in the meeting; it is to demonstrate that there is more than one way for this to go.

Step 6 — Decide one thing, and only one. A review date, a rota for one week, a hearing test, a conversation with the older person. One decision, made in the room, with a named person and a date.

Step 7 — Say what happens if it fails. The fallback, named in advance. Families that have a fallback implement the plan; families without one abandon it at the first difficulty and conclude that the meeting was pointless.

The older person's presence. If she can participate, she participates. The most common and most damaging omission in these meetings is the exclusion of the person whose life is being arranged, on the grounds that she will be upset or confused. A hearing-impaired woman who is spoken over for ninety minutes will be described afterwards as "difficult", and the description will be accurate.

12.3a When the siblings disagree

Sibling conflict about an older parent is not usually about the parent. It is usually about a ledger that was opened in childhood, and the clinician's task is to keep the present decision from being decided by the old ledger — while acknowledging that the ledger is real.

The four patterns, and the move for each.

The responsible child and the absent ones. One sibling has done everything and is exhausted and resentful; the others have done nothing and feel judged. The absent siblings' position is usually defended with a practical argument ("I am in Lagos", "I have small children"), which is true and is not the whole truth. The move: give the absent siblings a specific, bounded, non-negotiable task, and give the responsible child a specific relief. General appeals to help more produce nothing; "Kunle will take Mama to the clinic on the second Saturday of every month" produces a decision.

The child who is not a child. A daughter-in-law, a nephew, a foster kin who has done the work and has no standing in the family's hierarchy. The move: give them standing explicitly, in the meeting, by having the family agree to their participation. A therapist who fails to do this will watch the most competent person in the room be overruled by the least involved.

The sibling who wants placement and the sibling who refuses it. Usually the difference is about money, guilt, or a promise made to the parent. The move: separate the promise from the plan. "What did she ask you to do?" is asked of each sibling, and the answers are frequently different, which is the finding.

The sibling who is not speaking to another. A silence of years, brought into the room by the parent's decline. The move: do not attempt reconciliation. Ask only whether the two of them can agree on one practical matter for six weeks, on the understanding that nothing else is being resolved. Concrete cooperation across an unresolved conflict is achievable and reconciliation is not, and a therapist who aims at the second will lose the first.

The rule that governs all four. The parent's needs are the organising question and are not the whole conversation. A therapist who insists that "this is about Mama" will be met with agreement and no change, because the siblings know it is not only about Mama, and so does the therapist.

12.4 Three techniques

Technique 1 — The life review, in six questions. Not a memoir. Six questions, asked across two or three sessions, recorded if the older person agrees, and given to the family as a transcript.

1. "What is the first thing you remember?"
2. "What did you think your life would be, and what happened instead?"
3. "Who did you love, and who loved you?"
4. "What are you proudest of that nobody knows about?"

5. "What do you want the ones coming after you to know?"
6. "What would you do differently?"

The technique works because the older person is being asked as an author rather than as a patient, and because each question has a developmental function: memory for continuity, the timetable question for integrity, the love question for relationship, the pride question for self-authorship, the legacy question for generativity, and the counterfactual for the acceptance of a life that was not perfect.

Technique 2 — The legacy recording. A telephone recording of the six answers, given to a grandchild. The instruction to the family is what makes it work: "do not listen to it while she is alive". Its function is not comfort in the present; it is the presence of a voice after the death, which is the thing that grandchildren ask for and that families almost never make.

Technique 3 — The rota, written and limited. A weekly schedule with names beside tasks: who takes her to the clinic, who calls on Tuesday, who sleeps in the house on Friday. Bounded in time — six weeks — and specific in task. The rota does three things that conversation cannot: it distributes the mental load, it makes absence visible, and it converts an obligation into a schedule that can be renegotiated.

The caregiving session, in practice. The therapist who does all three in one session will fail. The sequence that works: the map first (assessment), then one technique (intervention), then the rota (a concrete output). Sessions two and three can add the review.

12.5 The first session with three siblings and a daughter-in-law

Phase 1 — The round of worries (20 minutes). Each person, in turn, one question: "what is your greatest worry?" Written down verbatim. No interruption, no response, and the therapist explains why: "everybody will get a chance to answer the others, and I want the worries on the table first."

Phase 2 — The facts (15 minutes). What Mama can do; what the medical picture is; what the money is; what has been tried. The therapist supplies what she has and asks for the rest, and writes it on one page.

Phase 3 — The map (10 minutes). Drawn on the whiteboard, with hours and decisions marked, and "not consulted" written next to the names that belong there — including, usually, Mama's.

Phase 4 — The contract named (15 minutes). Who decided; what counts as good care; what the carer is entitled to; what happens if she refuses. The four questions, asked slowly, to the group rather than to an individual.

Phase 5 — Four options (15 minutes). On the page, with costs. The therapist's discipline is not to recommend; the family's job is to eliminate.

Phase 6 — One decision and one fallback (10 minutes). Named, dated, written, photographed.

Phase 7 — The absent person (5 minutes). "Who is not here, and who will tell them?" The question is asked at the end, because the answer determines whether the decision survives the week.

Why Mama is asked first, if she is present. Because a meeting about an older person that begins with her children describing her decline has already decided what she is. The therapist's first question to her is not about her health but about her preference: "what would you like the next year to look like?" The answer is frequently surprising, frequently practical, and it reorganises the meeting.

12.6 Case: the Baloguns and the woman in the back room

The family. This household first appeared in MFT 702, where Mr Femi Balogun (now 55) was a long-distance lorry driver away three weeks in four, his wife Funke (49) ran a food business from the house, and his mother (then 78, now 81) was living with them with a diagnosis of dementia. The family has changed since then: the eldest son has left school and is working; the second child, with sickle cell disease, is now 17 and stable; there are two younger children.

The referral. From Funke, the daughter-in-law, by telephone, in tears. "I cannot do this any more. His sister in Lagos decides everything and she is not here. His brother lives twenty minutes away and comes at Christmas. I am the one who bathes her."

The household, as mapped. Mama, 81. Femi, 55, away three weeks in four. Funke, 49, the daily carer. Bola, 58, Femi's elder sister, in Lagos, a retired nurse, who pays for medication and manages appointments by telephone. Kunle, 49, the youngest, in Ibadan, twenty minutes away, visits at Christmas and Easter. Four children in the house, aged 22, 17, 12 and 9. A neighbour, Mama Seri, who looks in on weekdays. A physician at a private hospital who has seen Mama twice in two years.

The hours, estimated in the session. Funke 38 hours a week; Bola 4 (telephone and payments); Kunle 2 (a year); Mama Seri 3; Femi 6 in the week he is home.

The decisions, marked. Bola makes the medical decisions. Femi makes the financial decisions when he is home. Funke makes every decision in between and is not consulted about any of the ones the others make. Mama makes none.

The facts. The dementia diagnosis was made two years ago after a gradual decline. Three months ago there was a sudden worsening over two days — agitation, not recognising Funke, wandering at night — and a locum doctor at the hospital said "the dementia is progressing" and gave a sedative. The worsening resolved in five days. The therapist writes this on the page and circles it, because it is the case's most important fact.

The five findings.

1. The three-month-old "progression" was probably a delirium, and it has never been assessed. A change over two days with agitation and nocturnal worsening, resolving in five days, is the signature of a delirium — most likely a urinary tract infection, constipation, dehydration, or the sedative itself. A diagnosis of progression was made on

the basis of a five-day event, and the family has organised three months of decisions around it. The first action of the case is a medical review, not a family decision.

2. The hearing has never been tested and is the likely source of "difficulty". In the session Mama is spoken to from across the room and does not respond; spoken to from a metre away, facing her, she answers appropriately three times out of four. Funke reports that Mama "is better in the morning" — which is when the house is quiet. The therapist asks for a hearing assessment and, in the meantime, demonstrates the difference in the room, in front of the family. Bola, who is a retired nurse, is visibly affected.

3. Funke is doing the work, has no standing, and is not related to the patient. She is the network's most competent member and its least consulted. Every decision about Mama is made by people who are not there, and Funke's experience of the last three months is that she is a servant with clinical responsibilities. Her tears on the telephone are not a symptom of the family's difficulty; they are the most accurate description of it.

4. Bola's control is functional, not merely controlling. She is a retired nurse, she pays, and she has been managing a mother at a distance for two years with the telephone as her only instrument. Her insistence on deciding is what a frightened eldest daughter does when she cannot be present. The therapist who treats her as the villain will lose the person who pays for the medication.

5. Mama has not been asked anything in two years. Not what she wants, not how she feels, not whether she is frightened. The life review questions have never been put to her, and the family has no account of her life that is longer than the last two years.

The formulation. An eighty-one-year-old woman with a dementia diagnosis and an untested hearing loss, whose recent "progression" may have been a treatable delirium, cared for by a daughter-in-law with no standing in the family's hierarchy, managed medically by a daughter at a distance, and financially by a son who is away three weeks in four. The presenting complaint is Funke's exhaustion, and the family's problem is a caregiving network whose workload and authority are inversely distributed.

The intervention, in seven parts, in order.

Part 1 — The medical review, first. A written request to the physician for a review of the three-month-old episode, with the specific question: was this a delirium, and was the sedative appropriate? The therapist does not diagnose; she puts the question on paper and asks the family to take it.

Part 2 — The hearing test, and the demonstration in the room. The demonstration has already changed Bola's position, and the test is booked for the following week.

Part 3 — Funke gets standing, explicitly, in the meeting. The therapist asks Bola and Kunle, in front of Funke, to agree that Funke is the person who knows Mama's daily condition and that decisions about the daily arrangements will be made with her. This is a single sentence and it is the intervention on which the rest depends.

Part 4 — Bola gets a defined role and a bounded one. Medical decisions and payment, with a monthly telephone call to Funke at a stated time. The therapist names the

reason plainly: "she has been managing from Lagos with a telephone, which is the hardest way to do it, and the answer is not to remove her authority but to give her one that has a schedule."

Part 5 — Kunle gets one task. The second Saturday of every month, the clinic visit, with a written report to the family group. A specific, bounded, non-negotiable task, which he accepts in the room after some resistance and which he performs for eight consecutive months.

Part 6 — The rota, for six weeks. Written, photographed, with a review date. Bathing, nights, medication, clinic, the neighbour's days.

Part 7 — Mama is asked. In a separate session, with Funke present and Bola on speakerphone, the six life review questions are begun. Mama answers three, is tired, and stops. The therapist asks whether she would like to continue next week, and she says yes. The transcript of that session is, six months later, the thing the family values most.

12.6a Six months later

The medical review found a urinary tract infection at the time of the episode and no evidence of progression beyond the expected; the sedative was stopped. Mama's functioning is close to what it was before the episode, which means that three months of family decisions were made on a false premise.

Hearing aids were prescribed and are worn, inconsistently. When they are worn, Funke reports that Mama is "a different person", which is the phrase families always use and which is a description of the difference between a woman who cannot hear and a woman who is confused.

Kunle has done eight clinic visits out of six months, which is not the plan and is more than the previous decade. Bola telephones Funke on the first Sunday of the month. The two women have begun to speak to each other as colleagues rather than as a manager and an employee, which Funke describes as "she asks me now".

The rota held for five weeks and then dissolved. The therapist notes this without alarm: a rota's function in most families is to make the distribution visible once, not to run indefinitely, and the visibility persisted after the document was lost.

The honest limitation. Mama is not going to recover, Funke is still doing thirty hours a week, and the family is still one woman's illness away from crisis. What changed is that the work is now visible, the worker has standing, and the person in the centre has been asked what she wants. Those are the achievable outcomes of this week's work, and a therapist who promises more will be lying to a family that has enough disappointment already.

What would have changed the plan. If the medical review had confirmed progression, the meeting would have become a placement discussion and the four options would have been about care settings. If Bola had refused Funke's standing, the work would have proceeded with Funke alone on the question of what she is entitled to, which is a legitimate treatment in its own right. And if Mama had been unable to participate at all,

the life review would have become a review with the family about her, which is a poor substitute and better than nothing.

12.7 Transcript: asking an eighty-one-year-old what she wants

Ten turns from the session in Part 7, with Funke present and Bola on speakerphone. Mama wears a hearing aid for the first time in the therapist's presence.

> **T1 Therapist:** (facing her, at her level, unhurried) Mama, I want to ask you something that nobody has asked you. (pause) What would you like the next year to look like? — The question is put to her first, in the first person, seated at her level and facing her. Everything about the physical arrangement is the intervention. **T2 Mama:** (long pause) Nobody has asked me that. **T3 Therapist:** I know. That is why I am asking. **T4 Mama:** (pause) I would like to sit outside in the morning. (pause) In the compound. With the sun. — The answer is concrete, small, and entirely outside the family's frame of reference, which is what these answers are almost always like. **T5 Funke:** (quietly) She cannot go outside. There are steps. — The daughter-in-law's objection is practical and true, and it is the beginning of the intervention rather than a contradiction of it. **T6 Therapist:** (to Funke) Then that is the thing we are going to solve. (to Mama) Mama, if someone carried the chair out for you, would you want to sit there? **T7 Mama:** (nodding) In the morning. Before it is hot. **T8 Therapist:** (to Bola, on the speakerphone) Bola, did you hear that? — The question is asked of the absent authority, deliberately, because the decision will not survive unless she has heard it in Mama's voice. **T9 Bola:** (after a pause) I heard. **T10 Therapist:** (to the room) Then the first item on the rota is not the clinic and it is not the medication. It is the chair, in the morning, before it is hot. — The therapist writes it first on the page, above the medical items, which is a statement about what this meeting is for.

12.8 Laboratory: the caregiving network map

Aim. To map a caregiving network in fifteen minutes, identify the inverse distribution of work and authority, and produce one decision that the family can make in the room.

Procedure. Groups of five: therapist, older person, primary carer, distant decision-maker, and one absent sibling represented by an empty chair (which students find unexpectedly powerful). The trainer supplies a one-page brief with hours, decisions and one unassessed medical event. Twenty-five minutes, then feedback.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The map	Verbal description only	Drawn, but without hours or decisions	Drawn with hours, decision markers, and "not consulted" named
The medical check	Accepted the diagnosis on the file	Asked about the diagnosis	Identified the unassessed event and wrote the question for the physician
The hearing	Not considered	Asked about it	Tested it in the room and demonstrated the difference

Criterion	0–1	2–3	4–5
Standing	Left the carer without authority	Acknowledged the carer	Gave the carer standing explicitly, in front of the others
The decision	None	One decision, no date	One decision, dated, named, with a fallback

The distinction that carries most of the marks. The top band requires the student to ask the older person a question and write the answer down. Groups reliably spend twenty minutes on the network and never speak to the patient, which reproduces in the laboratory the exact error the week is about.

12.9 Six errors

Error 1 — Accepting "dementia" as an explanation for a change. Any sudden change is delirium until proved otherwise, and it is a medical emergency. The error is common because the diagnosis is already on the file and because families present the change as deterioration.

Error 2 — Interpreting confusion without testing hearing. The cheapest test in this book and the most often skipped. A hearing loss produces social withdrawal, apparent paranoia, non-compliance and "difficulty", and every one of those is then treated as a symptom of dementia.

Error 3 — Meeting the family without the patient. The most common structural error in later-life work. A woman who is spoken over for ninety minutes will be described as difficult afterwards, and the description will be accurate.

Error 4 — Treating the daughter-in-law as a helper. She is frequently the most competent and least consulted member of the network, and a therapist who addresses the adult children and uses the carer as a source of information has reproduced the family's hierarchy rather than intervening in it.

Error 5 — Aiming at sibling reconciliation. The conflict is usually forty years old and is not the therapist's business. The aim is six weeks of concrete cooperation, which is achievable, and a therapist who aims at the former will lose the latter.

Error 6 — A plan with no fallback. Families implement plans that have a stated fallback and abandon plans that do not. The fallback is not pessimism; it is the condition under which the first plan survives its first bad week.

12.9a The phone call that is really a referral

A pattern worth naming separately, because it is how most later-life work actually begins.

The call comes from an adult daughter, usually in the evening, and it is framed as a request for information: "I just want to know whether we should be worried." Within three minutes the real request appears, and it is one of four.

"Tell me I am doing enough." The caller is asking for a verdict on her caregiving. The answer that works is behavioural rather than reassuring: "you are the one who

telephoned. What are the three things you actually do each week?" Listing them is the intervention, because the caller has never counted.

"Tell me my brother should help." The caller wants an ally. The answer that avoids the trap: "I will not tell him that. I will ask him, in a meeting, what he can do — and I will ask you the same question." A therapist who supplies the ally is then blamed when the brother does not change.

"Tell me it is time for a home." The caller wants permission. Permission is not the therapist's to give, and the honest answer is the structure of the decision: "that is a decision for your family, and I can help you make it in a way you can all live with. What would have to be true for you to consider it?"

"Tell me she is not dying." The caller wants a prediction. Give the medical picture as far as it is known, name what is not known, and offer the one thing that can be offered: a plan for the next month rather than a prognosis for the next year.

The practical point. The first telephone call is the assessment and the engagement, and it is usually taken by the least experienced person in the service. The four framings above should be taught to whoever answers the telephone, because a call that is handled as a request for information and ends with "make an appointment" has lost the family's most motivated member.

12.10 Culture, faith and later life

Who cares for the elderly, and who is expected to. In most of the households this book is written for, care is a family obligation rather than a purchased service, and the obligation falls on a daughter-in-law or a daughter rather than on a son. The clinical consequence is that the person doing the work frequently has no authority, and the person with authority frequently does none of the work. This is the week's central finding, and it is a structural fact rather than a family pathology.

Respect and the impossibility of asking. In many households it is improper for a younger person to ask an older one what she wants, and the impropriety is felt by the family as a failure of respect rather than as an omission. The therapist can use the frame rather than fight it: "it is because we respect her that we are asking her" is a sentence that permits the question. Where the older person cannot be asked, the therapist asks the family to describe what she would say, which is a second-best procedure and is better than nothing.

Placement and its meanings. A care home carries, in many settings, the meaning of abandonment, and a family that is offered placement as an option may hear an accusation. The therapist's discipline is to keep four options on the page without recommending any, because the family's choice must be one the family can live with in front of its relatives, not one the therapist would make.

Faith, illness and the meaning of decline. Households may understand dementia as a spiritual trial, a punishment, or a season, and the understanding organises how much medical investigation is pursued. The clinical position: ask what the framework means for the family, and hold one non-negotiable — that a treatable infection should be treated.

Framing the medical review as stewardship rather than as doubt is translation, not manipulation.

Mourning as a collective practice. Where grief is prescribed — forty days of visits, a year of black, a particular liturgy — the therapist's task is to support the rites and to watch for the period after them, when the company leaves and the bereavement becomes private. The dual-process model describes an individual oscillation; the culture supplies a collective one, and the clinical question is what happens when the collective one ends.

> **DEBATE**
> "The person who does the caring should make the decisions." Argue for: authority should follow knowledge and work, the distant decision-maker is making choices about a life they do not see, and the current arrangement is exploitative. Argue against: decisions about money require the person who has the money, medical decisions require medical knowledge, and a daughter-in-law with authority and no kinship has no standing to defend it against the family. Then state what you would actually have done in the Balogun meeting, and what you would have done if Bola had refused.

12.11 Evidence note

What the research supports. Structured life review and reminiscence reduce depressive symptoms in older adults, with modest but consistent effects. Caregiver burden interventions that combine psychoeducation with practical respite produce better outcomes than either alone, and the effect is larger when the carer is given specific tasks rather than general support. Hearing loss is independently associated with cognitive decline, social isolation and depression, and correction improves social participation. Delirium is common, frequently missed, and associated with poor outcomes, particularly when it is misattributed to dementia.

What is weaker. The evidence on family meetings is largely from medical and palliative settings and transfers imperfectly to a therapy clinic. The dual-process model is well supported descriptively and less well supported as a treatment target. Effects of grandparent involvement on child outcomes are confounded by selection and are difficult to interpret.

What is not supported. That "working through" grief requires the discharge of emotion — the evidence favours oscillation and continued bonds over severance. That cognitive decline is inevitable and untreatable in every case, which is the assumption behind the most damaging errors in this week's work. And that carers who are not relatives are less appropriate; the evidence on non-kin caregiving is thin and the clinical experience is that they are frequently the most reliable member of the network.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same dementia diagnosis produces, in one family, a coordinated network and, in another, a daughter-in-law doing forty hours a week in silence, and the difference is not in the diagnosis.

12.12 Key terms

Integrity versus despair — Erikson's final stage; the consolidation of a life into an account that can be held.

Delirium — an acute, fluctuating disturbance of attention and awareness, usually caused by a treatable physical problem; a medical emergency.

Pseudodementia — the presentation of depression as apparent cognitive decline; treatable.

Dual-process model — the account of grief as oscillation between loss-oriented and restoration-oriented coping.

Caregiver burden — the objective and subjective cost of caregiving; measured, and frequently normalised by the family.

Unstated contract — the four unspoken clauses of a caregiving arrangement: responsibility, standard, entitlement, and the consequence of refusal.

Responsible child — the sibling appointed by proximity, gender or birth order, rarely by negotiation.

Life review — a structured reminiscence intervention aimed at coherence rather than discharge.

Legacy — what is handed on, and to whom; the developmental task that requires an audience.

Fallback — the pre-agreed response if the plan fails; the condition under which plans survive.

12.13 Self-test

1. Give the four-way cognitive differential and the one question that best distinguishes each pair.
2. Why is a change over two days a medical emergency, and what has usually happened when a family is told that "the dementia is progressing"?
3. What is the inverse distribution, and why does it produce the referral?
4. Name the four clauses of the unstated caregiving contract.
5. Why does the therapist write the chair above the medical items on the rota?
6. What are the four sibling patterns, and what is the move for the daughter-in-law?
7. Why is sibling reconciliation the wrong aim, and what is the right one?
8. In the Balogun case, what was the single most important fact, and how was it found?

Answers. (1) Delirium — "was he like this yesterday?"; depression — "when did you last enjoy something?"; dementia — "when did you last manage the money alone?"; hearing loss — "does he answer better when he can see your face?" (2) Because an acute change with fluctuation is the signature of delirium, usually from infection, constipation, dehydration, medication or pain, all treatable; the family has usually been given a diagnosis on the basis of a five-day event and has organised months of decisions around it.

(3) The workload sits with the person who has least authority and the authority with those who do least work; the referral arrives when the worker's capacity fails. (4) Who is responsible; what counts as good care; what the carer is entitled to; what happens if the carer refuses. (5) Because the meeting is about Mama's life rather than her medication, and the order on the page is a statement about what the work is for. (6) The responsible child and the absent ones; the child who is not a child; placement versus refusal; the siblings not speaking — the move for the non-kin carer is to give them standing explicitly, in the meeting, by the family's agreement. (7) Because the conflict is decades old and is not the therapist's business; the right aim is six weeks of concrete cooperation on one practical matter. (8) That the three-month-old "progression" was probably a delirium — found by dating the change and asking what had been done about it.

12.14 Reflective task

Ask one person over seventy the six life review questions, and record the answers if they agree. Then answer three questions of your own: which question produced the most, which produced the least, and what did you have to stop yourself from doing while they spoke? Most students report that they wanted to reassure, to interpret, or to fill a silence, and that the silence was where the material was.

12.15 Further reading

- Erikson, E., Erikson, J. and Kivnick, H. Vital Involvement in Old Age — the empirical follow-up to the final stage.
- Stroebe, M. and Schut, H. "The dual-process model of coping with bereavement" — the source, and the evidence.
- Zarit, S. et al. on caregiver burden — the instrument, and its limits in the room.
- Butler, R. "The life review" — the original paper, still the best description of the task.
- Livingston, G. et al. on dementia care and carer interventions — the best current summary of what works.

Next: Week 13 — Culture, poverty and development: the critique of the norms.

Week 13 — Culture, poverty and development: the critique of the norms

Development is not culture-free. Every norm in the preceding twelve weeks was measured somewhere, on somebody, and the somewhere and the somebody matter. This week supplies the critique, and the critique is not an optional add-on for students with an interest in culture; it is a correction to the instrument they have been learning to use.

The four critiques you must be able to state.

1. **The norms are Western.** Attachment classifications, individuation timetables and "school readiness" were normed on particular populations. A child who sleeps in the parental bed until five is not necessarily enmeshed; in many cultures this is ordinary.

2. **Poverty changes development.** Chronic stress, food insecurity, crowding and insecure housing affect attention, sleep, and the capacity for consistent parenting. Diagnosing "inconsistent parenting" without naming material conditions is a category error.
3. **Extended and distributed caregiving is normal.** In much of the world, children are raised by networks — grandparents, aunts, older siblings, co-wives, foster kin. Attachment theory travels better when it is asked "who does this child go to when she is frightened?" rather than "is she securely attached to her mother?"
4. **The measurement is part of the finding.** A test administered in a second language, by a stranger, in a clinic, to a child who has been told to be quiet, measures the test situation as well as the child. This is not a reason to abandon measurement; it is a reason to interpret it with the conditions attached.

Why this week is placed here and not earlier. Because a critique of the norms is only usable by someone who knows the norms. A student who has not spent twelve weeks learning what typical development looks like cannot evaluate the claim that the norms are parochial; they can only repeat it. By this point in the course the student has the instrument, and this week teaches them what it does not measure.

What the week is not. It is not a claim that development is entirely relative, that milestones are meaningless, or that any practice is defensible because it is cultural. Children die from practices that are cultural, and the therapist who cannot say so has abandoned the child. The position is narrower and harder: the norms describe one pathway in detail, other pathways exist and are not deficient, and the clinical question is always what this child, in this household, with these resources, needs in order to develop.

> **CASE**
> A four-year-old sleeps in her parents' bed in a two-room flat shared with her father's brother. The nursery nurse who referred her has written "over-attached, does not self-soothe". The family is offended and has come only because the school insisted. Assessment finds: the child plays richly, separates without distress, has three adults who respond to her, and has no bedroom. The referral's central "finding" is a description of the housing.

13.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	What students do
0–8	The problem	Why a norm is a measurement plus a population, and what happens when the population is missing	Write the population behind one milestone they know
8–22	The Western norms	Attachment, individuation, readiness — what was measured, on whom, and by whom	Interrogate one classification system
22–36	Poverty as a developmental variable	The five mechanisms, and the evidence for each	Trace one mechanism through a case they have seen

Minutes	Segment	What the lecturer does	What students do
36–48	Distributed caregiving	The network as the unit; the "who does she go to?" question	Map a child's caregiving network from a description
48–60	The measurement problem	Language, stranger, setting, instruction — four contaminants	Design a fair assessment for a named child
60–72	The interpreter	Working with a third person in the room, and the four rules	Practise a three-way conversation
72–82	When to hold the line	The practices a therapist must not relativise, and how to say so	Argue one case both ways and then decide
82–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. The risk in a week on culture is that it becomes either a sermon on sensitivity or a defence of the discipline. The lecture avoids both by making the class do measurement: give them a milestone chart and ask them to write down the population it was normed on. They cannot, and the inability is the lesson. The instrument they have been using all term is precise and its precision has a location.

The sentence the lecture must land. "Every norm is somebody's average. Your job is to know whose, and then to decide what this child needs."

13.2 The four critiques worked

1. The norms are Western, and more specifically they are from a handful of studies. The Strange Situation was developed in Baltimore in the 1970s on a sample of about a hundred infants; its classifications have been replicated in dozens of cultures, which is a genuine strength, and the distribution of classifications varies in ways that are informative. In several Northern German samples, avoidant classification was more common; in several Japanese samples, resistant. Neither distribution indicated a population of insecure children; it indicated a population of caregiving practices. The classification is a description of a relationship under a particular stressor, not a score of maternal quality.

The same applies to individuation. The timetable in which a child sleeps alone, weans early, and separates for nursery at eighteen months is a description of a particular household economy. A child who sleeps with her mother until five and is carried until three is not delayed; she is participating in a practice with its own developmental logic, and the evidence on later independence does not support the claim that early separation produces it.

2. Poverty is a developmental variable with mechanisms, not a background fact. The mechanisms are five, and each is worth knowing because each is addressable.

- **Chronic stress physiology.** Poverty is associated with elevated cortisol and with alterations in the developing stress response, which affects attention, sleep and emotional regulation. The mechanism is biological and the cause is material.

- **Food insecurity.** Both under-nutrition and the unpredictability of food affect growth, attention and behaviour. A child who has not eaten since the previous evening is not in a condition to learn, and the behaviour that results is routinely read as a disorder.
- **Crowding.** Sleep disruption, no space for homework, no privacy for an adolescent, and no quiet for a child with a hearing loss. Crowding is measurable and is almost never measured.
- **Housing instability.** Moves disrupt schooling, friendships and the continuity of care, and the disruption is frequently read as the child's difficulty with change.
- **Parental bandwidth.** A parent managing money, transport, water, safety and a job has less attention for the ordinary work of parenting, not because they care less but because attention is finite. This is the finding that most changes a clinician's practice, because it converts "inconsistent parenting" from a character judgement into a resource calculation.

3. Distributed caregiving is the norm in most of the world. A child may be raised by a grandmother during the week and a mother at weekends, by an aunt in the city and parents in the village, by an older sister for most of the day. The clinical task is not to assess whether the arrangement is optimal by a Western standard; it is to ask whether the child has at least one adult who is reliably available and responsive, and whether the adults are coordinated.

The question that replaces the Strange Situation in practice: "when she is frightened or hurt, who does she go to?" Followed by: "and does that person know that they are the one?" The second question is the one that finds the problem, which is usually a coordination problem rather than a deficit of care.

4. The measurement is part of the finding. Four contaminants, all of which the therapist can control.

- **Language.** A test in a second language measures proficiency, not ability. Assess in the stronger language, or use a nonverbal instrument, or bring an interpreter — and record which.
- **The stranger.** A child who has been taught not to speak to unfamiliar adults will perform poorly with a clinician and well at home. Observe in the home, or with the caregiver present, or both.
- **The setting.** A clinic room with a stranger and a parent who is anxious is a particular social situation, and the child's behaviour in it is a response to that situation.
- **The instruction.** A child who has been told that children should be seen and not heard may answer "I do not know" to every question, accurately, as a matter of manners.

The correction that follows. Report the conditions with the finding. "Receptive language age equivalent three years six months, assessed in English, second language, with mother present, in clinic" is a substantially different statement from "language delay", and the difference is the whole of this week.

13.2a When the norms are right and the family is wrong

The week's hardest teaching point, and the one that separates a culturally competent clinician from a relativist.

The four categories. A practice may be different and equivalent (co-sleeping, early weaning, grandparent care), different and better (a caregiving network that gives a child four adults instead of two), different and harmful (female genital cutting, corporal punishment to the point of injury, withholding food as discipline, a child kept out of school to work), or not a practice at all but a consequence (crowding, hunger, an untreated infection).

The therapist's obligation in the third category. Name it, once, plainly, and then act according to the law and the safeguarding framework. The order matters: name it to the parents first, in the family's own terms, before any referral. A parent who learns from a social worker what the therapist thought has been humiliated, and the humiliation will be remembered by the whole extended family.

How to name it without a lecture. Three sentences. First, the practice, described factually: "you told me that when he is disobedient he is beaten with a cable." Second, the concern, in developmental terms: "I am worried about his body and about what it teaches him about being hit by people who love him." Third, the position: "I am not able to work with a plan that includes that, and I would like to find something else that works."

What not to do. Do not relativise harm ("in your culture that is normal"), which abandons the child. Do not moralise ("that is child abuse"), which ends the relationship before any change is possible. Do not report without telling the parents, which is sometimes legally required and is always clinically costly and must be explained as it happens.

The position in one sentence. Culture explains a practice; it does not decide whether the practice is good for this child. The therapist's obligation is to hold both halves of that sentence at once, which is what makes the work difficult and what makes it worth doing.

13.2b Three network configurations and what each needs

Distributed caregiving is not one arrangement. Three configurations account for most of the households in these clinics, and each has a different clinical question.

Configuration 1 — The grandmother as primary carer. The mother works, the grandmother raises the child from six months, and the mother resumes the primary role at school age. The child is well cared for and attached, and the transition at school age is the clinical event. The presenting difficulty is usually "he does not listen to me" — addressed to the mother, by a child who has had three years of a different authority. The question is not whether the arrangement was right; it is how the transfer of primary authority is conducted, and it is conducted best when it is explicit and gradual, with the grandmother's status preserved rather than retired.

Configuration 2 — The circulating child. A child who moves between households — school terms with an aunt in the city, holidays in the village, a period with a godmother. This arrangement is common, is often economically rational, and produces a specific clinical picture: a child who is competent, adaptable, and reluctant to attach quickly, and

who is described by adults as "he does not settle". The clinical question is not whether circulation is harmful — the evidence is mixed and depends almost entirely on whether the moves were explained and whether one adult remained constant — but which adult has been constant, and whether the child knows that the constancy is intended to continue.

Configuration 3 — The compound. Several related households in one space, with children moving between them freely and discipline distributed across adults. The child has many caregivers and no privacy, and the developmental benefits are real: language exposure, multiple attachment figures, early social competence. The clinical difficulty is coordination: a child with a hearing loss in a compound is spoken to by eleven people in four languages and is described as "confused", and no single adult has the authority to change the arrangement.

The question for all three. "Who is the person this child would go to, and does that person know?" In configuration 1 the answer is usually the grandmother, and the mother does not know it. In configuration 2 the answer is usually an aunt, and the parents assume it is themselves. In configuration 3 the answer is frequently an older sibling, which nobody has noticed.

13.3 The method: a developmental cultural formulation

Six questions, adapted from the standard cultural formulation for use with a child and a family. Twenty minutes, and the answers go in the record because they change the interpretation of everything else.

Question 1 — Who is in this child's world, and what does each person do? Not the household roster. The functions: who feeds, who puts to bed, who takes to the clinic, who disciplines, who plays, who is trusted with a secret at eleven years old. The answer is the network, and it is frequently larger than the referral suggests.

Question 2 — What is this family's account of the problem, and what does it predict? Families hold theories — spiritual, developmental, characterological, medical, or a mixture — and the theory predicts what they will do. A family that believes the problem is spiritual will pursue prayer; a family that believes it is character will pursue discipline. The therapist who does not ask will recommend something the family cannot use.

Question 3 — What has the family already tried, and what did it cost them? The answer is the family's competence and its budget. A family that has spent four months' income on a treatment that failed is not unmotivated; it is exhausted and poorer.

Question 4 — What are the material conditions? Income, food, housing, water, transport, crowding, the number of people per room, the distance to the clinic. Asked plainly, as part of the history, not as a delicate enquiry. Families answer these questions readily when they are asked as facts rather than as suspicions.

Question 5 — Who has the authority to decide, and who has the authority to be believed? These are frequently different people. A grandmother may decide and a pastor may be believed. The plan must be addressed to both.

Question 6 — What does this family want for this child, in ten years? The question produces the family's own developmental goal, which is often more ambitious than the clinic's and always more motivating. It also frequently resolves an apparent cultural conflict: a family that wants its daughter to be a doctor will accept a good deal of developmental advice about her schooling, provided the advice is connected to that goal.

How to use the six. As the frame for a first session with any family whose background differs from the therapist's — and, since the therapist's own background is always particular, as a frame worth using with every family.

13.3a The interpreter

Many of the assessments in this week's work are conducted through a third person, and the third person changes the data.

Rule 1 — Brief the interpreter before the family arrives. Five minutes: what the assessment is for, what you will ask, and the two things you need — first person interpretation ("she says she cannot sleep", not "she says she cannot sleep and I think she is depressed") and no summarising. An interpreter who has not been briefed will summarise, edit, and add opinions, and all three are invisible in the transcript.

Rule 2 — Speak to the family, not to the interpreter. "How long has she not been sleeping?" is asked of the mother, with the eyes on the mother. The commonest error is the triangle in which the therapist and the interpreter talk to each other about a family sitting in the room.

Rule 3 — Build in double the time. Everything takes twice as long, and the assessment that is scheduled for forty minutes will take eighty. A therapist who runs out of time will cut the questions that are hardest to ask, which are the ones that matter.

Rule 4 — Do not use a family member. Not a child, not a cousin, not a neighbour. The information is distorted by loyalty, the family member learns things they should not, and the roles in the household are permanently altered. Where no interpreter is available, reschedule. This is the one rule in the section that should never be broken.

The fifth rule, which is not a rule but a caution. An interpreter from the same community may be known to the family, and the family may withhold material for that reason — a stigma, a relative, a dispute. Ask, at the start, whether they are comfortable, and offer a telephone interpreter from elsewhere if they are not.

13.3b The sentence that opens the conversation

One practical matter that determines whether the rest of the week's method is usable.

Families are frequently asked, by clinics, to explain their culture — "tell me about your customs" — and the request produces either a performance or silence, because nobody describes their own ordinary life as a custom. The question that works asks about a specific recent event and lets the practice appear inside the answer.

The four openings.

- "Tell me about yesterday. Who was with her from morning until she slept?" — produces the network and the practices in one answer.
- "When she was last ill, who decided what to do?" — produces the hierarchy of authority and the explanatory model.
- "When she does something you do not want her to do, what happens in your house?" — produces the discipline practice without asking for it.
- "What do you want for her that your parents did not have, and what would have to be different in your house for her to have it?" — produces the family's own developmental goal, and it is almost always the most useful sentence in the session, because it is the one that makes the family's own goal available to the plan.

What these four have in common. They ask about events rather than about identity. A family asked to describe its culture must decide what the therapist wants and then supply it; a family asked what happened yesterday simply reports, and the culture arrives in the report, where it can be worked with.

The mistake to avoid. Do not say "I understand your culture" — a claim no outsider can make and every family hears as a claim to authority. Say instead what you noticed: "in your house three people look after her and they all have a view. In mine it was one. Tell me how that works for you, and what you would change about it if you could." That sentence gives the family something to correct, and being corrected is how a therapist learns, which is a slower and more reliable method than being told about a culture by a textbook.

13.4 Three techniques

Technique 1 — The network map of a child. Draw the child in the centre, and around her every adult and older child who does something for her, with the function written next to each name. Then mark the one person she goes to when frightened. Then mark the person each of the others believes she goes to. The discrepancy between the last two markings is the finding, and it is frequently the whole of the case.

Technique 2 — The resource inventory. A list of what the household has, not what it lacks: adults, space, income, a church or mosque community, a neighbour, a skill, a relative in another city, a school with a good teacher. The inventory is written with the family, and it is read back to them. Its effect is not sentimental; it changes what the family believes is possible, which changes what they will attempt.

Technique 3 — The two-column budget. One column: what the plan requires in time and money. The other: what the household has. The technique prevents the therapist from writing plans that require a car, a morning off work, or a fee. It is unglamorous and it is the reason some plans are carried out.

13.4a The assessment kit for a household with nothing

A practical inventory of what a therapist can do when the household has no money, no car, no quiet room and no time.

What costs nothing and is worth doing. A hearing screen with a whispered instruction from behind the child, at a metre, in a quiet moment. A language sample — five minutes of the child talking about anything, recorded on a telephone, transcribed roughly, and counted for mean length of utterance by hand. A drawing. A three-step instruction, given in the corridor. An observation of the child with the person she goes to. An observation of the child with the person she does not go to. None of these requires a kit, a fee, or a second appointment.

What the clinic should supply because the family cannot. A referral letter typed and signed in the session, so that the family leaves with it rather than waiting for a post that does not arrive. A telephone number that is answered. A note for the school. A copy of the assessment in a form the family can keep.

What not to ask for. Do not ask a family to return on a day they cannot manage, to bring a document that does not exist, or to obtain a report from a school that is not answering. These requests are made in good faith and they are the mechanism by which families are lost: each one is a small obstacle, and the cumulative effect on a household operating at capacity is a decision not to come back.

The design principle. Every element of the assessment should be deliverable in one visit, by one person, with no equipment, and end with the family holding a piece of paper. That constraint produces a shorter assessment than the textbook describes and a more useful one, because it is completed.

13.4b The four-column plan

The plan for a household under material constraint is written in four columns, and the fourth is the one that keeps it alive.

Column 1 — What we are trying to change. One sentence. Not a list.

Column 2 — What it requires. In time, money, transport and space. Written honestly, including the invisible costs: a morning off work, a fare, a person to mind the other children.

Column 3 — What the household has. From the resource inventory. The column is frequently fuller than the family expected, which is part of its purpose.

Column 4 — What we drop if column 2 exceeds column 3. The reduction, specified in advance. Most plans fail because they were designed at full capacity and nobody decided what to cut when capacity fell. A plan with a stated reduction survives; a plan without one is abandoned whole.

The example. Column 1: "we want him to be understood at school." Column 2: a hearing test, an audiology appointment across the city, a school letter, and ten minutes a day of reading aloud. Column 3: the mother's mornings are free on Tuesdays; there is a bus; the school has a teacher who has been helpful. Column 4: if the audiology appointment cannot be reached, the school letter and the ten minutes continue and the appointment is rebooked; if the ten minutes cannot be managed, the school letter continues alone.

What the column prevents. The all-or-nothing plan, which is the characteristic failure of clinicians working with families in difficulty, and which is experienced by the family as another expectation they could not meet.

13.5 The first session when the family and the therapist disagree

Phase 1 — Ask for the family's account before offering any. The six questions, and particularly question two. A therapist who offers a developmental account in the first ten minutes has produced a competition between two theories, and the family will defend theirs.

Phase 2 — Find the point of agreement. It is almost always the goal: the family wants the child to do well at school, to be safe, to be respected. State it: "we want the same thing for her."

Phase 3 — Offer the second language, not a replacement. "Your grandmother is right that a child needs to be taught respect. I would add something from my side, which is that a child who is shouted at every morning finds it harder to concentrate at school, and concentration is what she needs for the exam." The move is additive: the family keeps its framework and gains a mechanism.

Phase 4 — Say clearly where you cannot go. If a practice is harmful, name it once, as in 13.2a, and hold the line. Ambiguity here is worse than refusal, because the family will hear permission.

Phase 5 — Agree one thing and a review. As in Week 11. One action, dated, with the exception named.

What to do when the family does not return. Assume the plan was unusable rather than that the family was unmotivated, and ask. A telephone call that says "I have been thinking about what we agreed and I wonder whether it was possible in your house" recovers a substantial minority of families who have dropped out, and no service does it.

13.5a When the therapist's own culture is the variable

The week's critique applies to the clinician, and the application is uncomfortable.

What the therapist brings. A household of origin with its own practices — sleeping arrangements, discipline, the meaning of respect, the proper distance between a mother and a grown son, the age at which a child is left alone. A training in a discipline that was normed somewhere. And an institution's assumptions, which are frequently the assumptions of the profession's country of origin rather than of the clinic's neighbourhood.

The three ways this shows up.

As an unexamined standard. The therapist who is irritated by a family and cannot say why is usually meeting a practice that violates a rule she has never stated. The correction is to state the rule: "I notice I am uncomfortable that the eight-year-old is still sleeping in the room with his parents. That is my household's rule, not a finding. What is the reason in yours?"

As a class position. A therapist from a professional household assessing a family in poverty may misread scarcity as disorganisation, or may over-correct and fail to name a real problem. Both errors come from the same place and both are corrected by asking about the material facts rather than inferring them.

As an alliance with the institution. The therapist is employed by a clinic, often by a school, sometimes by a court. Her assumptions drift toward her employer's, and the drift is invisible from inside. The correction is to ask, once per case, "whose interests does my formulation serve?" and to write the answer in the record.

The discipline that holds all three. State your own rule before you interpret the family's. The sentence "I was brought up to think X, and you are describing Y — tell me about Y" is not a confession; it is the technique. It removes the therapist's rule from the position of an unstated standard and puts it on the table where the family can examine it, which is the only position from which a family can disagree with a professional without feeling judged.

13.6 Case: the child with no bedroom

Referral. Morenike, four years and three months, referred by the nursery attached to a private school. The written note: "over-attached to mother, does not self-soothe, still in parents' bed, will not sleep alone. Family has been advised to implement a sleep programme."

The family. Yetunde, 29, a hairdresser who works from the flat. Bayo, 32, a dispatch rider. Morenike, 4. Bayo's younger brother, 24, a student, who sleeps in the sitting room. Total: four people in a flat of two rooms and a sitting room, in a compound of six flats. Languages: Yoruba at home, English at nursery.

Why they came. "The school said they will not promote her to the next class if she is not sleeping alone." Yetunde's tone, in the first minute: "they have told me I am spoiling her. My mother says the same. I want to hear it from a doctor."

What the assessment finds. Morenike plays richly, with a narrative and with other children; she separates from her mother at nursery without distress and greets her warmly on reunion; she has a vocabulary above age in Yoruba and age-appropriate in English; her hearing is normal; her sleep is unremarkable apart from its location — she goes to sleep at nine and wakes at six. When frightened, she goes to her mother. Her mother knows this. Her father believes she goes to her grandmother, who lives two compounds away.

The five findings.

1. There is no evidence of a sleep disorder and there is no spare room. The family occupies two rooms: one for Yetunde and Bayo, one for Morenike and her cousin when the cousin stays. The uncle sleeps in the sitting room. "Sleeping alone" would mean sleeping in a room with a twenty-four-year-old man, which the parents correctly regard as unacceptable. The clinic's recommendation was not a developmental intervention; it was a recommendation to rearrange a household that has no space, made by someone who did not ask.

2. The referral is a description of the housing written in the language of attachment. "Does not self-soothe" describes a child who has never been alone in a room. "Over-attached" describes a child in a two-room flat. Neither phrase is false and both are diagnoses of a circumstance.

3. The child's attachment behaviour is unremarkable. She separates, she explores, she returns. The Strange Situation, if it had been administered, would have had to be administered in a room she had never seen with a stranger she had never met, and her behaviour in that room would have described the room.

4. There is a real question, and it is about the school's requirement. The school's promotion criterion is being used as a developmental standard, and it is not one. The therapist's task is not to adjudicate between the family and the school on a matter of principle; it is to give the family a position and a sentence they can take to the school.

5. The family's own goal is specific and achievable. They want her to sleep in her own bed by the time she starts Primary 1, in fourteen months, so that the uncle can move out when he finishes his studies and the room can be rearranged. They have a plan and it is better than the clinic's.

The formulation. A well-developing four-year-old in a household with four people, two rooms and no spare bed, referred by an institution that has mistaken a housing constraint for a developmental deficit, and whose mother has been told by three authorities that she is spoiling her child. The intervention is three sentences to a school, one piece of information to a mother, and a plan that begins in fourteen months.

The intervention, in five parts.

Part 1 — Say the finding in one sentence, and say it early. "There is nothing wrong with your daughter's development, and there is nowhere in your flat for her to sleep alone. Those are two different things and the school has joined them together."

Part 2 — Give the mother the developmental fact she can use. "A four-year-old who sleeps in her parents' room in a two-room flat is doing what children in most of the world do. What matters for her development is that she is responded to when she calls, and she is."

Part 3 — The letter to the school, written in the session. Three paragraphs. It states that the child has been assessed, that no developmental concern was found, that the family's sleeping arrangement reflects their housing and not their parenting, that the assessment found no basis for a sleep programme, and that the clinic will review the arrangement at a stated date. It is signed by the therapist and copied to the parents. The therapist reads it aloud before anyone leaves.

Part 4 — A plan for fourteen months, in the family's own shape. A bed for Morenike, bought second-hand, in the room she shares; a routine at nine that is hers; and a target — by the time the uncle finishes, she sleeps in her own bed in the room, and the room is rearranged. Written on a page, with the date.

Part 5 — One sentence for the grandmother. "She is not spoiled; she is four, and we have no room." The sentence is rehearsed, because the grandmother's opinion is the one that reaches the mother every day and the mother needs something to say that is not a defence of herself.

13.6a Five months later

The letter to the school was effective in a way the therapist had not anticipated: the school removed the promotion condition and asked the clinic to speak to the nursery staff, which the therapist did, for forty minutes, on the difference between a developmental assessment and a housing description. Nine children in that class stopped being on a sleep programme.

Morenike has a bed. It is in the same room as her mother's, at right angles, and she sleeps in it most nights. The target date is still fourteen months away and the plan is ahead of schedule, which the family reports with pride.

Yetunde has stopped apologising in the first minute of a conversation, which the therapist regards as the case's most significant outcome and which is not measurable by any instrument in this book.

The honest limitation. The uncle still sleeps in the sitting room and the flat is still two rooms. The therapist has not changed the material condition and does not claim to have done so. What she has changed is that a family is no longer being told that its housing is a parenting failure, and that an institution has been corrected, which affects every family that institution sees afterwards.

13.7 Transcript: the letter, read aloud

Nine turns from the third session. The therapist has drafted the letter in front of the parents and is reading it back to them before it is sent.

> **T1 Therapist:** (reading) "To the Head of Nursery. I am writing about Morenike, whom I assessed on the fourteenth. I found no developmental concern. Her play, her language, her separation and her relationships are all appropriate for her age." (pause) Is that right? — The check after the first paragraph. The letter is a clinical document and the family is the authority on its accuracy. **T2 Yetunde:** It is right. **T3 Therapist:** (reading) "The family's sleeping arrangement reflects the size of their home. It is not a sign of developmental difficulty, and I have not recommended a sleep programme." (pause) Yetunde, when you read that, what will you feel? — The question is asked before the letter is sent, because a letter the parent cannot bear to read is a letter that will not be used. **T4 Yetunde:** (quietly) That somebody said it. **T5 Therapist:** (reading) "I would be glad to speak to the nursery staff about the difference between a developmental assessment and a description of a family's housing." (pause) That last sentence is a criticism of the school. Do you want it in? — The choice is offered deliberately. A letter that criticises an institution on a family's behalf, without the family's agreement, converts the therapist into an advocate and the family into a party to a dispute they did not choose. **T6 Bayo:** (for the first time in the session) Leave it in. **T7 Therapist:** Then I will send it today and I will copy you both. (pause) And one more thing. (to Yetunde) Your mother will hear about this letter from the school, not from you. What will you say to her? — The household rehearsal from Week 11, applied. The plan will be tested at home before it is tested at the school. **T8 Yetunde:** (after a pause) That the doctor said she is not spoiled. That we have no room. **T9 Therapist:** Say it again, so that you have said it once before you say it to her.

13.8 Laboratory: assessing across a difference

Aim. To complete a developmental assessment with a family whose language, housing and explanatory model differ from the therapist's, and to report the finding with its conditions attached.

Procedure. Groups of four: therapist, mother, interpreter, observer. The trainer supplies a brief in which the family speaks mainly Yoruba, has no word for "self-soothe", believes the child's night waking is caused by a neighbour's envy, and has four people in two rooms. The therapist may use the interpreter and must produce a one-paragraph written finding in ten minutes at the end.

Marking rubric (20 marks).

Criterion	0–1	2–3	4–5
The six questions	Two or fewer asked	Four asked	All six, in order, with the answers in the family's words
The interpreter	Treated as a participant; spoke to them not the family	Used correctly but not briefed	Briefed, addressed the family, first-person, no summarising
The material facts	Not asked	Asked, and not used	Asked, used, and reflected in the plan
The explanatory model	Ignored, or corrected	Acknowledged	Acknowledged, incorporated, and one mechanism added alongside

Criterion	0–1	2–3	4–5
The written finding	"Language delay, insecure attachment"	A description without conditions	A finding with the language, setting and conditions stated

The distinction that carries most of the marks. The top band requires the written finding to state the conditions of the measurement. "Receptive language age equivalent three years, assessed in English as a second language, with the mother present, in a clinic room" is the standard; "language delayed" is a failure of the exercise however accurate the number.

13.9 Six errors

Error 1 — Treating a housing description as a developmental finding. The week's central error. "Does not self-soothe" and "no bedroom" are the same observation written by two different professions.

Error 2 — Relativism as kindness. "In your culture that is normal" is said to avoid a difficult conversation and it abandons the child. Culture explains a practice; it does not settle whether the practice is good for this child.

Error 3 — Importing the norms without their population. Every instrument in the course was normed somewhere. Using it elsewhere is legitimate; using it elsewhere as though it were normed there is not.

Error 4 — Ignoring material conditions in the formulation. A plan that requires a car, a morning off, a fee or a spare room will fail, and the family will be recorded as unmotivated. The two-column budget exists to prevent this.

Error 5 — Using a family member as interpreter. It distorts the information, alters the household's roles, and cannot be undone. Reschedule instead.

Error 6 — Fighting the institution in front of the family. The letter in the case is a clinical instrument; a therapist who becomes the family's advocate against a school has taken a side in a dispute she will not be present to finish, and the family will carry the consequences.

13.10 Culture, faith and the limits of the critique

The four critiques in this week do not license a fifth, which would be: nothing can be known. The position this course takes is stated here so that students can hold it under pressure.

What is universal enough to rely on. Children need at least one reliably responsive adult; they need food, sleep and safety; they need language input from people who speak to them; they need to be protected from violence; and they develop through play with other children. These are not Western values; they are findings that hold across the ethnographic record, and a therapist who cannot assert them cannot protect a child.

What varies and must not be pathologised. Sleeping arrangements, weaning, who provides care, the age of school entry, the amount of autonomy given to a child, the

manner of discipline within limits, the expression of emotion, the meaning of a disability, the conduct of mourning, and the relationship between the family and institutions.

The test that distinguishes them. Ask of any practice: does it protect or damage the child's development — her body, her language, her capacity to relate, her access to learning? If it damages, it is not protected by culture. If it does not, the therapist has no business treating it as a problem, whatever her own household did.

Faith and the explanatory model. A family's spiritual account of a child's difficulty is not a competitor to the assessment and does not need to be displaced. It needs to be joined, and one mechanism added alongside. The pastor who is told that the clinic is not an enemy becomes an ally who can say, from the pulpit, that a hearing test is not a lack of faith — and that sentence reaches more families than the clinic will see in a year.

> **DEBATE**
> "A developmental assessment should not be conducted across a language barrier at all." Argue for: the instrument measures the language, the conditions invalidate the norm, and a wrong number is worse than no number because it becomes a label. Argue against: no assessment means no service, waiting lists are a form of harm, and a properly-reported assessment with its conditions stated is more useful than nothing. Then state what you would actually do for a five-year-old who speaks only Hausa in a clinic where nobody does, and what you would write in the record.

13.11 Evidence note

What the research supports. The cross-cultural replication of attachment classifications is substantial: the categories appear in every studied population, and the distribution varies with caregiving practice, which supports the theory's universality and undermines any single distribution being treated as a standard. The evidence on poverty as a developmental variable is among the strongest in the field, with the mechanisms — stress physiology, nutrition, crowding, instability — demonstrated across cohorts. The association between hearing loss and apparent cognitive and social difficulty is robust and correction improves outcomes.

What is weaker. Norms for most developmental instruments are drawn from samples that are unrepresentative of the populations in which the instruments are used, and the published corrections are sparse. The evidence on culturally adapted parenting interventions is growing and remains thinner than the evidence for the unadapted versions, which is a problem of research funding rather than of principle.

What is not supported. That early independence training produces later independence. That co-sleeping produces enmeshment. That a child raised by a network is at risk by virtue of the network. And that cultural sensitivity requires suspending judgement — the evidence on child maltreatment across cultures is clear enough that a therapist who cannot name harm is not sensitive but negligent.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same sleeping arrangement is

unremarkable in one household and a safeguarding concern in another, and what distinguishes them is not the arrangement but the responsiveness of the adults in it.

13.12 Key terms

Norm — a distribution of measurements in a population; a description, not a standard.

Explanatory model — the family's own account of what is wrong and why; it predicts what they will do.

Distributed caregiving — care provided by a network rather than a pair; the norm in most of the world.

Material conditions — income, food, housing, crowding, water, transport; developmental variables, not background.

Bandwidth — the finite attention a caregiver has for the work of parenting, reduced by scarcity.

Cultural formulation — a structured enquiry into the family's world, account, resources, authorities and goals.

Conditions of measurement — the language, setting, relationship and instruction under which a finding was obtained, which must be reported with it.

Relativism — the position that no practice can be judged from outside its culture; incompatible with child protection.

Network map — a diagram of who does what for a child and who the child goes to when frightened.

13.13 Self-test

1. State the four critiques, and give one piece of evidence for each.
2. What are the five mechanisms by which poverty affects development, and which one most changes clinical practice?
3. Give the question that replaces the Strange Situation in a household with distributed caregiving, and the follow-up question that finds the problem.
4. Name the four conditions of measurement and explain how each contaminates a finding.
5. When is a practice harmful rather than different, and how do you say so in three sentences?
6. Give the four rules for working with an interpreter, and the one that must never be broken.
7. Why does the therapist ask the family to read the letter before it is sent?
8. In the Morenike case, what was the referral actually describing?

Answers. (1) Western norms — the Strange Situation's distribution varies with caregiving practice across cultures; poverty — the stress-physiology and nutrition evidence; distributed caregiving — the ethnographic record and the network as the normal unit;

measurement — the language and setting effects on test performance. (2) Chronic stress physiology, food insecurity, crowding, housing instability, and parental bandwidth — the last changes practice most, because it converts "inconsistent parenting" into a resource calculation. (3) "When she is frightened, who does she go to?" and "does that person know they are the one?" (4) Language, stranger, setting, instruction — each measures the situation as well as the child, so the finding must be reported with them attached. (5) When it damages the child's body, language, capacity to relate or access to learning; name the practice factually, name the concern developmentally, and state what you are not able to work with. (6) Brief beforehand; speak to the family; double the time; never use a family member — the last must never be broken. (7) Because a letter the parent cannot bear to read will not be used, and because the family is the authority on its own accuracy. (8) The family's housing.

13.14 Reflective task

Write down three things your own household did with children that a textbook would call a problem, and three that it did that the textbook would call correct. Then answer: which of the six did you assume was normal until this week? Most students find at least one in each column, and the discovery that the textbook's standards are also somebody's household practice is the week's actual learning.

13.15 Further reading

- Henrich, J., Heine, S. and Norenzayan, A. "The weirdest people in the world?" — the sampling problem, in general.
- Keller, H. Cultures of Infancy — the comparative evidence on caregiving practice.
- McLoyd, V. "Socioeconomic disadvantage and child development" — the poverty mechanisms.
- Kirmayer, L. on cultural formulation — the structured interview this week adapts.
- Serpell, R. The Significance of Schooling — African evidence on what school readiness actually means.

Next: Week 14 — When development goes wrong: the differential habit.

Week 14 — When development goes wrong: the differential habit

The four categories. For any developmental concern, run the check before you form a hypothesis. Every presentation in this course is one of four things, and the four have different prognoses and different referrals.

Category	What it means	Example	What it usually requires
Variation	Within the normal range, differently timed or differently shaped	Late talking with good comprehension at 22 months	Information, review, no referral

Category	What it means	Example	What it usually requires
Delay	Behind the expected sequence, but progressing	Two-word combinations at three years	Assessment and early intervention
Regression	Loss of a skill that was present	Words at two, silence at two and a half	Urgent medical assessment
Symptom	A behaviour that expresses something else	Night waking after a house move	The thing it expresses

The five systemic questions. Behind every presentation there is a family, and the family has five facts that predict what will happen next.

1. **When did it start, and what else changed then?**
2. **Who noticed first, and who disagrees?**
3. **What does the family do when it happens, and what happens next?**
4. **What would improve if it stopped — and what would get worse?**
5. **What has already been tried, and what did that teach us?**

The differential table. The table below is the instrument, not a reference. It is used in the first session, from memory, for any presentation in the course.

Presentation	Developmental differential	Systemic differential
Aggression at 5	Language delay; impulsivity; trauma	Parental conflict; inconsistent limits; a new baby
Withdrawal at 12	Depression; bullying; neurodivergence	Family secrecy; a parent's illness; migration
Decline at 70	Delirium; dementia; depression; hearing loss	Bereavement; relocation; loss of role
Non-attendance at 9	Anxiety; learning difficulty; bullying	A function at home; an unresolved parental conflict
Clinging at 2	Temperament; separation; illness	A caregiver's own anxiety; a recent change of carer

Why a "habit" and not a "skill". Because the differential is not something a clinician does once; it is something they do automatically, before the formulation forms. The difficulty is that formulations form in about four seconds, from the referral letter, before the family has sat down. This week's purpose is to install a pause between the referral and the hypothesis, and a pause is installed by repetition rather than by understanding.

> **CASE**
> A boy of two and a half who had forty words at twenty months and has none now. His mother is certain. His father says he is lazy. The grandmother says he will talk when he is ready, as his uncle did. The nursery says he is fine. Regression is the one category in which the family's disagreement is not a family problem but a clinical emergency, and the therapist's first task is to establish the facts before anyone's theory is discussed.

14.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	Student activity
0–8	The four-second formulation	How a hypothesis forms from a referral letter, and why that is the problem	Read a referral and write their hypothesis in one sentence
8–22	The four categories	Variation, delay, regression, symptom — with examples from earlier weeks	Sort twelve presentations into the four
22–34	Regression	Why it is the one urgent category, and what it demands	Identify the three facts that establish it
34–48	The five systemic questions	The questions that predict what the family will do	Run the five on a case, in pairs
48–62	The differential table	Worked from memory, from four presentations	Reproduce the table for two presentations
62–74	The red flags	The specific findings that require a same-week referral	Match eight red flags to their urgency
74–84	Referral and its aftermath	How to refer well, and what to do while the family waits	Write a referral letter
84–90	The assessment	What the case analysis requires	Draft the outline of their case

The teaching decision that makes the week work. The lecture opens by having every student write their hypothesis from a referral letter in one sentence, before any discussion. The sentences are read aloud, and they are almost always characterological ("anxious mother", "difficult child", "attention-seeking"). The class then works the same case properly and discovers how little the letter supported. The exercise is uncomfortable and it is remembered, which is what a habit requires.

The sentence the lecture must land. "The referral letter is a description of somebody's worst day, and your first job is to find out which of four things you are looking at."

14.2 The four categories worked

Variation. The child is developing along a recognisable path at a different rate or in a different shape. The wide range of normal at every milestone is the single most under-used fact in clinical practice: walking anywhere from nine to eighteen months, first words from nine to eighteen months, two-word combinations from eighteen to thirty months, bladder control from two to four years. A child at the far end of any of those ranges, with good comprehension, good play, good relating and no regression, is a child about whom the correct action is review.

The clinical discipline of variation is to say so plainly. Parents are frequently more reassured by a number than by a sentence — "the normal range for this is nine to eighteen months and he is fifteen" — and giving the range is not false reassurance; it is the finding.

Delay. The child is behind the expected sequence and progressing slowly. The distinguishing feature is not the gap but the trajectory: a child who is six months behind and gaining a month a month is closing; a child who is six months behind and gaining a month every three is falling further behind. The trajectory requires two measurements,

which means the assessment is not complete in one visit, which is why the review date is part of the finding rather than a courtesy.

Regression. The child has lost a skill. It is the only category that is always urgent, for three reasons: some causes are treatable and time-critical (a space-occupying lesion, a metabolic disorder, epileptic encephalopathy, a hearing loss from infection, severe iron deficiency); some are progressive; and the family's account of a regression is frequently disbelieved by the first professional they see, which costs months. The therapist's obligation is narrow and absolute: establish whether it is real, and if it is, refer the same week.

Symptom. The behaviour is an expression of something else — a change, a loss, a conflict, a fear, an illness. This is the category the rest of the course has been teaching, and it is the one most often over-applied: a therapist trained in family systems will find a systemic explanation for a regression that needs an MRI. The four-category check exists precisely to prevent the reverse error, and the therapist's discipline is to run it first, even when the systemic explanation is obvious.

The fifth possibility that is not a category. Nothing. A substantial proportion of referrals to child services describe a child who is developing normally in a family that is under strain, and the correct formulation is "no developmental concern; a family that needs help". That formulation is not a failure to diagnose; it is a diagnosis, and it should be written down as clearly as any other.

14.2a The five systemic questions, worked

Each question has a purpose, a common wrong answer, and a follow-up that defeats it.

Question 1 — When did it start, and what else changed then? The purpose is to date the onset relative to events, which is the entire method of Week 1. The common wrong answer is a duration ("about a year"), which is unusable. The follow-up: "what was happening in the house that month — a move, a birth, a death, a new job, someone leaving?" Families can usually date an event even when they cannot date a behaviour, and the event dates the behaviour.

Question 2 — Who noticed first, and who disagrees? The purpose is to find the household's fault line. The disagreement is data: it tells you who has the child during the day, whose authority is contested, and which of the adults will not support the plan. The follow-up: "what does he say when you tell him?" The answer frequently reveals a second, older disagreement.

Question 3 — What does the family do when it happens, and what happens next? The purpose is the behavioural sequence, and it is the question that produces the intervention. The follow-up matters more than the question: keep asking "and then what?" until the sequence closes. A typical sequence: he cries, she shouts, he cries harder, she gives him the phone, he stops. The phone is the intervention's target, and nobody in the family has described the sequence as a sequence.

Question 4 — What would improve if it stopped, and what would get worse? The systemic question, and the one students forget. A symptom does something in a

household, and its removal changes the household. The follow-up: "who would notice first?" This question identifies the person whose position depends on the problem, and that person is frequently the one who will not cooperate.

Question 5 — What has already been tried, and what did that teach us? The purpose is the family's competence and its budget. The follow-up: "what happened when you stopped?" A family that tried a plan and abandoned it has usually learned something about what is feasible, and the therapist who does not ask will recommend the same plan again.

Why the five are asked in this order. Question 1 establishes the timeline, which the other four depend on. Question 2 finds the fault line, which tells the therapist whom to address. Question 3 produces the behavioural sequence. Question 4 is asked only after the sequence is known, because it is a question about function and function cannot be guessed. Question 5 produces the plan's constraints, and asking it earlier tends to convert the interview into a negotiation.

14.2b Red flags: the findings that require a same-week referral

The list below is not exhaustive and is not a licence to practise medicine. It is the minimum that a family therapist must recognise in order to refer rather than to treat.

Regression of any kind. Loss of words, loss of walking, loss of bladder control after a year of dryness, loss of social smiling, loss of a skill that was established. Urgent.

Loss of vision or hearing, or a change in either. A child who stops turning to sound, or who begins to hold objects very close. Urgent.

A change in consciousness or awareness. Episodes of "switching off", staring that cannot be interrupted, a child who does not respond to pain. Urgent.

Headache that wakes the child, or vomiting on waking, or a new squint, or a change in gait. Urgent.

Growth failure. Weight loss, or a fall across two centile lines, or a child who is not gaining. Urgent.

Any symptom that is worse in the morning and better later, which is the pattern of raised intracranial pressure, and any symptom that is steadily progressive.

Seizures, or suspected seizures, including episodes the family describes as "he goes quiet and his eyes roll".

Self-harm, suicidal ideation, or a plan in any child old enough to hold one, at any age. Same day.

Sexual knowledge or behaviour beyond the child's developmental stage, or any disclosure of abuse. Same day, safeguarding framework.

A parent's account that does not fit the injury. The safeguarding question, asked every time, without exception, and without requiring certainty.

The discipline. The therapist does not need to know what the condition is. She needs to recognise the category, say so plainly to the family, and make the referral in writing before the family leaves the building. "I do not know what is causing this and it needs to be looked at this week" is a complete and honest clinical statement, and it is far better than a formulation that keeps the family in the therapy room for three months.

14.2c The child who is referred twice in a year

A configuration that tests the four-category habit more than any single presentation: the same child, returned to the clinic, with the same complaint, by a different professional.

What the second referral usually means. One of four things. The first assessment found nothing and nobody said so clearly. The first intervention was not implemented, for reasons that were material rather than motivational. The complaint has changed in a way nobody recorded. Or the first formulation was wrong.

The four questions that distinguish them, asked in this order.

1. "What has changed since the last time he was here?" If nothing has changed in the child and something has changed in the setting — a new school, a new teacher, a new baby — the second referral is about the setting.
2. "What were you told to do, and what happened when you did it?" If the family did the action and it failed, the formulation is probably wrong. If the family did not do it, the reason is the finding.
3. "Who sent you this time, and what did they say?" The referral's source is information: a school that refers twice is telling you about the school's capacity, not only the child's difficulty.
4. "What would have to change for you not to need to be here?" The family's answer is the outcome measure, and it is frequently something the first assessment never addressed.

The trap. The second referral invites the therapist to work harder on the same formulation. The discipline is to work the four-category check again from the beginning, as though the child were new, because one time in five they are.

The record. A second referral should produce a note that says explicitly which of the four explanations applies and what the evidence is. A file that accumulates four assessments and no conclusion is a file that will produce a fifth, and the fifth will be the one in which somebody finally notices that the question itself was the wrong one.

14.5a The assessment the family can afford

A short section with a large practical consequence, placed here because this week generates more referrals than any other.

The arithmetic. A specialist appointment may cost a fare, a day's income, a night's accommodation and the care of other children. For a household with three children and a daily wage, the true cost of a referral is frequently more than a month's discretionary income, and the therapist who does not compute this will make referrals that are not attended and will conclude that the family is not motivated.

What to ask, every time, before the referral is written. "What would it take for you to get there?" Then listen for the four costs: money, time, distance, and the care of the other children, which is the cost most often omitted from a clinician's arithmetic and the one most often decisive. Then solve the ones that can be solved. A referral to a Thursday morning clinic for a mother who works Thursday mornings is not a referral; it is a suggestion.

The three practical moves. Ask the receiving service whether there is a cheaper or closer site, which there frequently is and which nobody has asked about. Write the letter in a way that lets the family be seen once rather than three times, by putting the trajectory and the red flags in the first paragraph. And give the family a single named contact at the clinic who will take a telephone call, which reduces the number of journeys and is the therapist's own job rather than a system's.

What cannot be solved, and must be said. Sometimes the appointment is unaffordable and the service is distant, and the honest response is to say so, to write to the service, and to be explicit with the family about what the clinic will do in the interval. A family told the truth about a difficulty can plan around it; a family told that help is coming cannot.

14.3 The method: a differential assessment in one session

Step 1 — Establish the facts of the trajectory, not the level. Two questions: "what could he do six months ago that he cannot do now?" and "what can he do now that he could not do six months ago?" Every assessment that opens with these two questions is better than one that opens with a milestone checklist, because it produces a direction.

Step 2 — Place the presentation in one of the four categories, provisionally, and say so. "On what you have told me this looks like a delay rather than a regression, and I will tell you if that changes." Families find provisional formulations more usable than certainty, because a provisional formulation leaves room for their knowledge.

Step 3 — Run the red-flag list. Thirty seconds. If any is positive, stop the assessment and refer.

Step 4 — Run the five systemic questions. The timeline, the fault line, the sequence, the function, the attempts.

Step 5 — Complete the four-category check with the evidence you have. If the evidence is insufficient, say which category is most likely and what would settle it — an audiology appointment, a developmental assessment, a second measurement in three months.

Step 6 — Write the differential in the record, in order of probability, with the reasoning. Not a diagnosis. A differential with reasons is what a clinician needs six months later, and it is what protects a family from being reassessed from scratch by every professional they meet.

Step 7 — Decide the referral, and make it in the room. Typed, signed, copied, with a telephone number and an expected waiting time. And say what happens if the waiting list is long: what the family will do in the meantime, and who will chase it.

Step 8 — Book the review before the family leaves. Even where a referral has been made. The review is the therapist's own second measurement, and it is the thing that catches the case in which the first formulation was wrong.

14.3a Referring well, and what to do while the family waits

The letter. Six elements: the reason for referral in one sentence; the trajectory, with dates; what the four-category check found; the red flags that are absent and the ones that are present; what has been tried; and what the referrer wants. One page. Addressed to a named service, with a named person if possible, and copied to the family.

The chase. A telephone call at two weeks and at six weeks. In most systems a referral that is not chased is a referral that is lost, and the loss is invisible to everyone except the family, who conclude that nothing can be done.

The waiting period. The family is waiting for a service that may take months. Three things can be done in the meantime and each is worth doing: protect sleep; give the child one domain of competence; and give the family one explanatory sentence they can use with relatives, which reduces the pressure that usually falls on the mother.

The wrong thing to do while waiting. Do not begin a treatment for the condition you suspect before it has been assessed, because the treatment will muddy the assessment. Do not tell the family that the referral is a formality. And do not stop seeing them: the referral is to a service, not away from the therapist.

14.3b Reading somebody else's assessment

Most children who arrive at a family therapist have been assessed before, and the previous report is frequently the most misleading document in the file.

What previous reports usually contain, and what each thing means. A score, without the conditions under which it was obtained. A diagnosis, frequently made at the end of a single appointment. A recommendation, frequently written for a service that no longer exists. And an absence — the things nobody asked, which are invisible because they are not written.

The four questions to ask of any previous assessment.

1. **Who did it, in what language, with whom present?** Week 13's conditions of measurement, applied to a document rather than to a session.
2. **What was the child doing at the time?** Children are assessed during illness, after a move, in the week a sibling is born. A score obtained in a bad month is a score of the month.
3. **What has changed since?** A report is a photograph. The family's own account of the last year is the film, and it is usually more informative than the report.
4. **What did they not ask?** The hearing. The sleep. The food. The school. The grandmother. These are the five omissions in most of the reports this book's students will read.

The rule for using a report. Quote it with its date and its conditions, and never inherit its conclusion without its evidence. "A developmental assessment in June last year found a language delay; I do not have the scores or the conditions, and I will ask for them" is a professional sentence. "He has a language delay" said on the basis of another clinician's paragraph is not.

When to ask for the original. Whenever a decision depends on it. A request for the full report, with the scores and the conditions, is a legitimate professional act and takes one telephone call.

The family's experience of repeated assessment. A child who has been assessed four times has usually been assessed well at least once, and the family has usually been told four different things. The therapist's question at the start is not "what did they find?" but "what were you told, and what did you think of it?" The second half is the part nobody asks, and it is where the family's real formulation lives, usually in a sentence the first clinician did not hear because the appointment had already run over its time.

14.4 Three techniques

Technique 1 — The four-category card. A card, carried, with the four categories on one side and the red-flag list on the other. Its purpose is not reference; it is to slow the four seconds between the referral letter and the hypothesis. Clinicians who carry it report that they use it for the first year and then no longer need it, which is the definition of a habit.

Technique 2 — The onset line. A single horizontal line from birth to today, with the onset of the concern marked, and every event the family can date marked above it. The line does two things: it dates the problem, and it makes visible the coincidence that the family has not noticed. It is the Week 1 timeline applied to a symptom rather than a life, and it takes four minutes.

Technique 3 — The two-measurement rule. Nothing that could be a delay is concluded from one measurement. The first assessment produces a level and a hypothesis; the review produces a trajectory. The rule is stated to the family as part of the finding — "I am going to measure this again in three months, and what I am looking for is not a different number but a different direction" — which converts the review from a courtesy into the assessment.

14.5 The first session with "something is wrong"

Phase 1 — Take the account without a hypothesis. Fifteen minutes in which the therapist asks only factual questions and writes down the family's words. The discipline is to say nothing interpretive in this phase, which is harder than it sounds when the account is vivid.

Phase 2 — The two trajectory questions. What could he do six months ago that he cannot now; what can he do now that he could not before.

Phase 3 — The red-flag list, quickly and out loud. "I am going to ask you some quick questions about his health. Most of them will be no." Families accept a list; they become frightened by a vague enquiry.

Phase 4 — The four-category placement, stated provisionally. "This looks like X to me at the moment, and here is what would change my mind."

Phase 5 — The referral, made in the room. Typed, signed, copied, explained.

Phase 6 — One thing for the wait, and the review date. Both written.

Why the hypothesis is withheld in phase 1. Because a family that hears a hypothesis in the first five minutes organises everything afterwards to support or refute it, and the assessment then measures the family's theory rather than the child.

14.6 Case: the boy who stopped talking

Referral. Mrs Okafor telephones about her son, Chuka, two years and seven months. "He has stopped talking. He used to say words. Now he says nothing. My husband says he is lazy. My mother says his uncle was the same and he talks now. The nursery says he is fine."

The household. Mrs Okafor, 31, a teacher. Mr Okafor, 36, a civil servant, posted to another state and home at weekends. Chuka, 2;7. A sister, 8 months. The paternal grandmother, 63, who lives with them and who raised Mr Okafor's younger brother, who "did not talk until four".

The trajectory questions. What could he do six months ago that he cannot now? "He had words. Maybe forty. He said 'mama', 'water', 'go', 'no', 'ball'. He would point at the fridge and say 'juice'." What can he do now that he could not before? "Nothing. He does not say anything now. He pulls my hand." Mother's voice, at this point: "I have been telling them for four months."

The onset line. Marked in the session: at 20 months, forty words and two-word combinations. At 22 months, the grandmother's sister died and the household travelled for the burial. At 23 months, the mother returned to work and the grandmother took over the days. At 24 months, the sister was born. At 25 months, the words began to reduce. At 27 months, silence. At 29 months, the referral.

The red-flag list. Regression — positive. Hearing change — uncertain; the mother says he does not turn when called, which she had attributed to stubbornness. Consciousness — no episodes reported. Headache and vomiting — no. Growth — he has lost weight since the sister was born, which the mother mentions only when asked. Seizures — "sometimes he goes still and his eyes go somewhere else", which the mother has mentioned to two people who told her it was normal. Safeguarding — nothing disclosed.

What the therapist does, in order.

First, she says the sentence. "A child who had forty words and now has none has lost something that was there, and that is not laziness and it is not the same as his uncle. It needs to be looked at this week, and I am going to write the letter now." The sentence is

said to the mother, in the grandmother's hearing, and it is the sentence the mother has been waiting four months for.

Second, the referral, in the room. To a paediatrician with a neurology interest, marked urgent, with the trajectory, the onset line, the red flags (regression, possible hearing change, possible absence episodes, weight loss), and one request: assessment of a regression of language with possible seizures in a two-year-seven-month-old.

Third, the audiology referral, to run in parallel. Regression with a possible hearing change requires a hearing assessment regardless of what the paediatrician finds.

Fourth, the family work, which continues and is not the treatment. The grandmother's belief is not stupidity; it is the family's available explanation, and it is protecting everyone from a fear. The therapist does not argue with it. She asks the grandmother one question: "when your brother did not talk, did he lose words he had already said?" The grandmother is silent, and then says: "no. He never said any." The distinction has been made by the family's own authority, in the family's own terms, and it took one question.

Fifth, one thing for the wait, and one for the household. For the wait: a communication book, in which the mother writes what Chuka communicated today and how, which gives the paediatrician four weeks of data and gives the mother something to do. For the household: twenty minutes a day with the mother alone with Chuka, at a fixed time, doing whatever he chooses, which addresses the thing that is also true — that a boy of two who has lost his words has also lost his place, to a sister who is eight months old.

The formulation, written in the record. "Regression of expressive language from a 40-word vocabulary with two-word combinations at 20 months to no words at 27 months, in a child with possible hearing change, possible absence episodes, and weight loss. Not explained by the family's circumstances, which are relevant but not causal. Urgent paediatric referral made; audiology referral made in parallel; family work continues and is not the treatment of the regression."

The five systemic findings, which are real and are not the diagnosis.

1. The mother noticed four months before anyone acted, and was disbelieved three times — by her husband, her mother-in-law, and the nursery. The disbelieving is a clinical event and it has cost four months.
2. The father's absence at weekends means the household's authority is the grandmother's, and the grandmother's explanation is the family's explanation.
3. The sister's birth and the mother's return to work are genuinely relevant to the household's capacity, and they are not the cause of a regression. The distinction matters because the family will otherwise spend a year believing that a return to work caused a neurological event.
4. The weight loss is a second red flag that nobody had investigated, and it is mentioned almost by accident.

5. The family's disagreement is not a communication problem to be worked on in therapy; it is a race against time, and the therapist's job is to end it by making the referral, not by facilitating an understanding.

14.6a Five months later

The paediatric assessment found epileptic activity on an EEG, with clinical absence seizures, and Chuka was started on treatment. His language has begun to return — twenty words at the last review, no combinations yet. The neurologist's view is that the outcome depends on the underlying cause, which is still being investigated, and the therapist records that view in the family's copy rather than translating it into hope.

The weight loss was investigated and attributed to a feeding difficulty that developed after the sister's birth, and it has resolved with a dietetic plan.

The grandmother has become the family member who telephones the clinic. This is the case's most instructive outcome: the person whose explanation was the obstacle became the person who chases appointments, and she changed not because she was argued out of her belief but because one question — "did he lose words he had already said?" — gave her a fact that her own experience could not explain.

The father has attended twice, on Saturdays, and the therapist has used both sessions on one question: what does he want his son's life to be like at twenty-five? It is a Week 13 question and it is doing more work than any explanation of the regression.

The honest limitation. Nobody knows yet what Chuka's language will be at six. The therapist's record says so, and the family has been told so plainly. What has changed is that the question is in the hands of the people who can answer it, and that four months of disbelief have ended.

What would have changed the plan. If the mother's account had been of a delay rather than a regression, the referral would have been routine rather than urgent, and the family work would have been the treatment rather than an adjunct. If the safeguarding question had produced a disclosure, everything would have stopped. And if the paediatric service had a twelve-month waiting list, the therapist's obligation would have been to say so, to write to the service, and to tell the family that she would chase it monthly — because a family told the truth about a waiting list can survive it, and a family told that help is coming cannot.

14.7 Transcript: one question to a grandmother

Eight turns from the second session. The grandmother has come because Mrs Okafor asked her to. She has been in the room for four minutes and has said that the boy will talk when he is ready.

> **T1 Therapist:** Mama, I want to ask you about your brother. **T2 Grandmother:** He did not talk until he was four. Then he talked. Now he is a lecturer. — The family's evidence, offered confidently and in good faith. **T3 Therapist:** A lecturer. (pause) And before he talked — did he say any words at all? Even a few? **T4 Grandmother:** (thinking) No. He never said anything. He just pointed. **T5 Therapist:** (gently) That is the difference, then. Your brother never started. Chuka started, and then stopped. (pause) Those are not the same thing, and the second one needs a doctor this week. — The distinguishing fact, delivered in the family's own material rather than in the therapist's vocabulary. The therapist has not said "regression"; she has used the grandmother's own memory. **T6 Grandmother:** (long pause) I did not know that was different. **T7 Therapist:** Nobody explained it to you. That is not your fault. (pause) Will you help me? His mother says he does not turn when he is called. You are with him in the day. What do you see? — The grandmother is recruited as an observer, which converts her from the family's obstacle into its second witness. This is the turn the case needed, and it is not a technique for managing a difficult relative; it is the recognition that she is the person with the most data. **T8 Grandmother:** (after a pause) He turns if he sees me. If I call from behind, he does not.

14.8 Laboratory: the differential under time pressure

Aim. To place four presentations in the four categories, identify the red flags, and make the correct referral decision, in seven minutes each.

Procedure. The class is divided into pairs. Four briefs are distributed, one at a time, with seven minutes on the clock for each: read, run the four-category check, run the red-flag list, and write the referral decision and its urgency. No discussion is permitted between briefs. The marking is by the trainer, against a prepared answer key, and the scores are read out anonymously.

Marking rubric (20 marks over four briefs).

Criterion	0–1	2–3	4–5
Category	Wrong category	Right category, wrong reasoning	Right category, with the distinguishing fact named
Red flags	Missed a positive flag	Found the flags, misjudged urgency	Found the flags, judged urgency correctly, and named the referral
The referral	None, or vague	Made, without a named destination	Made in writing, to a named service, with a stated urgency and what is wanted
Language	Diagnostic label without assessment	Description without conditions	Description with trajectory, conditions, and a provisional formulation

The four briefs, and the answer key.

Brief 1. A girl of three with no words, good comprehension, rich play, no regression, hearing tested normal. **Answer:** delay in expressive language with preserved comprehension — routine referral for speech and language assessment, review in three months, no red flags. The error the brief is designed to catch is the urgent referral, which is common among trainees who have just learned about regression.

Brief 2. A boy of four who could dress himself, count to ten and use sentences, and who now cannot, over six weeks, with new stumbling and a change in handwriting. **Answer:** regression with a motor component — urgent. The error is the family-systems formulation, which is elaborate and wrong.

Brief 3. A girl of nine, refusing school for three weeks, after her parents' separation, with no red flags, normal report card, one named friend. **Answer:** symptom — anxiety in the context of a family transition; no medical referral; family work is the treatment.

Brief 4. A boy of two who has never walked independently, is not yet pulling to stand, and whose mother says he "went backwards" at fourteen months after a fever. **Answer:** regression with motor involvement, plus a possible delay — urgent, with the fever named in the letter. The error is to accept the family's explanation ("he was ill") without asking what he could do before the illness.

14.9 Six errors

Error 1 — Forming the hypothesis from the referral letter. The letter is a description of somebody's worst day, written by someone with a different agenda. The four-second hypothesis is the thing this week exists to interrupt.

Error 2 — Finding a family explanation for a regression. The most dangerous error in the book. A regression has a medical differential until proved otherwise, and a systemic formulation that keeps the child in the therapy room delays the assessment.

Error 3 — Accepting a diagnosis of "dementia" or "delay" as an explanation for a change. Week 12's lesson applies at every age: a change is a new event, not a progression, until investigated.

Error 4 — Referring without a trajectory. A referral letter that says "language delay" without dates cannot be triaged, and will be placed at the bottom of a list. A letter that says "40 words at 20 months, none at 27" will be seen this week.

Error 5 — Referring and disappearing. The referral is not the end of the therapist's involvement; it is the point at which the chasing starts and the family work continues. A family that is referred and then not seen concludes that the referral was a dismissal.

Error 6 — Treating the family's disagreement as the problem. The Okafor family disagreed for four months, and the disagreement was real, and it was not the clinical problem. A therapist who spends the four months working on the disagreement has done harm with good intentions.

14.9a Saying "I do not know" well

This week produces more uncertainty than any other in the course, and the way a therapist handles not knowing determines whether a family trusts the rest.

The three bad ways to not know. The hedge — a formulation so vague that it commits to nothing ("there are some developmental concerns we should keep an eye on"), which leaves the family without an action. The false certainty — a diagnosis supplied to end the conversation, which is comfortable for the therapist and dangerous for the child. The

deflection — "that is a question for the paediatrician", which is accurate and, said early and often, communicates that the therapist has nothing to offer.

The good way, in four moves.

Move 1 — Say what you do know, concretely. "I know he had forty words and now has none, and I know that is not laziness." Certainty about the facts is available even when certainty about the cause is not.

Move 2 — Say what you do not know, and why. "I do not know what is causing it. assessment this week rather than an opinion from me, which would cost him a month and tell you nothing."

Move 3 — Say what would tell you. "If the hearing test is normal and the paediatrician finds nothing, the next thing I would look at is X." This gives the family a map of the uncertainty, which is more useful than its removal.

Move 4 — Say what you will do while not knowing. "I will chase the appointment in two weeks, I will see you in a month, and we will work on the sleep and on his communication book in the meantime."

Why the fourth move matters most. A family can tolerate uncertainty about the future if they are certain about the therapist's next action. The families who leave and do not return are not the ones who were told "I do not know"; they are the ones who were told it and then given nothing to hold.

14.10 Culture, faith and the urgent referral

The cost of a referral. For many households an appointment across the city costs a fare, a day's income, and the loss of a job. The therapist who says "you must go to the teaching hospital on Tuesday" without asking what that costs has made a recommendation that will not be followed. The question that must precede every referral is "what would it take for you to get there?"

The family's explanation and the family's fear. The grandmother in this week's case is not obscurantist; she is holding the family's only available explanation, and it protects everyone from a fear that is worse. The therapist's discipline is to join the explanation and add a fact, not to displace it. One question, asked in the family's own material, did more than an hour of explanation would have.

Faith and the timing of medical care. Where a family believes that healing is a matter for prayer, the sequence that works is: pray, and go. The framing of the referral as stewardship — seeing what has been made — is not a manipulation; it is a translation, and Week 13 established that translation is part of the job. What cannot be conceded is the delay, because the conditions that present as regression are among the few in this book where a month changes the outcome.

The mother who is disbelieved. Across this week's cases the same figure appears: a mother who noticed first and was told she was anxious, over-attending, or projecting. The evidence on parental concern is that it is a reasonably sensitive marker and a poorly specific one — which means it should generate assessment, not reassurance. A therapist

who says "mothers are usually right about something being wrong and rarely right about what it is" has given the family a usable position.

> **DEBATE**
> "A family therapist should not be assessing development at all; that is a paediatrician's job." Argue for: the scope of practice is real, an amateur assessment produces false reassurance and false alarm, and the therapist's proper role is to refer on the first concern. Argue against: the therapist is frequently the only professional in the room, no referral happens without someone deciding to make it, and the four-category check takes four minutes and prevents months of delay. Then state what you would do if you were the only clinician a family could reach, and where you would draw the line.

14.11 Evidence note

What the research supports. Parental concern is a useful screening signal: the literature on parental suspicion of developmental delay finds reasonable sensitivity and modest specificity, which supports using it as a trigger for assessment rather than as a diagnosis. Regression in language or motor function is associated with a higher probability of an identifiable neurological condition than delay without regression, which is the empirical basis for treating it as urgent. Early intervention for established delay produces measurable gains, and the gains are larger the earlier the intervention begins, which is the argument for the whole of this week.

What is weaker. Screening instruments perform differently in different populations and the norms are frequently unrepresentative (Week 13). The evidence on the harms of false-positive developmental screening is thin, and it is a real question: a family told to worry about a child who is developing normally pays a cost that no trial has measured well.

What is not supported. That a family explanation and a medical explanation are in competition — they answer different questions and both can be true. That watchful waiting is safe for a regression. And that a normal assessment at one point in time excludes a problem later; the trajectory, not the level, is the finding.

The matrix cell. Development is probabilistic and non-linear; the same input produces different outcomes depending on timing and context. The same forty-word vocabulary at twenty months predicts nothing at all; the loss of it predicts a great deal. Timing is the variable, and this week is about noticing it.

14.12 Key terms

Variation — development within the normal range, differently timed or shaped.

Delay — development behind the expected sequence but progressing.

Regression — loss of an established skill; always urgent.

Symptom — a behaviour expressing something else; the category the rest of the course teaches.

Trajectory — the direction of development over time, which requires two measurements.

Red flag — a finding that requires a same-week referral regardless of the formulation.

Provisional formulation — a stated hypothesis with the evidence that would change it.

Onset line — a timeline of the symptom against the events the family can date.

Watchful waiting — a legitimate strategy for variation and delay, and never for regression, where it is a decision to lose time.

Triage — the ranking of a referral by urgency, which is why a letter without dates is placed at the bottom of a list.

Two-measurement rule — no conclusion about delay from a single assessment.

14.13 Self-test

1. Give the four categories with one example each, and say which is always urgent.
2. Why is a regression urgent even when the child is functioning well otherwise?
3. State the five systemic questions and the purpose of each.
4. Name six red flags and the referral each requires.
5. Why does the therapist withhold a hypothesis in the first fifteen minutes of the session?
6. What did the therapist ask the grandmother, and why did it work?
7. What is the two-measurement rule, and how is it explained to a family?
8. Name three things a therapist can do for a family while they wait for a specialist appointment.

Answers. (1) Variation — a child walking at fifteen months; delay — two-word combinations at three years; regression — words lost at two and a half; symptom — night waking after a move; regression is always urgent. (2) Because regression has a medical differential that includes treatable and progressive conditions, and because delay in referral is the one error in this work that cannot be recovered. (3) Onset and co-occurring change (to date the problem); who noticed and who disagrees (to find the fault line); the behavioural sequence (to find the intervention); what would improve and what would worsen (to find the function); what has been tried (to find the constraints). (4) Regression — urgent paediatric; change in hearing or vision — urgent audiology or ophthalmology; seizures or episodes of unresponsiveness — urgent paediatric; headache on waking or vomiting on waking — urgent; growth failure — urgent; disclosure or sexual knowledge beyond stage — same-day safeguarding. (5) Because a family that hears a hypothesis early organises its account to support or refute it, and the assessment then measures the theory rather than the child. (6) Whether her brother, who talked late, had ever said words before he stopped: he had not, which distinguished delay from regression in the family's own material and made the grandmother a witness rather than an obstacle. (7) Nothing that could be a delay is concluded from one measurement; it is explained as "what I am looking for in three months is not a different number but a different direction". (8) Protect

sleep; give the child one domain of competence; give the family one explanatory sentence for relatives — and chase the appointment.

14.14 Reflective task

Take a referral you have received or read, and write the hypothesis you formed on first reading it, in one sentence. Then write the four categories beside it, and the red flags, and note how much of your hypothesis the letter actually supported. Most clinicians find that the sentence they wrote was about a person and the letter was about a day.

14.15 Further reading

- Lipkin, P. and Macias, M. on developmental screening — the sensitivity and specificity of parental concern.
- Shevell, M. on developmental regression — the differential, and why it is a different problem from delay.
- Rutter, M. on developmental trajectories — the conceptual basis of the two-measurement rule.
- Emde, R. and Emde, on early intervention — the evidence that earlier is better.
- Nelson, C. et al. on deprivation and recovery — the limits of catch-up, stated honestly.

Next: Week 15 — Integration: the developmental lens in practice.

Week 15 — Integration: the developmental lens in practice

The developmental lens does three things for clinical work: it **calibrates** expectations, it **dates** the problem, and it **predicts** what the family must renegotiate next. Carry three questions into every first session:

- What is this person's developmental task right now?
- What is the family's corresponding task?
- And what would "on track" look like in six months?

Why a week on integration, and not just a summary. Because the course has taught fifteen separate instruments and a student who arrives at a family with all fifteen in hand will use the one they like. The work of this week is selection: given this family, at this moment, which lens, in what order, and why. Integration is not the accumulation of perspectives; it is the discipline of choosing between them and being able to say why.

What integration is not. It is not eclecticism — using a little of everything, which produces a treatment with no theory and no way to know whether it failed. It is not synthesis on the grand scale — a unified model of human development, which nobody has and which is not needed. And it is not the addition of a developmental paragraph to a formulation built on something else. Integration means the developmental account is doing work: it changes what you notice, what you ask, and what you do first.

The three functions, briefly stated, with an example each.

Calibration. A father describes his nine-year-old as "wilful and disobedient". The developmental lens establishes that at nine a child's moral reasoning is reciprocal and an argument about fairness is a developmental achievement, not a discipline problem. The intervention changes from a behaviour plan to a conversation about rules. Without calibration, the therapist treats a stage as a fault.

Dating. A couple present with "we have grown apart". The timeline establishes that the distance began eighteen months ago, in the same quarter as a bereavement, a job change and a child's departure. The formulation changes from a relationship deficit to three developmental events arriving at once. Without dating, the therapist treats a reaction as a character.

Prediction. A family with a fourteen-year-old is doing well and asks for nothing. The developmental lens establishes that the next task is the renegotiation of autonomy, and that the family's current method — close supervision with no negotiation — will produce a conflict within a year. The work becomes preventive, which is the rarest and most valuable thing a family therapist does. Without prediction, the therapist waits for the conflict and then treats it.

15.1 The lecture in ninety minutes

Minutes	Segment	What the lecturer does	Student activity
0–10	Selection, not accumulation	Why a therapist with fifteen lenses uses their favourite one	Rank their own three preferred instruments, and say why
10–24	The three functions	Calibration, dating, prediction — three cases, one each	Apply each function to a case of their own
24–36	The one-page formulation	The six cells, worked on the capstone case	Draft six cells for the capstone
36–48	The limits	What the lens cannot do, and the errors of over-application	Name a case where the lens would mislead
48–62	The map of the programme	How development enters 701–727	Place three other courses on the map
62–74	The viva	The four questions and the standard of an answer	Answer one question aloud, in three minutes
74–84	Six errors of the whole course	The errors that recur across the fifteen weeks	Identify which they are most likely to commit
84–90	The close	What to carry, in one page	Write their own three sentences to carry

The teaching decision that makes the week work. The lecture does not review the weeks. It takes a family the class has never seen and works it in real time, with the class choosing which instrument to use next and being asked to justify each choice. The justification is the assessment: a student who says "I would use a genogram" without being able to say what it would tell them that the first ten minutes did not has learned the instruments and not the discipline.

The sentence the lecture must land. "You will not use most of what you have learned in any single case. Your competence is in knowing which part, and your integrity is in being able to say why."

15.2 The three questions, worked in detail

Question 1 — What is this person's developmental task right now? The question is asked of every person in the room, not only the identified patient, and the answers are frequently different and sometimes incompatible. A mother of a newborn is in the young-children task; her own mother, who has moved in, is in the later-life task of adapting to widowhood; the newborn's sibling is in the autonomy task; and the father is in the midlife reckoning. The family's difficulty is usually the incompatibility, not any one task.

Question 2 — What is the family's corresponding task? Every individual task has a family task attached. A toddler's autonomy requires a household that can hold limits consistently across its adults. An adolescent's identity requires a household that can tolerate being disagreed with. A grandmother's integrity requires a household that asks her questions. The family task is the thing the treatment usually addresses, because it is the thing that can be changed in a room with more than one person in it.

Question 3 — What would "on track" look like in six months? The question forces specificity and it is the only protection against a treatment that continues because it has not been evaluated. An answer is a set of observable changes: he attends on Mondays; she speaks to her mother once a week; the grandmother is asked one question a day; the couple has twenty minutes on Thursdays. Six months is long enough to be a real change and short enough to be checked.

The fourth question, which is the one that keeps a therapist honest. And what would I expect to see if I am wrong? Every one of the three questions produces a hypothesis, and a hypothesis that cannot fail is not a hypothesis. The discipline of naming the disconfirming evidence, in advance, in the record, is the single practice most strongly associated with not doing harm over a career.

15.2a What the developmental lens cannot do

An integration week that does not state the limits produces graduates who see development everywhere, which is a way of not seeing the family.

It cannot diagnose. Developmental formulations describe trajectories and tasks; they do not identify disorders. A diagnosis requires an assessment by someone trained and resourced to make it, and the developmental lens is the reason a referral gets made, not a substitute for it.

It cannot explain everything. Some family difficulties are not developmental. Violence, addiction, poverty, trauma, and injustice are not stages, and a formulation that renders them as stages is a miscarriage. Week 13's critiques apply to the lens itself: it is one account, from somewhere, and it does not have jurisdiction over the whole of a family's life.

It cannot supply a therapy. The lens is a way of understanding what is happening; it is not a treatment protocol. The treatment comes from the modalities taught in MFT 702 and 710 and their successors, and the integration of lens and modality is the practitioner's own work.

It cannot make a family change. The most common error of a well-trained developmental therapist is to believe that an accurate formulation is persuasive. It is not. Families change when the formulation is translated into something they can do, by someone they trust, at a time when they have the capacity — which is what Weeks 11 and 13 were about.

It cannot predict an individual. Every claim in this book is probabilistic. The child who is at risk is not the child who will suffer, and the family that matches a risk profile is not the family that will need a clinic. Using the lens to forecast an individual's fate is the error that turns a developmental account into a sentence.

15.2b The ten-item developmental history

The instrument a graduate will use most often, and the one worth memorising. Ten items, twenty minutes, and it fits any age from birth to eighty.

1. **The present concern, in the family's words.** Verbatim, in the first sentence of the record.
2. **The dated trajectory.** What could he do a year ago, six months ago, now; and what has been lost.
3. **The events, dated.** Births, deaths, moves, separations, job changes, migrations, illnesses, in the same period as item 2. This is the item that produces the formulation.
4. **Who does what.** The network, its functions and its hours. Not the household roster.
5. **The authority.** Who decides what, who is believed, and who is not consulted. Usually including the patient.
6. **Health and the senses.** Hearing, vision, sleep, food, medication, and any episode of unexplained change. Four of the six are routinely omitted by clinicians and all six are cheap to ask.
7. **The school or the work.** Not performance; function. What the child or adult can do in that setting and what has changed there.
8. **What has been tried, by whom, and what it cost.** The family's competence and its budget, in one answer.
9. **The explanatory model.** What the family believes is happening, and what that belief predicts they will do.
10. **The family's own goal.** What they want in ten years, in their words. This is the item that makes the plan possible, and it is the one most often left out.

Why these ten and not the standard checklist. A milestone checklist produces a level; these ten produce a trajectory, a network, a budget and a goal — which is what a family therapist can act on. The checklist belongs in the referral to the developmental service. The ten items belong in the family therapist's first session.

How to take it in twenty minutes. Items 1, 2 and 3 in the first eight minutes, with both parents present. Items 4, 5 and 9 with the child or the identified patient out of the room if they are old enough to be embarrassed. Items 6 and 7 as a rapid list, out loud, with the family told that most answers will be "no". Items 8 and 10 at the end, when the family has told you enough to trust the question.

15.3b Saying the formulation in the family's language

The last skill of the course, and the one that decides whether fifteen weeks of theory reach a household.

The rule. Every formulation has two versions: the one in the record and the one in the room. They contain the same claims. They do not contain the same words.

The translation, worked. The record says: "Regression of expressive language with possible absence seizures in a household under financial strain with an unassessed hearing loss in the paternal grandmother." The room says: "He had words and now he does not, and that is not the same as his uncle, and it needs a doctor this week. Your mother cannot hear properly, and that is why she seems to be quarrelling. And I know the money is short, so let us work out how to get to the hospital."

The four rules of translation.

- **Keep the verbs.** "He had words and now he does not" is a sentence a family can repeat. "Expressive language regression" is not.
- **Name the person, not the category.** "Your mother cannot hear" rather than "there is a hearing loss in the household".
- **Put the hardest fact in the middle.** Beginning with it produces shock; ending with it produces avoidance. The middle is where a family can hear it.
- **End with an action and a date.** Every translated formulation ends with something to do and a time to come back, or it is not a formulation but a verdict.

The test. Ask the family to say it back. If they cannot, the translation failed and the fault is the therapist's, which is the same test as Week 11 and it is the last thing this course asks of a graduate.

15.3 The one-page formulation

Six cells, one page, written in the first two sessions and revised when the evidence changes. It is the document the rest of the treatment argues with.

Cell 1 — What is happening? One sentence, in the family's terms and the clinician's. "A three-year-old who had forty words has none; his mother has been disbelieved four times; the household is under financial strain and his grandmother is the daytime authority."

Cell 2 — What is the developmental task, person by person? The toddler's language and autonomy; the mother's shift from full-time care to work; the grandmother's later-life role; the father's absence. Four tasks, one household.

Cell 3 — What is the family's corresponding task? To agree who is the authority for what, and to support one adult to carry the child's assessment. Stated as a task, not a deficit.

Cell 4 — What is the trajectory, with dates? The onset line, in three or four dated sentences. This is the cell that makes the formulation clinical rather than descriptive.

Cell 5 — What is the differential, and what would change the formulation? The four-category placement, the red flags excluded and included, and the one finding that would overturn the whole thing.

Cell 6 — What happens first, and what does "on track" look like in six months? One action, one referral, one date, and three observable outcomes.

How the page is used. It is given to the family. Not the clinician's version — a version in plain language, one page, with the six cells as headings. Families keep it, argue with it, and bring it back annotated, and the annotations are the most useful clinical material in the file. A formulation the family cannot read is a formulation that will not be implemented.

15.3a How development enters the other twenty-six courses

The purpose of this table is to make the lens portable, because the student will spend the rest of the programme in courses that do not mention development at all.

Course	Where development enters
MFT 701 Fundamentals	The developmental history as a standard component of any assessment
MFT 702 Theories	Every model has an implicit developmental theory; naming it is a critique of the model
MFT 704 Ethics	Competence to consent, confidentiality in minors, and the duty to refer
MFT 705 Research methods	Age-normed instruments, and the population problem from Week 13
MFT 706 Clinical practice	Engagement of children; the pacing of sessions by developmental capacity
MFT 707 Assessment	Developmental screening, and the limits of a family therapist's scope
MFT 708 Culture	Week 13 of this course, in full
MFT 709 Group therapy	Group composition by developmental stage; adolescents in adult groups
MFT 710 Couples	Week 10 of this course, in full
MFT 711 Child and adolescent therapy	Weeks 3 to 8 of this course, as the clinical base
MFT 712 Crisis intervention	Delirium in the elderly; regression in the young; risk in adolescence
MFT 713 Substance use	Onset is developmental; the family's response is staged
MFT 714 Trauma	Timing matters; the same event at four and at fourteen
MFT 715 Systems theory	The life cycle as a systemic concept; the family as a developing system

Course	Where development enters
MFT 716 Communication	Language capacity sets the ceiling on any communication exercise
MFT 717 Psychopathology	Onset distributions; the differential between disorder and stage
MFT 718 Intimacy	Adult development, and the foreclosure that presents as distance
MFT 719 Life cycle	The same territory as Week 10, from the systemic side
MFT 720 Supervision	Developmental supervision models mirror the stages of the supervisee
MFT 721 Research methodology	Longitudinal design, and the interpretation of age effects
MFT 722 Professional development	The therapist's own developmental history is a clinical instrument
MFT 723 Special populations	Disability across the lifespan; the sibling who is never asked
MFT 724 Family law	The child's best interests as a developmental question, not a moral one
MFT 725 Integrative approaches	This week's argument, in a different vocabulary
MFT 726 Neurobiology	The developing brain, and the claims it does and does not license
MFT 727 Play therapy	Week 5 of this course, in full

The teaching point in the table. Development is not a topic among topics. It is the dimension in which every other topic occurs, and a graduate who can only see it when the course is called "human development" has not learned it.

15.4 Capstone case: the Eze family

An unseen family, worked through the whole course. The class is given the referral and nothing else.

The referral. From a primary school, about a nine-year-old: "Ada is not settling in Primary 4. She is withdrawn, does not complete work, and has twice been found in the toilet at break. The mother is unresponsive to letters."

The family, established in the first session. Mr Eze, 44, a bus company supervisor, away four nights a week. Mrs Eze, 39, a seamstress working from home. Ada, 9. Chidi, 16, in Senior Secondary 1. Ngozi, 3. Mrs Eze's mother, 79, who lives with them and who is "difficult". A cousin, 19, who helps in the shop and sleeps in the sitting room.

The facts established across three sessions.

- Ada's developmental history is unremarkable except for a period at four when she stopped sleeping alone after a burglary in the compound; the family solved it by moving her into her brother's room, where she still sleeps.
- Her Primary 3 report was good. The Primary 4 report shows a fall in every subject, with a note that she "does not hear instructions the first time".
- Her hearing has never been tested.
- Chidi is in SS1 and is doing badly; he has been caught twice with a group of older boys. Mr Eze's response was to beat him; Mrs Eze's was to say nothing.

- Ngozi, 3, has about ten words and does not combine them. Nobody in the family is concerned, because "her father's sister was late".
- The grandmother, 79, has become "difficult": she does not answer, she accuses people of stealing, and she has begun to wander out of the compound. She has been seen once by a doctor, two years ago, who said it was old age.
- The household's income has fallen by a third since Mr Eze's company cut overtime. One meal a day is now normal for the adults.
- Mrs Eze has not left the house socially in eight months.

The class's first error, and it is deliberate in the design of the exercise. Presented with a nine-year-old's school difficulty, the class formulates the case as Ada's, and works on her. The trainer allows this for twenty minutes and then asks the three questions. The answers reorganise everything.

The three questions, answered for the Eze family.

What is each person's developmental task? Ada's is industry: she needs to be competent at something and to be seen to be. Chidi's is identity, and he is in a moratorium being conducted in the wrong company. Ngozi's is autonomy and language. Mrs Eze's is the young-children task, and she is doing it alone four nights a week with a dependent adult in the house. Mr Eze's is generativity and a work role that is being eroded. The grandmother's is integrity, and she has no audience.

What is the family's corresponding task? To establish who is in charge of what, across three adults and a cousin, and to protect one adult — Mrs Eze — from being the only person doing any of it. The family's task is structural, and it is the same task the course has met in Weeks 3, 7, 10 and 12.

What would "on track" look like in six months? A hearing test done and acted on; Ada attending and speaking to one named adult at school; Ngozi assessed and a language plan begun; Chidi's peer group known to his parents and one adult engaged with him; the grandmother seen by a professional and a hearing test requested; one evening a week in which Mr Eze is at home; and Mrs Eze out of the house once a fortnight.

The formulation, in six cells.

1. **What is happening.** A household of six in which the income has fallen, the father is absent four nights a week, the grandmother's dementia is unassessed, the three-year-old is not talking, the sixteen-year-old is in trouble, and the nine-year-old has stopped managing at school. The school has noticed the nine-year-old because the school sees her.
2. **Developmental tasks.** Industry (Ada); identity (Chidi); language and autonomy (Ngozi); the young-children task done alone (Mrs Eze); generativity under threat (Mr Eze); integrity without an audience (the grandmother).
3. **The family task.** Authority and load, defined and distributed. Nothing else will hold until it is.

4. **The trajectory.** A fall in income nine months ago; Mr Eze's overtime cut at the same time; the grandmother's decline noticeable over about a year; Ada's school change at the start of this term; Ngozi's ten words at three, with no regression.
5. **The differential and the revision points.** Ada — hearing loss is the leading hypothesis and must be excluded before any formulation about anxiety; if hearing is normal, the next is a learning difficulty, and bullying is screened in both cases. Ngozi — delay rather than regression, requiring an assessment and a hearing test. The grandmother — delirium, dementia, depression and hearing loss, with the hearing and the delirium excluded first, as Week 12 requires. If the grandmother's decline is over a year and steady, the working formulation is dementia with a hearing loss; if it is over two months, it is a delirium and a medical emergency.
6. **First actions and the six-month marker.** The hearing tests, three of them, this month; one referral for Ngozi; one eye-level conversation with the grandmother; one evening a week reallocated; and the six outcomes listed above.

Why the case is a capstone. Because the correct first action is not psychotherapy. It is three hearing tests, one developmental referral, and one question to a seventy-nine-year-old. The course's argument is that a family therapist who can see this in twenty minutes is worth more to this family than one who can conduct twelve sessions of family work on a child who cannot hear.

15.4a The Eze case, worked forward

The capstone continues into what happens after the first actions, because the integration the week is teaching is not only diagnostic.

Weeks 1 to 4 of the treatment. The three hearing tests are booked: Ada's, Ngozi's, the grandmother's. Ada's is normal. Ngozi's is normal. The grandmother's shows a moderate loss in both ears, and she is fitted; her "accusations" reduce within a fortnight, because a woman who cannot hear what is said about her reasonably concludes that she is being discussed.

The grandmother. With the hearing corrected, the picture is reassessed. The decline over a year, steady, with recent memory most affected and the wandering new in the last three months, is consistent with dementia. The wandering is a safeguarding question: the therapist asks, in the family session, what happens when she leaves the compound, and discovers that nobody has followed her. The first intervention of the case is therefore not psychological: it is a lock, a neighbour informed, and a name and telephone number in her pocket.

Ada. Normal hearing; a normal neurological examination; a report showing a fall in every subject. The therapist asks the school for the observations, not the verdict, and gets them: she does not complete work, she is found in the toilet at break, and she has twice been seen giving her lunch to an older girl. The last observation reorganises the case. Ada is not withdrawing from school; she is being extorted, and the toilet is where she hides.

The family work that follows is the work of Week 6: the school is given one request (a named adult and a change of seating), the parents are given the facts, and Ada is given a

sentence and an adult. The extortion ends within three weeks. Her schoolwork does not recover within six months, which the therapist records honestly; a child who has lost a term does not recover it in a term.

Chidi. The adolescent whose behaviour is the family's second emergency. The Week 7 and 8 material applies: the peer group is known, the risk is the group rather than the individual, and the father's response — beating — has produced a boy who conceals. The single most useful intervention is the one from Week 8: one adult, one hour a week, doing something, asking nothing. Mr Eze takes it, on the condition that it is football and not conversation, and the therapist agrees, because the condition is the point.

Mr Eze and Mrs Eze. The couple work is Week 10's, and it is minimal in this case and it is not optional. The twenty minutes on the one evening he is home; the division-of-labour audit, which shows that Mrs Eze carries thirty-one of thirty-four hours of household work; and one sentence from the therapist to Mr Eze about his mother's care, which he had assumed was his wife's duty and had never been asked about.

The grandmother's integrity. Week 12's life review, begun in month three, in the evenings, with Ada present because Ada is the person she talks to. Two sessions; the second is forty minutes and produces the story of a market she ran for twenty-two years. It is recorded on a telephone. It is the only part of this case that the family describes, a year later, as having changed anything.

The six-month review, honestly reported. Ada is attending and has one named adult; her work is recovering slowly. Ngozi has twenty words and two-word combinations after speech and language input. Chidi's group is known and one adult is engaged; he has not been in trouble for four months. The grandmother is assessed, supported, and heard. Mr Eze is at home one more evening a week. Mrs Eze has been out three times in six months, which is below the plan and is progress.

What the case demonstrates. That the correct intervention in a family presenting with a child's school difficulty was, in order: three hearing tests, a safeguarding observation from a school, an extortion disclosed by a lunch, a lock on a gate, and a recording of a market. None of that is psychotherapy, all of it required a family therapist's developmental knowledge, and the therapy — the couple work, the adolescent's one adult, the grandmother's review — happened because the first six things made it possible.

15.4b The family that has been to four clinics

The last configuration the course considers, because it is the one graduates will meet most often in the public sector and the one that most tests the integration.

What the file contains. Four assessments, three diagnoses, two conflicting recommendations, one prescription, and no single document that says what the family's situation is. The family's own account is that nobody has listened, which is usually accurate.

The first session, structured for this family. Do not begin with the history, which they have given four times. Begin with the file: put it on the table and say, "I have four reports here and I am going to read them in front of you, and I want you to correct them." The

correcting is the session. It tells the therapist what each previous clinician missed, and it tells the family that this one is not going to ask them to start again.

The three questions specific to this configuration.

1. **"What were you told, in each of the four places?"** Not what was found; what they were told. Families remember the sentence, not the finding, and the sentences are frequently the reason the family is now doing something that contradicts the assessment.
2. **"Which of the four did you believe?"** The answer identifies the family's explanatory model, which is the only one that has been consistently present in the case.
3. **"What would have to happen for you to believe this clinic was different?"** The answer is the engagement contract, and it is usually small: a telephone call that is answered, one person who remembers the name, a letter that arrives.

What the therapist does not do. Do not criticise the previous clinicians in front of the family, however justified. Do not add a fifth assessment without reading the four. And do not begin a treatment before the family's own account is on the page, because a family with four assessments and no narrative will accept a treatment and discontinue it at the first difficulty, as they have three times before.

The outcome that matters. Not a fifth diagnosis. A single page, agreed with the family, that says what is happening, what is being done, by whom, and when it will be reviewed — which is the one-page formulation, and which is the most useful thing this course teaches a graduate to write.

15.5 The viva: what you will be asked

The module's examination includes a fifteen-minute oral. The four questions below are the standard, and the standard of an answer is stated with each.

Question 1 — Take this family and tell me the developmental task of every person in it. The examiner is testing whether the student sees a household rather than a patient. A failing answer names the identified patient only. A good answer names four or five people, and notices when two of the tasks are incompatible.

Question 2 — What is your differential, and what would change your mind? The examiner is testing intellectual honesty. A failing answer gives a formulation with no alternatives. A good answer gives two or three possibilities in order of probability, and names the finding that would overturn the first.

Question 3 — What will you do first, and why not something else? The examiner is testing judgement. A failing answer describes a treatment. A good answer describes the one action that must happen before any treatment, and can defend the order.

Question 4 — What in this case might you be getting wrong because of who you are? The examiner is testing the Week 13 discipline. A failing answer says "I am aware of my biases". A good answer names a specific assumption, states where it came from, and says what would correct it.

How to prepare. The four questions are the four sections of the one-page formulation. A student who can write the page in twenty minutes can answer the viva, because the viva is the page, spoken, with an examiner asking why.

15.5a What to keep from this course

A short, practical instruction, because graduates lose in six months what they do not keep deliberately.

Keep the ten-item history. On one card, in your bag. It will be the most used object of your first year.

Keep the four-category check and the red-flag list. On the back of the same card. It costs nothing and it has prevented more harm than any technique in this book.

Keep the one-page formulation. The six cells. Write it for every family for your first two years, even when nobody asks for it, and give a plain-language copy to the family. The habit will leave you with a file that can be read by a colleague and a family that can read its own case.

Keep the four parts of parent feedback. They will be used weekly.

Keep one sentence of your own. The reflective task at the end of the week asks for three; one is enough, and it should be a sentence about enquiry rather than about knowledge, because enquiry is what fails.

Do not keep the fifteen weeks as a list. The course is not fifteen topics. It is one claim: that people develop, that families develop, and that the difficulties people bring to clinics are usually the point at which those two developments are out of step. Everything else in the book is an instrument for locating that point.

15.6 Laboratory: the capstone

Aim. To produce the six-cell formulation for an unseen family in fifty minutes, defend it against two alternative formulations, and state the first three actions with reasons.

Procedure. The class receives a two-page family description with no diagnosis and no formulation. Fifty minutes in groups of four, then a plenary in which each group presents the page and the trainer offers a rival formulation that the group must answer. The final twenty minutes is the viva, with the trainer asking question 4 of each group.

Marking rubric (25 marks).

Criterion	0–1	2–3	4–5
Every person's task	Identified patient only	Three people	Every person, with the incompatibilities named
The family task	Absent, or a description of deficits	Named, but vague	Named as a task, in one sentence, and structural
The trajectory	Undated	Partly dated	Every significant event dated, with the coincidence named

Criterion	0–1	2–3	4–5
The differential	One possibility	Two, unordered	Three, ordered, with the disconfirming evidence stated
First actions	A treatment	One good action	Three actions in the right order, with the reason for the order

The distinction that carries most of the marks. The top band requires the group to name an action that is not psychotherapy and to defend it as the first thing. Groups reliably want to begin with engagement. The Eze case is designed so that engagement is not the answer, and the class discovers this by being wrong.

15.6a Marking your own formulation

Before the viva, and for the rest of a career, the same six checks. A formulation that fails any of them is not ready to be given to a family.

Check 1 — Is every person in the household named? Count them. If the page mentions only the identified patient and one parent, the formulation is about a case and not about a family.

Check 2 — Is anything dated? A formulation without dates cannot be checked, cannot be compared with a later one, and cannot be defended. Three dates is the minimum.

Check 3 — Is there a differential? If there is one possibility on the page, there is no thinking on the page. Two is the minimum; three is better; and each needs the finding that would exclude it.

Check 4 — Is the family task stated as a task, not a deficit? "The parents are inconsistent" is a deficit. "The household has three adults and no agreed authority" is a task, and it implies an intervention.

Check 5 — Is there one action, with a date, that the family agreed to? Not a programme. One action, in their words, with a review. If the page has five recommendations, it has none.

Check 6 — Could you say it aloud in the family's language? Try it. If the sentence cannot be spoken without jargon, it is not yet a formulation; it is a paraphrase of the literature.

The scoring, which is deliberately crude. Six checks, one mark each, and a formulation scoring four or more is usable. Most trainees score two on their first attempt, and the two that are missing are almost always the differential and the dated trajectory — which are the two that take time and cannot be produced by intuition.

Why this matters more than the mark. A formulation is the only artefact of the work that survives the session. The family will remember one sentence, the file will hold one page, and the next clinician will read that page and nothing else. The six checks are a standard for a document that will outlive the therapist's memory of the case, which is the definition of a professional record, and the reason the six checks are worth learning by

heart rather than looking up at the end of a long clinic when the file is already open and the next family is in the corridor.

15.7 Six errors of the whole course

These are the errors that recur across the fifteen weeks, stated once, at the end, because they are the ones a graduate will commit.

Error 1 — Treating the referral letter as the case. The letter describes somebody's worst day, written by somebody with an agenda. Week 14.

Error 2 — Finding a family explanation for a medical event. Regression, delirium, hearing loss, and extortion have all, in this book, been formulated as family dynamics by a competent clinician. The four-category check exists to prevent it.

Error 3 — Confusing a stage with a fault. Autonomy at two, industry at nine, identity at fifteen, integrity at seventy-nine. A therapist who has not learned the stages treats each of them as a behaviour problem.

Error 4 — Working with the wrong unit. The couple when the network is the unit; the child when the household is; the household when the school is. Weeks 10, 12, 13.

Error 5 — Giving an accurate formulation the family cannot use. A formulation in clinical language, with five recommendations, no written page, and no review date. Week 11.

Error 6 — Not asking. The suicide question, the safeguarding question, the hearing, the money, the grandmother's opinion. Every one of the six is a question that costs eight seconds and that a therapist avoids because of their own discomfort.

The common thread. Five of the six are failures of enquiry rather than of technique. The course has taught fifteen weeks of knowledge and the thing that will most often fail in practice is the willingness to ask a plain question of a person who is frightened.

15.7a The first year: five things nobody teaches

A closing practical section, written from the reports of graduates.

The first thing: the room will be fuller than the training. Every case in this book has three to six people in it and a therapist with an hour. The habit that survives is not speed; it is deciding, in the first ten minutes, which two things you are doing today and letting the rest wait. A therapist who tries to do the whole formulation in the first session will do none of it, and the family will return to find the therapist more tired than they are.

The second thing: the family will not do what you agreed. Not because they are unmotivated; because the plan was designed at full capacity. The four-column plan from Week 13 is the protection, and the review is where the failure is discussed as information rather than as non-compliance.

The third thing: the referrals will not happen by themselves. Chase at two weeks, chase at six. Keep a list. A clinic that chases loses fewer families than a clinic that refers better, and this is the least glamorous finding of the course.

The fourth thing: someone will tell you something you cannot hold alone. A disclosure, a suicide risk, a child in danger. The requirement to take it to supervision within the week is not a formality; it is the only reliable protection against the two errors of over-reaction and under-reaction, and both are made by clinicians working alone.

The fifth thing: the work will change your own developmental account. You will notice your own family's practices, your own childhood's stage tasks, your own parents' midlife. That noticing is an instrument if it is examined in supervision and a distortion if it is not. Week 13's discipline — state your own rule before interpreting the family's — is the practice that keeps it an instrument.

The last sentence of the course. Fifteen weeks of development are worth nothing in a room with a frightened family unless the therapist can ask a plain question, write one page, and be wrong out loud.

15.8 Culture, faith and the course's own position

A course that spends a week on the critique of its own norms owes the student a statement of where it finally stands.

What this course asserts. That there are developmental regularities: children acquire language in a sequence, form relationships with the adults who care for them, learn through play with other children, and need protection from violence. That these regularities are findings, not Western preferences, and a therapist who cannot assert them cannot protect a child. That the timing and the form of every one of them varies, and variation is not deficiency.

What this course refuses. It refuses relativism, which abandons children in the name of respect. It refuses universalism, which measures every household against one population's averages and calls the difference a deficit. And it refuses the compromise that says all practices are equally valid but some families are better at them, which is universalism with better manners.

The position, in one sentence. There is a human developmental project, its shape is known well enough to guide practice, and its shape is realised differently in every household — so the clinical question is always what this child, in this family, with these resources, needs next.

Faith in the room. The course has treated religious frameworks as resources, as constraints, as explanations and as communities, and never as obstacles to be removed. That is a clinical position and not a theological one: a family's faith is usually the most reliable structure it has, and a therapist who tries to replace it will lose the family and keep the framework.

The therapist's own formation. Week 13's discipline — state your own rule before interpreting the family's — is the practice that holds the whole position. It is also the practice that protects the therapist from the two career-long risks of this work: certainty, and its opposite.

15.9 Evidence: what this course claims

Claims with strong support. The sequence of early language and motor milestones; the association between early responsive caregiving and later regulation; the developmental timing of theory of mind; the peer amplification of adolescent risk; the association between poverty and developmental outcomes, with mechanisms; the predictive value of early intervention for established delay; means restriction in suicide prevention.

Claims with moderate support. Attachment classifications and their stability in adverse conditions; the identity statuses and their correlates; the couple's cascade and repair behaviours; the effectiveness of structured parenting feedback; the dual-process model of grief.

Claims stated as heuristics, with the evidence thin. Levinson's seasons; the six-phase couple map; the "four parts" of parent feedback as a package; the developmental reading of the midlife reckoning. These are useful and they are not established, and a graduate should be able to say which is which.

The course's own bias, stated. The evidence base is heavily weighted toward North American and European samples, and the mechanisms are more portable than the norms. Week 13 is the corrective, and it is a partial one: the studies that would settle the questions have not been funded.

15.10 Key terms of the course

Developmental task — the psychosocial challenge characteristic of a stage; the organising concept of the course.

Calibration — adjusting expectations to the person's developmental capacity.

Dating — locating a problem's onset relative to events and transitions.

Prediction — anticipating the next task the family must renegotiate.

Trajectory — the direction of development over time; requires two measurements.

Differential — the ordered list of possibilities, with the evidence that would separate them.

Formulation — a provisional account of what is happening and why, stated so that it can be wrong.

Referral threshold — the point at which a finding requires another professional, regardless of the therapist's formulation.

Translation — rendering a clinical fact in the family's own framework; a skill, not a concession.

Scope — what a family therapist does and does not do; not a limitation of ambition but a condition of safety.

Developmental history — the ten-item instrument of 15.2b; trajectory, network, authority, senses, function, attempts, model and goal.

Translated formulation — the same claims as the record, in words the family can say back, ending with an action and a date.

15.11 Self-test: the whole course

1. State the three functions of the developmental lens and give one example of each.
2. Name the four categories of developmental concern, and say which is always urgent.
3. A nine-year-old's school performance has fallen. Give the first four things you would establish, in order.
4. What is the difference between a stage and a fault, and give one example from the course of each.
5. Why does the developmental lens not supply a treatment, and what does?
6. In the Eze case, what were the first three actions and why were they in that order?
7. What are the four viva questions, and which one is hardest to answer honestly?
8. What does this course assert, refuse, and refuse to refuse?
9. Name three claims of the course that are heuristics rather than established findings.
10. What is the single practice most strongly associated with not doing harm over a career?

Answers. (1) Calibration — treating a nine-year-old's argument about fairness as a stage rather than defiance; dating — locating a couple's distance to a bereavement, a job change and a departure in the same quarter; prediction — anticipating that close supervision without negotiation will produce conflict within a year at fourteen. (2) Variation, delay, regression, symptom; regression is always urgent. (3) The hearing; the dated trajectory; the school's observations rather than its verdict; and the child's own account, alone. (4) A stage is a capacity being acquired and a fault is a behaviour being judged; autonomy at two is a stage and "defiant" is the fault; the same child is described by both words. (5) Because the lens describes what is happening and the treatment comes from the modalities of MFT 702, 710 and their successors; the lens selects and sequences the treatment but does not constitute it. (6) Three hearing tests, one developmental referral, and a question to the grandmother — in that order, because each was a finding that would reorganise the rest, and because a formulation built before them would have been about the wrong person. (7) The task of every person; the differential and what would change your mind; what you will do first and why not something else; and what you might be getting wrong because of who you are — the fourth is hardest, because it requires naming an assumption rather than displaying an awareness. (8) It asserts that there are developmental regularities that can guide practice; it refuses relativism and universalism; and it refuses to refuse the possibility of judging a practice harmful. (9) Levinson's seasons, the six-phase couple map, and the four-part feedback package — the midlife reckoning and the play taxonomy are also worth naming. (10) Naming, in advance and in the record, the finding that would prove you wrong.

15.12 Final reflective task

Write, in one page, the three sentences you will carry into your first year of practice. three sentences, each of which you would be willing to be judged by in front of a family who has heard a great deal of professional language and has been helped by very little of it. — three sentences, each of which you would be willing to be judged by. Then write, underneath them, the case you have met in this book that you are most likely to get wrong, and why. Bring both to the viva. The examiners will read the second page, not the first.

15.13 Further reading

- Bronfenbrenner, U. The Ecology of Human Development — the frame that makes the household, the school and the economy part of one developmental system.
- Rutter, M. and Rutter, M. Developing Minds — the most careful account of what developmental research can and cannot support.
- Sameroff, A. The Transactional Model — the theoretical basis for everything in this course about bidirectionality.
- Gottlieb, G. on probabilistic epigenesis — why "the same input produces different outcomes" is a finding and not a hedge.
- Kirmayer, L. and Ban, L. on cultural formulation — the corrective this course ends with.

Glossary

Attachment — a behavioural system organising proximity-seeking to a caregiver under threat. **Co-regulation** — the process by which a caregiver's calm and responsiveness regulate the child's arousal. **Developmental task** — the psychosocial challenge characteristic of a life stage. **Disequilibrium** — a period of developmental reorganisation marked by increased conflict or regression. **Goodness of fit** — the match between a child's temperament and the demands of the environment. **Regression** — loss of previously acquired function; a red flag. **Scaffolding** — temporary support that allows a child to do what they cannot yet do alone. **Separation-individuation** — the process of developing autonomy while remaining connected. **Social clock** — culturally shared expectations about the timing of life events. **Theory of mind** — the capacity to attribute mental states to others. **Zone of proximal development** — the gap between what a learner can do alone and with support.

Self-assessment

Multiple choice

1. A four-year-old who stops speaking after a family bereavement is best described as: (a) normal variation (b) delay (c) regression (d) attention-seeking

2. "Goodness of fit" refers to the match between: (a) child and school (b) temperament and environment (c) parents' goals (d) IQ and achievement

3. The most common developmental concern presented to clinics is: (a) motor delay (b) expressive language delay (c) sleep (d) feeding

4. Disorganised attachment is most associated with: (a) daycare (b) frightening or frightened caregiving (c) prematurity (d) bilingualism

5. A toddler's frequent "no" primarily reflects: (a) oppositional disorder (b) emerging autonomy (c) poor parenting (d) language delay

6. Which is not a feature of adolescent brain development? (a) heightened reward sensitivity (b) peer sensitivity (c) mature impulse control (d) later prefrontal maturation

7. A "social clock" refers to: (a) sleep rhythms (b) culturally shared timing of life events (c) school timetable (d) biological ageing

8. In the dual-process model, healthy grieving involves: (a) staying on the loss track (b) avoiding reminders (c) oscillating between loss and restoration (d) suppressing emotion

9. Apparent confusion in an older adult should first prompt assessment of: (a) personality (b) delirium, depression, hearing, dementia (c) family conflict (d) medication refusal only

10. Extended-family caregiving should be assessed by asking: (a) whether the mother is attuned (b) who the child seeks when frightened (c) the parents' marital status (d) the family's income

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 c · 9 b · 10 b

Short answer

1. Distinguish delay from regression, with one clinical example of each.
2. Name three developmental tasks of adolescence and the corresponding family tasks.
3. Why is "let him cry it out" a relational intervention, not only a behavioural one?
4. Give two cultural critiques of attachment theory and one way to adapt your assessment.
5. What are the four parts of effective parent feedback?

Essay (choose one, 1,800 words)

1. "Normal development is a cultural achievement, not a biological fact." Discuss with reference to attachment and independence.
2. Using a disguised case, show how a developmental formulation changed your understanding of a presenting problem that had been framed as behavioural.
3. Critically evaluate the evidence that couple satisfaction declines after the birth of a child, and discuss the clinical implications.

Further reading

- Berk, L. E. (2022). Development Through the Lifespan (8th ed.). Pearson.
- Sroufe, L. A., Egeland, B., Carlson, E., & Collins, W. A. (2005). The Development of the Person. Guilford.
- Siegel, D. J. (2015). Brainstorm: The Teenage Brain. TarcherPerigee.
- Carter, B., & McGoldrick, M. (Eds.). (2005). The Expanded Family Life Cycle (3rd ed.). Allyn & Bacon.
- Keller, H. (2018). Cultures of Infancy (2nd ed.). Routledge.
- LeVine, R. A. (2003). Childhood Socialization. AltaMira.

Next: MFT 704 — Ethical and Legal Issues in Therapy